

Medplus Health Services (MEDPLUS)

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MedPlusal

MedPlus Health Services Limited

Date: June 1, 2023

The Listing Department

BSE Limited

Phiroze Jeejeebhoy Towers

Dalai Street, Fort,

Mum bai —400001

Scrip code: 543427

Dear Sir/ Madam,

Sub: Submission of earning call Transcripts The Listing Department

National Stock Exchange of India Limited

Exchange Plaza , 55h Floor

Plot No. C/1, G Block,

Bandra - Kuria Complex

Bandra (East), Mumbai -400051

Symbol: MEDPLUS

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements)

Regulations, 2015, we enclose herewith transcript of the earnings call of Q4 & FY 2022-23

held on May 26, 2023.

The same is available on Company's website at www.medplusindia.com

Kindly take the same on record.

For MedPlus Health Services Limited

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Manoj Kumar Srivastava

Company Secretary & Compliance Officer

FCS 7460

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"MedPlus Health Services Limited
Q4 FY'23 Earnings Conference Call "
May 26, 2023

MANAGEMENT : MR. MADHUKAR REDDY GANGADI – CHIEF
EXECUTIVE OFFICER AND MANAGING DIRECTOR –
MEDPLUS HEALTH SERVICES LIMITED
MR. SUJIT MAHATO – CHIEF FINANCIAL OFFICER –
MEDPLUS HEALTH SERVICES LIMITED
MR. CHETAN DIKSHIT – CHIEF STRATEGY OFFICER –
MEDPLUS HEALTH SERVICES LIMITED
MR. PRASAD REDDY BATTINAPATLA – ASSISTANT
FINANCIAL CONTROLLER – MEDPLUS HEALTH
SERVICES LIMITED

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Moderator: Ladies and gentlemen, good evening, and welcome to the MedPlus Health Services Limited Q4 FY '23 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be a n opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded . I now hand the conference over to Mr. Prasad Reddy from MedPlus. Thank you, and over to you, sir.

Prasad Reddy : Thank you, Faizal. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Q4 FY '23 earnings conference call to discuss the financial results, which were announced on 25 , May 2023.

We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, Chief Executive Officer and Managing Director , Mr. Sujit Kumar Mahato, Chief Financial Officer , and Mr. Chetan Dikshit, Chief Strategy Officer.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note t he disclaimer mentioning these risks and uncertainties on Slide 1 of the initial presentation shared with all of you earlier. Documents relating to our financial performance have been circulated earlier, and these have also been posted on our corporate web site.

I will now hand over the call to Mr. Madhukar for his any comments. Thank you, and over to you, Madhukar.

Madhukar Gangadi: Thank you, Prasad. Good evening, everyone. At MedPlus, we are proud to have a dedicated team of over 22,000 people who constan tly demonstrate discipline and hard work in delivering essential services to our valued customers. As of March 31, we have been serving the healthcare and households for communities in 552 cities across 7 states through our extensive network of 3,820 ph armacy stores.

During the current quarter, we have successfully expanded our presence into 55 additional cities. In addition to our pharmacy operations, MedPlus operates three integrated radiology centers, four slightly smaller radiology centers, which have up to the ult rasound machines and over 100 collection centers. These facilities play a crucial role in our commitment to providing affordable diagnostic services to our customers.

Our store expansion initiatives remain on track, and we continue to grow our results. Ove r the past 12 months, we have added a network - net total of 1,074 stores with 284 stores opening in Q4 alone. Notably, Karnataka and Tamil Nadu saw the highest number of store additions with 88 and 54 stores, respectively. Of the store openings in Q4, 58% were in Tier 2 and beyond, reflecting our strategic focus on these markets. Currently, out of our 3,822 stores 1,675 are located in Tier 2 cities and beyond, which are advantageous in terms of store economics to us. We will recognize the potential of thes e markets and aim to further expand due to the maturity of our operations and robust supply chain capabilities.

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During Q4, we experienced 19 store projects, slightly higher than the 17 in Q3. However, taking into account both openings and closures, we achi eved a net addition of 265 stores in Q4 , compared to 229 in Q3. Over the course for the next 12 months, or the past 12 months, we have successfully added a total of 1074 stores.

In terms of age of our store network, we have categorized our network expandin g's.

Approximately 30% of our stores are less than a year old, around 19% of our stores are in the second year of operation and the remaining has been operating for more than two years. To illustrate the impact of ou

r rapid store expansion in the age distr ibution of the network by the end of Q4, approximately 49% fall within extent two years. In comparison, during Q4 FY '22, only 40% of our stores were less than two years.

It's important to note that also within less than two years age bracket are still in the ramp -up pace. And from a financial perspective, they currently have a negative impact on our operating EBITDA. However, as these stores mature, we anticipate them cont ributing p ositively to our profitability.

We closely monitor the time it takes for our new stores to break even for stores opened between April '22 and September '22, approximately 65% of them achieved break even within six months of operations. Additional ly, around 71% of our new stores break even within 12 months of opening. At the end of the quarter, our network i s grown to 3,822 with 2 million -plus square foot compared to 2,748 and 1.6 million square feet at the end of March 2022. The average store size was 537 square foot. To give you a sense of spread, we have 2,715 stores less than 600 and 1,107 stores that are greater than 600 square feet.

With our expanded scale, we are strategically positioned to enhance our revenue share from private label products. Our private label range is designed to offer customers high-quality products at affordable prices. Currently, network offers over 900 thoughtfully curated SKUs to expanding both pharmaceutical and non-pharmaceutical categories. Private label sales accounted for 14.1%.

Moderator: Sorry to interrupt, Sir. This is the operator. The audio is not very clear, Sir - request if you can keep the mic slightly close to you.

Madhukar Gangadi: Sure. We're pleased to report that our efforts to increase the share of private label products in our customer shopping basket are progressing positively. Notably, our private label pharmaceutical range has shown promising growth contributing to 8.4% of our overall revenue. Further, our expanding presence in Tier 2 cities and beyond is making a significant impact on the revenue mix. Sales from these markets accounted for 33% of our revenues in the current quarter, demonstrating an increase of 30% in the same period last year. This indicates the success of our expansion strategy and the growing acceptance of our offerings in this market.

We continue to expand our coverage of pin codes for our online orders. This complements well with our physical stores. MedPlus will continue to focus on increasing the coverage of our two-hour delivery period or two-hour delivery offering. Store pickups as a share of online orders continues to maintain a higher share than home delivery, reflective of the convenience and

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accessibility of our store network. Our strategy on online remains unchanged. We have not spent heavily to acquire customers online and we'll continue to maintain our omnichannel as a profitable channel.

Now I request Sujit to give an update on our numbers.

Sujit Kumar Mahato: Thank you, Madhukar. Now on our quarter's performance. Our consolidated revenue was INR12,530 million, which has a growth of 29.7% on a year-on-year basis and 5.3% quarter-on-quarter. Our consolidated operating EBITDA stood at INR 406 million, representing 3.2%. This is a 37.6% year-on-year growth and 9.6% quarter-on-quarter improvement. Around 99% of our revenues is from our pharmacy operation. The pharmacy operating EBITDA was INR 447 million, representing 3.6%.

Our stores performing. I would like to update on a store older than 12 months. Revenue from these stores in Q4 was INR 10,737 million, or 88% of our pharmacy revenue. These stores had a store level EBITDA margin of 10.3%. The store level operating ROCE of these stores stood at 60.5%. A whole year on the store level EBITDA margin by age and vintage, while stores greater than 12 months had a margin

of 10.6%, this was 11% for stores greater than 24 months.

and 7.8% for stores in the age bracket of 13 months to 24 months.

If the allocated non-store related costs, the operating EBITDA of store greater than 12 months would be INR537 million, which translates to a margin of 5%. Our net working capital for Q4 was 64 days which comprised of the following: the inventory in our warehouse was 35 days. As you are aware, because of the same trajectory of new stores, their inventory turnover is lower in the first year. In quarter 4, the inventory level of our first-year store was 114 days. In comparison for our stores older than 12 months, the inventory was 39 days.

Now I request Chetan to update on our Diagnostics business. Over to you, Chetan.

Chetan Dikshit: Thank you, Sujit, and good afternoon, everyone. At the end of March '23, in our pilot market of Hyderabad, we had three full-service diagnostic centers, four Level 2 centers. So that's seven. Plus, we had just over 100 collection centers. In Q1 of FY '24, we expect to open our fourth full-service diagnostic center, and we have already gone live with three additional Level 2 centers. So that's seven at the end of Q4, being currently 10 and expected to be 11 by quarter end.

Here the short recap for those who are not already acquainted with our approach in diagnostics. Any customer of MedPlus advantage plan can avail the full range of radiology tests and pathology tests at 70% --75% discount to MRP. There are three differences in our model versus our typical peer. Firstly, we do not operate via franchise. Secondly, our collection centers are housed within our existing pharmacies. And thirdly, our plan is designed such that we do not depend on the referral network of patient walk-in.

We have launched the Network Advantage plan in February '22 up to 31st March '23, we have sold a gross of 75,000 plans. Since these plans are of one-year validity, going forward, we will be disclosing active plans and underlying lives. As on 31st March we had 93,000 active plans and 163,000 underlying lives. While we expect seasonality headwinds in Q1, we have crossed

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the milestone of 100,000 access plans, and we are now setting our sights on the 150,000 milestone.

Plan renewal is an important area for us. Given that diagnostics is not a high frequent usage, say, like grocery, our preferred metric for monitoring renewals will be renewed within six months from expiry. At this stage, it is early days for us to draw a trend. However, we will provide an update in next quarter's call.

That's our update on diagnostics. Handing back to Madhukar.

Madhukar Gangadi: Thank you, Chetan. So going forward, what can we expect from that cluster? We currently operate in a very attractive pharmacy space and our poised for growth on the back of our store expansion. We have a platform of 4,000-odd stores right now. And this allows us scale benefits, which we will continue to reap as we go forward. So our expansion plans continue to remain in place for the next one...

Moderator: Sorry to interrupt you, sir, the audio is not clear to us.

Madhukar Gangadi: Okay. Our cluster-based network enables profitable omnichannel service and we continue to expand this every quarter. Scale allows a larger share for our private label basket. And our diagnostic project has proven that we can use our pharmacy stores to cross-sell our healthcare solutions and we will explore other revenues that can add incremental sales without increasing cost.

So that's the end of our update. I request you to open the lines for questions.

Moderator: Thank you very much. The first question is from the line of Prakash Agarwal from Axis Capital. Please go ahead.

Prakash Agarwal : My first question is, I'm not sure if you already said, but what is your store opening guidance for fiscal '24?

Madhukar Gangadi: It will remain on the same lines Prakash, most likely anywhere from 800 stores to 1,000 stores.

Prakash Agarwal : Okay. And this year, we closed at 1,074 nets?

Madhukar Gangadi: At 1,074 nets, yes.

Prakash Agarwal : Okay. And these are largely in the existing states, or you plan to introduce any new state?

Madhukar Gangadi: Going forward definitely. We are looking to expand into Chhattisgarh, Kerala, and Madhya Pradesh but these will be small for us, we could say, between 20 stores and 25 stores in each of the three cities of Cochin, Indore, and Raipur. We will stabilize the operations and then look to expand from there.

Prakash Agarwal : Okay. But currently, in these states, there is no store as of now, is it?

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Madhukar Gangadi: These are completely new states for us, and we are going here because they are contiguous to the states in which we operate in a major way.

Prakash Agarwal : Okay. Perfect. And one clarity on the SSG growth of 12-month-plus stores, 15% is a very strong number. Just trying to understand, is there any one-off? How to read this, is this sustainable?

Madhukar Gangadi: See, definitely Q3 and Q4 are slightly better months for us, normally, I would say. So seasonality effect would be there to some extent. You obviously can't expect the same in Q1. Outside of that, a little bit of seasonality, there's nothing more to it. There is no one-off to the growth.

Prakash Agarwal : On a full year basis, how would this number?

Madhukar Gangadi: Full year basis, I can give you guidance on the same. We expect that we'll grow at least 20%, 25% at least for the full year. On the overall number, I'm not talking about the same store sales.

Prakash Agarwal : Yes. So if you got to pen in like SSG growth for 12-month-plus, that should be, what, 5% to 10% range or...

Madhukar Gangadi: Yes. I would assume at least that much.

Prakash Agarwal : Okay. Got it. And lastly, on the diagnostics, so quite a faster ramp-up now on the access and the membership. So -- and I think in one, two quarters, you'll be EBITDA neutral also. So what's the way forward? I mean, so would we expect to explore a new city, or we'll go more deeper within this place, or what is the plan here?

Madhukar Gangadi: We will fully expand into Hyderabad right now. We have around 10 centers at this point of time. We will add two more, and that will cover Hyderabad fully for us. We will continue to expand the membership. And I don't think, we will venture into a new city till we have grown significantly from this number. We currently, we're happy to tell you that we have around 100,000 members right now.

The plan is now for us to actually get moving on the B2B segment. Now almost 99% of it is B2C. And we think with the full network being set up across the city. We will be in a position to make the offering available to our B2B customers.

I'll let Chetan answer this, if you have anything to add Chetan.

Chetan Dikshit : No, I think, Madhukar, that's fine. Prakash if you have a follow-on, I can take it up.

Prakash Agarwal : Okay. If I may squeeze last one here. Omnichannel Q-on-Q growth is flattish. Has online being

little muted and we are back to off-line or how to read this data?

Madhukar Gangadi: Our omnichannel actually, if you look at the pure online segment of our omnichannel spike, that has been in fact, for various reasons. There are a certain set of customers who want to go online and there are -- and 18:25 minutes unfortunately, there's a lot of competitors out there who are moving them with higher discounts than you find. So those who are inclined towards that are probably going out there. Those who are there with us continue to stay with us and we're growing
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slowly out there. But I think until the noise

of all the people drops significantly, I don't think we will see any significant growth on there.

Prakash Agarwal: Noise is dropping? If you see most online guys are reducing discounts and noise, I guess, is dropping so...

Madhukar Gangadi: So we will see the effect of it, hopefully, going forward, Prakash.

Moderator: Thank you. The next question is from the line of Arshita Jain from Nuvama Group. Please go ahead.

Arshita Jain: Hello, Good evening, Congratulations on the good set. So my question -first question is on cash generation. So when I look at your three expansion plans, almost 1,000 stores in FY '24 maybe similar until FY '25? And with cash on this of about INR280 crores and when I see a average year-on-year after these -- negative INR8 crores. Just wanted to understand when can we see a good cash generation from our existing stores? And also, is there any plan to raise fund in the near future?

Sujit Mahato: I'll take this question. So on the balance sheet, as you are right, we have around INR280 crores of cash. And if you observe our books are net debt free, which means that there is enough headroom for leverage. But on an asset basis with the expansion that the company is going up. Easily, we can target 700 stores to 800 stores with the existing cash. We are mindful of the cash, but we would, at a certain point in time, go out for a line of credit for the working capital requirement.

Arshita Jain: Okay. And any plan for Q3 or just the working capital credit that you are looking for?

Sujit Mahato: At this point, it's only working capital.

Arshita Jain: Okay. Thank you. And secondly, you mentioned that there is some seasonal benefit in Q3 and Q4. Is that the reason why our private label non-pharma contribution received this quarter is from 5.1% to 5.7%, or is this something which is sustainable even in the coming quarters? Any kind of on that...

Madhukar Gangadi: Private label non-pharma does not have any seasonal kind of benefit out there. It is just that we are focusing on that a little bit more. We were at around 3%, 3.5% last year. And the effort is to actually get it up to 7% or 8% over the period of this year. Yes. So by non-pharma, what I mean is the portion which I'm talking about, the FMCG part of 3%, 3.5%. The focus is to make that come to 7% or 8%.

Arshita Jain: Okay. And what will lead to that? Is it our existing mature Tier 1? Or as we move to Tier 2, Tier 3, that will lead to that continuation?

Madhukar Gangadi: No, no. It's just a focus on it, which basically means we're expanding the range of products making them slightly better visible in the stores. We're also changing the format of some of the new stores out there to make it easier for customers to pick-up these products earlier. Most of our stores, almost all stores were served from behind the counter in the general goods are also
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served from behind the counter, makes it slightly more tougher for customers to pick those up. So our new stores are coming up with the self-serve model in the front, which makes it easier for us to sell the general goods on that.

Arshita Jain: Okay. Very helpful. And then just last one on diagnostics. Now, I think you started last year in February, now we are in the, almost more than one year over now. Is there any renewal rate that we are tracking for diagnostic customers or subscribers? It would be really helpful if can share that data?

Chetan Dikshit: Arshita, just to recap. In the past, we have said that we will intend to maintain the average quarterly loss at INR5 crores. We expect that phase to at least continue till end of Q2. You see it's important for these new centers that have been open to follow the same trajectory. So we expect to improve from Q3 onwards. But the phase that we had described earlier, that will at least continue till the end

d of Q2 of FY '24.

Arshita Jain: Okay. But my question in terms of the patients are already subscribing to our plans. Have they renewed it? Those who would have subscribed in February '22 or March '22?

Chetan Dikshit : All right . So that's a fair question. And I'm sorry, I misunderstood your question. So yes, we will talk more about renewals as we go forward. What I had mentioned was that the right way, we have determined that the right way to look at renewals is not exact on -time renewals, but renewals within a six months of the plan expiry. So it's still early days for us to give a guidance. What we have seen is that we are having 33% renewal from the March '22 cohort, and that's just 60 days since the plans expire. But this is very early. We need more time to form a view and subsequently to give you, our guidance.

Moderator: Thank you. The next question is from the line of Harith Ahamed from Avendus Spark . Please go ahead.

Harith Ahamed : Can you share the blended discount rate for the pharmacy retail segment that we're seeing currently for the quarter? And do you see this trending down basis or what you're seeing with respect to competition and then some of the challenges that they're taking, especially the online players.

Madhukar Gangadi: See, our discount now stands at 17.1% overall. Unfortunately, it does not come down as yet. While we are seeing slightly less advertisement from one or two of our competitors. There are at least two of them who are making in applies right now. And they're all talking about a 25% discount for the first three purchases even today. So we all know about the struggles of one of our competitors at least on the online site, but the thing is , it's not across the board. There are still people active. So that's the thing. Are we seeing any dropping in discounts, a little bit, but that has not really affected us as of now?

Harith Ahamed : And on the operating EBITDA margin for the Pharmacy Retail segment again. So we are around 3.5%, 3.6% for the last couple of quarters. How should we look at this number for FY '24, given that we'll have a higher percentage of our shares, which are above 12 months. So, and then the 1,000 -odd store additions that we're targeting. So overall, any guidance on this number?

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Madhukar Gangadi: Going forward, although I can't speak for Q1 right now because Q1 typically is not a great quarter for us. But over the period of one year, I think we will slowly start to see an increase in the overall margin. For us, we'll still have a significant number of stores which are below two years. Two years is where the actual potential of the store is seen. We expect a 10% kind of EBITDA at the two-year level. And today, 50% of our stores are under two years at this point. Given that we are going to be adding anywhere between 800 stores to 1,000 stores this year also, I don't expect a significant change. If any change were to happen, that most likely happen because of the benefits of scale on, let's say, reducing the corporate cost or maybe reducing warehousing cost or possibly increase in the margin on private label. But the downward track will continue to be good.

Harith Ahamed : Okay. And one on the diagnostics business. Chetan, you mentioned something a number for level 2 centers. So just trying to understand how different are these from the full-service centers and if you can also share the total gross block or investment in diagnostics till date?

Chetan Dikshit : Right. So I'll take the first part of your question. You see we are currently describing our centers as full -service diagnostic centers. Those are the ones that have MRI, CT and downwards. The next definition we use is level 2 centers. These are diagnostic centers, which have ultrasound and below. And in one case year or day, it may have an MRI, or it may have a CT. So th

ese are

our level 2 centers.

The third is the collection center, which is an extremely interesting feature because our collection centers are housed within our pharmacy. So going in the reverse order. Currently, we have over 115 collection centers in Hyderabad. And we currently have seven Level 2 centers. We have three full-service diagnostic centers live. And this quarter, we expect one more full-service center to go live. So at the end of the quarter, we will have four plus seven , and any new collection center that we open.

On your question on gross block, I'll request Sujit to step in.

Sujit Mahato: So on gross block as of 31 , March for the diagnostic business, it's around INR96 crores, and the net block is around INR90 crores.

Harith Ahamed : So those number of around INR170-odd crores that we are seeing on the balance sheet as assets under the segment? So what makes the difference?

Sujit Mahato: Sure. I can clarify that. So a large part of that close to INR77 crores is the right of use asset, which is the lease asset for our premises and centers. And there are other small components of other items.

Moderator: Thank you. The next question is from the line of Prakash Agarwal from Axis Capital. Please go ahead.

Prakash Agarwal : I just wanted to check on if there's an update on that automated warehouse that we were planning?

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Madhukar Gangadi: Yes. So Prakash, as you know, the Hyderabad warehouse today sends out 50% of all the tablets and all through the automated stuff. We were having a little bit of an issue with building of the new warehouse in Hyderabad. Hence, we have actually moved the first part to Chennai. Chennai, we have just taken the premises which is ready to go. We are looking to set it up with full automation. I hope that we should be able to get it out in possibly four to five months from now to lead that.

Prakash Agarwal : Okay. So the Hyderabad site, it's not happening anymore...

Madhukar Gangadi: It will happen. It will probably be the second one to go on. Hyderabad, we were planning to actually build the facility and then house the entire thing. So that has entered into a little bit of a stack, I would say, with the possession issues and all. So hence, we have leased the premises in Chennai and we're starting that out right away.

Prakash Agarwal : Okay. And the way we would be entering Madhya Pradesh, Chhattisgarh and Kerala would be like to start with the supply from adjoining states and to follow with their own warehouse or what is the plan?

Madhukar Gangadi: That's right. That's right. That's exactly right. So they are adjacent. So we'll be supplying from Nagpur for these two places, both Chhattisgarh and Madhya Pradesh and from Chennai for the Cochin side. Little inefficient, but we believe it will be much better than setting up a full year us for a 25 -store kind of operation. So till that becomes stable and profitable, we will not actually set up a warehouse.

Moderator : Thank you. The next question is from the line of Divya Daga from Vijit Global Securities. Please go ahead.

Divya Daga : Hello Sir, I have two questions. The first question is, sir, can you explain me about inventory? What about the inventory we have that gets old or expired?

Madhukar Gangadi: Sure. So all pharma branded inventory is bought on a fully returnable basis. So at expiry it is return to the manufacturer for full cost recovery out there. They exchange it for some other product.

Divya Daga : And what about private label, Sir?

Madhukar Gangadi: Private label that, of course, we have the inventory risk. So if we don't sell it, it basically just gets dumped.

Divya Daga : Okay. And can you provide me the number that in this year happened?

Madhukar Gangadi: The private label expiry you are saying?

Divya Daga : Yes, sir.

Madhukar Gangadi: I think a total -- ok

ay. Give me a second. So can we come back to you offline on this. This year, the number is slightly higher, I must say, and that is mainly because of the onetime COVID kind of inventory. Otherwise, it is usually manageable. Typically, we see that the overall profits on

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private label are so high. The gross margin is 80%. We are easily able to take a small inventory risk on that.

Divya Daga : Okay. My next question is, are we expecting 7% of operating profit margin sustainable in near future that we have in this quarter?

Madhukar Gangadi: So if you're talking about EBITDA growth from here where we are at around operating EBITDA of around 3.5%, 3.6%, that's going to be slightly more gradually, and it is going to be a result of both operating benefit as well as increase of new products coming in and the stores maturing. It will - so as of now, I can tell you that for all -- if the networks are all two years and more, we would actually be at an overall missing of around 7%. That's in December. Otherwise, if you take out the rents and everything else and if we account for regular operational EBITDA, that's going to be around 5%, for stores, which are above two years. The entire thing is 3.5%.

Moderator: Thank you. The next question is from the line of Ankit Bansal from AV Investments. Please go ahead.

Ankit Bansal : My question is the kind of model you are pursuing like operating in some of the states like in the South, when you will move to bigger cities or newer cities like Delhi, in North India, where like I can say of myself, I've never heard of MedPlus before the IPO. So what is your strategy? How will you go deep into that market? How will you operate? How you will attract customer of getting medicines from you rather than from the established player in the north, like Apollo

is a very well-established player in the North. What are your views on that?

Madhukar Gangadi: Sure. See, we currently operate in seven states, and we're going to three new states this year. But don't get the seven number fully out there. I know it's a smaller word, this thing of the overall 29 states, which we have. But the seven states account for around 40%, 45% of the overall market in the country. We are also there in all the big cities except Delhi. In six of the top seven cities in the country, we are there. We have 450 stores in Hyderabad, Bangalore, and Chennai, around 300 in Calcutta, 250 -odd in Pune and roughly around 100 stores in Bombay. So except for Delhi, we are every where.

So the plan is for us to, we believe that the markets in which we are there are big enough for us to grow for the next several years, but that doesn't mean that we'll not continuously expand, slowly into the contiguous states. So we'll continue to grow. We will take a call in Delhi when the time comes. As of now, we have more than enough market to actually grow into at this point.

Ankit Bansal : But sir, it will take your deep research to get into the Delhi market because it's a very unorganized market. People like to buy from their 1% type medicine shop. You have to change, I think your model, I don't know how would you do that. But on this strategy of opening new store and new stores. I don't think it's going to work out in states like North India. Because they are -- I have seen people, they are very much organized with the buying of their medicines from the particular shop. Instead of buying from a player like you, where they have not heard a single, name of you before the IPO. What are your comments? How could you will penetrate into like this market?

Chetan Dikshit : Ankit. This is Chetan. You've made some very good points, and we'll reflect on them. Could we move to the next question?

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Moderator: Thank you. We'll take the next question from t

he line of Siddhant Choudhary from Perpetuity Ventures. Please go ahead.

Siddhant Choudhary : So my first question is, we have seen a gross margin improvement of around 40 basis points quarter -on-quarter. So sir, what could be the possible reasons for it? And like what part of it is contributed by private label?

Sujit Mahato: On the private label, I think in the earlier portion of the speech, it was clarified that we have around 14% in terms of the total share in the sale. In terms of gross margin improvement, yes, we have seen some efficiencies in inventory management and lower provisions. So that contributed to an increase. And I think it was explained earlier as well.

Siddhant Choudhary : So, can I get a breakup of this expansion for private label. Is there any data related to this?

Sujit Mahato: I think we can do that off -line.

Siddhant Choudhary : Okay. No problem. My next question is the full year capex for this year was a round INR170 crores. Can you give a breakup between the diagnostic and pharmacy business?

Sujit Mahato: So on the diagnostics, I will clarify because that will help you. On the diagnostics as on 31 , March the gross block is around INR96 crores, and the net block as on 31 , March is around INR90 crores. The balance in the -- if you are referring to the segment results, there is a write - off use asset, which is under Ind AS 116 the lease assets and therefore, you can now make your calculation.

Siddhant Choudhary : Do we have a guidance for the next year for both pharmacy and diagnostics?

Management: When you see for the Diagnostics business -- for the most part, we have already announced our plans. There may be small investments here and there within the existing centers or some additional collection centers that we opened. But in context of what we have spent so far, no material move is expected in the next -- in the FY '24.

Siddhant Choudhary : And for the pharmacy business?

Madhukar Gangadi: For the pharmacy side, as we said, we're going to add anywhere between 800 stores to 1,000 stores. And each store typically takes an investment of roughly around INR30 lakhs and that includes the inventory in it. So doing the math, it should be from INR240 crores to INR300 crores.

Siddhant Choudhary : Sir, my last question is for the diagnostic business. Do we have a data for the number of tests that were conducted during the last quarter?

Madhukar Gangadi: Yes, we do have that, but I just don't have it hand y. We can probably take it offline.

Moderator: Thank you. The next question is from the line of Amit Kadam from Canara Robeco Mutual Fund. Please go ahead.

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Amit Kadam : Okay. So I just wanted to check with the management team. What was the reason for changi ng

the reporting in terms of your segments?

Sujit Mahato: So I'll take that. So I think if you observe, there is no significant change. Earlier, there were three reporting segments, wholesale, retail, and diagnostic. We have now merged because we have merged the wholesale also into retail because wholesale is not the main business for the company. And though it continues to be an operating segment, it no longer qualifies to be a reportable segment and that's the only change. That's not a significant number.

Amit Kadam: And like just maybe -- if I just take the diagnostic and if I move to the a segmental profit line.

So this quarter, maybe assuming that like last -- for last quarter, we had reported in this diagnostic testing services somewhere around profit for the segment at 46.8 million. Whereas now, if I just check the same number, what is it for the December quarter, it is something in the 62.69 million. Just wanted to know how to reconcile these two numbers?

Sujit Mahato: So I think what you have also you'll observe is we have now clarified on the un-allocable finance cost, which was earlier going

as part of pharmacy alone. So that presentation we have corrected it along with the segment change.

Amit Kadam: Okay. So maybe I'll just come back. I just held up with this reconciling this particular...

Madhukar Gangadi: Sure. We will do it offline.

Moderator: Thank you. We will take the next question from the line of Neelam Punjabi from Perpetuity Ventures. Please go ahead.

Neelam Punjabi: First of all, congratulation for some good set of numbers. So my first question is on the corporate cost. So if I back calculated since the disclosures that you have provided, it seems that our corporate costs have gone up sequentially. Could you please explain what is the reason behind this sir?

Madhukar Gangadi: Demand gone up slightly. I'm not sure if it is a significant number. We expect that the cost will come down as we go forward as the scale increases. But let me look it up and then come back to you on this.

Neelam Punjabi: Got it. Okay. Secondly, sir, in our operating EBITDA break slide, there are some preoperative expenses for pharmacy business of about INR4 crores for the quarter. What is it pertaining to?

Sujit Mahato: I think they represent two parts. One is the property, which the company leases, which have a gestation period of two to three months to operate, right, from the way -- from the time we sign up. And second is the higher considerable number of employees, which is required for opening the new stores on a go-forward basis. So that from our presentation perspective and an operational perspective, we call it out as preoperative.

Neelam Punjabi: Understood. Okay. My next question is on the diagnostic business. Could you just highlight your strategy for incentivizing our existing subscribers to renew their existing plans? How are we incentivizing them?

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Chetan Dikshit: Neelam, that's a very good question. So you're right, in bringing the focus on renewals.

Currently, there is absolutely no differential between a customer who is renewing and who is buying a new plan. And this was actually done intentionally because we wanted to gather data on what is the natural pace of renewals. In the coming months, we will introduce a differential between renewals and new sign-ups, which will automatically incentivize existing customers to renew on time.

Neelam Punjabi: Got it. Okay. And lastly, could you give a broader level outlook for the aspirations for top line growth and margins for the next two to three years?

Madhukar Gangadi: So I'm not going to talk about the next two to three, but for this year, definitely, we expect a top line growth of at least anywhere between 20% to 25% that would be the minimum. On the margin side, I expect it will more or less be flattish mainly because we will have a significant number of new stores coming up even now. If there is any benefit, it's going to come out of any automation which we do on the back end or the scale benefits which come out of increasing top line for us.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for closing comments.

Madhukar Gangadi: Thank you. I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com.

Moderator: Thank you, members of the management. Ladies and gentlemen, on behalf of MedPlus Health Services, that concludes this conference call. Thank you for joining us, and you may now disconnect your lines.

Call: Aug 2023

Medplus Health Services Limited

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August 10 , 2023

The Listing Department
BSE Limited The Listing Department
National Stock Exchange of India Limited
Phiroze Jeejeebhoy Towers Exchange Plaza , 5th Floor
Dalal Street, Fort, Plot No. C/1, G Block,
Mumbai – 400001 Bandra - Kurla Complex
Scrip code: 543427 Bandra (East) , Mumbai - 400051
Symbol: MEDPLUS
Dear Sir/ Madam,

Sub: Submission of earning call Transcripts for the quarter ended June 30, 2023.

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 , we enclose h erewith transcript of the earnings calls held on August 8, 2023 for the quarter ended June 30, 2023.
Kindly take the same on record. The same is available on Company's website at www.medplusindia.com

For MedPI us Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer
FCS 7460

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"MedPlus Health Services Limited Q1 FY '24 Earnings
Conference Call "

August 08, 2023

MANAGEMENT : MR. MADHUKAR GANGADI – CHIEF EXECUTIVE
OFFICER AND MANAGING DIRECT OR, MEDPLUS
HEALTH SERVICES LIMITED
MR. SUJIT MAHATO – CHIEF FINANCIAL OFFICER ,
MEDPLUS HEALTH SERVICES LIMITED
MR. CHETAN DIKSHIT – CHIEF STRATEGY OFFICER ,
MEDPLUS HEALTH SERVICES LIMITED
MR. PRASAD REDDY – MEDPLUS HEALTH SERVICES

LIMITED

MedPlus Health Services Limited
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Moderator : Ladies and gentlemen , good day and welcome to MedPlus Health Services Limited Q1 FY '24 Earnings Conference Call .

As a reminder, all participants' lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr . Prasad Reddy from MedPlus Health Services Limited. Thank you , and over to you .

Prasad Reddy: Thank you, Ranjan. Good evening everyone . On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Q1 FY '24 Earnings Conference Call to discuss the financial results of MedPlus for the first quarter of Financial year '24 which were announced on August 7, 2023 .

We have with us today the Senior Management Team represented by Mr. Madhukar Gangadi – Chief Executive Officer and Managing Director; Mr. Sujit Mahato – CFO and Mr. Chetan Dikshit – CSO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning the risk and uncertainties are on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance have been circulated earlier and these have also been posted on our corporate website.

I will now hand over the call to Mr. Sujit. Thank you, and over to you, Sujit.

Sujit Mahato: Thank you, Prasad , and good evening everyone . As of June 30th , we have been serving the healthcare and household needs of community in 581 cities across seven states through our extensive network of 3,975 pharmacy outlets. During the current quarter , we have successfully expanded our presence into 29 additional cities .

In addition to our pharmacy operation , MedPlus operates 4 full-service diagnostic centers, 7 level-two centers and 122 collection centers . These facilities play a crucial role in our commitment to providing affordable diagnostic services to our customers .

An important milestone in our diagnostic journey is we have crossed the 1 lakh active membership as at the end of the current quarter covering 180,000 lives are coming from the network .

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Our store expansion program remains on track as we continue to balance growth and profitability . Over the past 12 months , we have added a net total of 995 stores with 168 stores opened in Q1.

Notably, West Bengal and Karnataka saw the highest number of store additions with 53 and 32 stores respectively . Of the store openings in Q1, 53% were in Tier-2 cities and beyond reflecting our strategic focus on these markets . Currently, out of our 3,975 stores , 1,762 stores representing 44 % are located in Tier-2 cities and beyond. We recognize the potential of these markets and aim to further expand due to the maturity of our operations and robust supply chain capabilities .

During Q1, we experienced 15 store closures . Considering both openings and closures , we achieved the net addition of 153 stores in Q1 compared to 265 store opening in Q4 of the previous year .

In terms of age of our network, approximately 27 % of our stores are less than a year old. Around 22% of our store are in the second year of operation . The remaining 51 % of our store have been operating for two years or more .

To illustrate the impact of our rapid store expansion on the age distribution of our network, by the end of Q1, approximately 49% of our stores fall between less than two years age bracket . In comparison , during Q1 FY '23, only 45 % of our stores were less than two years old . It's important

to note that all stores in the less than two years age bracket are still in their ramp up phase. From a financial perspective , they currently have a negative impact on our operating EBITDA .

However, as these stores mature , we anticipate them contributing positively to our profitability . We closely monitor the time it takes for our new stores to break even . For stores opened between July 2022 and December '22, approximately 58 % of them achieve break even

within six months of operation. Additionally, as a cohort, all the stores combined achieved break-even in just four months.

Network and the store size:

As at the end of the quarter, our network has grown to 3,975 stores with 2.1 million plus square feet compared to 2,980 stores representing 1.7 million square feet at the end of June '22. The average store size is 542 square feet. To give you a sense of spread in store sizes, we have 2,855 stores less than 600 square feet and 1,120 that are greater than 600 square feet. With our expanded scale, we are strategically positioned to enhance our revenue share from private label products. Our private label range is designed to offer customers high quality products at affordable prices. Currently, MedPlus offers over 1,300 thoughtfully curated SKUs spanning across pharmaceutical and non-pharmaceutical categories.

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Private label sales accounted for 13.3% of our total revenue in Q1 FY '24, we have observed a reduced sale in our private label due to extended summer and delayed monsoon.

Furthermore, our expanding presence in Tier-2 cities and beyond is making a significant impact on our revenue mix. Sales from these markets accounted for 34% of our pharmacy revenue in the current quarter demonstrating an increase from 31% in the same period last year.

New offering:

MedPlus has introduced the MedPlus Advantage Pharma subscription plan for its customers in Hyderabad offering several exciting benefits. To become a member of this plan, customers are required to pay an annual subscription fee of Rs. 499. Initially, the company is offering the subscription at Rs. 49. As part of the plan, customers gain access to MedPlus branded products at discounted rates ranging between 50% and 80%. Initially, the plan covered 433 pharma products and the company is now planning to expand the offerings to include more products. The financial numbers, on our quarter's performance, the consolidated revenue was 12,843 million with a growth of 29.2% year-on-year and 2.5% quarter-on-quarter. Our consolidated operating EBITDA stood at 291 million representing 2.3%. Around 99% of our revenue is from our pharmacy operations. The pharmacy operating EBITDA is 343 million representing 2.7%.

On our store performance, I would like to update you on our stores older than 12 months. Revenue from these stores in Q1 was 11,124 million or 89% of pharmacy revenue. These stores had a store level EBITDA margin of 8.9%. The store level operating ROCE for these stores stood at 49.1%.

A word here on the store level EBITDA margin by age. While stores greater than 12 months had a margin of 8.9%, this was 9.7% for stores greater than 24 months and 6.1% for stores in the 13-to-24-month age bracket. If we allocated the non-store-related cost, then the operating EBITDA for the store greater than 12 months would be 452 million, which translates to a margin of 4%.

On our diagnostic numbers, diagnostics revenue has grown to 139 million in Q1 FY '24 compared to 32 million in Q1 FY '23 which is primarily due to the launch of new centers in Hyderabad. Diagnostic segment recorded an operating EBITDA loss of 46 million compared to a loss of 52 million in Q1 FY '23.

On the working capital details, our networking capital for Q1 was 66 days. The inventory in our warehouse was 35 days. As you are

aware, because of the sales trajectory of new stores, their inventory turnover is lower in the first year. In Q1, the inventory level of our first-year store was 114 days. In comparison for our stores older than 12 months, the inventory was 41 days. Now I request Chetan to update on our diagnostic business. Over to you, Chetan.

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Chetan Dikshit: Thank you, Sujit, and good afternoon everyone. To recap what Sujit has already mentioned in our pilot market of Hyderabad, we now have 4 full-service diagnostic centers, 7 level two centers and 122 collection centers.

In July of this year, we made two tweaks to the MedPlus Advantage plan. Firstly, we made the configuration scalable. So, now add-on participants, example, a family member can be included anytime during the one-year tenure of a plan.

Secondly, we increased the price of our plans by Rs. 150. Simultaneously, we introduced the same 150 as a discount for timely renewal. In effect, our old members will have the option to renew on time at the old price. There is no change to the benefits. Any customer of MedPlus Advantage can avail the full range of radiology tests and pathology tests at 75% discount to MRP.

As on 31st March , we had 92 ,000 active plans and 162 ,000 underlying lives .As on 30th June , we had 105 ,000 active plans and 186 ,000 underlying lives . We are now setting our sites on the 250,000 underlying lives milestone .

For this quarter , in April, we sold 333 gross plans per day . In May and June , this was 308 and 332 respectively .We had launched MedPlusAdvantage in March '22. After the completion of one year , our current observed on -time renewal rate is 15 % .In 60 days, post expiry , we have observed the renewal to be at 30 % . That's an update on diagnostics .

Here I am handing it over to Madhukar now.

Madhukar Gangadi: Thank you , Chetan. On June 17th , we launched a new product line in our stores .It's actually a fist on the private label which we had . Earlier our white label or private label products were sold under the name of MHS and Wynclark , and they were offered as substitutes as a convenience to customers for whom we did not have the full set of products on their prescription . These were sold at the same price as the original brands and all , and there was no price advantage . And as I said earlier ,these were sold purely in a convenience kind of product out there .

But over the last few quarters , we have been observing that more and more customers are now looking to actually take up or at least have been asking for unbranded or generic products .So, as a response to that and as a response to the fact that the Indian customer is now more aware than ever before and the fact that most of them now realize that 80 % of all the drugs which are sold in the country are off -patent and are actually effectively generic products with really no big difference on the efficacy of the drugs and everything else .

So, given that most people have now realized this and they have been asking for this kind of products , we thought it was the right time for us to introduce a set of products under the MedPlus brand.

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The big difference now is that while the earlier private labels were offered as just a convenience kind of thing substitute for a brand for which we did not have the stock at the point of time , we have now started offering these actively to customers who are looking to go for a less expensive product than the branded products out there .

So, this comes through a membership plan .The membership plan will beat the Rs. 491 per year kind of thing. We have started it at the Rs. 49 plan as an inaugural offer and what it offers to the customers is 50 % to 80 % discount on a broad set of medicines .We have started with 433 odd products . We expect

that we will extend this to around 800 over the coming months .

We have actually launched it only in Hyderabad and rest of Telangana as of now , and I am happy to tell you that the uptake by the customer has been fantastic . You know, we are actually seeing a large set of customers asking us for these products ,asking for the price contract , trusting the MedPlus brand and asking for the substitution .

So, we are happy to tell you that as of July , we are at a total of roughly 14% of all the products sold in our stores today are of the MedPlus brand . While it is sold at a discount , it's still margin accretive to us . It also helps in bringing in new customers into our stores .We also have been seeing a lot of customers who we had actually lost in the past few months walking back to the stores for the promise of the discount and all.

We expect as we go forward that the 14% will continue to increase . We also expect to launch these products across all the seven states in the country , and we expect this to not just add to our margin but also add to the overall top line of the company .

We believe the reason for which we , I would say , believe now we are at a stage where the MedPlus brand is accepted and we will now start selling at least offering customers two things out there . One, the best price on a branded generic product . Two, a MedPlus brand if the customer is so willing at a price which is like 50 % to 80 % discount to the MRP .

So, that's the update for us for this product . We would like to now take the questions from the audience .

Moderator: Thank you. We will now begin the question -and-answer session .The first question comes from the line of Ankit Bansal from AD Private Limited. Please go ahead.

Ankit Bansal: My question is , sir, your private label brand which you recently started , so can you please elaborate the kind of manufacturer you have been tied up with or another information which will be helpful for our investors in the growth of the company looking forward ?

Chetan Dikshit: So, we have shortlisted around 10 different manufacturers across the country . I don't want to give you many , many names out here right now , but these are people who are actually doing it for the largest brands in the country .They have the best , I would say , they have one of the

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better facilities in the country and for us, we are very confident that these guys can actually give us quality, which is on par with the best brands out there in the country.

Ankit Bansal: My next question is, sir, any idea of expanding to North India? Any in the mind, sir, any your view, sir?

Madhukar Gangadi: No, no, we are actually expanding into the contiguous states in which we are already present. We are going into Madhya Pradesh, Chhattisgarh and Kerala. We will definitely be looking at a North India expansion, but not at this time. There's not any plan as of now.

Ankit Bansal: Sir, are you seeing a healthy competition or competition is getting more tougher and tougher with the e-commerce like local manufacturers are also I am witnessing a lot of growth in their businesses. How you see them, sir?

Madhukar Gangadi: So, there are two parts to the question, I guess. One is the e-pharmacy side. We are seeing while a couple of players have actually reduced the amount of discounting and definitely reduce the amount of advertising and all, not everyone is doing that obviously. There is one or two players who continue to advertise. On the local manufacturers and all, I can't really comment on that. What we believe is that MedPlus is would say, the best purveyors for the complete set of branded generics. Our scale allows us to actually give a price which no one else can match. At the same time, we also have a size and scale and capability which allows us to actually make or at least have these drugs

contract manufactured at the best facilities across the country and give them at a price at which no one else can match. So, for us, we really don't see any of the smaller and local manufacturers being competition members.

Moderator: Thank you. Next question comes from the line of Nikhil Mathur with HDFC Mutual Funds. Please go ahead.

Nikhil Mathur: Sir, my question is on the revenue per store that the company is generating. I am looking at the pharmacy count numbers taking the pharmacy revenue, dividing it with the number of stores and kind of analyzing it by multiplying it by four. So, this quarter, the company is at 12.8 million. This has consistently been coming down, not very sharply, but if I look at the previous quarters from 13.2 to 13.3, it was in the first nine months of FY '23, and it's now down to 12.8. At the same time, if I look at the ageing of stores, the number of stores which are Year 1, as classified in your Investor Presentation, that has come down from 30% to 27%. So, I am not sure what I am missing here that despite the aging profile kind of improving, the revenue per store is trending down, not trending up.

Madhukar Gangadi: So, a couple of things. One, the store continues to mature till it actually gets to 24 months while yes, last quarter, we probably would have slightly less number of stores which opened. The overall number of less than two years still is pretty high. It is more than 50% of the overall network, and that is what is actually dragging the overall number down.

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For us, we really don't see any other impact other than a small seasonal impact on the Q1 which is normal. This time it has been an extended kind of summer. Typically, April and May by themselves, usually, the summer months don't really have any kind of, I would say, infections or morbidity concerns, which would otherwise result in some kind of medications sold. So, that's one.

And second, the extended summer also basically led to some of it. Other than that, I really don't see any problem.

Nikhil Mathur: So, adjustable seasonality, this number would have been much better. That's what you are trying to suggest?

Madhukar Gangadi: I would say, no, no, no. See, the big effect is definitely of the 50% stores which are under two years. Seasonal thing is it's a quarter kind of thing. That's all.

Nikhil Mathur: So, sir, I mean, we still have 900, I think 800, 900 store addition plans in this year, next year. At what point would there be some leverage available on revenue per store? And I think this is what will drive margins as well, right? Unless your revenue per store increases, I am not sure if the margins can trend up to what some of your companies, I mean, like a listed company which reports the pharmacy margins, they seem to be much, much higher than what MedPlus is at. So, when that kind of breakthrough happens, when the revenue per store increases and that kind of flows through to EBITDA?

Madhukar Gangadi: So, as we have been saying since the beginning of the year, last year we said we would actually do in 1,000 and 1,200 stores. We ended up at around 1,045. This year we have been driving towards 800 to 1,000 stores. We are conscious of the fact that margins need to improve and all, but more than that we are actually, I would say, extremely cautious about the money. We want to make sure that there is enough accrual. We don't need to really borrow too much to actually expand. So, for that reason, we are basically going between 800 to 1,000 stores this year. One.

Second, the margin expansion itself will actually come through several things .It's going to come through the maturing of the stores , (Inaudible) 23:45 will basically definitely help the stores become more profitable and as we go forward , as we move from the 4,500 -crore top line

closer towards the 6,000 odd for the next year , I don't expect that the warehousing and the corporate bus are going to keep in line with that .

So, there will be benefits of scale out there , but more than anything else I think our recent let's say, initiative of bringing on the MedPlus brand of private labels at 50 % to 80 % discount , I feel extremely confident that this is what is going to actually drive the top line. It's also going to drive the model store. As I said we just launched it on June 17th and as of now we are basically seeing the overall portion being around 14%. And this has just been launched only in Hyderabad and rest of Telangana .

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So, we are very confident that not only will this bring in more customers to the MedPlus store, it will also be extremely margin accretive as we move forward. The platform which we have built will basically now start getting monetized significantly because you require this kind of size to get such a big list of products to be manufactured on your own name .

Nikhil Mathur: And sir, one more question I had was , if I look at the employee costs and the other expense , can you give some ballpark indication , what percentage of these costs are sitting in the corporate overheads ? And how much of that is in store ? The employee expense and the other expenses , what proportion of these expenses are related to the corporate overheads ? And what percentage is related to stores ?

Madhukar Gangadi: Give me a second . See, I am not sure about the exact thing , but let me just give you the numbers . We on a two -year store , we expect a 10% liquidity at the store level and below that we have a 2% corporate expense and a 3% warehousing expense . That is standard .

Nikhil Mathur: This is for a two -year store you are saying?

Madhukar Gangadi: Exactly. So, the overall expenses, so, Yes, the two -year qualifier was for the EBITDA. So, 2 year to 2.5 year, that's when we hit the 10% EBITDA, but below that then you would always have that. So, no matter what the EBITDA at the store level is, 2% on corporate and 3% on warehousing would be the normal cost .

Nikhil Mathur: So, if your entire network system was to be two -year old , MedPlus as a console entity would be reporting a 5% EBITDA margin . Is that the right understanding ?

Madhukar Gangadi: As of now , yes, but as we go forward with scale benefit and with private label and everything else, we expect this will continue to improve .

Nikhil Mathur: And we are talking about pre -Ind-AS, right? Or not?

Madhukar Gangadi: Yes, we are. Yes. All rents and everything else taken into account .

Nikhil Mathur: Sir, I am not sure if it's there in the press release or Investor Presentation. What is the net debt position as of this quarter ?

Madhukar Gangadi: Net debt is zero as of this quarter.

Nikhil Mathur: Net debt is zero, okay. And one last question .How much inventory is sitting in the stores ? And how much inventory is sitting in the warehouses ?

Madhukar Gangadi: For any store which is more than one year old , 39 days of inventory sits in the store , and roughly around 30 to 35 days sits in the warehouse .

Nikhil Mathur: So, for a reasonably mature store, 50% is in the store?

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Madhukar Gangadi: So, after one year, it pretty much remains the same. I would say , for a store which does more than a lakh of rupees per day kind of stuff , it probably will come down to 28 to 30 days , but on the whole , I would say , across the Board, at the store level , it is roughly around 35 days . But the number starts falling significantly after one year or just after one year . Otherwise, the store starts off with Rs. 18 lakh inventory and a lot of times it ends up doing only Rs. 3 lakh per month , which means a six-month inventory in the initial

stage .

Nikhil Mathur: And sir, one final comment on discounting , please . I mean , we are hearing various new stories around e -pharmacies being under pressure because of funding constraints and all .Is there anyway to the argument that discounting should come down systematically and that should benefit MedPlus? Or would you keep discounting high in order to gain volume share, which has been the strategy till now ?

Madhukar Gangadi: See, we are definitely seeing slightly less noise on the discounting side from some of the e-pharmacy companies out there , but as I have been actually maintaining in the past too , e-

pharmacy is a small portion of the overall business out there , and what we are seeing is a lot of the small independent operators are selling a lot of trade generics as brands, and they are basically pushing up the discounts out there . So, there is some competition out there , and our own private label is at 50% to 80 % discount is I would say kind of move towards conquering the same . That's one.

Our own experience we have been seeing more and more people buying above 1,000. Our bills above 1,000 have actually increased . Hence, the discount has actually gone upside from 17 to 17.1.

Moderator: Thank you. Next question comes from the line of Tanmay Gandhi with Investec. Please go ahead.

Tanmay Gandhi: Sir, first question is on the store closure . So, this quarter we had almost 15 closures, and the average age is more than seven years . So, did that impact our financials or growth in this quarter?

Madhukar Gangadi: No, no. So, that's a pretty standard set of closures , I think , you have . The store , you are talking about store closures, right?

Tanmay Gandhi: Yes. So, I think , for eight stores it was because of relocation , but there is another item which is others , and we are not sure what is that . And also, if you can highlight if relocation leads to some different sales for that particular micro markets ?

Madhukar Gangadi: Yes. No, no, see, if it is not relocation , then it is definitely a business reason . It could be where we have had a problem renewing the lease or something like that . Yes, it could be visibility . It could be several things honestly .

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Tanmay Gandhi: So, assuming a matured store can do anywhere between 1 Cr to 1.5 Cr a year , right, so does that mean that you would have lost almost 8 to 10 crores of annual sales ?

Madhukar Gangadi: I don't see it that way because you are already relocating most of the stores . Some of them are actually going somewhere else anyways . So, in all these markets , there is almost no place where we wouldn't have and if it is not exact store to store relocation , we will probably just set up some parallel store . It's unlikely that I would , I wouldn't take that as a thing for losing revenue on them .

Tanmay Gandhi: And again on your new pen which you have launched for generics , right, so I understand that our private labels are around 13 % and what would be the revenue from these generic products ?

Madhukar Gangadi: See, earlier when we talked about the 13 , 13.5 private label , we were talking mainly about , we are talking about a whole set the complete set which was medicines , non-medicines. So, earlier , if you look at only the pharma side, it is around 8.

Now, that 8 has actually moved to 14 % and 14.5% overall but given that the new set of private labels which we have come with a big discount , I am basically comparing this as a MRP sale, which means if the entire sales in the store , the overall sale above Rs. 100 on MRP, the discount would be normally 17 % or whatever it is and all . To make sure that the comparison is like-to-like, we are basically saying these products comprise 14 , 14.5 out of the 100 on a MRP sale value .

Tanmay Gandhi: And our margin profile on this product would be inferior to private labels .

Is that correct

because your COGS would be similar ?

Madhukar Gangadi: On an absolute margin , this would be far better than the branded products , but it would definitely be lesser than the old private label which we had.

Tanmay Gandhi: But when we sell these generics , so is it fair to assume that it can also cannibalize our private labels , right? Because let's say , if a customer is not comfortable switching , let's say , a third-party brand to some other brand , right, then he would probably wouldn't want to actually go for an unbranded brand . So, is it fair to assume that we might actually it might actually cannibalize your private labels ?

Madhukar Gangadi: So, see, earlier we had a private label that was being sold at the full price and now we have got the MedPlus brand which is actually sold at a discount . The plan is to go forward and basically merge both these into one.

The reason for that is we want the store employee to be focused on selling this one product and not look to start and not I would say , be inclined to sell that slightly higher cost , higher margin product at the cost of , I would say , confusing the customer basically saying that okay , I don't have this product . I have something else which costs Rs. 80, something else which costs 40 ,

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and both are equally good . So, we don't want that kind of message going out. Hence, we have actually merged them.

Yes, initially the margins would definitely get affected slightly, I would say, but given that we are able, the overall set, if we were selling, let's say, 8% of private label earlier, now it is 14.5% of the combined set, we expect, and this is growing really rapidly, we expect that there will come a time when we would be at least Yes, or even as of now, I would say, the margins are more or less in line with what were earlier.

But the big difference is our announcement of the 50% to 80% discount on the MedPlus brand has actually brought in a lot of new customers, and we are seeing growth in the top line which will take care of all these other issues, if there are any.

Tanmay Gandhi: And last question from my end. So, can you share the same store sales growth number for stores more than two years old?

Madhukar Gangadi: I am not sure. I think we will come out of the number shortly, but I don't think I can tell you that right now.

Tanmay Gandhi: Can you tell us directionally whether it has improved or not? Because probably out of many other our competitors, right, probably we are offering highest discounts. So, has that actually resulted in some volume gains or better same store sales growth? If you can share the direction, that will also work.

Madhukar Gangadi: No, no, absolutely. The new private label initiative of MedPlus has definitely resulted in extremely good growth in the small set of, Yes, I would say, not really small. We have actually launched it in Hyderabad and Telangana. So, it is definitely good. So, we have definitely seen growth in July and August.

Moderator: Thank you. Next question comes on the line of Mehul Sheth with Axi Capital. Please go ahead.

Mehul Sheth: Sir, more on this recent launcher this year your own branded private label on the pharma side, so can you talk more about like how many SKU you are planning, currently have, and what is your expansion plan in that? Apart from that, what will be the exact as a percentage of sale in the current contribution is? And how the unit economics layout in this particular part of a business?

Madhukar Gangadi: I'm sorry, Mehul, the question, you are not very clear. I can't really make out. I know you are asking about the private label drug. And can you just repeat that?

Mehul Sheth: Let me repeat myself. So, sir, first, I want to know about how many SKU you currently have? And what are your expansion plan in

terms of brand coverage? Secondly, how the unit economic works in this part of a business? And thirdly, what is the exact contribution right now from this particular business as a percentage of sales?

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Madhukar Gangadi: What is the second question?

Mehul Sheth: Unit economics. How the unit economics play this part?

Madhukar Gangadi: So, we started with 433 odd products. We are now expanding it to 800. We continue to basically receive products from manufacturers across the country and you know, over a period of next three months, we will probably be there closer to that number. 750 to 800 is the number I am looking at. That's the number of SKUs. And these are all over the thing there is oral solids, tablets, capsules, everything out there, including ointments, creams and syrups and all. The contribution is roughly around 14%, 14.5% as of now in Hyderabad alone, Hyderabad and the rest of Telangana. We expect, and that's the number in July by the way, not in June. And going forward, we expect this number to go closer. Actually, I fully expect that as we increase the overall number of products, the 14.5 can easily go north of 20.

Unit economics? On unit economics side, a Rs. 100 MRP drug has a weighted average discount of roughly 55% to 60%. So, it actually ends up getting a net revenue of 40. Our margin on that would be anywhere from 60% to 70% on the net sales value. If you look at the MRP side, then, obviously, it will be slightly lesser, but on the net sales value it is fairly significant.

Mehul Sheth: Secondly, you were guiding your overall top line growth to be somewhere in the range of 25% to 30%. Now you are like, would you like to change your guidance considering there will be a scale up in your branded private label sales?

Madhukar Gangadi: Is your question the impact of margins between our traditional private label and the new MedPlus brand? We are struggling to comprehend your question. Did you ask us to make a comparison of the margin between our traditional private label and the new MedPlus brand private label?

Mehul Sheth: Yes, sir, that you can obviously provide, but I was also asking that, okay, you can finish this answer. Then I have one more question to ask.

Sujit Mahato: Old private label margin structure versus new private label?

Madhukar Gangadi: So, let's take both the products are selling at around Rs. 100 MRP. The old one would have had a net revenue of 83 and the new one probably has anywhere between 40 to 45. So, that would be the revenue side.

On the margin itself the products are made at more or less the same cost. So, we would

definitely , after GST , after the incentive to employees and after all the other costs , we would land at roughly around 21 % to 22 % on a MRP upon that . So, let me put it this way . Rs. 21 to Rs. 22 versus Rs. 50 on the whole if the price of the product were to be Rs. 100, and this is after taking into account all costs including GST and employee incentives .

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Mehul Sheth: So, I was asking about the overall revenue growth for the company . So, our expectation was that company can grow around 25 % to 30 % range , and now with addition of this new business venture , so would you like to change your top line guidelines or anything , like it will be plus 30% or something ?

Madhukar Gangadi: No, not really . See, this is going to be a significant but still a small portion of our overall business , but while it can go to 14 % to 20 % or whatever , it also will help us basically add that many customers . So, I really don't see any need to change the overall top line. I don't see any need. We will definitely let you know if we feel the need as we go forward .

Moderator: Thank you. Next question comes from the line of Madhav Marda with Fidelity . Please go

ahead.

Madhav Marda: Just two questions from my side . The first one was with this the MedPlus branded pharma range which you all are launching , if you just take any one SKU as an example , if somebody buys a branded generic version versus the one that MedPlus is selling , what would be the net saving that the customer gets ? Just trying to understand that bit .

Madhukar Gangadi: So, it's a little difficult to give an exact number because it would vary depending on the type of drug. If the product is a DPCO kind of product on the price control , then earlier he would normally get a discount of , let's say , 20% if he bought , if his overall bill size was above 1,000. Now, this discount would be anywhere from 30 % to 35 % or even 40 % discount . That's under price control .

On the ones which are not under price control , it could go anywhere from 50 % to 90 % discount . So, the net saving out there would be , let's say , 52, even if you take it as 80 % of the top end , so it would still be a 60 % additional discount on the product to the customer .

So, anyway , let me just qualify it by saying one more thing. While the entire range is 50 to 80 , what we are seeing is the overall average discount which we are giving is around 60. So, you would say around 40 % savings on a typical above Rs. 1,000 purchase of a branded drug right now.

Madhav Marda: 40% saving for the customer ?

Madhukar Gangadi: Yes.

Madhav Marda: And just the second question was basically , as we expanded more into the Tier-2, Tier-3 and beyond kind of locations , how does the unit economics change in terms of the kind of investment that we make and the sort of sales the store bit margins , if you could just help us understand that economics there ?

Madhukar Gangadi: Tier two and Tier-3, obviously , the rents are lower. The overall inventory would definitely be slightly lower and the cost of manpower is also lower out there and which also means that the

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top line is also slightly lower by at least 10 % to 15 % , but what we have seen is given that the product mix is usually better in terms of profitability for us because a lot of people accept the private label out there , we see that on a smaller top line, the net EBITDA is slightly actually higher in a small town at the store level

Madhav Marda: EBITDA margin is slightly higher. Okay.

Madhukar Gangadi: Yes. That's because the costs are lower and the product mix is better for us. There is no private label out there.

Madhav Marda: And what percent, okay, you mentioned how many stores are there in Tier-2, Tier-3 now? You will mention that , right ?

Madhukar Gangadi: I think , roughly around 30 % of our stores now are in Tier-2 and Tier-3.

Sujit Mahato: 53%.

Madhukar Gangadi: So, it's 53%. A correction out there. It's 44% on Tier-2 and Tier-3.

Moderator: Thank you. Next question comes from the line of Bino Pathiparampil with Elara Capital. Please go ahead.

Bino Pathiparampil: It's just a follow-up on from Nikhil, I think . This is regarding the revenue per store . You know, I believe many of your new store additions are happening in Tier-2, Tier-3 towns where the population will be more and maybe in distant suburbs of cities where the population will be less. So, is it possible that our revenue per store can remain a bit subdued compared to where it was earlier even if the stores mature ?

Madhukar Gangadi: You are saying because we have more stores in small towns, the revenue will not grow because we are in not in the bigger....

Bino Pathiparampil: Yes, the revenue per store for those stores will be lower compared to our mix of stores which a year back. So, will the overall revenue per store remain a bit subdued compared to what it was earlier?

Madhukar Gangadi: No, no, see, it's always going to be slightly lower than a Metro, but the costs out there and the

product mix is better for us. Hence it is actually a more profitable store. We end up actually, there is a lot of potential for opening these stores in small towns, and so once the supply chain is set up, once we have the central warehouse in the main capital of the city and once we are getting all the supply line set up out there, it is every additional store is profitable, and hence we pursue that. Yes, the top line will definitely be slightly lower than the ones in a big city.

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Bino Pathiparampil: Second, on the diagnostic part of the business. So, if I heard rightly, you mentioned that the renewal rate is somewhere in the 15% to 30% range. Are you happy with that? You think that is a sustainable business or what are the benchmarks out there?

Madhukar Gangadi: Yes. So, this is not an insurance kind of business where people have to necessarily come and renew on day one. We are seeing other subscription businesses across the world, and we see that wherever people don't need to, there is no big penalty on not insuring in exact time, people tend to renew over a period of six months. So, we are happy with the way things are going. I don't think this is as expected I would say.

Bino Pathiparampil: So, the business model as designed by you works fine even if the renewal rate is 15% to 30%. Am I correct?

Madhukar Gangadi: See, think of it this way. We have 12 large centers in Hyderabad. 11 actually set up and 12th one will get added on. We have 120 odd collection centers and we also have home service, and these can serve the entire city of Hyderabad which has a population of a crore people. Our model basically is dependent on profit from a subscription base.

And today within the year and most of the, within let's say 18 months where at least for a majority part of this period in which we started the business, since we started the business, we had only two centers. Despite that, we have actually gone to 110,000 members. We see no reason why it cannot be a 250,000 or even a 500,000-membership base as we go forward and from that I know some people there will be a rolling kind of renewal kind of thing out there. We are fine with that.

So, we are looking at a much bigger pool of subscribers as we go forward as the brand continues to grow and as more and more people become aware of the services we have.

Moderator: Thank you. Next question comes from the line of Ashita Jain with Nuvama (Edelweiss). Please go ahead.

Ashita Jain: Sir, my first question is on gross margin. So, just trying to understand this 130 bips Q-on-Q compression is purely because of the private label dip? Or are there any other factors in play which could reverse and we can see some margin accretion in coming quarters?

Madhukar Gangadi: See, the gross margin, if you actually look at it, the direct effect is of the 0.4% extra discount actually, and this is, I will say, kind of seasonal. There is also the, I would say, reduction in the private label itself. Private label typically in Q1 is slightly lesser only because of the reduction in the infectious standard diseases and all, which usually helped in terms of OTC and a lot of the antibiotic drugs.

So, both those combined have actually seen a pretty big dip in the overall gross margin, and we also have a small one-time inventory write down kind of thing, and that has also resulted in a slightly lesser than usual gross margin.

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Ashita Jain: So, since you have also introduced this MedPlus Pharma Advantage plan and which you are offering at 50% to 80% discount, so is it fair to understand that 21%, 21.5% kind of gross margins are kind of steady space?

Madhukar Gangadi: Yes. So, as I said earlier, despite the discount and despite whatever we do out there, it is still margin

accretive. It actually is slightly north of 20% out there for us on the MedPlus branded drugs. And the important thing out there is this. This is Agra. The MRP is in our control to a large extent within the guidelines of the government regulations and all, and 50% to 80% discount is also something which is in our control.

So, the goal for us would be to bring in as many customers as possible, increase the overall

customer base , and then we would have the perfect kind of platform to increase or decrease the discount as we go forward . 50 to 80 is a pretty wide range . I don't think it would be too difficult for us to increase our profits as we move forward .

Ashita Jain: And just lastly on this diagnostic , so excluding the fourth center losses , is it possible to share what was the EBITDA loss for the rest of the three ? Do you expect all the four centers to EBITDA neutral ? And then what are the steady state margins post that that you are expecting from this entire business ?

Madhukar Gangadi: Ashita, what we have been saying earlier and we are kind of on track is, we have been saying that these centers turn EBITDA neutral from the eighth month , roughly around the eighth month onward . And even though Q1 has been a seasonably weak quarter , we have been seeing the trajectory in that direction . But if you have a more specific question , then we will take it offline , please.

Ashita Jain: And just last one , just one clarification since you have launched only in June , this MedPlus Advantage Pharma . Where are you reporting it ? In branded pharma only or it's including private label. Your own MedPlus brand?

Madhukar Gangadi: Ashita , I think your question is where are we reporting the revenue for the new MedPlus branded pharma? Is that your question ?

Ashita Jain: Yes, the new plan.

Sujit Mahato: It is still , as of this quarter , it is still included in private label pharma. So, it is part of that earlier group of MHS and Wynclark .

Madhukar Gangadi: But its contribution in the last quarter would be really small given that we launched it only in Hyderabad and only on June 17th . So, I expect we probably have a slightly better kind of reporting this quarter , July, August , September .

Moderator: Thank you. Next question comes from the line of Nikhil Mathur with HDFC Mutual Fund. Please go ahead.

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Nikhil Mathur: Sir, I just wanted to extend the argument of Metro the non -Metro store expansion . So, in summary , how should we conclude ? Is the EBITDA per store better or worse versus a Metro store in non -Metro cities ?

Madhukar Gangadi: It's actually better .

Nikhil Mathur: EBITDA per store is better ?

Madhukar Gangadi: Yes.

Nikhil Mathur: And what about working capital ? I mean, is there any more elongated cycle of working capital in non -Metros?

Madhukar Gangadi: Not really . No, because the range of products which are asked for in a non -Metro store are much lesser , both in terms of medicines as well as general goods, and as we go forward , as the private label continues to increase , the MedPlus brand of private label , if we go north of 20 % , given the fact that our cost of that product is in the , I will say, low teens on a Rs. 100 product versus a 66 or 67 cost of products for a branded product and given that the small towns will actually sell more , hence you would stock more of that , we actually think that we will have less inventory in store . So, the return on capital would actually be better .

Nikhil Mathur: So, basically a non -Metro Store is a creative on both working capital as well as on EBITDA per store?

Madhukar Gangadi: Yes.

Nikhil Mathur: And sir, since you have 10 warehouses , can we get to know how well are these warehouses currently utilized ? I mean , are your warehouses back to capacities or there is a significant space yet to be properly

utilized in these warehouses ?

Madhukar Gangadi: No, no, I am afraid most of the warehouses are utilized almost fully . We are looking to expand a little bit here and there , but again , that's a little bit of CapEx . I don't think the OpEx will actually increase . As we actually made these warehouses more spacious and more easy to operate in , we expect the cost of running them will actually come down . No, as I answered your question , they are all being utilized well , and I don't really see too much expansion in the overall cost even as we expand the number of stores . It will change as we go , let's say , into newer states and as we expand into Kerala , Chhattisgarh and Madhya Pradesh. Right now we are operating from the warehouses in the states which are adjacent to those , but as we go forward , as the number of stores will become 20 to 50 and so on , we would definitely need to set up somewhere else , but that's not in this year .

Nikhil Mathur: And one final question related to this . Is there a scope for a lot of automation in your warehouses ? I mean , there are so much technological breakthroughs happening in terms of vertical cladding , energy cost saving . You can do a full dark kind of warehousing without the

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requirement of light and energy and all . Are there a lot of technological developments that

MedPlus as a company can deploy in its warehouses to kind of draw a lot more efficiencies ?

Madhukar Gangadi: Absolutely. So, we are now looking to automate our warehouse in Chennai fully, and this will probably happen over the next quarter or so . We, in fact, would like to tell you that we have actually developed the technology in -house . The proof of concept is already there . In fact, half of the oralor at least half of the tablets and capsules in our warehouse in Hyderabad are now picked from the automatic pick ing machine which has been developed by us . And the reason why we have not actually extended it fully is because it requires a particular kind of space and we are actually looking to build something in Hyderabad for that . That got delayed. So, we have actually now started to develop this in Chennai . We have rented a place and we are looking to actually automate that fully . So, we will let you know, but I am hoping that at least by the year end we should have it fully working .

Nikhil Mathur: In the medium to long term , what kind of implication can this have on your margins ? Any broad range that you can share , what kind of savings can be generated ?

Madhukar Gangadi: Given that our warehousing now is at a 3% overall cost and manpower is a big part of the overall cost , as we automate it and as we make it , first of all , the whole purpose of automation is to also making it super efficient , make sure that products are sent to our stores every single day without taking a break for holidays and vacations and festivals and all that. So, that, of course will make the overall thing much heavier , but we will also be able to save in the range of let's say 0.75 % to 1% on the overall cost . So, I would say a 3% cost would probably end up coming somewhere to 2% to 2.25%.

Moderator: Thank you. Ladies and gentlemen , due to time constraints , we have reached the end of question -and-answer session . I would now like to hand the conference over to Mr. Madhukar Gangadi for closing comments .

Madhukar Gangadi: Thank you. I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com for any further queries . Thank you.

Moderator: Thank you. On behalf of MedPlus Health Services Limited, that concludes this conference . Thank you for joining us . You may now disconnect your lines.

Call: Nov 2023

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November 10 , 2023

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Symbol: MEDPLUS
Dear Sir/ Madam,

Sub: Submission of earning call Transcripts for the quarter & half year ended September 30, 2023 .

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 , we enclose herewith transcript of the earnings calls held on November 8, 2023 for the quarter & half year ended September 30, 2023.
Kindly take the same on record. The same is available on Company's website at www.medplusindia.com

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer
FCS 7460

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MANOJ KUMAR SRIVASTAVA
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Conference Call
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MANAGEMENT : MR.MADHUKAR REDDY GANGADI MANAGING
DIRECTOR &CHIEF EXECUTIVE OFFICER,MEDPLUS
HEALTH SERVICES LIMITED.
MR.SUJIT MAHATO CHIEF FINANCIAL OFFICER,
MEDPLUS HEALTH SERVICES LIMITED.
MR.CHETAN DIKSHIT CHIEF STRATEGY OFFICER,
MEDPLUS HEALTH SERVICES LIMITED.
MR.PRASAD REDDY MEDPLUS HEALTH SERVICES
LIMITED.

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Moderator: Ladies and gentlemen, good evening and a warm welcome to the MedPlus Health Services Limited Q2FY24Earnings Conference Call.

As a reminder, all participant lines will be in the listen only mode. There will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing *then 0on your touchtone phone.
Please note that this conference is being recorded.

I now hand the conference over to Mr. Prasad Reddy from MedPlus. Thank you. And over to you, sir.

Prasad Reddy : Good evening everyone.

the MedPlus Q2 FY24 Earnings Conference Call to discuss the financial results of MedPlus for

the Second Quarter of Financial Year 24 which were announced earlier today. We have with us today the Senior Management team, represented by Mr. Madhukar Reddy Gangadi Chief Executive Officer and Managing Director; Mr. Sujit Mahato CFO and Mr. Chetan Dikshit CSO.

Before we begin, I would like to mention that

may be forward looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties are on Slide #1 of the Investor Presentation shared with all of you earlier. Documents relating to our financial performance have been circulated earlier and these have also been posted on our corporate website.

I will now handover the call to Mr. Madhukar. Thank you. And over to you Madhukar.

MadhukarReddy : Today I would like to talk to you about the four main features of the Indian Pharmacy retail industry as we see it and the opportunities these features actually present to us.

i) The industry itself is extremely fragmented.

ii) It is a big resilient, small and medium sized set of operators out there who still have nearly 85% of the overall market.

iii) The other factor would be the active involvement of government in promoting low cost generics.

iv) Marked absence of store brands in the country, you know like in U.S. and UK and all where Walgreens and CVS and all have their own store brands.

Just to come back to the main points out here, the Indian pharma retail industry as I said has over 900,000 retailers serving a population of 1.4 billion people. To put that into perspective Indian market has roughly 20 times as many pharmacies as in U.S. while our population is only four times that of U.S. The U.S. market has roughly 45,000 to 48,000 stores catering to their over 350,000 million people.

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So, 900,000 retailers for a market that is less than 200,000 crores makes the average per store any normal business standard

should be considered extremely low throughput and as a result unviable actually. The reason it still works in India is because of the prevalence of high margin trade generics. Some of the lower throughput stores have a 50% or more share of trade generics where often the margins are above 70%. This makes even a store that sells Rs. 20 lakhs per year viable with an average margin of 50% which takes care of the fixed cost and makes a salary level wage for the operator.

Then if trade generics is how the small operators survive the reason for the resilience of the mid to large size independent retailers is not really much different. As you all know the retailer and distributor margins in India are 20 and 10 for drugs not under DPCO and only 16 and 8 for those under price control.

enjoys both retail and distributor margins actually get a margin of around 20% or 21%. Despite this we see them competing with us and with the other large key pharmacy guys with a 20% discount. We have seen this happening in most South Indian markets, and we have seen them actually mushrooming, some of these stores.

What we have seen and what we have actually observed when we have shopped in this stores and what we have from industry knowledge is that most

t of these operators, a large number of

them at least do substitution for the prescription which they get with trade generics. So, an average bill of Rs. 1500 if 20% of it is substituted for a trade generic product, that they would still make Rs. 150 at least on that. The trade generic of Rs. 300 basically fetching them at least Rs. 150, 50% margin, which still makes an overall 10%, and which is how most people today seem to be competing.

So, in a super competitive scenario where online players are willing to lose money, and whereas large retailers like us are using our buying power to actually give, choose discounts we actually see a lot of the small operators still continuing to give the same discount and still continuing to actually compete with us.

While the first two factors depend on selling trade generics at full price the other third big factor on the list has the potential of changing the industry in the opposite direction: selling generics at hugely discounted price. Government has introduced Jan Aushadhi Stores a while back and the fact that they are even selling anything despite them not being located in the best places and not carrying the full assortment is proof of the fact that the Indian consumer has started realizing that 80% of the drugs sold in the market are only variations of a limited set of actual formulations.

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While the effect of these stores on the sales of regular pharmacy retail is not big as of now, if things were not to change then the effect will actually continue to grow should not be in any

doubt. The recent NMC notification, although it was withdrawn after a short while asking all doctors to write only the generic name in the prescription is one more step in the direction of a generics market. This we can be sure will come in some form or fashion back to take the branded generics out.

Another key feature or lack of, I would say, in the Indian Pharma retail market is the absence of store brands. In U.S. for example customers get to buy either the innovator product and if the innovator product is not longer on patent or if it comes off patent, the store brand. So, customers could still continue to buy the innovator product even if it is off patent for a slightly lesser cost. Or they would buy the store brand which would either be by Walgreens, Rite Aid or CVS there is nothing in-between.

Indian market on the other hand does not have this, largely because till now there was no large chain that had a trusted brand that customers could buy. Or no large chain that had the scale to get a full assortment of drugs manufacturing. You require a certain number of stores to be able to get the MOQ manufactured for, the large set of drugs. And you could perhaps get away with maybe the Top 50 or Top 100 but to actually get the Top 500 drugs or the Top 800 it would require substantial scale and till now there was no one actually there in the Indian market.

This has led to a proliferation of a large number of me-too branded generics resulting in pharmacies being forced to carry several thousand SKUs to cover the prescription while essentially stocking a few hundred actual therapeutic types only. So, all the above-mentioned four resulted in us taking our next step in our path to becoming the largest pharma retailer.

End June 2023 MedPlus launched over 400 drugs in Hyderabad and these drugs which could potentially provide substitutes for over 55% of the medicine needs of our customers. We launched them with a discount of 50% to 80% on the MRP and the MRP Maximum Retail Price itself was tagged to the price of the marked leader for the corresponding drug composition. We were able to do this because we obviously don't have any marketing cost, that we own the channel, and we are able to pass on all the sales to the customer. And we were also able to convince the customer of the value proposition. Our aim in doing so was to

take advantage of

the fact that the Indian market has matured and has become knowledgeable of the fact that 80% of the drug sold in the market are off patent and their generic substitutes which work just as well as the innovator product as long as they are manufactured in a good factory.

This is also aimed at bringing in customers who otherwise shop at Jan Aushadhi Stores and their like. And our aim was not just to get these customers but also anyone who understood the fact that all the drugs are generic in nature, but who was looking for slightly better, I would say better quality manufacturing than what was available in the market. So, to these customers also, our appeal actually worked well.

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MedPlus making MedPlus Store brand available at 50% to 80% below MRP also makes it difficult for small stores to sell at full price trade generics. And if they are not able to sell it, as I said, stores which otherwise sell Rs. 20 lakhs a year or Rs. 1.5 lakhs a month or Rs. 1.6 lakhs a month, if there access to the margin which is afforded by trade generics is taken away they actually become unviable and we expect that will be one of the ways in which the market will consolidate as small operators basically stop doing this.

MedPlus also in future, plans to make trade generics from companies like Cipla, Alchem, Zydus and the likes available at the same price as MedPlus Store brands. This will now make it tougher for stores that are substituting promoted brands with trade generics to compete with us and to offer the 20% discount which we actually give on branded products.

This approach not only positions MedPlus as a cost-effective choice, but also enables the company to capture a significant portion of the market that was previously dominated by at least was populated by I would say a large number of smaller, independent players. Thanks to our comprehensive control over the entire supply chain MedPlus can strategically adjust pricing and discounts, thereby optimizing profitability. This grants the company a competitive advantage in effectively managing its profit margins in achieve a favorable financial position in the market. Additionally, as more and more customers take to our products, we expect our inventory cost to come down significantly. As the cost of stocking MedPlus brands is only a 5th of the cost of stocking branded generics. We also expect fill rates to get much better in our stores for customers who have adopted the MedPlus brand. Needless to say, it is much more easier to stock 500 to 600 products in sufficient quantities versus stocking 47,000 SKUs that we now stock across our network.

MedPlus products were introduced in Hyderabad and the rest of Telangana at the end of June 2023 followed by their launch in Andhra Pradesh in 2023. Notably witnessed a substantial 15% increase in September compared to June, a growth 3 times higher than that observed in Bangalore and Chennai. Out of the 15% portion a large portion 9% has been contributed by the MedPlus Store brands. Furthermore, the regions outside of Hyderabad

and Telangana and Andhra Pradesh have experienced remarkable growth than compared to similar territories in Karnataka and Tamil Nadu.

We are happy with the way the customers have taken to our brands and we have now launched them across all the states in October.

and meanwhile I will now hand it over to Mr. Sujit to give our quarterly updates.

Sujit Mahato : Good evening, everyone. As of September 30th, we have been serving the Healthcare and Household needs of our community in 599 cities across 9 states through our extensive network of 4089 pharmacy stores. During the current quarter, we have successfully expanded our MedPlus Health Services Limited

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presence to 18 additional cities. In addition to our pharmacy operations in Hyderabad, MedPlus operates four full service diagnostic centers, se

ven Level-II centers and 120 collection centers.

These facilities play a crucial role in our commitment to providing affordable diagnostic service to our customers.

An update on the network, our store expansion program remains on track as we continue to balance growth and profitability. Over the past 12 months, we have added a net total of 761 stores to which 139 stores opened in Q2. Notably West Bengal and Karnataka saw the highest number of store addition with 32 and 26 outlets respectively.

Of the store openings in Q2 48% were in Tier-II cities and beyond, reflecting our strategic focus on these markets. Currently out of our 4089 stores around 1819 stores reflecting 44% are located in Tier-II cities and beyond. We recognize the potential of these markets and aim to further expand due to the maturity of our operations and robust supply chain capability. During Q2 we experienced 25 store closures. Considering both openings and closures we achieved a net addition of 114 stores in Q2 compared to 153 stores in Q1.

In terms of the age of our store network approximately 21% of our store are less than a year old. Around 26% of our stores are in their second year of operation and the remaining 53% of our less than two years age bracket are still in their ramp up stage. From a financial perspective they continue to have a negative impact on our operating EBITDA. However, as these stores mature we anticipate them contributing positively to our profitability.

We closely monitor the time it takes for our new stores to breakeven. For stores open between October 2022 and March 2023 approximately 54% of them achieved breakeven within six months of operations. Additionally, as a cohort all the stores combined achieved breakeven in just five months.

In terms of the network store size, as at the end of the quarter, our network has grown to 4089 stores. We are 2.1 million plus square feet compared to 3320 stores and 1.8 million square feet at the end of September 2022. The average store size is 538 square feet. To give you a sense of spread in store sizes, we have 2958 stores that are less than 600 square feet and 1131 stores that are greater than 600 square feet.

In terms of the revenue mix with our expanded scale, we have strategically positioned to enhance our revenue share from private label products. Our private label range is designed to offer customers high quality product at affordable prices. Currently MedPlus offers around 1400 thoughtfully curated SKUs spanning across pharmaceutical and non-pharmaceutical category. Private label sales accounted for around 14% of our total revenue. Furthermore, our expanding presence in Tier-II cities and beyond is making a significant impact on our revenue mix. Sales from these markets accounted for 34% of our pharmacy revenues in the current quarter demonstrating an increase from 31% in the same period last year.

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Financial numbers -

Consolidated Revenues

Our Consolidated revenue was Rs. 14,086 million with growth of 25.7% year-on-year and 9.7% sequential basis quarter-on-quarter. Our consolidated operating EBITDA stood at Rs. 410 million representing 2.9%.

Pharmacy

Around 99% of our revenues is from our pharmacy operations. The pharmacy operating like to update on our store older than 12 months, revenues from these stores in Quarter 2 was Rs. 12,520 million or 91% of pharmacy revenue. These stores had a store level EBITDA margin of 9%. The store level ROCE of these stores stood at 50.6%, a word here on the store level EBITDA margin by vintage.

While stores greater than 12 months had a margin of 9% this was 9.8% for stores greater than 24 months and 6.5% for stores in the 13 months to 24 months age bracket. If the allocated non-store related cost than the operating EBITDA of stores greater than 12 months would be Rs. 530 million which translates to a margin of 4.2%.

Diagnostics

Our diagnostic numbers,

Diagnostic revenues has grown to Rs. 182 million in Quarter 2 compared to Rs. 58 million in Quarter 2 FY23 which is primarily due to launch of the new centers in Hyderabad. Diagnostic segment recorded an operating EBITDA loss of Rs. 29 million compared to a loss of Rs. 53 million in Q2.

Working Capital

Our networking capital for Q2 was 64 days. The inventory in our warehouse was 37 days. As you are aware because of the sales trajectory of new stores, their inventory turnover is lower in the first year. In Q2 the inventory level of our first year stores was 110 days. In comparison for our stores older than 12 months, the inventory was 41 days.

Now I request Chetan to update on our Diagnostic business. Over to you Chetan.

Chetan Dikshit: Good evening, everyone. To recap what Sujit has already said, in our pilot market of Hyderabad we have four full-service diagnostic centers, eight Level-II centers and 120 plus collection centers. In July 2023 we had increased the price of our plans by Rs. 150. This was a 15% increase. On earlier occasions we had modified prices but at the test level. So, this was our first instance at testing price increase at scale and it has not affected the offtake of our plans. This is positive.

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In July we sold 348 gross plans per day. In August and September this was 381 and 408 respectively. As on 30th June, we had 105,000 active plans and 186,000 underlying lives covered under our plans. As of 30th September we have 117,000 active plans and 207,000 underlying lives. Our current observed on-time renewal rate is 15%, but in 6 months post-expiry we have observed the renewal rate to be 40%.

Moderator : Thank you very much. We will now begin the question and answer session. The first question is from the line of Tanmay Gandhi from Investec. Please go ahead.

Tanmay Gandhi : My first question is on profitability, so if we look at the operating matrices for this quarter, so we have probably the best quarter in last 8 quarters right, in terms of store aging. So, we have only 20% of the stores less than 12 months; in terms of private labels, we are very close to all time high of 14.1% or 14.2%, right. And still in terms of operating EBITDA margins we are nowhere close to the recent peak of 5%. So, what exactly is dragging our EBITDA margins?

Madhukar Reddy : In terms of, in the Investor Presentation we have guided on the second last slide that the drag from the new stores and the stores which were opened in the last 12 to 24 months, they are still a drag on our P&L and on a matured state basis we should see those inching northwards.

Tanmay Gandhi : But even if we look at the EBITDA margins for stores which are more than 12 months old and within that also we have a very favorable mix in terms of stores which are more than two years old. Still, it is only 4.2% versus the 5% which we had seen. And we have a very high contribution coming from gross margins, what exactly is dragging our margins even in case of older stores?

Madhukar Reddy : Tanmay for us the overall maturity is only at the level of 24 months and more, 24 to 30 is when we actually hit the 10% gross margin. And below that then we have the apportion of cost of the

So, there, the mix is not quite there. So, that we definitely need these to come to the level of that Rs. 15 lakh odd sales per month, per store kind of number for it to actually get to that number.

Tanmay Gandhi : No sir actually my question was --

Madhukar Reddy : But we are going in the right direction.

Tanmay Gandhi : --More than two-year-old stores are at 53% which is 5 quarter highs and still we are not 5 quarter high in terms of margins. But anyways I understand. And secondly in terms of store additions, we had initially guided for 1000 plus stores and now I think we are at 800 to 1000 stores, so just wanted to understand that do you want to adjust your guidance given that we have only added a

bout 260 stores in the first two quarters of the year.

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Madhukar Reddy : So, in the beginning of the year we said 800 to 1000, we were looking more at 800 stores. I really think we will actually be able to ramp up in the Q4. We may be somewhere close to that but if there is any need for us to adjust the number,

Tanmay Gandhi : So, have we seen any, are we facing any challenges in terms of store opening, maybe some regulatory challenges or some bandwidth related challenges because in last two quarters it has meaningfully come down. And again, this coincides with our new initiatives of diagnostics, of generic or rather MedPlus branded generic side. So, are there any constraints in terms of

bandwidth or these are just timing issues in terms of regulatory approvals.

Madhukar Reddy :

have been some regulatory issues here and there, those we expect are normal. While we should have basically had something like 400 stores, we are at some 260. But these numbers are usually adjusted towards the end of the overall year anyway. Last four years our Q4 has been the best and so we expect that we will continue to ramp up going forward. Definitely not a bandwidth issue. Yes, we are going into some new states, that is taking us a little bit of time. Kerala, Madhya Pradesh and Chhattisgarh but that is to be expected. I would say whatever adjustment we have made is mainly to make sure that we can just get to the guided number of 800 odd stores, all.

Tanmay Gandhi : Last question from my end, so we have recently obtained an enabling resolution for QIP and as of September 2023, we already have about Rs. 220 crores of cash on books. And anyways our quarterly store additions have come down. So, can you give us some color on where will you deploy such kind of funds? And what is the exact number of fundraise you are looking at in the near term?

Madhukar Reddy : No, as you said this is mainly an enabling resolution only. If we do decide to basically go get the money then it will be only to bolster the overall balance sheet and we will use the money in the best way, but yes, I would say at this point it is just being ready to raise if necessary. And again just to clarify, if at all we end up actually doing a raise then it will be for growing the pharmacy faster, I would say one. And second definitely on the diagnostic side. And of course we will require some money for working capital as we go forward.

Moderator : Thank you. The next question is from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed : Can you talk a little bit about the seasonality in the business, the second half versus the first half? When I look at our quarterly margins for the pharmacy retail segment, last year second half was much stronger. So, should we expect a similar trend this year as well?

Madhukar Reddy : So, there is definitely a little bit of seasonality, not sure if extends all the way to the second, third and fourth quarters and all, but first quarter typically we have seen a softening normally during

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the peak summer months and if especially the summer month gets extended then you definitely y because people obviously tend to

too much seasonality on this, Harith. There is always the festival factor which comes in during the extended Dusshera, Diwali periods and all and the vacations period, that is not a time in which people typically tend to go to hospitals or to doctors and all, nothing elective gets done during those days. Doctors themselves are on vacations and all. So, we see a small dip during those times, but otherwise there is not much I would say.

Harith Ahamed : Couple of questions on the MedPlus branded private label that we have launched. So, when I look at the contribution of our revenues this quarter. And it has come d

probably leading to this lower share. But we were also ex

pecting the higher volumes to kind of

offset the impact of discounts, right. So, how should we look at the contribution from private label pharma, should it inch back to the previous level of close to 9% that we saw in the September quarter FY23?

And second one on the same foray would be on our plans to expand beyond Hyderabad and rest of AP, Telangana into our other key markets?

Madhukar Reddy : Obviously the second part of the question answers the first one. We have seen a significant increase in volumes where we have introduced private label on the 50% to 80% discount. In Hyderabad, rest of Telangana and in Andhra where we did it successively, we started in Hyderabad in end of June then after that in July, we did it in rest of Telangana and in September we did it in Andhra. We have actually seen on a GMV basis close to 15% pharma private label in these states. So, if we have not seen the overall thing, it is because Hyderabad and rest of Telangana are a portion of the entire country. But we are now going to the reporting on a GMV basis all our sales, on this overall to show you the effect of the overall private label and all. And you will see a significant increase in these.

Just to give you an idea here Harith, one second, so we introduced roughly around 433 products in the end of June in Hyderabad. Those account for 55% of all the medicine needs which we have, which is you know if we say that 80% of all my sales are medicines and out of that 55% that 44% of overall sales. We had substituted 44% of the medicines which we sold. What we saw at the end of the quarter was that 15% of our sales now are coming from these discounted products. So, which means 15% out of 44%, which is a third of whatever we offered, was coming in from sales of private label products in Hyderabad and rest of Telangana, and slightly higher in rest of Telangana and Andhra actually.

Harith Ahamed : And then our preparedness to go beyond Hyderabad and AP, Telangana with this --?

Madhukar Reddy : So, we have now launched except in Karnataka and Maharashtra where we had a little bit of hurdle but otherwise, we have launched across the rest of the country. And probably we will be
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doing that also in those two states also, shortly but yes. So, as of at least now, as of November 1st at least you can assume that we are there across the country with our private label, in all the states in which we are working, that is, with our private label offering.

Moderator : Thank you. The next question is from the line of Vilina Jain from Perpetuity Ventures. Please go ahead.

Vilina Jain : So, firstly on the pharmacy business, there were 14 store closures for which in the presentation you have mentioned that the reason is others. So, what is basically involved in this?

Madhukar Reddy : So, for us the others are mainly due to various reasons it could have been lease which has expired or we have changed it because of location problems or whatever which have come up in the middle. So, as we can see, the age of the other store is also 5.6, the average age is 5.6, so they are not stores which we just took and made a mistake and had to move, these are stores which where we found better locations to actually move to, so that would largely be the cost. I can get back to you with the exact details after the call though. But the fact that the average age of these stores is 5.5 years should tell you that these were regular stores which we had to move for other reasons.

Vilina Jain : Secondly on the diagnostic business when do we expect the business to breakeven? And what are the target active subscriptions by the end of FY'24?

Chetan Dikshit : Vilina, we want to take advantage of Q3 and Q4, Q4 tends to be slightly strong seasonally. But at this stage we are not giving firm guidance. And on the second part of your question was in terms of what are the active plans goals that we have set, see we

kind of look at the active plans

as well as the active lives the target that we are currently looking at is 250,000 lives, we expect to close the year below 150,000 plans sold, active plans. Again we are not in a position to give guidance at this stage.

Vilina Jain : Any plans to take the diagnostics business to other cities or states?

Chetan Dikshit : Well at this stage we do, see we do have residual legacy diagnostics business in few cities. These are legacy from our earlier phase. What we talk about as the current experiment that is being carried out in Hyderabad, there is no plans to go outside of Hyderabad as of now.

Vilina Jain : Lastly of the active subscriptions that is 117,000 currently, how much of this is renewed and how much of this is new subscription base?

Madhukar Reddy : Of this roughly 20,000 of those which have gotten renewed.

Moderator : Thank you. The next question is from the line of Lokesh Manik from Vallum Capital. Please go ahead.

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Lokesh Manik: My question first was a clarification in the DRHP it is given store payback period is 3 years, maturity is 4 years. In your presentation you are reporting numbers at 2 years plus. The presentation shows stores greater than 12 months are operating at 8.89% that is the margin. So, I am just confused as to what is the exact or common average and what is the maturity period for a store that you consider?

Madhukar Reddy : So, the maturity period continues to be the same which is around 24 to 30 months or so. The payback period is 3 years, is what we say. If we are getting stores more than one year at 8.9%

Lokesh Manik: Yes

Madhukar Reddy : No, . I

Anyway, just

to walk you through that, we expect our stores to breakeven anywhere from three to six months, which is when they have 0% EBITDA. At the one-year timeframe they probably have around 3% to 4%. And at the end of two years, they should have roughly around 7% to 8% or 10% and that could go on up to being a 30-month thing, in which they are around 10% EBITDA at the store level for an average.

Lokesh Manik: My second question was on store closures, what we have ideally seen in the retail industry is the store closures, the age of these stores is usually younger. In our case these are older. And what is happening is that stores with an average age of six years will get closing, means significantly impacting our return matrix given these are maybe 60%, 80% according to the ROC generating higher EBITDA margins stores, greater throughput stores. Given that we are operating on thin margins so what are the steps we have taken to avoid this? As far as my understanding store closure is 5% of the number of the stores, roughly get closed every year. And these are in the higher 5, 6 years age bracket. So, just your view on what we are doing to avoid this?

Madhukar Reddy : So, it is not as high as 5% but we will get back to you on that. The reasons for store closures for us are the average lease for us is around 9 years. So, if we feel or at the 9-year period if you see

that the landlord is not renewing the lease, for various reasons or we are not happy with the location, we could tend to move so there in that case, we will probably move. And those movements which happen because of landlord not renewing or we not being happy with the rent and all, they could significantly increase the overall age of the store closure rate t

Second, for us unlike most store, we open at the rate of around 50 to 100 stores a month, sometimes, right. So, what that means for us is --. So, in the way we do it and largely because of the way the economics of the store are structured. We set up a store at roughly around Rs. 25 lakhs to Rs. 30 lakh cost, out of which Rs. 2 lakhs to Rs. 3 lakh goes towards the rental deposit and Rs. 6 lakhs to Rs. 7 lakh goes towards the build of the cost store and rest is all inventory.

So,
fact that we have great availability in

our stores, and we have the best value pricing for the

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customers we are confident that we could actually go into any market in which there is a reasonable density and even if there is competition, we could actually grab business from those guys.

So, hence we go location by location, market by market and set up the store in the best available location at that time. And we constantly look to upgrade the location as our business increases or as new locations open up in the same market. And the reason we do this is because inventory, store deposit and everything, all of those are completely recoverable. 80% to 90% of the cost of our fit-out is modular furniture and the stuff which can be moved from one store to another. So, we actually lose less than Rs. 2 lakhs when we move the location. So, it's a big, let us say a problem if we have to move the store, especially when we come across a location which is better than what we currently have.

Lokesh Manik: So, in the DRHP it is mentioned the attrition rate was about 20% to 30%, DRHP is about 21%, so are we seeing the drop from there now at that attrition level?

Madhukar Reddy : See most of the people are working at minimum wage down anytime soon. We are figuring out different ways of solving this problem, but yes we would be happy to solve it, no, attrition has not come down.

Lokesh Manik: You did because of competitive intensity, or they are opening up their own after getting --?

Madhukar Reddy : Neither, see obviously there is always different kinds of things out there. It is definitely not competition. Some of the people, after coming to Hyderabad to setup or to any other big cities to actually work with us for a few years, may end up going to the small towns to go setup a store. It is neither this nor that, it is

just that, people today definitely are --. Seeing retail is almost like the first step for most people who come in from small towns into big cities. They come in, get some experience and then try to move on to better jobs as they go forward. We also expect that they work with us only for two to three years we did for longer than that.

Lokesh Manik: In diagnostics business, when do you plan to breakeven, if I am not mistaken we have invested over the last few years about Rs. 200 crores in terms of CAPEX --

Madhukar Reddy : No, I don't, we have invested only around Rs. 110 crore odd, you probably are seeing some other lease old things out there as capitalized expense. So, our investment is only Rs. 110 crore. We are now more or less breaking even at all the central level I would say. At the company guidance on the exact time of breaking even. But our sales continue to grow up and our losses continue to go down. And we are seeing a huge uptick by all the, I would say citizens of Hyderabad, we expect that if the value proposition of what we have is really good and we are seeing a really good uptake. So, we feel confident about the prospect of the old business.

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Lokesh Manik: On the strategy you are getting operational leverage through scale, so if I understand correctly a fixed cost is about 2%, so there is not much fixed cost left to create operating leverage. So, then are you then looking at expansion of gross margin because the chain that you are competing with that is a retailing distributor, gross margin level is about 28% to 35%. We are at 21.4% so then is that where you are expecting operating levels to kick in closing to that gap?

Madhukar Reddy : No, I still think operating leverage is possible as we move forward and automate our warehouses and all, that has taken us slightly longer than what we expected, but that will happen. So, today 3% of the cost goes towards warehousing and 2% on corporate. As we grow from Rs. 4,500 crore to whatever number it is this year cost or the warehousing cost will grow in tandem

with that. There is still a lot of room for

leverage out there, I think.

Gross margin, definitely but that is going to come on the back of better adoption by customers

or at least higher penetration of private label among our customers for us. So, private label is where we expect even on the discounted private label model which we have today, we still expect margins to be better as we go forward.

Moderator : Thank you. The next question is from the line of Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee : I wanted to understand the growth rate that you mentioned in the 12 month plus at around 15%. So, for those stores are growing?

Madhukar Reddy : Saion, we are currently not sharing those numbers whatever is there in the presentation is what it is. We expect the older stores to grow slightly slower than this obviously. But given the fact that we now have two levels out there private label in non-pharma products which will continue to gain traction, second, our own private label which is now going to go at that 50% to 80% discount, we expect growth is going to be much higher than inflation, much higher than the cost

Saion Mukherjee : On this experience on generic, Hyderabad you have launched it in June and there is some month of experience. So,

now with the kind of transition, you know is it more than 100, how much more than 100 or less than 100 if you can provide some color on that. And how we should think this number going forward?

Madhukar Reddy : So, I will give you a longish answer for this. I explained the four main kind of features of our Indian industry and all, and I also said how we are going to go and take more top-line. I expect our EBITDA to grow both as a result of higher HSG as we go forward as we take market share from the smaller independent retailers and all (1).

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And 2) also as the private label grows from the current 15% pharma which is in Hyderabad to 20% and above. We have already seen a pretty significant growth in the overall top-line and I expect that to continue. The margin growth though, will take a little while, because although the private label is more profitable than the branded generic, it has also in the first stage replaced the slightly more profitable, full cost private label which we had earlier. So, right now what we have is a MedPlus brand but earlier to that we had a MedPlus subsidiary brand which we used to sell at full price. So, cannibalization of that has resulted in a slight degrowth of I would say of a margin of there, what has been more than made up by the increase of overall sales, and by the increase from 8% to 15%. So, for us is Hyderabad trending at a better EBITDA, definitely, compared to earlier. The exact numbers, I think you will probably be able to see it as we go forward in the next quarter, when we give the results for the full country.

Saion Mukherjee : And finally one question is the plans that you have, is there any scope for acquisition both at the distributor level or at the retail level, is that something which makes sense given the plan for expansion that you have at this point?

Madhukar Reddy : We are always willing to look at acquisitions out there. But as of now we have nothing in mind. We are not really seeing any great targets for acquisition. But I'm pretty sure at some point we will continue to evaluate as they say and if there is a good deal out there on table we will take it.

But as

Saion Mukherjee : But you would pursue, I mean this is mainly because of valuation --

Madhukar Reddy : Absolutely .

Saion Mukherjee : So, the issue is valuation because if the industry is consolidating, I would assume that you would have target which we would like to consolidate with the largest player. And you could then add value. So, I was wondering if something which we can see in the near term.

Madhukar Reddy : See there are couple of things 1) Valuation, definitely, but 2)

also the kind of models, right. If

their business philosophy is completely different and the way they actually go about getting their customers and their value proposition to customers is completely different than it may not really blend very well with our own model. Then the only way in which we could basically run it is have two independent brands supplied by the same backend. And that could be one possible thing, but we have not really seen any I would say immediate opportunity out there. But yes, if we see

for complimentary models we obviously work at, we are a value player and we basically do discounting and everything else but there could be someone out there who caters to a different audience and who is profitable and which can be acquired at a decent price, we could perhaps look at it, at that time.

Moderator : Thank you. The next question is from the line of Madhav Marda from FIL. Please go ahead.

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Madhav Marda : My question basically was that as we see this mix of private labels, new private label which we have launched, the MedPlus Store Brand Generics I think if I picked up right, you are saying

gross margins are better. And we are seeing higher volumes in the stores where we have launched them which means there should be better operating leverage. Is it fair to understand as we go ahead the EBITDA that we make per store should structurally improve because we get better gross margin and operating leverage? I am taking like a slightly longer view if you could think from like a one-or two-year

Madhukar Reddy : 100%, as I said earlier, the reason why we have so many stores out there is because a lot of them depend on trade generics for their viability. Store which does Rs. 5000 and Rs. 10000 per days is selling 30%, 40%, 50% of its overall thing making around 50% to 60% margin. Now that we not a promoted brand, then we are there with a 50% to 80% discount on that. And in Hyderabad and rest of Telangana we are also selling trade generics at the same price. And we are testing out to see how this works. You know if trade generics are available at that number, at 50% to 80% discount that automatically means lot of the smaller stores will find it very difficult to sell it at that full price. And if they tried to sell, at a price which matches with us, the low throughput will not allow them to actually survive. We will see a consolidation, we will see the benefit of that coming to us, that is for sure. And again, as I said the Indian consumer is also getting, I would say more knowledgeable about the overall medicines and all, we are also seeing a bunch of s not a big deal, we know MedPlus, we know all these brands are same, now that you guys have something to offer, we are more than happy to actually take it and save money. So, we are seeing a lot of those people come too. So, between both, between some of the stores not giving up and sending their customers over to us and people switching to our thing, both in terms of top-line and on the margins, I expect we will see a positive outcome as we go forward.

Madhav Marda : So, payback period for store that we introduced obviously just mathematically should come down right, over the period of time as these unit economics of a store increases, our sort of ROIC profile I mean I am just 1+1 but that should happen, I would assume?

Madhukar Reddy : No, long run definitely, mainly because of this, the cost of private label is 1/5th of what it cost for us to buy a brand. So, at the sale of Rs. 18 out of Rs. 30 lakhs basically going into inventory for us. As the inventory cost comes down, as our overall investment in the store comes down definitely the ROCEs will improve. And as we continue right, today we are making let us say after all the discounts which we give and after all the incentives which we give our employees for selling our private label, we still make at least I would say 75% to

a 100% more than what

we make on branded generics after all the discounts.

Now this number where it is can only go up from here, because today the average discount in Hyderabad is roughly around 58% to 59% or even 60% on the MedPlus private label. This need not always be that number, it can easily move anywhere from 60% to 55% or even one lever which we have and that will actually improve our overall margin.

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And the normal inflation which the government allows every year of 10% right on the drug price, that will also, some of it we will pass to the customer, because you know that 10% does get passed on us, 50% to 55%

expect that the price of manufacturing it will go up as much.

Moderator : Thank you. The last question of today is from the line of Sayantan Maji from UBS. Please go ahead.

Sayantan Maji : My question is regarding the private label portfolio. So, if I look at the private label portfolio and considering that there would have been some cannibalization to the earlier trade generics that we were selling. So, is it fair to assume that say if I just look at the 14% portfolio, 14% revenues that are coming from private labels to the gross margins sequentially could have come down and considering that the pharmacy gross margin has increased so in the branded pharma business probably the gross margins would have increased quarter-on-quarter and hence the average discounting levels would have come down? Is that a fair assumption or something is not correct?

Madhukar Reddy : No, so you are saying our margins because of the cannibalization of our earlier higher margin private label will have come down that is right. Earlier in Hyderabad we had roughly around 7% to 7.5% So, we assume that all 7.5% has been

substituted by this, then we would have lost roughly around 2% on that. But the additional 7% which we actually got here would have basically secured another I would say another 1% would have basically been made up by the extra private label which we sold and that would have replaced the regular branded generic. Now what we are also not seeing is the fact that our sales also grew significantly because of this. So, a growth of even 10% extra at a 20% margin more than makes up for all this. And we expect that we will continue to grow as we go forward. From now on, I would say in Hyderabad whatever margins we have lost because of merging both the old and new ones that effect is long gone, so we will continue to grow from here.

In the other states though, in Karnataka and all these other places, we have now, till now kept

these separate so we continue to operate the old private label as it is. We are seeing some cannibalization but not complete so to that extent we will see some loss, but overall since the private label is growing out there we expect to make more money out of that.

Sayantana Maji : Another question related to private label is when you had chosen 400 SKUs which cater to 55% of the customers. So, are these the ones who have probably the sale base were lower earlier or are these top selling SKUs that you see in branded pharma. And the 15% which is as you explained 1/3rd of the addressable market that you were able to convert. So, do these customers had higher awareness or is there some below the line activity which has been done to create awareness which led to this uptick in private labels?

Madhukar Reddy : Two things, everyone is aware because the private labels was sold only to people who had MedPlus membership and MedPlus membership was sold at some price, super discounted price,

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original price which we thought of was Rs. 499 but we ended up selling it for Rs. 49 as initial sell. But they had to actually become a member, pay that money to actually obtain this discount. So, they were told. And in the stores when they came in and we also had some marketing m

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which went outside we also did a little bit of advertisement and all. We talked about only this, that 80% of all the drugs in the country are off patent, all these are just branded generics or generics only. And we basically just said that MedPlus with a 4000-store network and a Rs. 4500 crores as of last year as the size and capability of getting these drugs manufactured in the same places and to the same level of quality as any other larger brands. And given that we did not have any marketing cost and all and we made it very clear that the discount was coming to them ost. And so

customers bought that and a significant portion of them ended up actually buying as I said a 3rd of whatever we put out on offer basically got taken up. And these are fast moving products by the way, these are not products in which the fill rate is a problem or anything like that, these are the, the fastest moving products.

Sayantana Maji : So, that is substitution,

were able to fulfill 8 out of 10 and the 2 was the gap and you available for the customer.

Madhukar Reddy : Exactly yes.

Sayantana Maji : So, your CAPEX if I just do a CAPEX per store it comes to around Rs. 1.4 million which is Rs. 14 lakhs so we have fit out cost of Rs. 6 lakhs, Rs. 2 lakhs to Rs. 3 lakhs of deposit, so where is the remaining Rs. 5 lakh to Rs. 6 lakhs coming from assuming that none of the CAPEX is going into diagnostics for this quarter?

Madhukar Reddy : --; capitalized or something

little bit of a maintenance, we actually did tell the investors last time that we are going to refurbish I would say refit roughly around 1000 stores to accommodate the private label products, non-pharma private label products. which would now start going from being shelved from behind the counter to in front of the counter. So, it is going to be, it should have change it so much. Happy to come back to you with more details on this please reach out to us after this.

Moderator : Thank you. Ladies and gentlemen, that was our last question for today. I now hand the conference back to the management for their closing remarks. Thank you. And over to you.

Madhukar Reddy : Thank you everyone for the call.

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Moderator : Thank you very much. Ladies and gentlemen, on behalf of MedPlus Health Services Limited that concludes this conference. Thank you for joining us today and you may now disconnect your lines.

Call: Feb 2024

Medplus Health Services Limited

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February 7 , 202 4

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Scrip code: 543427 Bandra (East), Mumbai - 400051
Symbol: MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Earning Call Transcripts for the quarter ended December 31, 2023 .

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 , we enclose herewith transcript of the earnings calls held on February 5 , 202 4 for the quarter ended December 31, 2023 .

Kindly take the same on record. The same is available on Company's website at www.medplusindia.com

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer
FCS 7460

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by MANOJ
KUMAR
SRIVASTAVA
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"MedPlus Health Services Limited
Q3 FY'2 4 Earnings Conference Call "
February 05, 202 4

MANAGEMENT : MR. MADHUKAR REDDY GANGADI – CHIEF
EXECUTIVE OFFICER AND MANAGING DIRECTOR –
MEDPLUS HEALTH SERVICES LIMITED
MR. SUJIT MAHATO – CHIEF FINANCIAL OFFICER –
MEDPLUS HEALTH SERVICES LIMITED

MR. CHETAN DIKSHIT – CHIEF STRATEGY OFFICER –
MEDPLUS HEALTH SERVICES LIMITED
MR. PRASAD REDDY – ASSISTANT FINANCIAL
CONTROLLER – MEDPLUS HEALTH SERVICES
LIMITED

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Moderator: Ladies and gentlemen, good evening, and a warm welcome to the MedPlus Health Services Limited Q3 FY '24 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Prasad Reddy from MedPlus. Thank you, and over to you, Mr. Reddy.

Prasad Reddy : Thank you, Michelle. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Q3 FY '24 Earnings Conference Call to discuss the financial results of MedPlus for the third quarter of financial year 2024, which were announced on 2nd February 2024.

We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, Chief Executive Officer and Managing Director; Mr. Sujit Mahato, CFO; and Mr. Chetan Dikshit, CSO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties are on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website.

I would now hand over the call to Sujit, the CFO. Thank you, and over to you, Sujit.

Sujit Mahato : Thank you, Prasad, and good evening, everyone, on this call. We are pleased to share that as of December 31st, we have been serving community in over 600 cities across 10 states, including the 3 new states of Madhya Pradesh, Chhattisgarh and Kerala through our network of 4,233 pharmacy stores. Additionally, in Hyderabad, MedPlus operates 4 full-service diagnostic centers, 8 level 2 centers and over 120 collection centers offering essential diagnostic services at affordable rates.

Let me cover the network in terms of openings and closures of our outlets. As informed during the last quarter, we have taken a thoughtful approach towards store expansion, prioritizing scale-up of MedPlus-branded pharma and non-pharma products and also maintaining a balance between overall growth and profitability.

Over the past 12 months, we have added a net total of 676 stores. On a gross basis, this was 758 stores, with 164 stores opened during Q3 alone. Most of the new stores were opened in Tamil Nadu and Telangana with 55 and 24 stores, respectively.

This strategic focus of growth reflects our commitment to meet the evolving needs of our community while maintaining a focus on sustainability and financial health. We expect the new store openings for the fiscal year 2024 to be around 600 stores.

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In the third quarter, 59% of our store openings were in Tier 2 cities and beyond, underscoring our strategic emphasis on these markets. Presently, out of our 4,233 stores, 1,908 stores, representing 45%, are situated in Tier 2 cities and beyond. We acknowledge the growth potential inherent in these markets.

Throughout Q3, there were 20 store closures. Taking into account both openings and closures, we achieved a net addition of 144 stores during the current quarter, a notable increase compared to the 114 added in the previous Q2 quarter.

In terms of the network age. Approximately 18% of our stores are in the first year of operation, around 27% of our stores are in the second year of operation and the remaining 55% of our stores have been operational for 2 years or more. It is noteworthy that all stores in the less than 2 years

are

age bracket are still in their ramp-up phase. From a financial standpoint, they continue to exert a negative impact on our operating EBITDA.

However, as these stores mature, we anticipate a positive contribution to our profitability. We closely monitor the timeframe for our new stores to reach breakeven. For stores opened between January 2023 and June '23, approximately 59% of them achieved breakeven within 6 months of their operations.

As a cohort, all stores combined reach breakeven in just 4 months. In terms of the store size as at the end of the quarter, our network has grown to 4,233 stores, covering 2.2 million square feet compared to 3,557 stores over 1.9 million square feet as of the end of December '22. The average store size was 535 square feet. To give you a sense of spread in store sizes, we had 3,082 stores less than 600 square feet and 1,151 stores that are greater than 600 square feet.

In terms of revenue mix, with our expanded scale, we are strategically positioned to increase our revenue share from our private label products. Our private label range is crafted to provide customers with high-quality products at competitive prices.

Presently, MedPlus offers over 1,000 carefully selected SKUs spanning across pharmaceutical and non-pharmaceutical category. Private label sales currently constitute 14.5% of our total net revenue. However, our growing presence in Tier 2 cities and beyond is significantly impacting our revenue mix. Sales from these markets comprise 35% of our pharmacy revenue in the current quarter, marginal increase from 33% in the same period last year.

I'm pleased to provide an update on the impact of the launch of MedPlus-branded products in Telangana state. In quarter 1 FY '24, prior to the launch, the share of private label pharma stood at 6.61% of the total GMV in Telangana. However, following the launch and in quarter 3 FY '24, the gross merchandising value notably increased to 15.03% of the total GMV in Telangana. Following the promising results observed in Telangana, the company initiated the launch of MedPlus branded products in other states starting October '23 onwards. Prior to the introduction of MedPlus-branded pharma products, sales of private label pharma products accounted for 8% of the gross merchandise value in quarter 2 of FY '24.

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However, subsequent to the launch and throughout the reported period, the private label share of GMV surged to 11.2%. The increase in private label GMV indicates the positive reception from customers and validate our commitment to delivering high-quality products under the MedPlus-brand umbrella.

Now coming to the financial numbers. On our quarter performance, our consolidated revenue was INR14,415 million with a growth of 21.1% year-on-year and 2.3% quarter-on-quarter. Our consolidated operating EBITDA stood at INR466 million, representing 3.2%. A round 99% of our revenue is from our pharmacy operations.

The pharmacy operating EBITDA is INR505 million, representing 3.6%. In terms of our store performance, I would like to update on our stores older than 12 months. Revenue from these stores in quarter 3 was INR12,953 million or 91% of pharmacy revenue.

These stores had a store level EBITDA margin of 9.6%. The store level operating ROCE of these stores stood at 52.5%. A word here on the store level EBITDA margin by age. While stores greater than 12 months had a margin of 9.6%, this was 10.1% for stores greater than 24 months and 7.2% for stores in the 13 to 24 months age bracket.

If we allocated the non-store related costs, then the operating EBITDA of stores greater than 12 months to the INR588 million, which translates to a margin of 4.5%. Quick numbers. Diagnostic revenue has grown to INR196 million in Q3 '24 compared to INR97 million in Q3 of FY '23, which is primarily due to launch of the new

centers across Hyderabad.

Diagnostics segment recorded an operating EBITDA loss of INR34 million compared to a loss of INR33 million in quarter 3 of fiscal year '23. Our net working capital for quarter 3 was 66 days. The inventory in our warehouse was 38 days.

As you are aware, because of the same trajectory of new stores, their inventory turnover is lower in the first year. In Q3, the inventory level of our first year store was INR111 million. In comparison for our stores older than 12 months, the inventory was 42 days.

Now I request Chetan to share an update on our diagnostic business. Over to you, Chetan.

Chetan Dikshit: Thank you, Sujit, and good afternoon, everyone. As you know, Q3 is a seasonally weak quarter for Diagnostics in great part due to the festival and holiday season. However, we have 8% quarter-on-quarter growth in Q3. In October, we sold 405 gross plans per day. In November and December, this was 369 and 357, respectively.

As on 30th September, we had 117,000 active plans and 207,000 underlying lives. As of 31 December, we had 127,000 active plans and 228,000 underlying lives. Our current observed on-time renewal rate was 19% in Q3 versus 15% in Q2.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question

is from the line of Ashita Jain from Nuvama. Please go ahead.
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Ashita Jain: My first question is on your store opening guidance. I think for this year, your store down to 600. What is the store opening guidance for the next 2 to 3 years or 3 years for FY '25? Can you give color on that?

Madhukar Gangadi : Sure. Thanks, Ashita. So as Sujit said earlier in this call, we have focused primarily on increasing the overall private label, both in pharma and non-pharma. That's going to be our focus for the first 1 or 2 quarters this year also. We've just introduced over 600 products. And even as the product line is expanding, we are also making sure that products are available across the entire network and all.

And we're also ensuring that our employees are fully trained and are able to sell and everything else. So as I said, the primary focus is going to be on this. We will most likely end up doing only around 600 to 700 stores the next year, the coming year, to o. But we will keep you guys updated as soon as our figures are finalized.

Ashita Jain: And secondly , you just spoke about last few quarters about entry into the newer market. What's the update on that?

Madhukar Gangadi: It is still early going for the three states, one is Kerala, Chhattisgarh and Madhya Pradesh, but we are, I would say, pretty encouraged by the response in all these places. We will put up a small -- a very small working kind of warehouse to make sure that we are able to service all our stores properly and all.

But in general, both Madhya Pradesh and Chhattisgarh are doing well. We expect to grow faster than that. There are -- I would say, the growing is a little bit slower in Kerala for us, but we continue to expand in that state also.

Ashita Jain: Any particular reason for this Kerala -- slower uptick in Kerala?

Madhukar Gangadi: So see, a lot of the initial store opening depends on the regulator over there. There are some regulators who are slightly more sticky about every single detail. And in some states, it is not as much. It is a state-wise thing. So it is a slow growing in the beginning stages, I would think. But yes, that's about it is. Otherwise, it's not an issue. The market itself is pretty good in Kerala.

Ashita Jain: Understood. And I think phenomenal job on the private label side, now we're close to 14.4% pharmacy presence. How do you think this number would settle in the next year, given our initiatives on the MedPlus branded labels?

Ma

dhukar Gangadi: It's really difficult to say. We are very encouraged with the way the initial two states have responded with the initial two regions where we started in June have responded. And also under the early listing out there in the other states, it is following the same path. Just on the pharma side alone.

I think given that 80% of our sales comes from pharma right now, and we will be able to hit or at least we'll be able to, I would say, have substitutes for at least 60% of the entire inventory, which is 70% at least of the medicines. I think for us to get to 20% will not be very tough on the pharma side over the next one year.

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Moderator: Thank you. The next question is from the line of on Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee: Sir, on the private level, the MedPlus brand. So I mean what you see at a GMV level, how the ratio has moved? If you get your share at an overall company level like how much it was before, let's say, when you launched in first quarter and now?

Sujit Mahato: Yes, I'll take that, Saion thank you for the question. In quarter two, in terms of the GMV, the private label pharma was 8.96% overall. And end of quarter three, the same percentage on the same basis of GMV reflects at 11.78% If this is helpful.

Saion Mukherjee: So this is for the overall, entire pharmacy...

Sujit Mahato: Overall. Only for the private label pharma.

Saion Mukherjee: Understood. Okay. And the other question I wanted to check with you is that you have provided stores less than 12 months revenue split. How has been the revenue growth for stores, which are, let's say, more than two years old? I mean, is that growth very slow? I mean, is that a right assessment that it's in single digits?

Sujit Mahato : In fact close to the inflation number. If we get all stores put up to FY21 end, it is inflation -plus.

Saion Mukherjee: Okay. But generally, the pharma market growth is around 9%, 10%, right? And if there is price increase that is also there in pharma. So I was wondering why this number is a bit lower? Is there

sort of a de-stabilization with the new store opening? How do you sort of read this number?

Madhukar Gangadi: Yes. So Saion, while definitely there is some price inflation and unfortunately the DPCO rules which kick in once every while, also reduce the prices a little bit. So there's also that impact. For us, I expect that the growth will be slightly over inflation as we go forward. There may be some cannibalization in some existing states, but I don't really expect that to mute the sales any more than it should.

For us, the other main reason could be also this. We are not really -- except for places where we're reporting the private label, we are not taking the GMV route to actually explain the sales growth. So in some of those cases where private label medicines have actually taken off, you would see a slight dip in the top line because of the lower value at which we sell the discounted private label products, the new pharma private label products.

Saion Mukherjee: Right. Sir, is it possible to share the growth numbers on a like-to-like basis at the GMV level? Is that possible to -- like you reported 21% growth, how much would have been the growth at a like-to-like basis?

Madhukar Gangadi: Sujit, do you want to take that?

Chetan Dikshit : Madhukar, I'll take that. Saion, our reporting what we have done is we've benchmarked it to peers. But we'll take your feedback on board. And see if there's more disclosure that we can give going forward.

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Saion Mukherjee: Okay. And finally, are there any changes in the discounting in your normal branded products due to competition and behaviour of online players? Any changes that you have witnessed in the recent past?

Madhukar Gangadi :

We have not increased the discounts anywhere. And if your question was, if the competition is actually coming down, are you taking by decreasing it? No, not as of now.

Saion Mukherjee: Okay. Thank you.

Moderator: Thank you. We'll take the next question from the line of Gaurav Nigam from Tunga Investments. Please go ahead.

Gaurav Nigam: Sir, first question was on, I was looking at your presentation, you used to give this private label number of pharma SKUs. That number from Q2 to Q3 presentation is dropped from almost 1,100 to 783. Just wondering, is this a correct number to look at? And what's the reason if this is correct?

Madhukar Gangadi: We will have to come back to you. I don't think we have really decreased the overall number. If anything, given that we have been continually increasing the pharma private label and also looking at other product, I guess not to decrease the overall number of products. Sujit, do you want to add anything else?

Sujit Mahato: I will come back in a while.

Gaurav Nigam: Understood, sir. Another question was on, can you provide the average level of pharma discounts that was there in Q3? And has it quarter-on-quarter, has it increased or reduced, any sense of direction?

Madhukar Gangadi: Pharma discounts on branded products has not really increased significantly. There may be a little bit of increase in 1 state, West Bengal, where the earlier hurdle was -- after the earlier hurdle we used to give a 10% discount, we moved it to 15%, but that was at least 2 to 3 quarters ago. Otherwise, I see no reason for any increase in discount.

If as a blended this thing it is coming out a slightly higher discount, it could be because of our private label products. Our private label products, the new ones sell at an average of 50%. So if you combine the entire pharma products, then that could increase the discount. But I think if you look at purely the branded ones, there should be no increase or there should be very little increase.

Gaurav Nigam: Understood, sir. And sir, what is the guidance for diagnostics capex in FY'24 and '25, sir?

Madhukar Gangadi: Do you want to take that Sujit or Chetan?

Chetan Dikshit : Yes, sure. I can take it. Apart from maintenance level capex, we don't have anything really planned in terms of big machines or new centers. If there is a change in view, we will definitely update you.

Gaurav Nigam: In FY'25, sir, as well, you're saying?

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Chetan Dikshit : Yes. For the foreseeable horizon, our capex program has been rolled out. In some of the centers, we may have minor capex maybe an additional ultrasound, ambulance. So which is why I cannot

spoke about it like maintenance capex. But there is no plan on the cards for new centers or new cities or new big machines. We will update as and when anything like that is on the cards.

Gaurav Nigam: Understood, sir. And sir, just a last question. Is there any update on the QIP? I think you have taken a shareholder approval. So just wondering if there is anything in the plan going forward.

Chetan Dikshit : Thank you for asking that question. We maintain our stance that what we have taken was shareholder enabling provision just to keep that optionality and that is for a full duration of 12 months. As and when the circumstances require us to raise additional funds, we will then have the option of both looking at equity and debt. As of now, it is an enabling provision, and there is no action expected on it in the near-term.

Gaurav Nigam: Understood, sir. Thank you for answering my question.

Moderator: Thank you. We'll take the next question from the line of Kunal Randeria from Axis Capital.

Please go ahead.

Kunal Randeria: Thanks for taking my question. Madhukar, your private label sales contribution on the pharma side has actually gone down on a year-on-year basis, right? But your GMV has gone

up, I

believe, so does that mean Wynclear MHS had a lot less discount than made by MedPlus brand?

Madhukar Gangadi: Yes. So earlier Wynclear and even a little bit of the trade generics which we sold, were sold at the same price as branded products. Since then, we have decided to basically go with a store generic thing, promote MedPlus brand and give it the full discount. So while a response to this has been very good, there has also been a little bit of cannibalization of our original Wynclear brand. And so in some places, that has come down a little bit. And so overall margin will have come down because of that.

Kunal Randeria: Okay. So do you still sell under Wynclear.

Madhukar Gangadi: Sorry?

Kunal Randeria: Do you still sell under the Wynclear brand or have you stopped it?

Madhukar Gangadi: No, we do. So we will continue that at least in the acute kind of therapies and all. In Telangana, the entire state of Telangana, we have reduced the price of Wynclear to the level of the MedPlus brand. So we sell it at the same price, all the products have been sold at the same price. But in the states in which we have started after this, in the rest of the country in the seven states, Wynclear continues to sell at the old price.

It is some of the chronic -- for some of the chronic therapies, the sales has come down. Most likely it will come down to next to nothing or zero in a while, we'll let that happen over a period of time. But we will continue to actually sell the Wynclear private label at the full price for acute therapies.

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The idea is that for those who are not very price sensitive, those who are not members of the MedPlus discount plan and all, and those who need the -- and those who are seeking early convenience, from them, we will get the full price. Otherwise, anyone else who is price sensitive will get the MedPlus brand at the full discount.

Kunal Randeria: Got it. And just one last one on this. So this is more like your -- it's been a few months since you launched Made by MedPlus. Let's say, as it gets bigger and people recognize the brand, do you plan to reduce discounting slightly, let's say, from 70% to a 60% or something like that?

Madhukar Gangadi: So today, our overall discount is at around 60%. We would say 50% to 80%, but the average falls at 60%. That is something we will see as we go forward, depending on how the market is evolved. Our primary thing right now concerns to grab market share and increase the top line. Once it is fully established, people understand the quality of the product and people are buying MedPlus -- are asking for it by name and then coming and buying it. Then we could always play around either with the top line, either with the MRP price, increase it slightly or perhaps decrease the discount to a slightly lower number. But that is not something which we are going to decide right now.

Kunal Randeria: Got it. Just one more if I can. Have you introduced it across the states or is it just Telangana at the moment?

Madhukar Gangadi: No, it is across all the seven states right now. As I said, only in Telangana, we have killed our old private label. Everything is now available at the new MedPlus private label brand rate. But in the other states, both co-exist.

Kunal Randeria: Got it. Thank you and all the best.

Moderator: The next question is from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed: I joined the call a bit late, so my apologies if this was asked before. So my first one is on store additions. I see that we are tracking around 400 stores added year-to-date FY'24. So I remember that previously, we had guided for roughly 1,000 stores addition, 800-plus. So what would be the updated number for FY'24? And is there a revision in the guidance for FY'25 and beyond in terms of net additions?

Madh

ukar Gangadi: Sure. The number of stores, the gross additions this year will be close to 600, around the 600 mark, the net additions may be slightly lower. And for the coming year though, the number could be around the same number. The focus this year is going to be on private label. We want to make sure that we get it to the 20% kind of mark on the pharma side. So that's going to be the thing -- both by increasing the range, increasing the awareness and also increasing the education level of our own employees in being able to sell this product.

Harith Ahamed: So based on your experience so far on the MedPlus branded private label side, can you share some of the lessons or challenges that you've encountered in this foray? Is it on the sourcing side or is it in terms of getting subscribers? And if you can comment on the willingness that you're

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seeing among patients to switch from the brand to the private label, so if you could share some of the takeaways on these areas, that would be helpful.

Madhukar Gangadi: Sure. So you have seen a pretty good response to our products in Telangana where we launched it first. The results from this are only a quarter. Actually we started only in the middle of October or late October in the rest of the country. So the numbers are still not there. But in Telangana, we have seen a very good response.

What I think is that the early adopters, people are buying the fact that all the products available in the market are generic only. And MedPlus is able to give a very good price only because it does not have any marketing or distribution costs. So people are getting that. So some of the early adopters have come on board.

We think as we go forward -- but there still remain some questions. I think we have our work cut out in trying to educate the customer. The common question we encountered is this.

Whenever our guys have gone and said 50% to 80% discount on our brands, they have said customers have come back and said is this a generic product?

So we are taking the time to explain that all the products in the country are generic. There's really no difference. It's all the same product anyway. The only way you can differentiate one from the other is on the quality of its make. So we are countering that using several different explanations out there. And we are seeing a lot of people converting once that happens.

So this will happen not just through our stores, but also through media, influencers and all sorts of other testimonials and all. Again, to come back to your question, how is the response? Pretty good. The early adopters are on board. People like the fact that they are saving a lot of money. We're seeing a steady increase. But I think if you want to hit the next, let's say, level of growth, we'll have to answer some of these questions, which we are now in that process today.

Harith Ahamed: Okay. Just a clarification, Madhukar, you mentioned that you've launched it across all the states that you operate in, but in select stores. So I was just wondering that from an operations standpoint, wouldn't it be easier to launch it across all the stores in a certain market versus launching it in select stores?

Madhukar Gangadi: No, I may have communicated it wrongly. We have actually launched it across all states, all stores.

Harith Ahamed: All states, all stores.

Madhukar Gangadi: I don't know why it came out like that. Yes, yes, now it is all states, all stores.

Harith Ahamed: And last one, I didn't get 1 number that you shared previously, the store level margins for stores of more than 24 months of maturity?

Madhukar Gangadi: Yes, Sujit, do you want to take that?

Sujit Mahato: Yes. It's around 10.1, Harith.

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Harith Ahamed: Alright. Okay. That's all from side. Thank you.

Moderat

or: Thank you. The next question is from the line of Madhav Marda from Fidelity International. Please go ahead.

Madhav Marda: My first question basically was that as we target this 20% private label mix in the pharma portfolio, how do you see the gross margins for the company trending, say, in the next -- I think you said that you can hit the 20% mark over the next year -or-so. So if you look a couple of years out once we have that Mark, how do you see the gross margin profile? Like I think we're trending at about 21.5%, 22%. How do you see this changing for the company?

Madhukar Gangadi: So Madhav, we'll try and give you a guidance as we go forward on that. So it's going to be a

function of 2 things. One, as we replace the branded product, we will see an increase of around 0.1 or 14%, overall...

Madhav Marda: Sorry, I didn't get you, you said half...

Madhukar Gangadi: No, no. As we replace the branded product, right, on the branded product, post -GST, post everything and all, we probably get around 13% overall margin and this one basically gets us around 22% to 23%. So there'll be that increase as we replace that. But in the initial phase, though, we may end up replacing some of our own private label, which is Wynclark. While we continue to basically run both of those, I think the consumers who are buying chronic product, the medicines for chronic ailments and who are buying the Wynclark brand on that, may switch over to MedPlus brand.

And there, for every one person, we will probably end up losing around 0.3%. So in the conversion -- if you convert maybe 2% or 3% out there, you'll probably end up losing 1% gross margin on the Wynclark side. But after that, it will all be a gain.

Madhav Marda: Okay. No, sir, you're saying Wynclark...

Madhukar Gangadi: So Madhav, sorry to interrupt there. See, our goal in trying to actually get this MedPlus brand popular and get the word out to everyone about the quality and the pricing of that is not to convert the existing customers into the MedPlus brand, but we actually get customers to increase their top line by bringing in customers from the other mom-and-pop and other organized stores. We are seeing a lot of people actually doing that. So we expect the benefit to us is going to be more on the top line than on actual margin side.

Madhav Marda: Okay. Okay. Understood. So -- got it. So would that mean that if earlier, like you said the store level EBITDA margin for 24 months plus stores were actually 10.1%. Essentially, what we're saying is that there is gross margin and then there's EBITDA margin. So in your view, if we get higher volumes at the existing stores, would you have a view on where the store level margins for the mature stores could move? Because I guess that's what we are finally trying to target?

Madhukar Gangadi: Sure, sure. So for us, I mean just -- if we were to increase our sales by, let us say, 10%, over the current level, right, the average store, let's say, is doing around INR15 lakh out there, if you were

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to increase the top line without really changing anything else by 10%, that's -- that will increase sale by INR1.5 lakhs. The margin will be INR30,000 extra. And that would actually increase the overall EBITDA by 2%.

Madhav Marda: Okay. Okay. So basically, the sort of older version versus the new version of stores, you could get 200 basis points higher margin, as this mix of private label goes up because we'll sell more basically at the same stores, right?

Madhukar Gangadi: The idea is to be able to sell more because if it was purely for margin accretion, then we will just continue and the old private label and just continue that. What we feel is introducing a store generic concept for the first time in India, getting people to -- see, our ability to convert a normal customer into the Wynclark customer for a higher

margin is only so much given that our pricing was not any lesser than the other brands.

Now when we talk about it and when we tell people about the MedPlus brand and its quality and we also talk about the price advantage to them, we see a lot more people, a lot more receptors. So, A, conversion of our existing customers; but B, more importantly, we have only about 15% to 20% of any other markets in this city, even though we are really big.

We feel the balance 80%, do you feel we'll be able to win over at least 10% to 20% of the balance 80% who are not shopping with us based on the price. And that's what we're looking for. Yes.

Moderator: The next question is from the line of Sayantan Maji from UBS Group.

Sayantan Maji: My first question is on the portfolio of private labels. How much of it -- sir, how much of the current pharma sales are these private label seeing? So I remember a number that you had given earlier was somewhere around 55%. So has it increased now, now that we have launched it nationally? Or is it still somewhere in that range?

Madhukar Gangadi: No, it has increased slightly. So earlier when we talked about it, it was 80% of our overall sales comes from medicines and for 55% of that we had a substitute. So on the overall scale, we had substitute for 44%. Now that 55%, I think, has now moved to around 60% or 62%, but we have also placed an order for a lot more medicine. So I expect that it will give you up to 70% as we go forward.

Sayantan Maji: Okay. So if I say, let's say, 70% of 80%, which is, say, 55%, so out of this 50%, 55%, you are expecting a 20% share of pharma private label or is it on the total sales?

Madhukar Gangadi: No, we are expecting on the total thing. See, on this 56%, the 20% is already there almost because even leaving aside Telangana, the rest of the country is at 11.2%. So out of the 56%, 20% would be only 11.2%, right? So we already have that. And in Telangana it is actually 14%. So the number is slightly higher. So when I'm talking 20%, I'm talking 20% on the overall number, so

which will actually become 20% and 56%.

Sayantana Maji: Okay. Okay. So this is basically the share of GMV, not the net revenues?

Madhukar Gangadi: Yes.

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Sayantana Maji: Okay. Got it. And also, I think in the opening remarks, you had mentioned that the share of pharma private labels has increased from 8% to 11.2%. And later in the call, you said 8.96% to 11.78%. So is it the difference between the timing? Or is it -- can you just explain like where is the difference coming from?

Madhukar Gangadi: Sure. Sujit, do you want to take that?

Sujit Mahato: Yes. We had the first number, 6.61% to 15% was an update on only Telangana, which we have launched end of June. And the second one, 8%, going up to 11.2% is all states put together, excluding Telangana. It's the only once during October, and that's relevant to the quarter.

Sayantana Maji: Okay. Understood. Okay. That's clear. And my second question is on Diagnostics. So in Diagnostics, we are ramping up in Hyderabad. So -- and I remember you're saying that it is a pilot. And once the pilot is successful, you will expand to other cities. So when do we -- when -- at what point will the decision be taken to expand into other cities based on the results of the pilot in Hyderabad.

Madhukar Gangadi: Sure. Chetan, I'll take this. So yes, no, this continues to be a pilot still. We today are at around 120,000 subscribers serving roughly around 220,000 people out there. The goal is for us to get to at least 200,000 number before we can take a call on expanding in the current situation.

Sayantana Maji: Okay. Thank you so much for taking the questions.

Moderator: We'll take the next question from the line of Tanmay Gandhi from Investec.

Tanmay Gandhi : Hi sir. Thanks for taking my questions. Sir, firstly, if you can clarify the p

private label pharma

sales on the right. So is it possible to split it between MedPlus branded and the normal private labels which we used to have even earlier on a net revenue basis?

Madhukar Gangadi: Sure. We'll see if you can do that. But for Telangana, that is Hyderabad and rest of Telangana state, all of them are lumped into 1 category right now. To the customer, it doesn't matter because he's getting the same price. To us also, it doesn't matter. So we're just not differentiating them on there, but we should be able to give you the actual split.

In the other places out of the 11.6% or 11.7%, I think around 5% is probably the old Wynclear.

I'm not sure, Sujit, do you -- if you have the right numbers, you can just go ahead.

Sujit Mahato: So it's around 11.2% on a net revenue basis, the old and the new put together.

Tanmay Gandhi : No, sir, actually. No, no, sir, actually, I want it on a net revenue business which we report, right? So I want to split off...

Sujit Mahato: I mentioned it is on a net revenue basis. I'm mentioning on a net revenue basis. But both the old as well as the new put together, as Madhukar explained that in the state of Telangana, we have not differentiated between the erstwhile model and the current MedPlus brand. And therefore, on a total basis, this is around 11.2% net.

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Tanmay Gandhi : Okay. Got it. Okay. And secondly, now that we are -- the pace of store addition has come down. So how should we look at the corporate level operating margins? So should we see a meaningful uptick or you plan to invest in some other initiatives like Diagnostics or the MedPlus -branded generics which you have launched?

Madhukar Gangadi: Okay. See, on the Diagnostics, as we said earlier, we're not going to be doing any other investments. We want to -- I think we are fully invested, except for some small maintenance capex, as Chetan said earlier. We will be wait to see how it is shaping up. We are doing well on the B2C side. We also want to see some traction on the B2B and post that we will figure out how to actually expand either on investments in the same place or in going to new cities.

On the private label side, there is no significant investment for us. The cost of the medicines is not very high, and it is just replacing one part from the other that's all. So I think the benefit of adding lesser stores, should be, I would say, more on allowing us to focus on the private label and possibly on the profitability also as we go forward.

Tanmay Gandhi : Sure. So sir, if we look at your operating EBITDA bridge that you provide, right, what are the various drags on the reported numbers. So I think the biggest drag comes from the new store additions and given that now we have a bigger base and obviously, the addition will also be lower in FY '25. So is it fair to assume that there will be a significant uptick in EBITDA margins? Or it will be steady expansion on this?

Madhukar Gangadi: No please. assume for now it is going to be a steady growth only in that. We will see how the private label and all the top line and all go up. I know the cost will be slightly lower, but I don't think that will -- I think the bigger driver for us is -- are the first 2 which is top line and private label component.

Chetan Dikshit : Tanmay, I think you have the modelling inputs over the last many quarters. The reason why you heard Madhukar say what he said was because we do see the new private label having a fairly large opportunity. And we may have additional non store operating costs like marketing, etcetera, which is what Madhukar said, what we said.

Moderator: Mr. Gandhi, we were not able to hear sir.

Tanmay Gandhi : Sorry, I lost you Chetan, sir, but my question was that given that we are right now just giving our store addition guidance, should we assume good uptick in margins because as Ma

dhukar

had said that we are not spending a lot on private label side? Sorry, I missed you. I didn't expect [inaudible].

Moderator: Sir I would request you to kindly unmute yourself and speak. Chetan sir and Madhukar sir.

Chetan Dikshit : I am afraid we couldn't comprehend what was being said. At least at our end it was quite garbled.

Moderator: Mr. Gandhi I would request you to kindly use your handset to ask question please.

Tanmay Gandhi: Yes, is there any better?

Moderator: Yes.

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Tanmay Gandhi: Sir, so my question was that, so because we are reducing our store additions for the next year, and obviously, now we have a better base as well, right, larger base, so should we assume a significant uptick given that we are not spending much on marketing as Madhukar has said, right? So I just wanted to clarify that what are the various drags on the margins?

Because if we are lowering our store additions, then ideally there should be -- the gap between 12-month -plus stores margins and the reported margins for -- at the company level should be narrower, right?

Madhukar Gangadi: No, what you're saying is right, Tanmay. Logically, if we have lesser new stores, yes, definitely. But for us, the drag of that was never very big anyway. We will continue to report stores, which have more than one year old and all. I think it should definitely be slightly better as we go forward.

Moderator: The next question is from the line of Neelam Punjabi from Perpetuity Ventures. Please go ahead.

Neelam Punjabi: My question is on the Diagnostics business. So if I look at the latest quarter, we have already reached an INR80 crores annualized run rate for the diagnostic business. However, we are still reporting our losses segmentally. So by when can we turn profit in our Diagnostic franchise?

Madhukar Gangadi: Yes, sure. Chetan, do you want to take that?

Chetan Dikshit: Yes, sure. I can take the first crack at it. Neelam, the Diagnostics business also has a similar structure like pharmacy in the sense that a lot of operating costs sit both at the center and outside the centers. So because the Diagnostics business has costs which are at the HO level, which are currently not diversified across many cities, the segment continues to be dragged by those costs.

The improvement in the performance of the centers naturally, will make up for some of that.

And we do expect to report the segment as profitable. But what we haven't disclosed so far is the center level profitability for diagnostics as we do in pharmacy which is why if you can just keep that framework in mind that what's ramping up is the revenue and the profitability of the diagnostic center.

But the HO level costs, which are actually at a multicity level that degree of preparation, whether it is procurement or absorb various lines of sub business, that continues to be a drag because our current operations are limited to Hyderabad.

Neelam Punjabi: Okay. But however, then can you help us understand what's the center level EBITDA margin, at least like for the mature center, the oldest -- first the Gachibowli center, if that will be helpful for us to understand then?

Chetan Dikshit: Sure Neelam. If you allow us, we'll do that in the next quarter.

Neelam Punjabi: Sure. Okay. And 1 clarification I needed. For the private label for our pharmacy business, we gave a target of about 20%, right? So is that on the net revenue or GMV basis?

Madhukar Gangadi: Sujit?

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Sujit Mahato: Yes. I think we are talking on a GMV basis, please.

Moderator: We'll take the next question from the line of Lokesh Manik from Vallum Capital. Please go

ahead.

Lokesh Manik: I have just one question from my end is being on the store addition in this quarter is net addition of 144 stores, but the rent expe

nse is flat at INR53 crores. So can you please help me reconcile this?

Chetan Dikshit: Yes, I'll take that. So if you observed last year for the similar period, over the period, we opened around 1,100 outlets at various points in time during the year. And in the current year, during the nine months, we have opened 400 -plus outlets. So that is the full impact taking care of the timing issue. But for that, there is no -- but for that, I would say we have the 5% to 7% annual contract base increase. So that's the major reason for the variance in the rental expense.

Lokesh Manik: Okay. But for this quarter, we should have expected a higher rent expense I mean because we are in line for the stores which are not there in the last quarter. So quarter -to-quarter, it's flat Q2, Q3 it's INR53 crores.

Chetan Dikshit: Yes?

Lokesh Manik: Yes, Q2, Q3 is flat at INR53 crores, and there is an increase in store addition of INR144 crores. I'm just trying to understand how the rent expense is...

Chetan Dikshit: Yes. It's in line with the contracts which we have entered the lease rent till and there's no exceptions in those numbers.

Moderator: Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for closing comments. Over to you, sir.

Sujit Mahato: I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you, and have a great day.

Moderator: Thank you, members of the management. Ladies and gentlemen, on behalf of MedPlus Health Services, that concludes this conference call. We thank you for joining us, and you may now disconnect your lines. Thank you.

Call: Feb 2024

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Dear Sir/ Madam,

Sub: Submission of Earning Call Transcripts for the quarter ended December 31, 2023 .

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 , we enclose herewith transcript of the earnings calls held on February 5 , 202 4 for the quarter ended December 31, 2023 .

Kindly take the same on record. The same is available on Company's website at www.medplusindia.com

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer
FCS 7460

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Q3 FY'2 4 Earnings Conference Call "
February 05, 202 4

MANAGEMENT : MR. MADHUKAR REDDY GANGADI – CHIEF
EXECUTIVE OFFICER AND MANAGING DIRECTOR –
MEDPLUS HEALTH SERVICES LIMITED
MR. SUJIT MAHATO – CHIEF FINANCIAL OFFICER –
MEDPLUS HEALTH SERVICES LIMITED

MR. CHETAN DIKSHIT – CHIEF STRATEGY OFFICER –
MEDPLUS HEALTH SERVICES LIMITED
MR. PRASAD REDDY – ASSISTANT FINANCIAL
CONTROLLER – MEDPLUS HEALTH SERVICES
LIMITED

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Moderator: Ladies and gentlemen, good evening, and a warm welcome to the MedPlus Health Services Limited Q3 FY '24 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Prasad Reddy from MedPlus. Thank you, and over to you, Mr. Reddy.

Prasad Reddy : Thank you, Michelle. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Q3 FY '24 Earnings Conference Call to discuss the financial results of MedPlus for the third quarter of financial year 2024, which were announced on 2nd February 2024.

We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, Chief Executive Officer and Managing Director; Mr. Sujit Mahato, CFO; and Mr. Chetan Dikshit, CSO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties are on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website.

I would now hand over the call to Sujit, the CFO. Thank you, and over to you, Sujit.

Sujit Mahato : Thank you, Prasad, and good evening, everyone, on this call. We are pleased to share that as of December 31st, we have been serving community in over 600 cities across 10 states, including the 3 new states of Madhya Pradesh, Chhattisgarh and Kerala through our network of 4,233 pharmacy stores. Additionally, in Hyderabad, MedPlus operates 4 full-service diagnostic centers, 8 level 2 centers and over 120 collection centers offering essential diagnostic services at affordable rates.

Let me cover the network in terms of openings and closures of our outlets. As informed during the last quarter, we have taken a thoughtful approach towards store expansion, prioritizing scale-up of MedPlus-branded pharma and non-pharma products and also maintaining a balance between overall growth and profitability.

Over the past 12 months, we have added a net total of 676 stores. On a gross basis, this was 758 stores, with 164 stores opened during Q3 alone. Most of the new stores were opened in Tamil Nadu and Telangana with 55 and 24 stores, respectively.

This strategic focus of growth reflects our commitment to meet the evolving needs of our community while maintaining a focus on sustainability and financial health. We expect the new store openings for the fiscal year 2024 to be around 600 stores.

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In the third quarter, 59% of our store openings were in Tier 2 cities and beyond, underscoring our strategic emphasis on these markets. Presently, out of our 4,233 stores, 1,908 stores, representing 45%, are situated in Tier 2 cities and beyond. We acknowledge the growth potential inherent in these markets.

Throughout Q3, there were 20 store closures. Taking into account both openings and closures, we achieved a net addition of 144 stores during the current quarter, a notable increase compared to the 114 added in the previous Q2 quarter.

In terms of the network age, approximately 18% of our stores are in the first year of operation, around 27% of our stores are in the second year of operation and the remaining 55% of our stores have been operational for 2 years or more. It is noteworthy that all stores in the less than 2 years

are

age bracket are still in their ramp-up phase. From a financial standpoint, they continue to exert a negative impact on our operating EBITDA.

However, as these stores mature, we anticipate a positive contribution to our profitability. We closely monitor the timeframe for our new stores to reach breakeven. For stores opened between January 2023 and June '23, approximately 59% of them achieved breakeven within 6 months of their operations.

As a cohort, all stores combined reach breakeven in just 4 months. In terms of the store size as at the end of the quarter, our network has grown to 4,233 stores, covering 2.2 million square feet compared to 3,557 stores over 1.9 million square feet as of the end of December '22. The average store size was 535 square feet. To give you a sense of spread in store sizes, we had 3,082 stores less than 600 square feet and 1,151 stores that are greater than 600 square feet.

In terms of revenue mix, with our expanded scale, we are strategically positioned to increase our revenue share from our private label products. Our private label range is crafted to provide customers with high-quality products at competitive prices.

Presently, MedPlus offers over 1,000 carefully selected SKUs spanning across pharmaceutical and non-pharmaceutical category. Private label sales currently constitute 14.5% of our total net revenue. However, our growing presence in Tier 2 cities and beyond is significantly impacting our revenue mix. Sales from these markets comprise 35% of our pharmacy revenue in the current quarter, marginal increase from 33% in the same period last year.

I'm pleased to provide an update on the impact of the launch of MedPlus-branded products in Telangana state. In quarter 1 FY '24, prior to the launch, the share of private label pharma stood at 6.61% of the total GMV in Telangana. However, following the launch and in quarter 3 FY '24, the gross merchandising value notably increased to 15.03% of the total GMV in Telangana. Following the promising results observed in Telangana, the company initiated the launch of MedPlus branded products in other states starting October '23 onwards. Prior to the introduction of MedPlus-branded pharma products, sales of private label pharma products accounted for 8% of the gross merchandise value in quarter 2 of FY '24.

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However, subsequent to the launch and throughout the reported period, the private label share of GMV surged to 11.2%. The increase in private label GMV indicates the positive reception from customers and validate our commitment to delivering high-quality products under the MedPlus-brand umbrella.

Now coming to the financial numbers. On our quarter performance, our consolidated revenue was INR14,415 million with a growth of 21.1% year-on-year and 2.3% quarter-on-quarter. Our consolidated operating EBITDA stood at INR466 million, representing 3.2%. A round 99% of our revenue is from our pharmacy operations.

The pharmacy operating EBITDA is INR505 million, representing 3.6%. In terms of our store performance, I would like to update on our stores older than 12 months. Revenue from these stores in quarter 3 was INR12,953 million or 91% of pharmacy revenue.

These stores had a store level EBITDA margin of 9.6%. The store level operating ROCE of these stores stood at 52.5%. A word here on the store level EBITDA margin by age. While stores greater than 12 months had a margin of 9.6%, this was 10.1% for stores greater than 24 months and 7.2% for stores in the 13 to 24 months age bracket.

If we allocated the non-store related costs, then the operating EBITDA of stores greater than 12 months to the INR588 million, which translates to a margin of 4.5%. Quick numbers. Diagnostic revenue has grown to INR196 million in Q3 '24 compared to INR97 million in Q3 of FY '23, which is primarily due to launch of the new

centers across Hyderabad.

Diagnostics segment recorded an operating EBITDA loss of INR34 million compared to a loss of INR33 million in quarter 3 of fiscal year '23. Our net working capital for quarter 3 was 66 days. The inventory in our warehouse was 38 days.

As you are aware, because of the same trajectory of new stores, their inventory turnover is lower in the first year. In Q3, the inventory level of our first year store was INR111 million. In comparison for our stores older than 12 months, the inventory was 42 days.

Now I request Chetan to share an update on our diagnostic business. Over to you, Chetan.

Chetan Dikshit: Thank you, Sujit, and good afternoon, everyone. As you know, Q3 is a seasonally weak quarter for Diagnostics in great part due to the festival and holiday season. However, we have 8% quarter-on-quarter growth in Q3. In October, we sold 405 gross plans per day. In November and December, this was 369 and 357, respectively.

As on 30th September, we had 117,000 active plans and 207,000 underlying lives. As of 31 December, we had 127,000 active plans and 228,000 underlying lives. Our current observed on-time renewal rate was 19% in Q3 versus 15% in Q2.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question

is from the line of Ashita Jain from Nuvama. Please go ahead.
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Ashita Jain: My first question is on your store opening guidance. I think for this year, your store down to 600. What is the store opening guidance for the next 2 to 3 years or 3 years for FY '25? Can you give color on that?

Madhukar Gangadi : Sure. Thanks, Ashita. So as Sujit said earlier in this call, we have focused primarily on increasing the overall private label, both in pharma and non-pharma. That's going to be our focus for the first 1 or 2 quarters this year also. We've just introduced over 600 products. And even as the product line is expanding, we are also making sure that products are available across the entire network and all.

And we're also ensuring that our employees are fully trained and are able to sell and everything else. So as I said, the primary focus is going to be on this. We will most likely end up doing only around 600 to 700 stores the next year, the coming year, to o. But we will keep you guys updated as soon as our figures are finalized.

Ashita Jain: And secondly, you just spoke about last few quarters about entry into the newer market. What's the update on that?

Madhukar Gangadi: It is still early going for the three states, one is Kerala, Chhattisgarh and Madhya Pradesh, but we are, I would say, pretty encouraged by the response in all these places. We will put up a small -- a very small working kind of warehouse to make sure that we are able to service all our stores properly and all.

But in general, both Madhya Pradesh and Chhattisgarh are doing well. We expect to grow faster than that. There are -- I would say, the growing is a little bit slower in Kerala for us, but we continue to expand in that state also.

Ashita Jain: Any particular reason for this Kerala -- slower uptick in Kerala?

Madhukar Gangadi: So see, a lot of the initial store opening depends on the regulator over there. There are some regulators who are slightly more sticky about every single detail. And in some states, it is not as much. It is a state-wise thing. So it is a slow growing in the beginning stages, I would think. But yes, that's about it. Otherwise, it's not an issue. The market itself is pretty good in Kerala.

Ashita Jain: Understood. And I think phenomenal job on the private label side, now we're close to 14.4% pharmacy presence. How do you think this number would settle in the next year, given our initiatives on the MedPlus branded labels?

Ma

dhukar Gangadi: It's really difficult to say. We are very encouraged with the way the initial two states have responded with the initial two regions where we started in June have responded. And also under the early listing out there in the other states, it is following the same path. Just on the pharma side alone.

I think given that 80% of our sales comes from pharma right now, and we will be able to hit or at least we'll be able to, I would say, have substitutes for at least 60% of the entire inventory, which is 70% at least of the medicines. I think for us to get to 20% will not be very tough on the pharma side over the next one year.

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Moderator: Thank you. The next question is from the line of on Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee: Sir, on the private level, the MedPlus brand. So I mean what you see at a GMV level, how the ratio has moved? If you get your share at an overall company level like how much it was before, let's say, when you launched in first quarter and now?

Sujit Mahato: Yes, I'll take that, Saion thank you for the question. In quarter two, in terms of the GMV, the private label pharma was 8.96% overall. And end of quarter three, the same percentage on the same basis of GMV reflects at 11.78% If this is helpful.

Saion Mukherjee: So this is for the overall, entire pharmacy...

Sujit Mahato: Overall. Only for the private label pharma.

Saion Mukherjee: Understood. Okay. And the other question I wanted to check with you is that you have provided stores less than 12 months revenue split. How has been the revenue growth for stores, which are, let's say, more than two years old? I mean, is that growth very slow? I mean, is that a right assessment that it's in single digits?

Sujit Mahato : In fact close to the inflation number. If we get all stores put up to FY21 end, it is inflation -plus.

Saion Mukherjee: Okay. But generally, the pharma market growth is around 9%, 10%, right? And if there is price increase that is also there in pharma. So I was wondering why this number is a bit lower? Is there

sort of a de-stabilization with the new store opening? How do you sort of read this number?

Madhukar Gangadi: Yes. So Saion, while definitely there is some price inflation and unfortunately the DPCO rules which kick in once every while, also reduce the prices a little bit. So there's also that impact. For us, I expect that the growth will be slightly over inflation as we go forward. There may be some cannibalization in some existing states, but I don't really expect that to mute the sales any more than it should.

For us, the other main reason could be also this. We are not really -- except for places where we're reporting the private label, we are not taking the GMV route to actually explain the sales growth. So in some of those cases where private label medicines have actually taken off, you would see a slight dip in the top line because of the lower value at which we sell the discounted private label products, the new pharma private label products.

Saion Mukherjee: Right. Sir, is it possible to share the growth numbers on a like-to-like basis at the GMV level? Is that possible to -- like you reported 21% growth, how much would have been the growth at a like-to-like basis?

Madhukar Gangadi: Sujit, do you want to take that?

Chetan Dikshit : Madhukar, I'll take that. Saion, our reporting what we have done is we've benchmarked it to peers. But we'll take your feedback on board. And see if there's more disclosure that we can give going forward.

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Saion Mukherjee: Okay. And finally, are there any changes in the discounting in your normal branded products due to competition and behaviour of online players? Any changes that you have witnessed in the recent past?

Madhukar Gangadi :

We have not increased the discounts anywhere. And if your question was, if the competition is actually coming down, are you taking by decreasing it? No, not as of now.

Saion Mukherjee: Okay. Thank you.

Moderator: Thank you. We'll take the next question from the line of Gaurav Nigam from Tunga Investments. Please go ahead.

Gaurav Nigam: Sir, first question was on, I was looking at your presentation, you used to give this private label number of pharma SKUs. That number from Q2 to Q3 presentation is dropped from almost 1,100 to 783. Just wondering, is this a correct number to look at? And what's the reason if this is correct?

Madhukar Gangadi: We will have to come back to you. I don't think we have really decreased the overall number. If anything, given that we have been continually increasing the pharma private label and also looking at other product, I guess not to decrease the overall number of products. Sujit, do you want to add anything else?

Sujit Mahato: I will come back in a while.

Gaurav Nigam: Understood, sir. Another question was on, can you provide the average level of pharma discounts that was there in Q3? And has it quarter-on-quarter, has it increased or reduced, any sense of direction?

Madhukar Gangadi: Pharma discounts on branded products has not really increased significantly. There may be a little bit of increase in 1 state, West Bengal, where the earlier hurdle was -- after the earlier hurdle we used to give a 10% discount, we moved it to 15%, but that was at least 2 to 3 quarters ago. Otherwise, I see no reason for any increase in discount.

If as a blended this thing it is coming out a slightly higher discount, it could be because of our private label products. Our private label products, the new ones sell at an average of 50%. So if you combine the entire pharma products, then that could increase the discount. But I think if you look at purely the branded ones, there should be no increase or there should be very little increase.

Gaurav Nigam: Understood, sir. And sir, what is the guidance for diagnostics capex in FY'24 and '25, sir?

Madhukar Gangadi: Do you want to take that Sujit or Chetan?

Chetan Dikshit : Yes, sure. I can take it. Apart from maintenance level capex, we don't have anything really planned in terms of big machines or new centers. If there is a change in view, we will definitely update you.

Gaurav Nigam: In FY'25, sir, as well, you're saying?

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Chetan Dikshit : Yes. For the foreseeable horizon, our capex program has been rolled out. In some of the centers, we may have minor capex maybe an additional ultrasound, ambulance. So which is why I cannot

spoke about it like maintenance capex. But there is no plan on the cards for new centers or new cities or new big machines. We will update as and when anything like that is on the cards.

Gaurav Nigam: Understood, sir. And sir, just a last question. Is there any update on the QIP? I think you have taken a shareholder approval. So just wondering if there is anything in the plan going forward.

Chetan Dikshit : Thank you for asking that question. We maintain our stance that what we have taken was shareholder enabling provision just to keep that optionality and that is for a full duration of 12 months. As and when the circumstances require us to raise additional funds, we will then have the option of both looking at equity and debt. As of now, it is an enabling provision, and there is no action expected on it in the near-term.

Gaurav Nigam: Understood, sir. Thank you for answering my question.

Moderator: Thank you. We'll take the next question from the line of Kunal Randeria from Axis Capital.

Please go ahead.

Kunal Randeria: Thanks for taking my question. Madhukar, your private label sales contribution on the pharma side has actually gone down on a year-on-year basis, right? But your GMV has gone

up, I

believe, so does that mean Wynclear MHS had a lot less discount than made by MedPlus brand?

Madhukar Gangadi: Yes. So earlier Wynclear and even a little bit of the trade generics which we sold, were sold at the same price as branded products. Since then, we have decided to basically go with a store generic thing, promote MedPlus brand and give it the full discount. So while a response to this has been very good, there has also been a little bit of cannibalization of our original Wynclear brand. And so in some places, that has come down a little bit. And so overall margin will have come down because of that.

Kunal Randeria: Okay. So do you still sell under Wynclear.

Madhukar Gangadi: Sorry?

Kunal Randeria: Do you still sell under the Wynclear brand or have you stopped it?

Madhukar Gangadi: No, we do. So we will continue that at least in the acute kind of therapies and all. In Telangana, the entire state of Telangana, we have reduced the price of Wynclear to the level of the MedPlus brand. So we sell it at the same price, all the products have been sold at the same price. But in the states in which we have started after this, in the rest of the country in the seven states, Wynclear continues to sell at the old price.

It is some of the chronic -- for some of the chronic therapies, the sales has come down. Most likely it will come down to next to nothing or zero in a while, we'll let that happen over a period of time. But we will continue to actually sell the Wynclear private label at the full price for acute therapies.

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The idea is that for those who are not very price sensitive, those who are not members of the MedPlus discount plan and all, and those who need the -- and those who are seeking early convenience, from them, we will get the full price. Otherwise, anyone else who is price sensitive will get the MedPlus brand at the full discount.

Kunal Randeria: Got it. And just one last one on this. So this is more like your -- it's been a few months since you launched Made by MedPlus. Let's say, as it gets bigger and people recognize the brand, do you plan to reduce discounting slightly, let's say, from 70% to a 60% or something like that?

Madhukar Gangadi: So today, our overall discount is at around 60%. We would say 50% to 80%, but the average falls at 60%. That is something we will see as we go forward, depending on how the market is evolved. Our primary thing right now concerns to grab market share and increase the top line. Once it is fully established, people understand the quality of the product and people are buying MedPlus -- are asking for it by name and then coming and buying it. Then we could always play around either with the top line, either with the MRP price, increase it slightly or perhaps decrease the discount to a slightly lower number. But that is not something which we are going to decide right now.

Kunal Randeria: Got it. Just one more if I can. Have you introduced it across the states or is it just Telangana at the moment?

Madhukar Gangadi: No, it is across all the seven states right now. As I said, only in Telangana, we have killed our old private label. Everything is now available at the new MedPlus private label brand rate. But in the other states, both co-exist.

Kunal Randeria: Got it. Thank you and all the best.

Moderator: The next question is from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed: I joined the call a bit late, so my apologies if this was asked before. So my first one is on store additions. I see that we are tracking around 400 stores added year-to-date FY'24. So I remember that previously, we had guided for roughly 1,000 stores addition, 800-plus. So what would be the updated number for FY'24? And is there a revision in the guidance for FY'25 and beyond in terms of net additions?

Madh

ukar Gangadi: Sure. The number of stores, the gross additions this year will be close to 600, around the 600 mark, the net additions may be slightly lower. And for the coming year though, the number could be around the same number. The focus this year is going to be on private label. We want to make sure that we get it to the 20% kind of mark on the pharma side. So that's going to be the thing -- both by increasing the range, increasing the awareness and also increasing the education level of our own employees in being able to sell this product.

Harith Ahamed: So based on your experience so far on the MedPlus branded private label side, can you share some of the lessons or challenges that you've encountered in this foray? Is it on the sourcing side or is it in terms of getting subscribers? And if you can comment on the willingness that you're

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seeing among patients to switch from the brand to the private label, so if you could share some of the takeaways on these areas, that would be helpful.

Madhukar Gangadi: Sure. So you have seen a pretty good response to our products in Telangana where we launched it first. The results from this are only a quarter. Actually we started only in the middle of October or late October in the rest of the country. So the numbers are still not there. But in Telangana, we have seen a very good response.

What I think is that the early adopters, people are buying the fact that all the products available in the market are generic only. And MedPlus is able to give a very good price only because it does not have any marketing or distribution costs. So people are getting that. So some of the early adopters have come on board.

We think as we go forward -- but there still remain some questions. I think we have our work cut out in trying to educate the customer. The common question we encountered is this.

Whenever our guys have gone and said 50% to 80% discount on our brands, they have said customers have come back and said is this a generic product?

So we are taking the time to explain that all the products in the country are generic. There's really no difference. It's all the same product anyway. The only way you can differentiate one from the other is on the quality of its make. So we are countering that using several different explanations out there. And we are seeing a lot of people converting once that happens.

So this will happen not just through our stores, but also through media, influencers and all sorts of other testimonials and all. Again, to come back to your question, how is the response? Pretty good. The early adopters are on board. People like the fact that they are saving a lot of money. We're seeing a steady increase. But I think if you want to hit the next, let's say, level of growth, we'll have to answer some of these questions, which we are now in that process today.

Harith Ahamed: Okay. Just a clarification, Madhukar, you mentioned that you've launched it across all the states that you operate in, but in select stores. So I was just wondering that from an operations standpoint, wouldn't it be easier to launch it across all the stores in a certain market versus launching it in select stores?

Madhukar Gangadi: No, I may have communicated it wrongly. We have actually launched it across all states, all stores.

Harith Ahamed: All states, all stores.

Madhukar Gangadi: I don't know why it came out like that. Yes, yes, now it is all states, all stores.

Harith Ahamed: And last one, I didn't get 1 number that you shared previously, the store level margins for stores of more than 24 months of maturity?

Madhukar Gangadi: Yes, Sujit, do you want to take that?

Sujit Mahato: Yes. It's around 10.1, Harith.

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Harith Ahamed: Alright. Okay. That's all from side. Thank you.

Moderat

or: Thank you. The next question is from the line of Madhav Marda from Fidelity International. Please go ahead.

Madhav Marda: My first question basically was that as we target this 20% private label mix in the pharma portfolio, how do you see the gross margins for the company trending, say, in the next -- I think you said that you can hit the 20% mark over the next year -or-so. So if you look a couple of years out once we have that Mark, how do you see the gross margin profile? Like I think we're trending at about 21.5%, 22%. How do you see this changing for the company?

Madhukar Gangadi: So Madhav, we'll try and give you a guidance as we go forward on that. So it's going to be a

function of 2 things. One, as we replace the branded product, we will see an increase of around 0.1 or 14%, overall...

Madhav Marda: Sorry, I didn't get you, you said half...

Madhukar Gangadi: No, no. As we replace the branded product, right, on the branded product, post -GST, post everything and all, we probably get around 13% overall margin and this one basically gets us around 22% to 23%. So there'll be that increase as we replace that. But in the initial phase, though, we may end up replacing some of our own private label, which is Wynclark. While we continue to basically run both of those, I think the consumers who are buying chronic product, the medicines for chronic ailments and who are buying the Wynclark brand on that, may switch over to MedPlus brand.

And there, for every one person, we will probably end up losing around 0.3%. So in the conversion -- if you convert maybe 2% or 3% out there, you'll probably end up losing 1% gross margin on the Wynclark side. But after that, it will all be a gain.

Madhav Marda: Okay. No, sir, you're saying Wynclark...

Madhukar Gangadi: So Madhav, sorry to interrupt there. See, our goal in trying to actually get this MedPlus brand popular and get the word out to everyone about the quality and the pricing of that is not to convert the existing customers into the MedPlus brand, but we actually get customers to increase their top line by bringing in customers from the other mom-and-pop and other organized stores. We are seeing a lot of people actually doing that. So we expect the benefit to us is going to be more on the top line than on actual margin side.

Madhav Marda: Okay. Okay. Understood. So -- got it. So would that mean that if earlier, like you said the store level EBITDA margin for 24 months plus stores were actually 10.1%. Essentially, what we're saying is that there is gross margin and then there's EBITDA margin. So in your view, if we get higher volumes at the existing stores, would you have a view on where the store level margins for the mature stores could move? Because I guess that's what we are finally trying to target?

Madhukar Gangadi: Sure, sure. So for us, I mean just -- if we were to increase our sales by, let us say, 10%, over the current level, right, the average store, let's say, is doing around INR15 lakh out there, if you were

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to increase the top line without really changing anything else by 10%, that's -- that will increase sale by INR1.5 lakhs. The margin will be INR30,000 extra. And that would actually increase the overall EBITDA by 2%.

Madhav Marda: Okay. Okay. So basically, the sort of older version versus the new version of stores, you could get 200 basis points higher margin, as this mix of private label goes up because we'll sell more basically at the same stores, right?

Madhukar Gangadi: The idea is to be able to sell more because if it was purely for margin accretion, then we will just continue and the old private label and just continue that. What we feel is introducing a store generic concept for the first time in India, getting people to -- see, our ability to convert a normal customer into the Wynclark customer for a higher

margin is only so much given that our pricing was not any lesser than the other brands.

Now when we talk about it and when we tell people about the MedPlus brand and its quality and we also talk about the price advantage to them, we see a lot more people, a lot more receptors. So, A, conversion of our existing customers; but B, more importantly, we have only about 15% to 20% of any other markets in this city, even though we are really big.

We feel the balance 80%, do you feel we'll be able to win over at least 10% to 20% of the balance 80% who are not shopping with us based on the price. And that's what we're looking for. Yes.

Moderator: The next question is from the line of Sayantan Maji from UBS Group.

Sayantan Maji: My first question is on the portfolio of private labels. How much of it -- sir, how much of the current pharma sales are these private label seeing? So I remember a number that you had given earlier was somewhere around 55%. So has it increased now, now that we have launched it nationally? Or is it still somewhere in that range?

Madhukar Gangadi: No, it has increased slightly. So earlier when we talked about it, it was 80% of our overall sales comes from medicines and for 55% of that we had a substitute. So on the overall scale, we had substitute for 44%. Now that 55%, I think, has now moved to around 60% or 62%, but we have also placed an order for a lot more medicine. So I expect that it will give you up to 70% as we go forward.

Sayantan Maji: Okay. So if I say, let's say, 70% of 80%, which is, say, 55%, so out of this 50%, 55%, you are expecting a 20% share of pharma private label or is it on the total sales?

Madhukar Gangadi: No, we are expecting on the total thing. See, on this 56%, the 20% is already there almost because even leaving aside Telangana, the rest of the country is at 11.2%. So out of the 56%, 20% would be only 11.2%, right? So we already have that. And in Telangana it is actually 14%. So the number is slightly higher. So when I'm talking 20%, I'm talking 20% on the overall number, so

which will actually become 20% and 56%.

Sayantana Maji: Okay. Okay. So this is basically the share of GMV, not the net revenues?

Madhukar Gangadi: Yes.

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Sayantana Maji: Okay. Got it. And also, I think in the opening remarks, you had mentioned that the share of pharma private labels has increased from 8% to 11.2%. And later in the call, you said 8.96% to 11.78%. So is it the difference between the timing? Or is it -- can you just explain like where is the difference coming from?

Madhukar Gangadi: Sure. Sujit, do you want to take that?

Sujit Mahato: Yes. We had the first number, 6.61% to 15% was an update on only Telangana, which we have launched end of June. And the second one, 8%, going up to 11.2% is all states put together, excluding Telangana. It's the only once during October, and that's relevant to the quarter.

Sayantana Maji: Okay. Understood. Okay. That's clear. And my second question is on Diagnostics. So in Diagnostics, we are ramping up in Hyderabad. So -- and I remember you're saying that it is a pilot. And once the pilot is successful, you will expand to other cities. So when do we -- when -- at what point will the decision be taken to expand into other cities based on the results of the pilot in Hyderabad.

Madhukar Gangadi: Sure. Chetan, I'll take this. So yes, no, this continues to be a pilot still. We today are at around 120,000 subscribers serving roughly around 220,000 people out there. The goal is for us to get to at least 200,000 number before we can take a call on expanding in the current situation.

Sayantana Maji: Okay. Thank you so much for taking the questions.

Moderator: We'll take the next question from the line of Tanmay Gandhi from Investec.

Tanmay Gandhi : Hi sir. Thanks for taking my questions. Sir, firstly, if you can clarify the p

private label pharma

sales on the right. So is it possible to split it between MedPlus branded and the normal private labels which we used to have even earlier on a net revenue basis?

Madhukar Gangadi: Sure. We'll see if you can do that. But for Telangana, that is Hyderabad and rest of Telangana state, all of them are lumped into 1 category right now. To the customer, it doesn't matter because he's getting the same price. To us also, it doesn't matter. So we're just not differentiating them on there, but we should be able to give you the actual split.

In the other places out of the 11.6% or 11.7%, I think around 5% is probably the old Wynclear.

I'm not sure, Sujit, do you -- if you have the right numbers, you can just go ahead.

Sujit Mahato: So it's around 11.2% on a net revenue basis, the old and the new put together.

Tanmay Gandhi : No, sir, actually. No, no, sir, actually, I want it on a net revenue business which we report, right? So I want to split off...

Sujit Mahato: I mentioned it is on a net revenue basis. I'm mentioning on a net revenue basis. But both the old as well as the new put together, as Madhukar explained that in the state of Telangana, we have not differentiated between the erstwhile model and the current MedPlus brand. And therefore, on a total basis, this is around 11.2% net.

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Tanmay Gandhi : Okay. Got it. Okay. And secondly, now that we are -- the pace of store addition has come down. So how should we look at the corporate level operating margins? So should we see a meaningful uptick or you plan to invest in some other initiatives like Diagnostics or the MedPlus -branded generics which you have launched?

Madhukar Gangadi: Okay. See, on the Diagnostics, as we said earlier, we're not going to be doing any other investments. We want to -- I think we are fully invested, except for some small maintenance capex, as Chetan said earlier. We will be wait to see how it is shaping up. We are doing well on the B2C side. We also want to see some traction on the B2B and post that we will figure out how to actually expand either on investments in the same place or in going to new cities.

On the private label side, there is no significant investment for us. The cost of the medicines is not very high, and it is just replacing one part from the other that's all. So I think the benefit of adding lesser stores, should be, I would say, more on allowing us to focus on the private label and possibly on the profitability also as we go forward.

Tanmay Gandhi : Sure. So sir, if we look at your operating EBITDA bridge that you provide, right, what are the various drags on the reported numbers. So I think the biggest drag comes from the new store additions and given that now we have a bigger base and obviously, the addition will also be lower in FY '25. So is it fair to assume that there will be a significant uptick in EBITDA margins? Or it will be steady expansion on this?

Madhukar Gangadi: No please. assume for now it is going to be a steady growth only in that. We will see how the private label and all the top line and all go up. I know the cost will be slightly lower, but I don't think that will -- I think the bigger driver for us is -- are the first 2 which is top line and private label component.

Chetan Dikshit : Tanmay, I think you have the modelling inputs over the last many quarters. The reason why you heard Madhukar say what he said was because we do see the new private label having a fairly large opportunity. And we may have additional non store operating costs like marketing, etcetera, which is what Madhukar said, what we said.

Moderator: Mr. Gandhi, we were not able to hear sir.

Tanmay Gandhi : Sorry, I lost you Chetan, sir, but my question was that given that we are right now just giving our store addition guidance, should we assume good uptick in margins because as Ma

dhukar

had said that we are not spending a lot on private label side? Sorry, I missed you. I didn't expect [inaudible].

Moderator: Sir I would request you to kindly unmute yourself and speak. Chetan sir and Madhukar sir.

Chetan Dikshit : I am afraid we couldn't comprehend what was being said. At least at our end it was quite garbled.

Moderator: Mr. Gandhi I would request you to kindly use your handset to ask question please.

Tanmay Gandhi: Yes, is there any better?

Moderator: Yes.

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Tanmay Gandhi: Sir, so my question was that, so because we are reducing our store additions for the next year, and obviously, now we have a better base as well, right, larger base, so should we assume a significant uptick given that we are not spending much on marketing as Madhukar has said, right? So I just wanted to clarify that what are the various drags on the margins?

Because if we are lowering our store additions, then ideally there should be -- the gap between 12-month -plus stores margins and the reported margins for -- at the company level should be narrower, right?

Madhukar Gangadi: No, what you're saying is right, Tanmay. Logically, if we have lesser new stores, yes, definitely. But for us, the drag of that was never very big anyway. We will continue to report stores, which have more than one year old and all. I think it should definitely be slightly better as we go forward.

Moderator: The next question is from the line of Neelam Punjabi from Perpetuity Ventures. Please go ahead.

Neelam Punjabi: My question is on the Diagnostics business. So if I look at the latest quarter, we have already reached an INR80 crores annualized run rate for the diagnostic business. However, we are still reporting our losses segmentally. So by when can we turn profit in our Diagnostic franchise?

Madhukar Gangadi: Yes, sure. Chetan, do you want to take that?

Chetan Dikshit: Yes, sure. I can take the first crack at it. Neelam, the Diagnostics business also has a similar structure like pharmacy in the sense that a lot of operating costs sit both at the center and outside the centers. So because the Diagnostics business has costs which are at the HO level, which are currently not diversified across many cities, the segment continues to be dragged by those costs.

The improvement in the performance of the centers naturally, will make up for some of that.

And we do expect to report the segment as profitable. But what we haven't disclosed so far is the center level profitability for diagnostics as we do in pharmacy which is why if you can just keep that framework in mind that what's ramping up is the revenue and the profitability of the diagnostic center.

But the HO level costs, which are actually at a multicity level that degree of preparation, whether it is procurement or absorb various lines of sub business, that continues to be a drag because our current operations are limited to Hyderabad.

Neelam Punjabi: Okay. But however, then can you help us understand what's the center level EBITDA margin, at least like for the mature center, the oldest -- first the Gachibowli center, if that will be helpful for us to understand then?

Chetan Dikshit: Sure Neelam. If you allow us, we'll do that in the next quarter.

Neelam Punjabi: Sure. Okay. And 1 clarification I needed. For the private label for our pharmacy business, we gave a target of about 20%, right? So is that on the net revenue or GMV basis?

Madhukar Gangadi: Sujit?

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Sujit Mahato: Yes. I think we are talking on a GMV basis, please.

Moderator: We'll take the next question from the line of Lokesh Manik from Vallum Capital. Please go

ahead.

Lokesh Manik: I have just one question from my end is being on the store addition in this quarter is net addition of 144 stores, but the rent expe

nse is flat at INR53 crores. So can you please help me reconcile this?

Chetan Dikshit: Yes, I'll take that. So if you observed last year for the similar period, over the period, we opened around 1,100 outlets at various points in time during the year. And in the current year, during the nine months, we have opened 400 -plus outlets. So that is the full impact taking care of the timing issue. But for that, there is no -- but for that, I would say we have the 5% to 7% annual contract base increase. So that's the major reason for the variance in the rental expense.

Lokesh Manik: Okay. But for this quarter, we should have expected a higher rent expense I mean because we are in line for the stores which are not there in the last quarter. So quarter -to-quarter, it's flat Q2, Q3 it's INR53 crores.

Chetan Dikshit: Yes?

Lokesh Manik: Yes, Q2, Q3 is flat at INR53 crores, and there is an increase in store addition of INR144 crores. I'm just trying to understand how the rent expense is...

Chetan Dikshit: Yes. It's in line with the contracts which we have entered the lease rent till and there's no exceptions in those numbers.

Moderator: Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for closing comments. Over to you, sir.

Sujit Mahato: I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you, and have a great day.

Moderator: Thank you, members of the management. Ladies and gentlemen, on behalf of MedPlus Health Services, that concludes this conference call. We thank you for joining us, and you may now disconnect your lines. Thank you.

Call: May 2024

Medplus Health Services Limited

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June 01, 2024

The Listing Department
BSE Limited The Listing Department
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Phiroze Jeejeebhoy Towers Exchange Plaza , 5th Floor
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Mumbai – 400001 Bandra - Kurla Complex
Scrip code: 543427 Bandra (East), Mumbai - 400051
Symbol: MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Earning Call Transcripts for the quarter and year ended March 31, 2024.

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the earnings calls held on May 29, 2024, for the quarter and year ended March 31, 2024.

Kindly take the same on record. The same is available on Company's website at www.medplusindia.com

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer
FCS 7460

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"MedPlus Health Services Limited
Q4 FY'24 Earnings Conference Call"
May 29, 2024

MANAGEMENT : MR. MADHUKAR REDDY GANGADI – CHIEF
EXECUTIVE OFFICER AND MANAGING DIRECTOR –
MEDPLUS HEALTH SERVICES LIMITED
M R. SUJIT MAHATO – CHIEF FINANCIAL OFFICER –
MEDPLUS HEALTH SERVICES LIMITED
M R. CHETAN DIKSHIT – CHIEF STRATEGY OFFICER –
MEDPLUS HEALTH SERVICES LIMITED
M R. SRINIVAS – MEDPLUS HEALTH SERVICES
LIMITED

MedPlus Health Services Limited
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Moderator: Ladies and gentlemen, good evening, and a warm welcome to the MedPlus Health Services Limited Q4 FY '24 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Srinivas from MedPlus. Thank you, and over to you.

Srinivas: Thank you, Yashashri. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Earnings Conference Call to discuss the financial results of MedPlus for the fourth quarter of FY '24, which were announced on 28th May. We have with us today, the senior management team represented by Mr. Madhukar Reddy Gangadi, CEO and MD; Mr. Sujit Mahato, CFO; and Mr. Chetan Dikshit, CSO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website.

I would now hand over the call to Sujit. Thank you and over to you, Sujit.

Sujit Mahato: Thank you Srinivas and a very good evening everyone, on this call. We are pleased to share that as of March 31, 2024, we have been serving community in over 640 cities across 10 states through our network of 4,407 outlets. Also, the company operates 4 full-service diagnostic centers, 8 level 2 centers and over 110 collection centers, offering essential diagnostic services at affordable rates.

On the update of the network, over the past 12 months, the company has added a net total of 585 stores, gross additions being 670 stores with 199 stores opened during Q4 alone. Most of the new stores were opened in West Bengal and Karnataka with 55 and 37 stores, respectively, during the Q4. This reflects our commitment to meet the evolving needs of our community while maintaining a focus on sustainability and financial health. We expect the new store openings for the fiscal year '25 to be around 600 stores.

In the fourth quarter, 54% of our store openings were in Tier 2 cities and beyond, underscoring the company's emphasis on these markets. At present, out of our 4,407 stores, 2,008 stores, approximately 45%, are situated in Tier 2 cities and beyond. We acknowledge the growth potential inherent in these markets. Throughout Q4, there were a total of 25 store closures. Taking into account both openings and closures, we achieved a net addition of 174 stores during the quarter compared to 144 stores added in the previous quarter 3.

In terms of our store network age, around 41% of our stores are operational for less than 2 years. And the remaining 59% of our stores have been operational for 2 years or more. It is

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noteworthy that all stores in the less than 2 years age bracket are still in their ramp-up phase.

As these stores mature, we anticipate a positive contribution to our profitability.

As a guardrail, we closely monitor the time frame for our new stores to reach breakeven. For stores opened between April 2023 and September 2023, approximately 70% of them achieved breakeven within the first 6 months of operations. As a cohort, all stores combined reached breakeven in just 4 months.

In terms of our store side, as at the end of the quarter, our network has grown to 4,407 stores with 2.3 million plus square feet compared to 3,822 stores w

ith 2 million square feet at the end

of March '23. The average store size was 530 square feet. To give you a sense of spread in store sizes, we have 3,234 stores, less than 600 square feet and 1,173 outlets that are greater than 600 square feet.

In terms of the revenue update, we are strategically positioned to increase our revenue share from private label products. Our private label range is crafted to provide customers with high-quality products at competitive prices. Presently, MedPlus offers over 1,100 carefully selected SKUs spanning across pharmaceutical and non-pharmaceutical category.

Private label sales for quarter 4 FY '24 constitute 15.2% of our total reported revenue.

Moreover, our growing presence in Tier 2 cities and beyond is significantly impacting on our revenue mix. Sales from these markets comprised 35% of our pharmacy revenues in the current quarter, marking an increase from 33% in the same period last year. The following is the impact of the launch of MedPlus branded products across our network.

In Q1 FY '24 prior to the launch, the share of private label pharma stood at 7.9% of total gross merchandising value compared to 13.6% during the current quarter. The increase in the private label GMV share indicates the positive reception from customers and validates our commitment to delivering high-quality products under the MedPlus brand umbrella.

Now on our financial numbers. On our quarter performance, our consolidated revenue was INR14,905 million, with growth of 19% year-on-year and 3.4% quarter-on-quarter. Our consolidated operating EBITDA stood at INR581 million, representing 3.9%. Around 99% of our revenue is from our pharmacy operations. The pharmacy operating EBITDA is INR593 million, representing 4%.

On our store performance, I would like to update on our stores older than 12 months. Revenue from these stores in Q4 was INR13,532 million or 94% of our pharmacy revenue. These stores had a store level EBITDA margin of 10.3%. The store level ROCE of these stores stood at 52.2%. A word here on the store EBITDA margin by age, while stores greater than 12 months had a margin of 10.3%, this was 10.7% per store greater than 24 months and 7.6% for stores in the 13 to 24 months age bracket.

If we allocated non-store related costs, then the operating EBITDA of stores greater than 12 months would be 685 million, which translates to a margin of 5%. Our diagnostics numbers.

Diagnostics revenue grew to 232 million in quarter 4 FY '24 compared to 119 million in MedPlus Health Services Limited

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quarter 4 FY '23. Diagnostic segment recorded an operating EBITDA loss of 11 million compared to a loss of 37 million in Q4 FY '23. However, it is noteworthy that -- I'm happy to report that the central level operating EBITDA stood at 33 million.

On our working capital, our net working capital for Q4 was 67 days. The inventory in our warehouse constituted 33 days. In Q4, the inventory level of our first year store was 118 days in comparison for our older stores, the inventory was 47 days.

Now I request Chetan to update on our diagnostic business. Over to you, Chetan.

Chetan Dikshit: Thank you, Sujit, and good afternoon, everyone. As you know, Q4 is a seasonally strong quarter for Diagnostics. In January, we sold 361 gross plans per day. In February and March, this was 369 and 357, respectively. As on 31st December, we had 127,000 access plans and 228,000 underlying lives. As on 31st of March, we had 133,000 access plans and 247,000 underlying lives. Our current observed on-time renewal rate was 21% in Q4 versus 19% in Q3.

That concludes our update for the quarter. I request that the host open the line for questions.

Moderator: Thank you very much. We will now begin the question-and-answer session. We'll take a first question from the line of Kunal Randeria from Axis Capital.

Kunal Randeria: Just firstly on the revenue side. If I take a

n average revenue per store, it seems to be

consistently sort of going down. For FY '24, it was around 37,000 per day. And a couple of years back, it was 40,000. I don't think the store maturity can be a factor here none of mature stores still have gone up. So what do you attribute this to? Is it because you have more stores in Tier 2 where the revenue is lower per day, what exactly can explain this factor?

Madhukar Reddy: Sure. So it's probably a combination of both, stores -- smaller stores in Tier 2, Tier 3 areas, which we always expect will be slightly lesser any way. That proportion has been growing.

And second, with the introduction of the new private label products, the top line also would have compressed a little bit. I wouldn't say it is a huge impact given that it has only gone up from 7% to 13.7% overall. So -- but both factors would be there. For us, though, we tend to look more at the EBITDA and to see if the stores are in line for a 24 month to 30 month EBITDA margin of around 10% at the store level.

Kunal Randeria: Okay. Secondly, just to take this private label point that you mentioned, our private label contribution is around 15.2% by the end of Q4. I believe the private label gross margin should be higher than your average. But if I see on a year-on-year basis, the consol gross margin is pretty similar. I just want to understand what has changed. Is there some extra provision that you have taken if you can just explain this?

Madhukar Reddy: No. Actually, there is a significant change in our private label strategy in the last one year or so. We used to sell our medicine private label earlier at the same price as the regular brands and the same discount as regular brands. While we continue to maintain the MRP at the same level, we have increased the discount significantly now.

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And for medicines, the discount now has gone up from 17% to nearly 55%. While this is a great way for us to attract new customers, and we are doing that, we're seeing a huge response to that, it definitely means that the margins would have gone down slightly. And even on the general goods also, where given the fact that we have roughly around 250 to 270 products. These products are doing very well. They're high-quality products made for us specifically. While they match the quality of the regular brands, we thought that it would be a great idea for us to give a customer one more reason for them to actually buy this product. And we are not normally a destination for people to come and buy toothpaste or dish washes or any other home cleaning or other equipment and all. We are in a position where we can basically be seen as a convenience store almost, where we have a customer walking in. We would like to encourage them to spend a little bit more in our stores and hence, we're encouraging them by basically reducing the price slightly.

We've seen a pretty big impact because of that, and we continue to see it. But that's one of the reasons why the private label margins would have not really bumped up the overall margins for us.

Kunal Randeria: Got it. I mean so if I were to summarize, it is because WYNCLARK margins would have been higher than made by MedPlus, and that is why you're not seeing that jump?

Madhukar Reddy: Yes. No, no, absolutely. So on both accounts, actually, we have also gone towards market share growth from the strategy on the private label non-pharma also. So both of them and we are actually seeing fantastic revenues out there. It is yet -- again, the impact will be seen as we go forward, but we are seeing the growth, yes, for both the reasons. MedPlus would have had slightly higher margins, and so would have our older private label non-pharma products.

Kunal Randeria: Sure. And just one more, if I can squeeze in. So a month ago, there were some news items saying that there are a couple of association with ch

emist in Karnataka and Telangana who

have been pushing the CDSCO to take action against MedPlus. So if you help us understand what exactly happened and where things stand now today.

Madhukar Reddy: So every time we go there and do this kind of, let's say, I won't say disruption, but at least logical progression. We have built the scale and now we are ready to launch our own products. And obviously, because we can pass on the benefit to gain more market share, we are doing that. There will be some people hurt and that's what has happened with the smaller stores. They have gone and approached all the -- both their local associations, local drug bodies from which nothing actually happened.

So ended up writing a letter to the CDSCO. Nothing concrete out there but given that we have been advertising our 50% to 80% discount quite prominently everywhere. I think in some cases, some of the departments have picked up the samples to check which -- we are more than happy to let them do. So that's about it. I don't think there has been any other impact as of now.

Kunal Randeria: So there is no litigation I think legal kind of court cases going on now?

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Madhukar Reddy: Absolutely none not at all. See Kunal while we all know that the government is behind this. Government wants everyone to basically go towards the generic side. Government wants everyone to basically pick one and save money on that and that's been their thing out there. And the drug departments and everyone recognize the fact that -- this is going to be a part of the overall business as we go forward.

I don't think anything which benefits the customers so in such a large fashion I don't think this is going to really find any kind of opposition from the government. Yes traders and all may get affected and may approach them, but I don't see any action happening from that side.

Kunal Randeria: Okay got it. Thanks and all the best.

Madhukar Reddy: Thank you.

Moderator: Thank you. We'll take our next question from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed: Hi, good afternoon. Thanks for the opportunity. So my first question is on the private label, the pharma private label number that you shared which is at 13-%odd percent of GMV for the quarter. So will you be able to give a breakup between the old private label and the new MedPlus brand private label for this 13-%odd number? And some colour on how this number or how the share has ramped up in different geographies, maybe in Telangana where you've launched earlier and in other states, how the progress has been?

Madhukar Reddy: Sure. So I'll try and get you the exact breakup, but to my knowledge at least from what I remember I don't think old Wynclear is now more than 1% or 2% of the overall sales maybe 2% of overall sales is what I think will be the number, but I'll get back to you on that. On the launch itself while October, November we launched this across the country. We have gone about using advisement only in very selective areas.

And so we started in AP, Telangana where we advertised and then we basically did it in Tamil Nadu and now in West Bengal. In all these places, it has actually gone up pretty significantly.

In -- let me actually -- Sujit why don't you step in and give the actual numbers, if you have any, for the different states Tamil Nadu and all.

Sujit Mahato: So in Tamil Nadu where we launched it post our marketing campaign March exit on pharma GMV sales we are at 19.3%. And in Telangana we ended at 15.9% and for all the other details maybe we can connect offline and can share the data.

Harith Ahamed: Okay. And Madhukar for -- previously you had shared a goal of around 20% share from a private label. Do you still maintain that?

Madhukar Reddy: Absolutely. We continued to increase the number of products out there. There's been a slight delay in bring

ing all the products into the market. We started off with around 433. Now we are around 550, 560. I think we are adding another 200 products. So that process is on and we are also seeing a larger number of people coming to buy from our stores. There will always be
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some people who will probably drop out for whatever reason, the doctors would have told them or whatever.

But we have seen the ever increasing number of new people and the existing ones. So we're tracking that. And for us the 20% number should not be a difficult one to beat. If you actually look at only the medicine part, given that 80% of our sales is medicines and given that something like 15.6% even in Telangana and slightly more in Tamil Nadu is already from this, you could say that 20% of all which we sell in medicines is already MedPlus private label.

Harith Ahamed: Okay. And one more question on the margin profile of our pharmacy retail segment. So when I look at the maturity profile of our stores and less than 12-month stores which accounted for 30% of our overall stores in March '23 now it's around 15%, but we haven't seen a commensurate improvement in our margins for pharmacy retail segment.

It's broadly in line with what we -- where we were last year. I know there are a lot of moving parts. But if you can help us understand how we should think about this trajectory going forward FY '25 as well, we have a lower number of store additions so that will aid maturity mix even further. So some broad sense on how we should think about margin profile for just the pharmacy business?

Madhukar Reddy: Sure. So a couple of reasons why the margins have not gone up to the level which we would have normally expected. One, we switched from our old private label to new private label. In the initial stage it would basically remain a slight decrease in overall margin. So if you actually look at it our overall margin on private label has come down slightly because of a huge mix of the new ones which are coming out there. So that's one.

The second thing is we also spent a little bit of money on advertising these. So compared to what we would have normally spent there has been an increase in marketing. And while it is not really significant, but it is enough to basically offset that small increase we would have had. We're happy to tell you that, that advertisement is really paying off. More and more people are coming to ask and request for the MedPlus brands in AP, Telangana and Tamil Nadu out there, but that's the past. That's something which we have taken initially.

Harith Ahamed: Understood. That's all from my side. Thanks for taking my question. Thank you.

Moderator: Thank you. The next question is from the line of Jatin Chawla from RTL Investments. Please go ahead.

Jatin Chawla: So the first question is on the new private label side what are the sort of retention rates that you're seeing I know the absolute numbers are going down, but are you tracking retention rates for people who are subscribing and how is that panning out?

Madhukar Reddy: We are tracking. We're not sharing the numbers right now. I don't think that is being -- we will try and see if we can do it at a later date, but right now no. The retention rates are in line with the usual rates which we have for all customers. It is no different.

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Jatin Chawla: Okay. Got it. On the pharma GMV share numbers that you shared for Telangana and Tamil Nadu, it seems Tamil Nadu has done far better despite the new private label strategy there being there for a shorter period of time. So what are you doing right there? And is there any learning that you can kind of extrapolate to other states?

Madhukar Reddy: So a couple of things. Yeah so one thing, of course, we have to note that Tamil Nadu started with a base of around 9.6% private

label and then that went up to 19%, whereas Telangana started with a much smaller base at around 6.9% and it has gone up. And the second thing is in Telangana, we really did not spend too much money on any kind of advertisement. We just did a couple of press conferences and did a little bit of unpaid kind of advertisement. And a very low level newspaper kind of ads, whereas Tamil Nadu, we decided to step it up a little bit and actually, I would say, spent at least twice or thrice of what we had spent in Telangana. So two things, the base as well as increased exposure, both of them are actually working in Tamil Nadu. We expect -- and we were running at Tamil Nadu just to see if the spend will actually yield the way we want it to.

We are still to decide on that once we are fully convinced, and that's what we decide -- that's what we end up doing across the country.

Jatin Chawla: Got it. When I look at your mature store ROCE number that you share every quarter, if I look at an annual basis, it seems that number has gone down from like 60% on an average to close to 50%. So this 10% decline, is this largely driven by the margin impact on the private label? Or is there any other factor which is driving that?

Madhukar Reddy: It's unlikely that it is mainly the margin impact, I would say, possibly some of it, but it is actually largely because of slight increase in store level inventory. And that is because we have introduced around 550 to 600 new products. Each of these products have gone to existing stores. We have not yet -- or over a period of time, our overall inventory of the other brands as this displaces that, that will come down.

But meanwhile, there's a slight increase in inventory, and that is what is resulting in the decreased ROCE out there. But as you go forward, given that our cost of private label is so much lesser than the branded drugs and all, as we figure out which portion of the branded drugs is actually coming down as we start to reduce that inventory, I think it will come back to normal.

Jatin Chawla: Okay. Just one last question. So you said that your -- so both private label pharma and private label non-pharma margins have come down this year. Between the two, which of it has a bigger impact on gross margins this year?

Madhukar Reddy: Possible -- definitely, the private label pharma, I would say, because we were at around 8%, 8.5% private label with 70%, 75% kind of margin or slightly more than 80% of kind of margin out there. Now that portion, which is at 80%, has come down to something like 1% or 2%.

And it is all filled up by the new product, which is at roughly around 70% margin overall.
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But this 70% also is on a lower top line. So what used to sell at roughly around INR83 on INR100 MRP, now sells at roughly around INR45 to INR42 -- INR42 to INR45. So it is slightly lesser margin on the new private label. But even the slightly lesser margin is on nearly half of the overall thing. So this would have impacted it significantly.

Jatin Chawla: And this impact when in Tamil Nadu, you reached like 19% share -- GMV share, has that share -- is this impact now negative? Or do you need still higher share for this impact to be negative?

Madhukar Reddy: No. I think more or less negative because in Tamil Nadu, we probably are at around 1%, 1.5% or less than 2% of the old private label. And if that comes down to 1% or 2% or less, then -- and we are still maintaining the same margin. So from there, any growth in the new private label is going to see a growth in margin for us.

Jatin Chawla: Okay. Thank you for answering my questions.

Moderator: Thank you. We'll take our next question from the line of Madhav Marda from Fidelity. Please go ahead.

Madhav Marda: Hi good evening thank you so much for your time. I had a question on the -- two questions. First one was on the GMV growth. Could you help us und

erstand how much is the GMV

growth, which in my understanding would be a proxy for volume growth for the company's...

Madhukar Reddy: Sure. As Sujit actually said that the overall GMV for pharma is roughly around 13.7%, I think. And in states where we have gone and advertised and actually made big, I think it is in excess of around 16% in Tamil Nadu and all.

Madhav Marda: And I mean the GMV growth, the mix -- I got the mix, but how much has this growth...

Madhukar Reddy: Got it. The mix, which I told you about was 16%, when you look at it on GMV basis. Let me just come back to you on the actual growth itself. So yeah... Sujit, go ahead.

Sujit Mahato: On a year-on-year basis, that is 27.4% in terms of the revenue in terms GMV.

Madhav Marda: Which I should take as proxy for volume growth, right, in a sense that 27% growth in volume growth...

Madhukar Reddy: Yes.

Madhav Marda: Okay. And then my second question is on the margins. I think in the morning in the television interview, you did mention that we spent about INR10 crores extra on promotions for the new private label in Tamil Nadu. So just wanted to understand that -- and I think you also mentioned that in FY '25, if I picked it up right, we expect the pharmacy operating margins to be 4.5% to 5%. So just wanted to check, first did I pick the comment right? And if you could share your outlook on this for FY '25 for the pharmacy operating EBITDA margin.

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Madhukar Reddy: Got it. So I'm not sure if the exact number is 10% for last year. I think some of it would have actually gone to this year also. But I think the overall increase in spend last year was more than INR10 crores extra than -- more than usual. Not sure if it is exactly in Tamil Nadu. I'll come back to you on that, Madhav.

We started the campaign in Tamil Nadu on February 25th. So entire February -- that last part of February and March would have gone to last year. But the number INR10 crores or slightly higher were spend for private label across the country before that. And on the growth itself. So if you -- I think we exited the year with around 4% overall. We expect that we'll at least grow this by at least 1% as we go forward, given that we're adding lesser number of stores, and we'll also get some more operational benefits now as we go forward. For the full year.

Madhav Marda: Sir, which was my question was that in quarter four, you're saying we were at 4%. For FY '25 as a whole, can this number be, I think, 4.5% to 5%. Is that what you said in the television? Or if you could just share your guidance for FY '25 for the full year, given we have lesser stores coming in as well.

Madhukar Reddy: So definitely we're higher than what we are and probably exit at least at 1% higher than this. But for the full year, we may go slightly above where we are because we'll still see the impact of the private label -- the old private label coming down slightly, and that will be there. But right now, at this point, why -- I don't want to give you the full margin guidance right now, but you can definitely expect some improvement out there.

Madhav Marda: FY '25 to be higher than exit of FY '24, you're saying, right?

Madhukar Reddy: Yes, definitely.

Moderator: Thank you. We'll take our next question from the line of Ashish Jindal, an individual investor. Please go ahead.

Ashish Jindal: Sir, I saw a few of your podcasts and it was a great way to market. I would appreciate if you could do more such podcasts in more languages. I guess it would give MedPlus more receptibility in newer territories. Sir, my question is that what are the challenges in getting B2B diagnostic business provided that you offer such a value to our customers?

Madhukar Reddy: The B2B part is not like the usual business of the other diagnostic companies. And other diagnostic companies say that -- they're basically saying that they are picking up sa

mple from

other small centres and processing it, and that's their B2B business. For us, the B2B is in subscription. Our business is selling subscriptions. Today, we sell B2C for -- subscriptions to people who come to our stores, who come to our diagnostic centres and who go online to buy. And the B2B would mean we would go to large companies and sell this as part or as -- or on a like-to-like basis, we would say, equal to insurance kind of product.

Companies normally pay INR25,000 a year for a family to get there to get themselves covered for health insurance. We are basically saying maybe they could pay INR1,000, INR500 or some number like that for the family for them to get covered for the diagnostic part. So that's

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the B2B part. Obviously, it's good to get that business going, and that's what we are looking to do this year. But the selling cycle out there is much longer, and it does take a while to convince them. B2C on the other hand is doing very well for us. B2B is what we're looking to crack this year.

Moderator: Thank you. We have our next question from the line of Bino Pathiparampil from Elara. Please go ahead.

Bino Pathiparampil: Hi good afternoon, sir. A couple of questions. First, I was looking at your deck, the share of private label in pharma has not changed much from last year's level, 8.4%-8.5%. The increase has come from private label others.

Could you put that in perspective because I thought with the private label pharma launch, that should have gone up, right?

Madhukar Reddy: Yes. So it looks like it has not gone up too much because you are looking at the net prices. Our prices on the new private label have actually gone down significantly, as I said earlier. What we used to sell at INR83 on a INR100 market, today we sell it around INR72 to INR75. So you would have seen a decrease because of that.

But if you look at the GMV part, last year where we ended at 8.1% -- or last year was 8.1%.

This year is 13.7%. And this got started only in the later part of the year for the rest of the country. But you're still seeing the growth. On the FMCG side, it used to be -- it has gone up there around 1.4% on the FMCG side.

Bino Pathiparampil: Understood. On the diagnostics market side, what is our latest plan and thoughts? Are you planning to expand it further? Or still we are in the figuring out the business model stage?

Madhukar Reddy: I'm going to let Chetan answer that question.

Chetan Dikshit: Yes, sure. On Diagnostics, we will continue to focus the model here in Hyderabad before we look at expanding in meaningful way outside of Hyderabad.

Bino Pathiparampil: Understood. But within Hyderabad, you're adding more labs or centres or anything like that?

Chetan Dikshit: There will be some maintenance capex type additions like ultrasound or something similar, but there is no large scale capex that is currently scheduled for Hyderabad. So no big diagnostic centres of -- no heavy capex changes, something more like maintenance capex.

Bino Pathiparampil: Understood. And just a bookkeeping. The consol level, the tax rate this year is around 10% odd. Could you let us know how it could progress over the next 2, 3 years at the consol level tax rate?

Madhukar Reddy: Over the next couple of years, we expect the tax rate to be normalized between 24%, 25% of the guidance.

Moderator: Thank you. Next question is from the line of Madhav Marda from Fidelity. Please go ahead.

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Madhav Marda: Thank you for the follow-up. My only other question was that if you -- like do you think from like a 3-, 5-year view on the pharmacy operating EBITDA margin, which is at 4% in quarter 4.

As we have slow maturity and I guess the new private label once it starts giving you the volume growth that there are some signs as that scales up across the country. And I

think our

stores in the smaller towns, you all have indicated in the past that it's better margin.

Just wanted to get like where could the sort of company-level operating EBITDA margin look like in 3, 5 years. If you could give us some sense. And are we thinking of the drivers correctly for the margins? Or are we missing something?

Madhukar Reddy: Yes, definitely. Same-store sales growth as the stores mature and go forward; 2, slightly better EBITDA in the smaller towns and all and 3 -- even when the top line is slightly lesser; and 3, private label. While private label comes with a couple of caveats out there, we still have to get the old private label completely out of the system. And as we get it out, you will lose a little bit of margin out there.

And it will also come with a little bit of spend. We will end up spending a little bit of money just to get the word out. What we think is this, Madhav, we're building a brand and we're getting people to come in and ask for the MedPlus brand out there. We're giving with discount. We feel as we go forward, we can play around with the discount a little bit. We can definitely play around with MRP. We allow to increase the MRPs every year and all. So all those will be great leverage for us.

But in the near term, though, while we build a brand, the margins may be flattish or grow up slightly. In the very near term, this thing, you will see some seasonality effect. Q1 is typically

expected to be slightly lower because of seasonality and all that. But otherwise, on the overall side, I would say we will continue to grow and get better margins. The levers are, as we said, smaller stores, smaller towns, slightly better EBITDA and private label.

Madhav Marda: Okay. So, the old private label in quarter 4, you said it was 1% to 2% for GMV, is that right?

Madhukar Reddy: In some of the places, I would say in Tamil Nadu and all probably down to that level. But in other places like Maharashtra and a couple of other places where we have not fully launched it with any kind of advertisement, it may still be slightly higher than that at this point.

Madhav Marda: Okay. And we plan to straddle it anyway, right? Like in most of the part -- most of our stores, except 1 or 2 states, we do plan to keep the old private label as well? Or what's the plan?

Madhukar Reddy: But it will probably come down to just the acute kind of products being sold in that. So, what we would say is we sell a subscription plan to people want to buy the chronic -- who want to buy the MedPlus brand. So, our thing is okay, we differentiate between the person who is really price sensitive and he wants to become a member and he wants to take the full advantage, he will get that.

But someone who is not -- who is seeking only convenience and fast service and not really looking to become a member and is just buying it one off, will probably end up buying some MedPlus Health Services Limited

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of our private label in the old brand for acute needs. So, it will probably give us different margin. But I don't expect that to be more than 1% or so as we go forward.

Moderator: Thank you. We have our next question from the line of Lakshminarayanan from Tunga Investments. Please go ahead.

Lakshminarayanan: A couple of questions. First, when you say your store is mature, at what stage you call the store as mature is it after 2 years or after 3 years?

Madhukar Reddy: So, 24 to 30 months would be the stage, slightly. It depends on the place and all. Smaller towns tend to plateau much faster. They will not grow beyond a certain level, whereas the larger cities like Bangalore or Bombay or something like that will tend to continue to grow even after like 24 to 30 months. And it is needed too because these stores typically come with a much higher rent and all we expect them to do a lot more. So, they continue to grow at 30 months. Yes, 30, 36 months.

Lakshminaray

anan: And when it comes to around 30, 36 months, from that what is the typical same-store sales growth? I mean usually, what nominally attracts?

Madhukar Reddy: Typically, we would say inflation plus. But given the fact that we have a big price advantage over other competitors and all, we expect that we're at least grow at around 10% of those stores.

Lakshminarayanan: Okay. When you say inflation, it is the pharma inflation or the general inflation, you usually model it?

Madhukar Reddy: Pharma inflation, 20% of it is the general goods. So, it's come with general goods plus pharma inflation that are up there.

Lakshminarayanan: Got it. And you say 10%, is it -- I mean, including the taxes? Or how do you think because what we understand is that the pharma companies themselves can take their prices up in that range inflation, right? So if you're growing at inflation, I just want to understand whether it is including taxes, excluding taxes and how does the pharma inflation play a role in this?

Madhukar Reddy: Pharma inflation is the main thing out there. One of the other things is also -- I won't say inflation at least we keep adding new categories out there and all. So that also caused a slight bump. Unfortunately, pharma inflation also comes with I would say, a reverse this thing out there. It comes with the DPCO price rules, which rise from time to time and we extend to drop the prices a little.

So it is not always going up or it is not always going up for the entire set of products there are a set of products which go up and some which could actually get moderated or even come down a little bit. So, on the whole -- because we would definitely be dependent on the pharma inflation and taking some market share from the other competitors, I would say 10% would be a good number to shoot for.

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Lakshminarayanan: 10% is our realization net of taxes?

Madhukar Reddy: Growth. Yes.

Lakshminarayanan: Yes. And second, from an operating cash flow point of view what is your goal or aspiration for the next couple of years? When I say operating cash flows, I mean net of lease rentals and so on. How do you think that would pan out? What's the aspiration for the management for not just 1 year but like 2, 3 years down the line?

Sujit Mahato: So over a period -- the way we think is -- you are right. From the operating cash flow, if you remove the lease rentals, which goes back to the pre-indale and then also look at the capex which the company is currently investing. Because we are in the growth phase it would continue to be minus and then we would leverage whatever working capital is required through a separate credit line as and when needed. But once we reach a stability that's the point where the company starts generating operating free cash flows.

Lakshminarayanan: And typically, in your expectation, how far we are? Are we 2 years out or it's like it will take out the 5 years?

Sujit Mahato: See, if we grow at the current speed I think in a couple of years, we will be adding surplus cash because the base starts growing and the base gets maturing then the level of cash inflows versus what we spend on an average of 600 stores per year, it would definitely inch towards the surplus cash.

Lakshminarayanan: Sorry to again harp on that point. Are we -- I mean, we are in -- 2025 started. So we expect by '27, '28? Or I mean how should investors look at it?

Madhukar Reddy: Okay. So for us, if you look at it, as Sujit said if we were to go only at the rate of around 600 stores a year or even 800 stores a year then the overall investment for us would be in a year within INR200 crores to INR220 crores. That is taking the investment around INR25 lakh to INR30 lakh per store.

Given that we are at something like INR170 crores on EBITDA right now and we should expect to grow pretty significantly as we go forward, I don't think it will be too

long before

because below the EBITDA, we don't have any debt as of now. There's only a timing mode of depreciation. Otherwise, it's just taxes.

So for us to actually get to this number, it's not going to be too far. And it all depends on what else happens in the company. If the margin were to go up by 1%, obviously at 1%, 1.5% as we bring in leverage -- the benefits of leverage and private label, then this, of course, can be achieved much faster. But on a conservative level it is what Sujit said in the next 2 to 3 years we should be having enough cash to fund any kind of growth.

Lakshminarayanan: All right. My third question is related to discounting. Just want to understand how the discounting has actually panned out for the industry in the last 1 year. What we anecdotally see is that discounting has reduced in some pockets. Just want to hear your thoughts on the

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discounting process both online as well as offline? And how are you thinking about as we move forward?

Madhukar Reddy: Sure. So, if you look at the discounts, I'm pretty sure given the amount of money which online payers usually spend on advertising and all they have a very high level of visibility. And so what they do tends to basically be seen as the industry's numbers. Definitely, they reduced their discount a little. Across the board, I think, they have all reduced their discount, at least for the lower tickets have reduced the discount.

But please bear in mind that the overall online business is actually less than 5% for the country, which basically means the rest of the business is in the hands of the mom-and-pop operators and the other brick-and-mortar operators. And there, we haven't seen any signs of reduction of discounts. If anything, people are now selling more and more generic drugs. They are mixing prescriptions with generics and are able to offer 20 and at times even 25% discount. So on the ground, it is a mess. People are using at least -- stores seem to be using all sorts of techniques to basically offer a higher and higher discount. And a lot of times, it is by just mixing either. I won't say, yeah, generics mainly. So, on the, and to answer your question, yes, the online guys have reduced it. No significant impact to us, honestly, whether they've reduced or increased. The offline players, though, are where they are, if anything, they're actually increasing the discounts.

Lakshminarayanan: Thank you, so much for your detailed answers.

Moderator: Thank you. We have our next question from the line of Saion Mukherjee from Nomura Securities. Please go ahead.

Saion Mukherjee: Just one question on private label farmers. So the reported number is 8.5% for the quarter. If

you were to split it between old and new, can you do that just to understand how much of cleanup of the whole private label is still left?

Madhukar Reddy: I can't give you the exact number right now, Saion, but I think it is roughly around 2% right now. But to give you an idea, I think -just hold for one second. Yes, probably around 2%. I'll come back to you with the GMV equivalent for the private label.

Saion Mukherjee: So this 2% is new -- or is it new, sir?

Madhukar Reddy: Sorry, the 2% is old or new. Okay. Sujit has the numbers. Why don't I ask him to answer the question?

Sujit Mahato: So in terms of GMV for the whole year, the breakup is 5.3% is the new product, and the old private label, because we launched it only in October, still continues for the whole year at 5.4%. For the quarter, it should be a different number, just give me a second. I'll give you that as well. And for the quarter, the old private label has shrunk to 2.9%, and the MedPlus brand has moved to 10.7% in terms of GMV.

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Saion Mukherjee: Okay. So a large switch has already happened. Yes. Sir, just to give -- get some colour , when

you think about this new private label, the volumes that you've got, how much of this volume has come from new customers who are not -- who didn't buy earlier from MedPlus and how much is from the existing customer? So do you look at the mix of the new private label currently?

Madhukar Reddy: A little difficult to split that exactly but in the states where we have seen the private label taking off primarily because we have advertised it slightly better out there, we have seen a slight growth in the overall sales, and we're also seeing a lot of new customers walking into our stores. So difficult for me to tell you. But yes, there's a certain percentage of people who are converting existing rights. And there's also significant people -- number of people who are coming in, who are otherwise not buying with us or who had stopped buying for whatever reason.

Saion Mukherjee: Okay. You don't have the split at this point. But do you think -- just qualitatively, like more than 50% you think would be new customers? Or it is old customers, which would be the majority of this new private label currently?

Madhukar Reddy: Up to 50% could be new people.

Saion Mukherjee: Okay. And you mentioned it's more sort of focused on chronic, right? So is there a disproportionate contribution from chronic here?

Madhukar Reddy: So I would say it is definitely represented more by people who are buying for MRP values, which are more than thousand and more. The reason I say that it is everyone who comes in to buy chronic medication also has some parts of acute also or some parts of general you would say you know you basically have a bunch of products like pantoprazole and vitamins and everything else, which are not necessarily chronic.

And they could also have some of the other acute products. But the lesser ticket value, the guy who walks in for just immediate need that definitely is slightly lesser on the retrospect. And the reason being that guy doesn't have time to actually listen to the explanation of our pharmacists. Neither is he is going to go become a member paying any kind of money as small as it is to get that advantage because ticket size is small, discount really doesn't matter, and we've not really changed the brand at that point.

Saion Mukherjee: Thank you.

Moderator: Thank you. Ladies and gentlemen, we'll take that as a last question for today. I would now like to hand the conference over to management for closing comments. Over to you.

Madhukar Reddy: Thank you, ladies and gentlemen. I thank all the participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at IR at www.medplusindia.com

[medplusindia.com](http://www.medplusindia.com)

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Moderator: Thank you, members of the management. Ladies and gentlemen, on behalf of MedPlus Health Services, that concludes this conference call. We thank you for joining us, and you may now disconnect your lines. Thank you.

Call: Aug 2024

Medplus Health Services Limited

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August 7, 2024

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Scrip code: 543427 Bandra (East), Mumbai - 400051
Symbol: MEDPLUS

Dear Sir/ Madam,

Sub: Transcripts of Analyst / Earning Call / Conference Call:

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the earnings conference call held with analyst on August 5, 2024 for the quarter ended June 30, 2024.

Kindly take the same on record. The same is available on Company's website at www.medplusindia.com

Thanking You,
For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer
FCS 7460

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"MedPlus Health Services Limited
Q1 FY'25 Earnings Conference Call "
August 05, 2024

Management: Mr. Gangadi Madhukar Reddy - Chief Executive Officer
and Managing Director
Mr. Sujit Kumar Mahato - Chief Financial Officer
Mr. Chetan Dikshit - Chief Strategy Officer
Mr. Srinivas - Sr. Manager Finance

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Moderator: Ladies and gentlemen, good day, and welcome to the MedPlus Health Services Limited Q1 and FY '25 Earnings Conference Call. As a reminder, all participant lines will be in the listen only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Srinivas. Thank you and over to you sir.

Srinivas: Thank you, Shajal. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Q1 FY '25 earnings call to discuss the financial results of MedPlus for the first quarter of FY '25 which were announced on the 2nd August 2024. We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, CEO and MD; Mr. Sujit Mahato, CFO and Mr. Chetan Dikshit, CSO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward looking in nature, and may involve risks and uncertainties. Please note, the disclaimer mentioning these risks and uncertainties on slide 1 of the investor presentation shared with all of you earlier, documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website. I would now hand over the call to Sujit. Thank you. And over to you, Sujit.

Sujit Mahato: Thank you, Srinivas, and a very good evening to everyone on the call. We are pleased to share that as of June 30, we have been serving communities in over 650 cities across 10 states through our network of 4,444 pharmacy outlets. Also, the company operates four full service diagnostic centers, eight level two centers and over 110 collection centers, offering essential diagnostic services at affordable rates.

I would now brief upon the openings and closures of our outlet. Over the past 12 months we have added a net total of 469 gross 568 store editions with 66 stores opened during quarter one. However, planned new store openings for FY '25 remains at 600. In the first quarter, 44% of our store openings were in tier 2 two cities and beyond. At present, out of our 4,444 stores, 2,031 representing 45% are situated entire two cities and beyond. We continue to acknowledge the growth potential inherent in these markets.

Throughout Q1, there were 29 store closures. Taking into account both openings and closures, we achieved a net addition of 37 stores during the quarter, compared to the 174 stores added in Q4. In terms of our network, the age of the network is around 36 % of our stores are operational for less than two years and the remaining 64% of our stores have been operational for two years or more. It's noteworthy that all stores in the less than two years age bracket are still in their ramp up phase.

As these stores mature, we anticipate a positive contribution to our profitability. As a guardrail, we closely monitor the timeframe for our new stores to reach breakeven. For stores opened between July 2003 and December '23, approximately 64% of them achieved breakeven within six months of operation. As a cohort, all stores combined reach breakeven in just four months.

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In terms of our store size, as at the end of the quarter, our network has grown to 4,444 stores with 2.3 million plus square feet compared to 3,975 stores representing 2.1 million square feet at the end of June 2023. The average store size was 529 square feet. To give you a sense of spread in store sizes, we have 3,269 stores, less than 650, and 1,175 stores that are greater than 600 square feet.

On our revenue mix, we are strategically positioned to increase

our revenue share from private

label products. Our private label range is crafted to provide customers with high quality products at competitive prices. Presently, MedPlus offers over 880 carefully selected SKUs spanning across pharmaceutical and non-pharmaceutical categories.

Private label sales for quarter one FY '25 constitute 15.8% of our total revenue. Moreover, our

growing presence in tier 2 cities and beyond is significantly impacting on our revenue mix. Sales from these markets comprised 36% of our pharmacy revenues in the current quarter, marking an increase from 34% in the same period last year.

The following is the impact of the launch of MedPlus branded product across our network. In quarter one FY '24, prior to the launch, the share of private label pharma stood at 7.9% of total GMV compared to 15% during the current quarter. The increase in the private label GMV share indicates a positive reception from customers, and validates our commitment to delivering high quality products under the MedPlus brand umbrella.

Now on the financial numbers. On our quarter performance, our consolidated revenue was INR14,888 million with growth of 15.9% YoY and remains flat quarter-on-quarter. Our consolidated operating EBITDA stood at INR435 million representing 2.9%. Around 99% of our revenue is from our pharmacy operations. Revenue from pharmacy operations grew by 24% year-on-year on GMV basis and by 15.3% year-on-year on net basis. The pharmacy operating EBITDA stood at INR432 million representing 3%. The pharmacy operating EBITDA adjusted for special marketing spend of INR95 million stood at INR527 million, representing 3.6%.

Now on our stores performance, I would like to update on our stores older than 12 months. Revenue from these stores in quarter one was INR13,604 million or 9.4% of pharmacy revenues. These stores had a store level EBITDA margin of 9.3%. The store level operating ROCE of the stores stood at 46.1%.

Award year on the store level EBITDA margin by age, while stores greater than 12 months had a margin of 9.3%, this was 9.7% for stores greater than 24 months and 6.8% for stores in the 13month to 24month age bracket. If we allocated non-store related cost, then the operating EBITDA of stores greater than 12 months would be INR525 million, which translates to a margin of 3.8%.

On our diagnostic numbers, diagnostic revenue grew to INR242 million in quarter one FY '25 compared to INR139 million in quarter one of FY '24. Diagnostic segment recorded an operating EBITDA of positive INR3.3 million compared to a loss of INR46 million in quarter one FY '24.

However, central level operating EBITDA stood at INR31 million.

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Working capital. Our net working capital for Q1 was 69 days. The inventory in our warehouse stood at 40 days. In quarter one, the inventory level of our first year store was 104 days. In comparison, for our stores older than 12 months, the inventory was 47 days.

Now I request Chetan to update on our diagnostic business. Over to you, Chetan.

Chetan Dikshit: Thank you Sujit, and good afternoon, everyone. As you know, Q1 is a seasonally weak quarter for diagnostics. With that context, in April we sold 406 gross plans per day. In May and June, this was 408 and 442 respectively. As on the 31 March, we had 133,000 active plans and 247,000 underlying lives covered under our plans. As on the 30 June, we had 141,000 active plans and 270,000 underlying lives. Our current observed on time renewal rate was 24% in Q1 versus 21% in Q4. That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you very much. The first question is from the line of Tanmay Gandhi from Investec.

Please go ahead.

Tanmay Gandhi: Hi, sir, the quarter looks a relatively weaker one, and I understand that this is a relatively weaker quarter on

a seasonal basis, but I just want to understand.

Moderator: Sorry to interrupt you sir. May I request you to please use your handset?

Tanmay Gandhi: Yes. Sir, my question is that, you know, whether do we need to revisit our guidance or are we reiterating our guidance of 20% plus top line growth and, you know, maintaining 4Q level margins?

Madhukar Reddy Gangadi: No, I don't think we need to change anything out there. As we mentioned earlier, Q1 is typically slow, and that's what it is. I don't think there's anything more to look beyond that honestly.

Tanmay Gandhi: Sure. And the 16% growth looks very weak, given specifically if you look at pharma companies' growth, even the largest player had reported almost 16% year-on-year growth. So just wanted to understand that, have you plateaued in terms of market share against even our same store sales growth also looks a little tepid. Just wanted to understand that, you know, is it something, is it a market specific trend or there is someone off in our numbers?

Madhukar Reddy Gangadi: No, no, Tanmay, it's actually 23%. You know, given that significant portion of our sales is now coming from MedPlus private label, and these are all selling at, you know, half the price of our regular brands, right. So, any growth in this is going to definitely affect the overall top line. So, if you look at it MRP to MRP, and if you compare like to like out there, our growth is actually 23%.

Tanmay Gandhi: Understood. So GMV growth is 23%. Got it. So, the 20% plus growth which you have guided for, that is for reported revenue or that is for GMV revenue?

Madhukar Reddy Gangadi: No, that's actually for normal reported revenue only for this year.
 Tanmay Gandhi: Understood. And sir, you know, our store generics are relatively better in terms of gross margins, right, but still, we have seen a good 50 bps decline sequentially. So just wanted to understand that, you know, is there any inventory write off during the quarter?
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Madhukar Reddy Gangadi: See, while the private label margin is definitely better than the regular branded margin and all, there are two things happening here. One, we are cannibalizing into the old private label, which had a slightly higher margin.

And this one, as you know, while it is better, it is not as good as the old private label. We actually have 52% to 55% overall discount on this product. So, the margin is not going to be as accretive.

So, if you look at a INR100 MRP equivalent for private label and for a branded generic, the regular one, if after the discount on a branded generic, we get something like 13 or 0.13 or 13% or something like that.

On the new private label, you end up getting anywhere from 23 to 26. This is on the INR100 MRP. If you look at on the net sales, it is going to be around 60, 64. So, it's not going to be as accretive. On the inventory and all. I'll let actually Sujit answer that, to see if there's any other adjustment. As far as I know, the private level growth is definitely accruing to us. The margins.

Sujit Mahato: Yes. In addition, what I would like to highlight is since we are speaking on the sequential quarter, Tanmay, in terms of the other business income and the data fee which we had recorded in quarter four, we had also highlighted during that time that to the extent of 0.3% of the gross margin.

There is a shortfall, which means that that was a Q4 event, which we also expect during the current fiscal. To that extent it has impacted our gross margin, and some amount of inventory provisioning has also impacted, which is another 0.3%.

Tanmay Gandhi : Sure. Got it. That's all from mine. Thank you.

Moderator: Thank you. The next question is from the line of Adrit Chaturvedi from Nomura Financial Services and Securities. Please go ahead.

Adrit Chaturvedi: Hello. Good evening. Yes, I was just trying to gauge the baseline

margin performance of the

stores. So, what I'm looking at is in 1Q FY '24, the overall retail pharmacy had an operating EBITDA margin of 2.7%. And on the like for like basis, if I look at 12 month plus operating EBITDA margin that is reported in this quarter, that's 3.8%.

So that's roughly like 110 bps of increase on these stores, the baseline stores, excluding the stores that were not added particularly. So, I was just trying to understand if you could qualitatively let me know how much of this 110 bps is just your efficiency in terms of margin profitability, and how much is it just ramp up of a lot of stores that were on the platform at 12 months. Or if you could just give me a number at how much is the operating EBITDA margin for 24 month plus stores as of now?

Sujit Mahato: Yes, sure. So, in terms of year-over-year, the way we have to look at is due to the change in the private label pharma mix, which has increased from 7.98% to 9.5%, there was an additional margin. Net of the cannibalization of our old private label, we were able to increase the gross margin by 60 basis points.

Additionally, we had an impact, what I just mentioned a few moments ago, we had an unusual inventory provision around 0.3% which impacted our gross margin. In addition to that, on our private label non-pharma products, due to the mix change from 5.7 up to 7%, we had a positive

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gross margin impact of another 25 basis points. So, with this, you will be able to broadly reconcile the year-on-year gross margin.

Adrit Chaturvedi: Okay, so what is your sense, just to follow up quickly, that out of this 110 bps increase that we are seeing on the baseline stores, how much could be attributed to just organic margin improvement and how much could be attributed to the ramp up of the new stores that were there on the platform last year? So, if you could, like out of the 110 bps, is there some sort of attribution that could be done?

Sujit Mahato: I think we'll take that up offline, please. I don't have that handy.

Adrit Chaturvedi: Okay. And just finally for me, could you share the private label at MRP growth for the new and the old?

Madhukar Reddy Gangadi: Yes. So as of last quarter, the private label stands at 15%. This is on GMV. I think it moved from 13.5% to 15%. So, the growth over the quarter was 1.44.

Adrit Chaturvedi: And like a YoY growth? Could there be a YoY growth in the private label, new and old?

Sujit Mahato: Last year, it stood at 7.9% overall and now it stands at 15.

Adrit Chaturvedi: Okay. And how much of this could be the new one, the new private label? And how much is old? Are you only talking about the new private label because there's obviously cannibalization?

Sujit Mahato: Yes. Out of this, I think, Yes. The old private label is at around 1.8% and the balance is new private label.

Adrit Chaturvedi: Okay, thanks a lot. Thank you. That'll be it.

Moderator: Thank you. The next question is from the line of Madhav from Fidelity Investments. Please go ahead.

Madhav: Hi, good evening. Thank you so much for your time. I wanted to understand that given that there's a big product mix, category mix change which is playing out in the company, is it more accurate to look at the company on a per store? Like should we look at EBITDA per store or gross profit per store?

Would that be a better reflection of the profitability? Because if you look at that special marketing spend which we're doing of INR9.5 crores in quarter one, and if you assume that that's not going to be a recurring expense, EBITDA up a sort of, in my understanding, probably improves quite well on a year-over-year basis. So firstly, am I thinking in the right direction? If you could share your thoughts.

Sujit Mahato: Sure, Madhav. See, as we had said earlier, we were running a pilot in Tamil Nadu, and that on that ground we ended up spending

ending INR9.5 crores. While it is, you know, we didn't get what we expected, I don't think we're going to go and spend any more money or at least to this level in the next in the coming quarters. So, this is not a recurring expenditure.

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So, if you take that out, definitely from a two -- I think from a 3, it goes up to 3.6 for MedPlus pharmacy and EBITDA. And as your question about, you know, should we do it store by store? That, Yes, we can take it offline. I don't think -- we don't have the data right now, but, Yes, that would be one way of looking at it.

Madhav: Got it. And then just a second question was, I think you all said that the GMV growth was about 24% versus 15%, 16% on a sales basis. So that 24% basically reflects volume growth at the network level, right? Is that how we should read into it?

Sujit Mahato: Absolutely. Absolutely. Yes. It is just the Delta between 83, which is the selling price for us at INR100 MRP for a branded generic, versus 42 to 43, which is for the MedPlus brand. So, every time you do that, the percentage of these drugs which are sold at 42 INR100 MRP are going to be reflected in a slightly higher GMV growth.

Madhav: Got it. And just the last question was that in some of the stores, like in Tamil Nadu or West Bengal, where the net plus private label mix, you know, has probably, like, you know, is higher versus some other parts of the network. If, you know, maybe, if not now, maybe in the quarter two call, if you could give us some more color in terms of how the profitability is in those stores, given that, you know, you would have like, probably good product mix change that played out. So just let me know directionally how the network would look two, three years out for the company as a private label.

Madhukar Reddy Gangadi: We will try and, you know, do it as we go forward. But right now, while the growth has been very encouraging and it is something which we are banking on, I can't give you the exact numbers, like model but certainly we can work out at least.

Madhav: Thank you.

Moderator: Thank you. The next question is from the line of Vinod Chandra, who is an Individual Investor. Please go ahead.

Vinod Chandra: Thank you for the opportunity, sir. I have two questions. One is generally we receive a notice about our store suspension, and there are a lot of notices about come in a month. So, is there a specific reason behind those store suspension or can we not control well in advance before receiving these notices? So that is the first question. Should I ask the second one as well? Okay, Yes. The second question is about like, I know, like you are reporting and showing the store level operating EBITDA, right. Or ROCE, we say around 46%. So, is it like something, if you can give us the ballpark figure about like what would be the store level ROE figure rather than operating ROCE? So, these are the two questions I have. Thank you.

Madhukar Reddy Gangadi: Okay, great. And answer to your first question, it's see the suspensions, unfortunately, some of them are at least a part of the business, given a large network of 4,444 stores. And as you know, healthcare, especially pharma retail is a highly regulated business. And there's always some small issue or the other for which you receive a small suspension.

For us, it is not material and it never came up earlier because it was not, any nonmaterial stuff was not supposed to be. We were not asked to report any nonmaterial, you know, kind of events.

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Now, of course, without changing the rules, we're doing everything out there. But again, as an answer, these are largely documentary issues, documentation kind of issues. So, this is something which we try to actually keep pace and try to reduce them. And other thing is, in a bunch of the susp

ensions which we received, which were for slightly higher number of days, we have actually gone to the court and gotten a stay. So, I don't think that's going to really impact us too much. We will fight that and get it out. But Yes, it is, I would say, unfortunately, something which we have to live with, not material, and we are trying our best to basically keep it under control as much as possible. We will definitely evaluate the store level ROE figure. We'll try and come back to you. Right now, we have -- we are reporting what we see RPI supported right now, but we will consider this, then come back.

Vinod Chandra: Okay. Thank you.

Moderator: The next question is from the line of Akansha Gupta from Solidarity Investment Managers. Please go ahead.

Akansha Gupta: Hello. Yes, so my question is around the gross margins. Could you please give us the breakup of the gross margin segment wise? That is the private label pharma, the private label non-pharma and the branded pharma and brand FMCG businesses.

Sujit Mahato: Yes. As a reference, for branded pharma, we will guide between 13% and 13.1%, private label Pharma, which was our old private label in the range of 74% to 76%. The newly launched MedPlus store level brand in the range of 64% to 66% gross margin. Branded non-pharma products in the range of 10.5% to 11%, and private label non-pharma product in the range of 16% to 18%.

Akansha Gupta: Okay. Okay, thanks. That's all from my side.

Moderator: Thank you. The next question is from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed: Hi, good evening. Thanks for the opportunity. In terms of store additions, this quarter seems to be on the lower side. While we had guided for around 500 stores, 600 stores for FY '25, are we still maintaining that number?

Madhukar Reddy Gangadi: Yes, Harith, so there's no change in guidance. If you have seen the last couple of years also, we usually do slightly better in the second, third and fourth quarters usually. This year, Q1 was unduly, I would say, kind of depressed because there were a couple of reasons. One of the main reasons was elections, I would say.

We actually had a lot of attrition and we had a lot of people -- and we were finding it a little difficult to hire the right kind of people during that month. And so almost everything, due to various things, actually, it got a little delayed. And I had already mentioned that our overall focus MedPlus Health Services Limited

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on MedPlus brand has increased a lot. And while that is not the main reason, that's where the company has been focused. For us, the guidance remains the same, one.

We are looking at our complete network. And in addition, so this year, while the net will remain more or less the same, we could have a few more closures than usual, because we have added quite a few stores in the last three years, could be anywhere from 30 to 50 more than usual, if we are looking at the entire network out there, and so that may change the number a little bit, but I don't think the addition itself will be affected too much.

Harith Ahamed: Okay. And this 15% GMV share of the pharma private label, how should we think about this number progressing over the next few quarters? And probably where do you see this reaching by FY '26?

Madhukar Reddy Gangadi: I think we'll probably be able to see the trajectory a little bit more clearly after this quarter. You know, when we started initially, obviously there were a lot of early adopters who came in immediately, and after that, it has been moving steadily at around 1.5% to 2% per quarter. If you look at it, I think last quarter was at around 13.56% and today it is 15%. That's 1.44% growth. We expect as we go forward, it will maintain at this level and also go up slightly, but I will come back to you guys with the guidance on growth quarter on quarter maybe in the next

quarter or

so. Probably have a more clearer idea.

Harith Ahamed: Thanks, Madhukar. One question for Sujit, in the slide where you've shown the IndAS adjustments in the interest income line for the quarter, the IndAS, adjusted interest income is

lower by around INR2 crores versus the reported interest income of around INR4.3 crores. So exactly is this adjustment, if you could give some color.

Madhukar Reddy Gangadi: I would say these adjustments are for the security deposits, what we have placed for the property.

But I will just look into this and review the details offline.

Harith Ahamed: Right. Thank you very much. That's all my side.

Moderator: Thank you. The next question is from the line of Aejas Lakhani from Unifi Capital. Please go ahead.

Aejas Lakhani: Yes, hi. So firstly, could you call out what was the entire ad and marketing expense for the quarter versus same thing last year and last quarter?

Madhukar Reddy Gangadi: Sure. As this year, we ended up spending around INR9.5 crores in this quarter versus Q1 of last year was INR1.3 crores.

Aejas Lakhani: Okay. And so, what was the same number for the previous quarter?

Madhukar Reddy Gangadi: Previous quarter was 8.5.

Aejas Lakhani: Okay. So, you know, I just want to understand that, you know, you made these spends in a timeframe when, you know, you just said on call that elections and there were some challenges and delays and attrition on account of that. So how do you measure the effectiveness of the spends?

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Madhukar Reddy Gangadi: No, no effectiveness so, I said the store openings did not happen because of that. Keep that aside.

That has nothing to do with the effectiveness aspect, right.

Aejas Lakhani: Okay. But, sir, could I understand that when you think of these, you know, these larger spends, which is more than normal course of business, how do you measure the effectiveness? And then what is the outlook for this line item for FY '25?

Madhukar Reddy Gangadi: We are not going to be doing any more of this in the next two quarters for sure. We actually spent this money in two different states. One was in West Bengal and the other was Tamil Nadu.

Largely the money was spent in Tamil Nadu. And for us, we are internally looking at a couple of things.

One, the number of new customers we have acquired and the growth of private label, MedPlus plan private label. So, those two would be the three key criteria for us to actually decide if the ad is worth spending on or not. But obviously, you can only get the, I would say you can only assign so much value to the immediate growth, the benefit to the brand and the benefit to the MedPlus brand. It is, you know that is going to come over a period of several quarters, much more difficult to actually much more difficult to measure.

Aejas Lakhani: Got it. And sir, the private label pharma, which is 9.1, could you just tell me that what is the split of 9.1 in old and new?

Madhukar Reddy Gangadi: Sure. So, the 9.1, which you're talking about is the net sale return number. I think the old one is around 1.8 and the balance goes to a new one.

Aejas Lakhani: Okay. And sir, when do you expect this runoff of the 1.8? I mean, do we expect it during the course of FY '25 or do you expect a sharper rundown?

Madhukar Reddy Gangadi: We will most likely see a slightly more sharper rundown as we go forward. Maybe in the next two quarters it should actually go down, but it will probably be always there to some extent. But from now, I think we'll probably end up going to slightly under one in the next two quarters.

Aejas Lakhani: Got it. And sir, I just want to understand more at the strategy level that why this decision to run down that old brand and replace it or replace it with MedPlus, because as you would I mean, there has been an impact on margins on account of this entire movement. And why not just

introduce the MedPlus brands, and let that automatically or naturally run down or progress with time? Why have we done this intervention which has been sort of forced to run that down and increase the MedPlus? Could you just help me triangulate that at a strategy level?

Madhukar Reddy Gangadi: No, no. It is not a question of running it down intentionally. We always had two different brands, but when customers walked in and they saw the entire set, the customers who are buying the old private label were people who are willing to take a substitute. The customers who are willing to take the new one are also the same set. Now, given that there is an alternative where they can get a discount for a small membership fee, most of them have chosen to go here.

Aejas Lakhani: Got it. Okay. Got it. So, sir, from here on, we are expecting, your growth guidance is 20% for, on a reported basis for FY '25, correct?

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Madhukar Reddy Gangadi: That's right.

Aejas Lakhani: And sir, operating margins. What is the outlook there?

Madhukar Reddy Gangadi: So, we'll continue to grow from here. Q1 typically is our slowest quarter normally. So, it should definitely be better than what it was last year. So, Yes, that's all I can tell you.

Aejas Lakhani: Okay, thank you and all the best.

Moderator: Thank you. The next question is from the line of Kunal Randeria from Axis Capital. Please go ahead.

Kunal Randeria: Hi, just one question, continuing on the previous participant's question. Madhukar, do you expect a sharper impact from canalization of your private label brand the next one or two quarters? And after that, that impact should completely go?

Madhukar Reddy Gangadi: I think so. Kunal. It will be there for a while because we have inventory. There are customers who are willing to buy. As you remember, as we told in the past, our previous set of private label also had the full set of chronic medicines too. So, some of them where, you know people who are on drugs like atenolol, telmisartan and all, they'll probably continue.

We have the stock. We'll probably switch them over to this at some point of time or they may themselves basically ask for this. So that part, that tailed stuff will continue, but most of the acute stuff will probably end in the next two quarters or so. But for us, whatever the impact may be of cannibalizing, this slightly more profitable portion of our business will most likely start coming down in the next two quarters.

As we continue to grow this. This is also accretive to us. This also gives us more margin, but as long as it comes at the cost of branded generics, it is good for us. If it comes at the cost of our old private label, not obviously. So that impact, I think will go down in the next two quarters.

Kunal Randeria: Sure. And just with the older private labels, have you changed the discounting practice or is it still, you know, trying to get in line with MedPlus discounting or same old method?

Madhukar Reddy Gangadi: So, if you're talking about my old private label, in the state of Telangana the old private label has always been sold at the same price as MedPlus as a new one. But in the rest of the states, we'd actually continue to sell it at the old rate out there. So, there was some confusion. We'll probably end up, you know, taking out some of that and, you know, redoing it in the MedPlus side as we go forward. Was that the question, Kunal?

Kunal Randeria: Yes, that was the question. Basically, I just wanted to understand whether on the old label also, are you giving a 50% to 80% kind of discount as you are giving it on made by MedPlus?

Madhukar Reddy Gangadi: Yes. In Telangana, yes, we are. We started on that note. We started off with that value proposition earlier to the customers. But in the rest of this country, while we started with maintaining the old private label at the old

discount. There are some products, maybe out of the

600 odd products, probably around 200 odd, which we will end up actually moving to the new pricing, the MedPlus pricing. This is just to expedite the overall sale of the inventory and to MedPlus Health Services Limited

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make sure that nothing goes into expiry. For us, obviously, we have both the inventories. We want to make sure that everything sells and we are left with nothing end of the day. And so part of that will be moving into a new price.

Kunal Randeria: Got it. Thanks.

Moderator: Thank you. The next question is from the line of Vilina Jain from Perpetuity Ventures. Please go ahead.

Vilina Jain: Hi. In your opening commentary, you mentioned INR95 million of special marketing spend. Could you elaborate on that? Also, secondly, you have been talking about Q1 being seasonally weak. If you could highlight some of the reasons for the same.

Madhukar Reddy Gangadi: Sure. The INR95 million got spent in Tamil Nadu and West Bengal, largely in Tamil Nadu. I think around INR7 million to INR7.5 million probably got spent there and INR2 million to INR2.5 million got spent in West Bengal. This advertisement was spent on, mostly on newspapers, some bit on TV, very little, some on radio and little bit on the social media, YouTube and everything else, and a little bit on podcasting and influencers and all those other things. That's one.

And I think, Yes, on the seasonality side, see typically April and May, the sales actually fall for two reasons. People end up actually leaving large cities in which we are largely placed. They move out, they go out for vacations, they go out of town and everything else and all. Even the senior citizens do end up actually, or, you know, people who are in their 50s also end up going back to their home towns and all once a vacation start for kids, that's one.

And second, in general, the overall burden of, I would say, disease on the infection side is much, much less. And so that whole set of antibiotics and painkillers and fever tablets and everything else, which are sold normally throughout the year and especially so in June, July and August are almost nil in April and May. And that's why Q1 is usually weak.

Vilina Jain: Okay, so regarding the marketing spend, is this more to do with the private label increase that is happening and going forward, how should we see the marketing spend increasing?

Madhukar Reddy Gangadi: So, we are probably bringing it down to a maintenance level of around something like INR10 million a quarter, which is INR1 crores, INR1.2 crores. But Yes, all the money has been spent on private label only. So, the whole idea is trying to get educate customers as many as them as possible, to kind of make them aware that all medicines sold in India, 90% of medicine sold in India are just generics, we're just buying them under different brand names, MedPlus now makes the same medicines.

And because MedPlus doesn't have any marketing spend or any, let us say, the channel spend, we sell it through our own stores, we are able to offer this big discount, and that's the value proposition to the customer, and that's what we have been pitching, and that's something which will continue all the way to a much, much lower level.

Vilina Jain: Understood. Thanks a lot.

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Moderator: Thank you. The next question is from the line of Vinayak Mohita from Axis Bank. Please go ahead.

Vinayak Mohita: Hello? Yes, hi. Good evening. I just have this one question on the store numbers for the greater than 12 months. So, accounting for the INR9.5 crores of advertisement expense, one time advertisement of expenses, if we removed that, then is it fair to assume that the operating margin for the greater than 12year, 12months stores would be more towards the 4.5% to 5% range like from th

e 3.8% that you have reported in the current quarter?

Madhukar Reddy Gangadi: Actually, it will go up by 0.6%.

Vinayak Mohita: Understood. 0.6% or 4.4%, perfect. And sir is it fair to assume that -- so when we look at the metrics on a standalone basis from what the quarter has been reported, they look to be deteriorating. But it would be fair to assume that x of those one time expenses and considering how the quarters would pan out going forward now on a YoY basis, we should continue to see an improving trajectory on the margins front for the stores, on a store level, that would be a right assumption to have?

Madhukar Reddy Gangadi: Definitely.

Vinayak Mohita: Understood, and so just wanted to understand from a longer term perspective, while your aspiration would be to be at 100% own brands, maybe, maybe not. But just wanted to understand over the next three to five years, where do you see this own brands for yourself settling down in the entire mix of your business? What's your aspiration out there for us to be able to visualize? How could the mess as a whole look like maybe three to five years forward from respect to private brands specifically?

Madhukar Reddy Gangadi: See, it is not yet one year since we have launched the brand across the country. We are already at around 15% overall. I feel very confident that this number will go up significantly, but to give you the actual trajectory, I think we'll have to wait a couple of quarters. So, it is possible that the early adopters have all come in and basically bought while we are seeing a gradual increase every quarter.

I just want to wait for a couple of quarters before I can predict that. But just to give you an answer, while it looks like store brands and all are typically capped at around 20%, 30%, 40% and all, the one number we should actually bear in mind is that even in a country like U.S. and places like Walgreens and CVS and all, 90% of what the store actually fills prescriptions are generic, and these are all, you know, the law, of course, basically allows them to actually substitute.

And the store then basically decides whether it wants to do its own private label or it has a preferred player, which is also a generic. There's no brand, right. So, the aspiration would be to go to that level over a period of time, but that will mean, you know, a lot of education of customers and maybe at some point the government comes in and basically changes the rule a little bit for us to do it.

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But, Yes, even if it were not to go to that number, I see no reason why you cannot settle somewhere in the 30s or 30% plus kind of number. But again, I will reserve that overall, this thing for at least another couple of quarters before I actually can make prediction out there on that.

And the reason why we are super excited about the whole thing is we are making slightly lesser

margin than our old private label, but please bear in mind that people are now buying the MedPlus brand. They are seeing it. It's not a substitute, it's not something else which we are just giving them. It is the MedPlus brand.

And now, as they get used to it and as they start buying it, and all, we have in our hands an ability to change the MRP every year and the government allows us to do that. You are allowed to do it by 5% or 10% odd. And we also have a chance to basically move the discount around a little bit to make it closer to 50, today it's 50 to 80, we can move it closer to 50, or we can even shift it. So, for those two reasons, we feel that the entire thing is going to be based around this whole strategy of private label.

Vinayak Mohta: Understood. And is there any reference in the global context wherein pharmacy has been able to replicate what you are trying to do? Because please do correct me if I'm wrong. From what I had read a couple of on a couple of pharmacies

ies, none of these pharmacies were able to cross the 20%, 22% mark on the private label side. So, do you have any reference on the similar company that I might have missed out on? Maybe my data is not correct. So, if you could just help me out there.

Madhukar Reddy Gangadi: Okay, so two things here. So one is, you know, is the store selling it on its own name? Right, then, Yes, maybe 20%, 30% probably is the number. Walgreens and CVS and all do that much, and I'm pretty sure other pharmacies across the world also have at least that much. But the number to actually bear in mind is that while these people may not sell on their name, when the patient walks into the store and hands in a prescription, all the pharmacist is asking him is, okay, your insurance says it has to be, it can be generic or your doctor has written, dispensers is written. If the doctor says dispensers written, then it is exactly given, the brand is given, but then the copay is very high and all that sort of stuff. But in 90% of the prescriptions, the pharmacist is filling a generic only. And the generic is not, let's say X company's generic or Y company generic or Z company generic. It is the generic which is chosen by Walgreens. And as far as the customer is concerned, it is the store generic only. It could be made by whoever it is, right? It is a store generic. So, yes, you know, you could actually go all the way up to 70%, 80% also in such cases.

Vinayak Mohta: Got it. And one last question. Have we in the recent one year stage, any kind of regulatory problems or something on the MedPlus brand products, like any complaints or any action on that front on the products part?

Madhukar Reddy Gangadi: No, no. Obviously what we are doing is, you know, doesn't go very well with the thousands and hundreds of thousands of modern part retailers, so they've all got a complaint. You know, drug authorities have come, they picked up the samples, they've tested them, everything is fine.

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So, for us, obviously, you know, we are disrupting a big market. It is going to have some people who are not going to be super happy about it. So, there have been people who have been calling on us and, you know, checking out all the things and all, but, Yes, that's about it. Because whatever you're doing is 100% worship.

Vinayak Mohta: Perfect, perfect. Great. Thank you so much and all the best.

Moderator: Thank you. Ladies and gentlemen, we will take that as the last question. I would now like to hand the conference over to Mr. Madhukar for closing comments.

Madhukar Reddy Gangadi: Thank you. I thank all participants on this call for your interest in the MedPlus journey. Our investor relations team can be contacted at ir@medplusindia.com. Thank you.

Moderator: On behalf of MedPlus Health Services Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

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Dear Sir/ Madam,

Sub: Submission of Transcripts of Earnings Call held on November 13, 2024 :

In continuation of our earlier letter dated October 25, 2024 and November 12, 2024 regarding the investor call on November 13, 2024 and pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the earnings conference call held with analyst on November 13, 2024 for the quarter and half year ended September 30, 2024.

Kindly take the same on record. The same is also available on Company's website at www.medplusindia.com

Thanking You
Yours faithfully
For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer
FCS 7460

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"MedPlus Health Services Limited
Q2 FY25 Earnings Conference Call"

November 13, 2024

Management : Mr. Gangadi Madhukar Reddy - Chief Executive Officer
and Managing Director
Mr. Sujit Kumar Mahato - Chief Financial Officer ,
Mr. Chetan Dikshit - Chief Strategy Officer ,
Mr. DRN Srinivas - Sr. Manager Finance

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Moderator : Ladies and gentlemen , good day and welcome to the MedPlus Health Services Limited Earnings Conference Call.

As a reminder, all participant lines will be in the listen only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" and then "0" on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Sreenivas. Thank you and over to you, sir.

Sreenivas: Thank you. Good evening , everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to MedPlus Q2 FY25's Earnings Call to discuss the Financial Results of MedPlus for the 2nd Quarter of FY25, which was announced on 12th November 2024.

We have with us today the senior management team represented by Mr. Madhukar Reddy – CEO & MD; Mr. Sujit Mahato – CFO, and Mr. Chetan Dikshit – CSO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on slide one of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier and these have also been posted on our corporate website.

I will now hand over the call to Sujit. Thank you, and over to you, Sujit.

Sujit Mahato: Thank you, Srinivas. And good evening, everyone, on this call.

We are pleased to share that as of September 30th, we have been serving communities in over 680 cities across 12 states through our network of 4,552 pharmacy stores. Also, the company operates four full service diagnostic centers, eight level two centers, and over 100 collection centers offering essential diagnostic services at affordable rates. In terms of the update on our network, over the past 12 months, we have added a net total of 463 stores, gross 561 store additions with 132 stores open during quarter two and 198 stores open during the first half of the current fiscal. In the 2nd Quarter , 59% of our store opening were in tier 2 cities and beyond. At present, out of our 4,552 stores, 2,102 stores are situated in tier 2 cities and beyond. We continue to acknowledge the growth potential inherent in these markets. Throughout Q2, there were 24 store closures. Taking into account both opening and net of closures, we achieved a net addition of 108 stores during the quarter compared to 37 stores added in quarter one. In terms of our store network age, around 30% of our stores are operational for less than two years and the remaining 70% of our stores have been operational for two years and more. It's noteworthy that all stores in the less than 2-years age bracket are still in the ramp up phase. As these stores mature, we anticipate a positive contribution towards profitability. As a guardrail, we closely monitor the timeframe for our new stores to reach breakeven. For stores open between October
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23 and March 24, approximately 59% of them achieve breakeven within six months of operation. As so far , all stores combined reached breakeven in just five months.

In terms of the store size, as at the end of the quarter, our network has grown to 4,552 stores with 2.4 million plus square feet compared to 4,089 stores and 2.1 million square feet at the end of September 23. The average store size was 529 square feet. To give you a sense of spread in store sizes, we have 3,357 stores less than 600 square feet and 1,195 stores that are greater than 600 square feet.

In terms of revenue mix, we are strategically positioned to increase our revenue share from private label products

. Our private label range is crafted to provide customers with high quality products at competitive prices. Presently, MedPlus offers over 1,100 carefully selected SKUs spanning across pharmaceutical and non-pharmaceutical categories. Private label sales for Q2 constitute 18.4% of our total revenue. Moreover, our growing presence in tier 2 cities and beyond is significantly impacting on our revenue mix. Sales from these markets comprise 36% of our pharmacy revenues in the current quarter, marking an increase from 34% in the same period last year. The following is the impact of the launch of MedPlus branded products across our network. In quarter one, FY24, prior to the launch, the share of private label pharma stood at 7.9% of total GMV compared to 16.3% during the current quarter. The increase in the private label GMV share indicates a positive reception from customers and validates our commitment to delivering high quality products under the MedPlus brand umbrella.

Now on our quarter performance in terms of the financial numbers, our consolidated revenue was Rs. 15,762 million with growth of 11.9% year-on-year and 5.9% quarter-on-quarter. Our consolidated operating EBITDA stood at Rs. 739 million representing 4.7% pre IndAS. Around 99% of our revenue is from our pharmacy operations. Revenue from pharmacy operations grew by 18.4% year-on-year on GMV basis and by 11.9% year-on-year on net basis. The pharmacy operating EBITDA stood at Rs. 717 million representing 4.6%.

Our stores performance, I would like to update on our stores older than 12 months. Revenue from these stores in Q2 was Rs. 14,438 million, 94% of pharmacy revenue. These stores had a store level EBITDA margin of 10.2%. The store level operating ROC of these stores stood at 58.1%.

A word here on the store level EBITDA margin by age. While stores greater than 12 months had a margin of 10.2, this was 10.6 for stores greater than 24 months and 7.7 for stores in the 13 to 24 month age bracket. If we allocated non-store related costs, then the operating EBITDA of stores greater than 12 months would be 752 million which translates to a margin of 5.2%. On our diagnostic segment, diagnostic revenue grew to Rs. 283.1 million in quarter two compared to 181.5 million in quarter two of this last fiscal. Diagnostic segment recorded an operating EBITDA of 21 million, 7.4%, compared to a loss of 29 million in Q2 last year. However, central level operating EBITDAs stood at 62 million. We did a restructuring of our legacy diagnostics MedPlus Health Services Limited
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operations in Nagpur, Bengaluru, and Vijayawada. This has resulted in a one-time expense of Rs. 19.7 million, of which Rs. 11.3 million is non-cash in nature. The net operational savings from closure of these centers is around Rs. 7.5 million per quarter. If we exclude the one-time expenses from Q2, then the revised operating EBITDA would be 11.1%, or Rs. 31.5 million instead of the 7.4% mentioned earlier.

An update on our working capital, our net working capital for Q2 was 61 days. The inventory in our warehouse was 38 days. In Q2, the inventory level of our first year stores was 95 days. In comparison, for our stores older than 12 months, the inventory was 40 days.

Now I request Chetan to update on our diagnostic business. Over to you, Chetan.

Chetan Dikshit: Thank you, Sujit, and good afternoon, everyone. Q2 is a seasonally positive quarter for diagnostics. In July, we sold 459 gross plans per day. In August and September, this was 466 and 461, respectively. As on 30th of June, we had 141,000 active plan and 270,000 underlying lives. As on 30th of September, we had 148,000 active plans and 299,000 underlying lives. Our current observed on-time renewal rate was 25% in Q2 versus 24% in Q1.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you. We will now begin the question-and-answer session. We have the first question from line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: A couple of questions from my end. No as we scale our private label and expand stores what kind of working capital cycle can we envisage because if I see last few years inventory days have been around 83 days on an average and payable days 18 days, is there a scope of improvement on this. If yes then how will that happen, that's my first question. Private level contribution has been increasing. So, if you could give us some more sense is it scaling across pharma, wellness, FMCG all parts of the business, where do you see the margin improvement trajectory if this contribution increases? And lastly of the 600 stores which we are planning to add, is it all leased? Is there franchisee of the current 4,500 stores? What is on lease or

franchisee? And incrementally, how do we look at free cash flow generation this year and say next year considering our expansion plans? So, these were my three questions. Thank you.

G. Madhukar Reddy: I'll take the first two questions. I think the first one was on the overall inventory and payable and working capital. So, inventory levels are also a function of the number of new stores. If I open, let's say, 200 stores in a quarter, each store is going to have at least 18 plus 18, that is 36 stores.

And given that the sales in the first quarter are going to be less, it's going to look like a lot of inventory days for the amount of sales out there on that day. We haven't opened as many stores in the last two quarters, so working capital would have gone down slightly. As we go forward, obviously, that is not the only thing only lever on the working capital, we also have been optimizing inventory in warehouses, as well as in the stores. That is something which we will
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continue to improve. But that said, one should remember that MedPlus, as with any pharmacy store, the success of such a store depends purely on the fill rate or your reputation of getting the right fill rate. So, you can't cut it too fine and not have enough stock and turn away customers.

The cost of lost sales is far higher than the cost of inventory. So, we are always mindful of that. So, we try to optimize it. We don't, we try to make sure that the fill rate is right. So, that's all.

The impact of private label on the other hand, is going to be really good, but it'll take a while for it to happen. The reason is that the private label basically costs us on a product or on a typical product which costs Rs. 100 in MRP, PL cost is roughly around Rs. 12 versus branded pharma costs around Rs. 68. So, if you were to replace the branded pharma sales by private label pharma sales over a period of time, you would definitely see a huge decrease in the working capital. But we are not at the stage as of now. We started only last year in June. And in fact, across the country, we only launched it in October or November. So, right now, whatever private label we have, is in addition to the branded pharma. So, we really don't know which brand, see while we know what category we are selling, we are selling around 700 odd products and for the top 100 products we have a significant, in fact for almost everything which we have, we probably are the largest sellers in that category. But we don't know, we are probably taking up sales of all the 10 brands which are out there. So, we can't really start stocking any of those. It will be a while before we understand and we get to a new model in which we can decrease the inventory. So, the impact of private level inventory while it is going to be there, it's going to take a while for it to actually clear out.

Prakash Kapadia: Private level contribution will increase over a per

iod of time, so what could be the margin trajectory for us with increased private level?

G. Madhukar Reddy: So, private level is two components one is the pharmacy side, I think of which the 17% which you see out there that is a net sales. Pharmacy component would be around 10 or 10.5 and the rest would be non-pharma part which is 10.5 and 7.9, that's the split for private label right now. 10.5 is private label pharmacy, and 7.5 is the FMCG products out there. Both of them are going up. We expect that on a net sale basis, this will increase by maybe around 0.5% to around 0.16:27 every quarter. And additional margin which you get on that is 50%. So, if you increase it by 0.7, you probably increase your margin by 0.3 or 0.35. The margin of private label is 50%. Now, but we also look at these numbers on a MRP basis. The reason is that a product which we would otherwise sell at 83. On a Rs. 100- MRP, where we give a discount of 17 and sell it at 83. Now the same product is getting sold at 45 on a discount of 50% or 55% out there. So, obviously the net sales is going to be lower when you do this, so the percentage grows slower. But to get a apple-to-apple kind of comparison, we usually do a MRP thing. And right now, I think we have MRP, which counts the volume, is at around 16%. We ended the quarter with a 16% sale, and we expect that we will grow this by 1% to 1.25% every quarter. So, that's on the pharmacy side. And obviously, every quarter, we are going to see a small increase in margin because of this. The second thing is, on the general goods, while we are increasing the overall sales, the margin on those is actually the same as the rest of the company. It is not hugely margin accretive. The
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way we see it is, it adds to our customer base. There are a lot of products which we sell, it's obviously it's our own brand and our strategy in that area is to make sure that we sell those

products at a price which is unavailable in the market. For instance, and the reason we do that is because we feel that a customer has got several different options to go and buy a soap or shampoo or toothpaste from. And the reason they would actually pick it up from our store is because, A, it is convenient, they come to us and B, if they pick it up, we want them to come back for the product. We don't want it to be a one-time purchase. And we want them to come back because the product is great and it is available at a price at which no one else sells. So, our private labels actually sell at a price which is lesser than any other private label in the market. So, much lesser than the brands. And that's one reason why it has been growing. But there it brings us customers, loyal customers. It adds to our top line, but the margin is still 20 % or 21%. So, it really doesn't add too much to the margin.

Prakash Kapadia: And lastly, the free cash flow and the operating metrics in terms of franchisee or lease stores? How do we look forward and what kind of free cash flow can we expect this year and in the medium term?

G. Madhukar Reddy: So, all our stores are leased. A very small portion of our stores are franchised. We don't know how that will play out. We will see as we go forward. But in the past, we have basically franchised our stores only in locations where it was difficult for us to reach and service that place. So, we would actually do it in smaller locations. But we are now trying it out in some of the larger cities, too. But it is a very small portion of our overall stores. Every one of our stores is leased. And on the free cash flow, I'll actually let Sujit answer that question on how the free cash flows will play out.

Sujit Mahato: I think based on our network we have presented, you would have observed 1,425 million for the quarter. And this has the components of two things, one, less number

of new stores opened, so that has helped us accrue cash. But otherwise, at least a significant portion of this will continue to accrue as we move forward.

Prakash Kapadia: You alluded to cash flow being better because of slightly lower stores which were added during the quarter. If I were to take the broader annual guidance of say around 600 stores, will this free cash flow accretion happen despite the addition is what we are saying?

G. Madhukar Reddy: I think with lower editions we were able to do this. If we continue with the planned additions, this will be a bit lower.

Moderator: Thank you. We have the next question on the line of Yash from Stallion Asset. Please go ahead.

Yash: My first question is that you've seen almost 200 basis point gross margin expansion right, 23.7 So, this number sustainable going forward?

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Sujit Mahato : See this is on the back of the growth in private label. I don't see any reason why it should not continue, the gross margin.

Yash: And how many stores in terms of percentage are able to do the less than two-hour delivery?

G. Madhukar Reddy: Right now, it is across 3 or 4 cities, Hyderabad, Bangalore, Chennai, and Calcutta. But we have been facing a little bit of a problem in getting people as delivery boys and all. So, we have actually decreased it to a little, to a small extent in some of the cities. So, I think what we typically do is have roughly around 30 to 40 hubs in each city, around 10% of our stores. And these are designated as delivery hubs and from there we do the two-hour delivery. I think I will have to get back to you, but we may have decreased a few hubs in the recent past.

Yash: And since you've opened slightly less number of stores, so any sort of guidance on could be the store opening run rate going forward, are you satisfied with the number of stores that you've opened this quarter or will this sort of be maintaining this sort of run rate?

G. Madhukar Reddy: So, we opened around 145 stores, we shut down around 30 -35. So, that would mean around 108 stores which we opened net this quarter. And I think the overall thing for this year is probably around 145 right now. We had actually guided to a 600 store number, but we could not achieve our usual number in the first quarter because of elections and a couple of other things. So, that will be lost. We will most likely in this year do a store count of anywhere between 400 to 450

stores of net addition. That's the plan for us.

Yash: And last question, the tax rates have been very volatile. So, is there some sort of a sustainable tax rate number that you can sort of let us know?

G. Madhukar Reddy: I think the tax rate what we are talking into the quarter will sustain.

Yash: Because I think 19% is something that's sustainable, going forward ?

G. Madhukar Reddy: That's a factor of the extra deduction what the company claims on section 80JJAA, which is directly linked to the additional number of employees which we hire for our store opening. And that's been a factor of our new store opening s as well .

Moderator: Thank you. We have the next question from the line of Jatin Chawla from RTL Investments. Please go ahead.

Jatin Chawla: My first question is, you spoke about the fact that on a GM P basis, your private label share is 16% and you see scope for a 1% increase every quarter. Till what level do you see this scope? How do you see this evolving, let's say, 18 to 24 months from now? Do you see it going to like over let's say 8 quarters 24 %-25%.

G. Madhukar Reddy: I see no reason why it should not. 1% is a fairly conservative kind of estimate, I would think. Today we have substitutes for roughly around 75% of the products which we sell as medicines.
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We will try to increase this a l

ittle bit. But more importantly, we are finding that some of the smaller towns in all filed a little late. But with all of them coming on board, and in some of the newer states obviously where MedPlus is not as well known , we saw that the uptake was not as high, while the rest of the countr y, there are places where we have as high as 20% private label contribution on MRP basis. We saw some of the newer states at around 8% to 10% and all. We are putting in efforts to get the w hole message out about the brand and everything else in those places. We see as they also continue to pick up over a period of time, the overall number will definitely go up. And 8 to 10 quarters, 1% each, I think obviously difficult to say that far out. But I see no reason to believe that it will not be done.

Jatin Chawla: And it's been a year now, so what are your learnings on this? Your hits and misses and any areas of improvement on this? And second question related on the subscription price that the customer pays for getting private labels. So, any thoughts on how that could shape up?

G. Madhukar Reddy: We were always reasonably sure that the chronic patients, the guys who are buying medicines for their chronic ailments would actually buy these and that has been actually proven. But we are also, I would say pleasantly surprised why those who would otherwise buy only the acute, they also are buying in reasonably large numbers. So, across the board, sales of our products are reasonably high. For us, I think the only thing we need to do is to just make sure that the product is available in every one of our stores, and we continue to increase the range of products available. Today, I think we are probably around close to 700, but we could add another 150 products as we go forward to increase the overall percentage. On, I think the second ques tion was?

Jatin Chawla: Subscription price and how do you see that shaping up ?

G. Madhukar Reddy: We started off at 49 in Telangana and I think a couple of months back, we increased it to 99 in Tamil Nadu, Rs. 99, so we left it there. So, this quarter we've added up a couple places, but Yes, we are going to slowly experiment with this, increasing the subscription price as we go forward.

Jatin Chawla: Just one last question , on the sustainability of margins that we have seen this quarter, so when I look at your last few years of results that you have shown, 2 Q-3Q, it's not as if the revenues go down materially. They are either like flat tish or marginally up, and then 4Q is normally seasonally a very strong quarter. So, going forward, looking out over the next two quarters, do you see this 2Q kind of margin sustainable or are there some one -offs that you would like to call out?

G. Madhukar Reddy: No, I don't think there's anything out there. The gross margin is going to be equally good, if not better. I see no reason for it to come down. And the sales, as you mentioned, should be flat or

slightly up. No reason for it to be any worse than the 2nd Quarter .

Moderator: Thank you. We have the next question from the line of Umakant Sharma from SAFE Enterprise.
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Umakant Sharma: So, I have a few questions Firstly, could you just through some color that there are you know? Many of these quick commerce companies are planning to enter the pharmacy retail space. So, could you just throw some color as to how do you plan to compete with them and do you also have any plans to establish an online presence for yourself? Secondly, in terms of brand building standpoint, could you just highlight what activities are we undertaking in terms of getting that brand awareness, building that brand awareness, brand recall and all of that? Here are the first two questions.

G. Madhukar Reddy: So, I don't think we'll actually do anything on the quick commerce side as of now. We are going to wait and watch and see how that comes along. So, we have been online for a long time, since 2014. 7% of our business now I think comes from online side. We have not given any additional discount or given any additional benefits for online buyers. Hence it has been where it is. On the quick commerce side, if pharmacy is tough for e-pharmacy or for the internet side. I think it'll be even more tougher for quick commerce , but we will see. We'll wait and see how that plays out. Honestly, we don't want to really jump onto anything right now. On the brand building side, no, actually, whatever we are doing, we are doing in the stores, we are actually creating a little bit of testimonial kind of thing. We are trying to give as much information as possible to the customer. We are talking about the fact that 90% of all medicines sold, even in a country like US, are generic. And what we have in India is all generic. So, we believe that our best bet is for people to get educated on the whole aspect. We are not really doing anything on no extra spend for us, outside of that.

Umakant Sharma: But are we picking up any marketing efforts from our side to educate them about the private labels and all of that?

G. Madhukar Reddy: We are doing a little bit of the influencer stuff, some podcasts, a lot of information going out to our regular customers, very pinpointed kind of thing, but nothing ATL. I don't think we spent anything , I think we spent around a crore or so last quarter. So, I don't see us spending a little more than that, a couple of crores last quarter. So, that's going to be the extent of the effort for us.

Umakant Sharma: And sir, on our private label pharmacy side, could you just, like you said, on a GMV level, we are at 16% right now, correct? Could you just split that number between the discounted private label and the non-discounted private label?

G. Madhukar Reddy: So, everything is discounted, but some of it is to members who are, and that is the 50 %-55% discount. That is roughly around 14.6 and the other is probably around 1.5 -1.6.

Umakant Sharma: And sir, would it be, like I believe you mentioned that incrementally we should see the scaling up of the private label discounted fees going up incrementally as well , could you share some insights, let's say, from the stores which we had launched in the very initial stages? What would
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be the level of penetration within those stores, let's say in Hyderabad, Telangana side? What would be our private label penetration within those stores?

G. Madhukar Reddy: I think in Hyderabad it's probably north of 17 and Telangana it is over 20% , rest of Telangana.
Umakant Sharma: And sir just one book keeping question which is rental cost per store per month I am talking about, per store per month, and this quarter it has gone to about 43,800 levels. So, is there any one-off or is there, could you just throw some color around those rental costs?

G. Madhukar Reddy: Now there's no one off, but we will look at it and come back to you if there's anything out there.

Moderator: Thank you. We have the next question from the line of Vijay Shah from RCL Investments.

Please go ahead.

Vijay Shah: My question is the MedPlus brand which you are primarily selling only at your stores, can you expand it to other stores like primarily the stores which are not chain and they are like mom & pop stores?

G. Madhukar Reddy: No, we are not thinking about it right now. We'll give it a thought. But no, I believe people have access to different kinds of generics out there. They probably buy from Cipla, one of the good brand anyway. The MedPlus brand is sold at a 50 %-55%-60% discount rate. And most of the small independent operators sell generics at full cost. And that's how they survive. They have low throughput,

Rs. 3 or 4 lakh a month. They sell 50% of their stuff at a full margin of 60% or 70%. Now, if they were to take MedPlus product and that is openly available in MedPlus stores and we advertise it extensively as being available at a 50% to 80% discount, that won't really work well for an independent operator, so I doubt if anyone would actually pick it up.

Vijay Shah: And secondly, I mean, see quick commerce has shown a lot of success across many categories. So, what would you be looking for in quick commerce to actually go on that platform? So, what are the data points which will convince you decisively that whether you should be on that platform or not?

G. Madhukar Reddy: We'll see how the customers are responding to it because there is a need of prescription. Someone has to upload a prescription, right? And that takes a while. And that doesn't really make it possible for people to deliver in 10 minutes. So, that automatically increases the time. And then again, the range of products required in medicines are far too many. They are not like the generic 500 or 1,000 brands which are or 1,000 SKUs which are sold in quick commerce across all locations. So, for us, it is several hundred brands, and each one, each zip code has a different one and fill rate is a very critical thing for a lot of people. So, if you're buying medication for your monthly needs, then your long list of medication, every one of them has to be there, otherwise they won't really go out there, and they are not really looking for that very quickly. The only guys who are looking for that really quick 10 -minute delivery would be the guys who are probably suffering from headache or diarrhea. And those are all mostly OTC products. I am not really sure if the need for the commerce is as high as it is when you are missing a critical
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component of something you're cooking out there in your home or some urgent need. But that said, we'll look at it. We don't know. We could be surprised by how the customers take to it. So, we are monitoring it closely. We'll see what happens.

Vijay Shah: Just being a customer for my parent, one of the things which we find is that they can't go down many times. When you are sick, you don't want to go down to the medical store. Even the medicine definitely comes in 30 minutes, it is a really good proposition because most of the local stores do not deliver even in 30 minutes and you have to call them so many times. So, it actually the delivery of medicine for sick people and older people might be a very good proposition.

G. Madhukar Reddy: Absolutely I agree. We will see how the people will react.

Moderator: Thank you. We have the next question from the line of Raghavendra, who's an investor. Please go ahead.

Raghavendra: Madhukar Reddy sir, so a special question for you regarding diagnostics. So, in one of the conference calls, you have mentioned that you're exploring B2B channels for diagnostic penetration. So, now given the OPD policies are common across IT employers and as well as all are linked to the TPAs. Any plans of seeing collaboration with the TPAs and ITAs in place for the diagnostic process? It's a very complicated.

G. Madhukar Reddy: We are exploring all options, including working with brokers, insurance brokers, insurance companies, TPAs, and going directly to companies and having this as an add-on product to whatever they have. So, typically, an OPD product costs around Rs. 5,000 to 6,000 for each company. A lot of companies find it very difficult to pay that money, given that their overall outflow on the basic insurance now is around 25,000, and that's quite an expense. And the OPD then is either a hit or miss in the sense that there could be some people who use it, and they could use it to the extent of 10,000, but what they are seeing is a large portion of

people don't use it at

all. So, it's actually a last -minute proposition for the employer who pays that money. So, I am not sure how many people are really taking it up at that expense. There are obviously some very, I would say, profitable companies who will do it. But we feel that if we were to offer our product, which actually costs Rs. 2000 for a family of two plus two to get a 75% discount on every single diagnostic test they can think of for the year and that 2000 then is also converted into wallet points which they can use at a full price, MRP. If that product were to be available to companies at maybe at a discounted price of 30 % to 40 % or even 50% discount for depending on the size of the employer and all that. We feel that there'd be a pretty good reason for companies to actually buy. We have approached a bunch of them. We have a pretty decent set of small to medium sized companies based in Hyderabad who have actually taken it up and have given that option to their customer, to their employees. We think as we go forward, more and more people will sign up.

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Raghavendra: To follow up on this, so on the diagnostic full-service centers, we still miss OPG. So, and only for the OPG, for example, myself, I am a subscriber, so I need to mark the other competitors do. So, is there a plan of when you actually say it as a full-service diagnostic center,

G. Madhukar Reddy: We are looking at that, but Yes, not an immediate thing.

Raghavendra: So, we found that the majority of the interiors are actually done by your own company, CustomFurnish. So, just wanted to understand the relationship between CustomFurnish and also by MedPlus and how much we are actually paying as competitive pricing?

G. Madhukar Reddy: I was the founder of CustomFurnish and a majority shareholder of that company. And that company used to supply lot of the furniture needs for MedPlus at a time when we were not growing in that space. Now MedPlus has set up its own furniture making unit. It's a small unit, but it supplies all of MedPlus' needs and going forward, it will also supply all of the diagnostic needs. So, I am not sure exactly what is the value, but I can tell you this for sure. The price at which Custom Furnish sold MedPlus is not a price you can find anywhere else in the market. It is the least possible listing out there.

Moderator: Thank you. The next question is from the line of Mehul Sheth from HDFC Securities. Please go ahead.

Mehul Sheth: One question on gross margin side. So, when you are saying that you will be able to sustain the gross margin, so should we consider that as an H1 gross margin or the Q2 gross margin?

Chetan Dikshit: Historically as you know, we've been used to talk about 21, 22. Currently this is 23 plus. If you want to be conservative, you could take the average for Q1 and Q2 as H1. But if you want a management sense, we are actually optimistic above the H1 number.

Mehul Sheth: One more question on your private level when you say you have around 16% of G MV contribution can you split between acute and chronic side? Even if chronic can be more of a like a sticky in nature, so can you split between that too?

G. Madhukar Reddy: Chronic is actually 50% or more and the reason why that is, while one would expect that people who are buying medication for a long -standing kind of disease are a little less open to changing over, the fact of the matter is they are the people who are actually spending a lot of money, month -on-month. And they are probably coming to our store regularly and they know our store employees well. And so when we talk to them about the product and the fact that it is made in the best factories and it is exactly the same as everything else out there and given the fact that the spend is much higher per month and they can save a lot more by

switching, they are the ones

who actually listen, who take the time to actually understand and basically switch. So, the chronic for us is actually slightly more than 50%. So, that's where it is and that's the good part. Again the question which 22.7 is actually split into the MedPlus pharmacy and the diagnostic margin. So, I don't know if the diagnostic margin can go much further, mainly because we

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already are optimized out there. Now it's just going to go on the topline. The MedPlus is actually

22.7. So, you will see the pharmacy component continuing to grow.

Mehul Sheth: And one last question on your expansion. So, from your presentation, we can see you recently opened a store in Uttar Pradesh. So, are you looking to expand it further in that state, so given it's a much larger state ?

G. Madhukar Reddy: No, we actually 3 stores. one in Delhi, one in Noida, and one in Gurgaon. So, it is Delhi, NCR. While it is in UP, it is more or nearly the NCR area. So, we are just testing it out. We are looking to see how well they grow and everything else and all. Delhi is the last of the remaining big seven cities in which we are not there, in the top seven cities. So, we said , let's just open one and figure out how things are right now and all. All the three stores are actually doing well. We will monitor it a little bit more and then maybe do 4 or 5 or 6 stores in each of those regions to slowly start gaining, let's say a foothold in that area. Two reasons actually, one, it is the last of the big cities in which we are not there. And second, as we, at some point, whenever we decide to do ATL, if at all we decide to do ATL for Me dPlus we want to make sure that we are there in every one of the capitals so that we can supply online in that state. So, being in Noida, if we were to get an order from anywhere in UP, allows us to tie up with the courier and send the MedPlus products out across the state. So, that's the reason for it. But Yes, we are not really immediately looking to expand in any big way out there.

Moderator: That was the last question. I 'd like to hand it over to the management for closing comments.

Srinivas: Thank you. I would like to thank all the participants on this call for your interest in the Me dPlus journey. Our investor relations team can be contacted at ir@metplusindia.com. Thank you.

Moderator: Thank you. On behalf of MedPlus Health Services that concludes this conference. Thank you for joining us and you may now disconnect your line.

Call: Feb 2025

MedPlus Health Services Limited

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February 06, 2025

The Listing Department The Listing Department
BSE Limited National Stock Exchange of India Limited
Phiroze Jeejeebhoy Towers Exchange Plaza,
Dalal Street, Fort, Bandra Kurla Complex,
Mumbai 400 001
BSE Scrip Code: 543427 Bandra (East), Mumbai – 400 051
NSE Symbol: MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Transcript of Earnings Call :

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the Earnings Call held with analyst / institutional investors on February 03, 2025 for the quarter ended December 31, 2024.

The same will be available on the website of the Company at www.medplusindia.com and also on the websites of BSE Limited and National Stock Exchange of India Ltd. viz. www.bseindia.com and www.nseindia.com respectively .

Thanking You
Yours faithfully

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer

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“MedPlus Health Services Limited
Earnings Conference Call ”
February 03, 2025

Management: Mr. Gangadi Madhukar Reddy - Chief Executive Officer and Managing Director
Mr. Sujit Kumar Mahato - Chief Financial Officer ,
Mr. Chetan Dikshit - Chief Strategy Officer ,
Mr. DRN Srinivas - Sr. Manager Finance

Moderator: Ladies and gentlemen, good day, and welcome to MedPlus Health Services Limited Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Srinivas. Thank you, and over to you, sir.

DRN Srinivas : Thank you, Sagar. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to MedPlus Q3 FY '25 Earnings Conference Call to discuss the financial results of the company, which were announced on January 31, 2025. We have with us today the senior management team represented by Mr. Madhukar Reddy, Gangadi, Chief Executive Officer and Managing Director; Mr. Sujit Mahato, CFO; and Mr. Chetan Dikshit, CSO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note, the disclaimer mentioning these risks and uncertainties on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website.

I will now hand over the call to Sujit. Thank you, and over to you, Sujit.

Sujit Mahato: Thank you, Srinivas and good evening, everyone, on this call. Our company remains committed to balance growth with profitability, while continuously enhancing our operational efficiency.

We are strategically strengthening our back-end operations and infrastructure to support long-term scalability and ensure seamless execution.

As we prepare for the next phase of expansion, we remain focused on optimizing our existing network, while laying the strong foundation for opening new stores across the 13 states in which we operate. As an update, we have added four additional warehouses to enhance their availability at our existing outlet and further supporting our endeavour in opening new stores. This disciplined approach will enable us to drive sustainable growth and enhance value for all the stakeholders.

On our network, we opened 87 stores during the current quarter. Over the past 12 months, we have added a net total of 379, gross of 484 store additions. Throughout quarter 3, there were 27 store closures. Taking into account both openings and closures, we achieved a net addition of 60 stores during the quarter, compared to the 108 stores added in quarter 2 and 37 stores in quarter 1, totaling 205 stores on YTD basis. We expect a total of 300 net store additions in the current financial year.

In terms of our network -- in terms of our stores' network age, around 27% of our stores are operational for less than 2 years, and the remaining 73% of our stores have been operational for 2 years or more. As a guardrail, we closely monitor the time frame for our new stores to reach breakeven.

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For stores opened between January 2024 and June 2024, approximately 55% of them achieved breakeven within 6 months of operations. As a cohort, all stores combined reached breakeven in 6 months. These include stores which have been opened in the new states.

In terms of our store size, at the end of the quarter, our network has grown to 4,612 stores with 2.4 million plus square feet compared to 4,233 stores and 2.2 million square feet at the end of December '23. The average store size is 528 square feet.

On the revenue mix, presently, MedPlus offers over 1,200 carefully selected SKUs spanning across pharmaceutical and nonpharmaceutical categories.

Private label sales for quarter 3 FY '25

constitutes 19.6% of our total revenue. The following is the impact of the launch of MedPlus brand products across our network. In Q1 FY '24, prior to the launch, the share of private label pharma stood at 7.9% of total GMV compared to 17.7% during the current quarter.

On the financial numbers, now on our quarter's performance, our consolidated revenue is INR 15,614 million with growth of 8.3% on a Y-o-Y basis and a degrowth of 0.9% on a quarter-on-quarter basis. Our consolidated operating EBITDA stood at INR 799 million, representing 5.1%. Around 99% of our revenue is from our pharmacy operations. Revenue from pharmacy operations grew by 12.3% year-on-year on GMV basis and 7.9% year-on-year on net basis. The pharmacy operating EBITDA stood at INR 780 million, representing 5.1%.

On a store performance, I would like to update on our stores older than 12 months. Revenue from these stores in quarter 3 was INR 14,388 million, representing 94% of pharmacy revenue.

These stores had a store level EBITDA margin of 11%. The store level operating ROCE of these stores stood at 61.7%.

A word here on the store level EBITDA margin by age. While stores greater than 12 months had a margin of 11%, this was 11.3% for stores greater than 24 months and 8% for stores in the 13 - to 24 -month age bracket. If the allocated non -store related cost, then the operating EBITDA of stores greater than 12 months would be INR 815 million, which translates to a margin of 5.6%. On our Diagnostics numbers, Diagnostics grew -- revenue grew to INR 274.7 million in quarter 3 compared to INR 196 million in quarter 3 of the last year. Diagnostics segment recorded an operating EBITDA of INR 22.1 million, representing 8.1% compared to a loss of INR 34.1 million in quarter 3 last year. However, central level operating EBITDA stood at INR 49 million. On working capital, our net working capital for Q3 was 61 days. The inventory in our warehouse was 36 days. In Q3, the inventory level of our first -tier stores was 88 days. In comparison, for our stores older than 12 months, the inventory was 40 days ...

Moderator: Sorry, sir, we have lost the audio from your line. Ladies and gentlemen, the line for the management seems to have disconnected. Please stay connected while we reconnect the line back.

Ladies and gentlemen, we have the line for the management reconnected. Please go ahead, sir.

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Sujit Mahato: Our update on the working capital. Our net working capital for Q3 was 61 days. The inventory in our warehouse was 36 days. In Q3, the inventory level of our first year stores was 88 days. In comparison for our stores older than 12 months, the inventory was 40 days.

Now I request Chetan to update on our Diagnostics business. Over to you, Chetan.

Chetan Dikshit: Thank you, Sujit, and good afternoon, everyone. Q3 is a seasonally weak quarter for Diagnostics. In October, we sold 413 gross plans per day. In November and December, this was 433 and 419, respectively.

As on 30th of September, we had 148,000 active plans and 299,000 underlying lives. As on 31st of December, we had 152,000 active plans and 315,000 underlying lives. Our current observed on-time renewal rate was 26% in Q3 versus 25% in Q2.

That concludes our update for this quarter. I request the host to open the line for questions.

Moderator: Our first question comes from the line of Saion Mukherjee from Nomura Securities.

Saion Mukherjee: So my question is around the growth in the Pharmacy business. So you mentioned, the GMV growth of 12.3% this quarter, now, obviously, there is some slowdown in new store addition that might have played a role. And also the -- if I look at the branded business, right, branded pharma number, probably, we are growing in low single digit, which is obviously much lower than what the pharma market

growth is.

So can you just throw some color on how the dynamics with respect to private label is playing out? Is there a very significant substitution that's happening from private label to branded, which is leading to very slow growth in branded? And now going forward, how are you thinking about the overall GMV growth in the pharmacy in the quarters ahead, please?

Gangadi Madhukar Reddy: So if you look at the growth in the pharma...

Moderator: Sir, we have lost the audio from your line once again.

Gangadi Madhukar Reddy: I think you have to reconnect.

Moderator: Ladies and gentlemen, the line for the management seems to have been disconnected. Please stay connected while we reconnect the line for the management back. We have the line for the management reconnected. Please go ahead, sir.

Gangadi Madhukar Reddy: Thank you. So as I was saying, we actually look at the overall pharma sales. And for us, it is not -- there's no real division between private label and brands and all. Growth is growth. We offer the customer an alternative when it comes to the store. We tell them, "Okay, we have got what you requested for." And we also have an option in the private table, and that is at a 50% to 80% discount.

Now one thing may get substituted for the other, depending on the customer's comfort with the overall thing and all. So for us, as long as we are growing the overall pharma pie, we are fine.

Now the growth itself, I know, was a little muted actually this quarter, and that is partly because

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the actual net sales would be slightly muted, I would say, mainly because the private label sells at a much lower rate and all.

But yes, outside of that and a little bit of seasonal, I would say, effect having plenty of holiday

season and all, especially in January and all, we did see a slightly, I would say, lesser sale than expected, but I expect that to get back.

Saion Mukherjee: Are you willing to guide any number for the GMV growth? Or how should we think about it over the next year or so?

Gangadi Madhukar Reddy: So it should be the usual listing of inflation plus kind of thing or it should be in line with the typical pharma market growth out there. So as we keep saying that for us, it is more important that we maintain the store -level EBITDA of 10% or 11% rather than just trying for same -store sales growth and all.

Some amount of cannibalization is always going to be there when we do this when we set up so many stores. But I'm fairly comfortable in saying that we should growth on the 2 -plus year stores also at; inflation and slightly above that.

Saion Mukherjee: Right. And my second question would be on your private label and gross margin. In this quarter, we have seen a significant pickup in private label, which is also reflected in gross margins, much higher than what you had guided in the previous quarter. So how should we think about that going forward?

Gangadi Madhukar Reddy: So think about the private label as going up by roughly around 1% every quarter on the volume basis, which is on MRP basis, which means additional margin of roughly around 15 to 20 basis points maximum per quarter. We could have 1 or 2 slightly better than normal kind of quarters and all, but I'm fairly comfortable we should be able to do 1% every quarter.

Moderator: The next question comes from Sudarshan Agarwal from Axis Capital.

Sudarshan Agarwal: Yes. So continuing on the previous participant's question, apart from the fact that your share of private label will kind of go up, what are your thoughts about the discounting policy in the private label?

I know right now, you are at 60, 65. At what levels do you think that maybe you could take a lower discount into consideration? And additionally, adding to this point, so let's say, the final brand that you are discounting against takes a hike, does that g

et reflected in your discount price also?

Gangadi Madhukar Reddy: Okay. About our private label discount itself, we advertise a 50% to 80% discount based on the molecule. And I think the average discount tough comes out to around 50%, 51%, that's all, because most of the higher selling drugs are all around the 50% discount.

On -- I don't expect that we'll really reduce this at any point of time. It may go down slightly over a longer period of time, I would think. But as of now, given where the discounting is on MedPlus Health Services Limited

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various generic products and all, I don't think we'll be able to reduce this much lower than what it is right now.

I think the second part of the question, if I understood it right, was, is the discount reflective of what the original brands we're substituting for. Not -- if that is what -- that was your question, no, not necessarily. We set the price based on the average price of the top 2 or the leader -- the brand leader out there because most of the people would end up actually buying those.

And in places where the thing is fragmented a lot more, the market is fragmented a lot more, and there are 1 or 2 or 3 players, all of them having more than 20%, 25% market share, then we will probably take an average outcome somewhere towards the middle of that and then set the price -- set the discount such that the net price to the customer is around that. The discount is off the average of these 3.

Sudarshan Agarwal: Got it, got it. And one more additional. While I understand the net store addition this year you have trended. For next year, are you guiding any number that we should kind of take into account, 500, 600 or it could be lower than that?

Gangadi Madhukar Reddy: No, I think it will be at least 600 stores for -- I don't think it will be less than 600 stores. This year, we have had a few things which we had to actually take care of because of our focus on private label and a couple of other things.

We also are -- as Sujit spoke in this call, we are also in the process of strengthening the back end. We have actually launched a bunch of warehouses across the state, in which we are there right now.

These will help us actually reach the stores faster and help us to actually grow faster, too. So a lot of that work is on. I expect that we'll be done in this quarter and we'll be able to start with the usual growth next year onwards.

Moderator: The next question comes from Tanmay Gandhi from Investec.

Tanmay Gandhi: Congrats on a good set of numbers. Sir, my question is again on private labels, right? So on the GMV, which is they are already at around 20% level, right, for the Pharmaceuticals, so my question is that do we need to put in more efforts to actually increase the contribution of the private labels from here on? Or do you think that, that it is going to be more organic?

Gangadi Madhukar Reddy: Tanmay, if you mean are we going to spend a lot of money advertising, I don't think so. What we'll end up doing is probably increasing the number of products which we have in private label. We are today, for the overall set of medicines, we probably are there at roughly around 68%. I expect insulins, which cover around 10% of the overall medicine sale, and some branded products will be another 5%, so which means that leaves at least around 85% to be targeted. I expect, if not 85%, it will probably take the 68% to maybe 75% to 77% or so. So increasing the number of products would be one thing. Making this available on a much better level than what we have been able to would be another thing.

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And third, as the number of people who buys these continue to increase, I expect that over a period of time, the word will spread, and we will basically see a lot more people asking for the product. It may not be -- we may

not really need to push as much as we are doing right now at this point.

Tanmay Gandhi: Yes. So sir, is it fair to assume that for the next year, the core focus area for the management will be store addition?

Gangadi Madhukar Reddy: The focus will be definitely on adding new stores also, 600 stores odd. And then same -store sales growth. Private label will be -- will continue to grow anyway. So that's going to be 1% per quarter is what we're aiming at. It could be slightly better, but I think we'll be actually able to hit that number easily without too much of a profit.

Tanmay Gandhi: And sir, one more question. The 4 new warehouses which you have launched, right, so that is largely for the upcoming stores, which you are planning to add in FY '26. Are these also to cater to your own private labels and overall increase the volumes at your existing stores?

Gangadi Madhukar Reddy: No, no. These are actually to increase the overall operational efficiency. For instance, that we have actually added around 3,000 stores in the last 4 years. In some of the states, we have got now around 1,000 stores each. Karnataka, we got 1,000 stores. Tamil Nadu, we have 1,000 stores. And AP Telangana combined, we have more than 1,200 stores.

So some of states are big, and we have not been able to reach the extreme parts as easily. And so supplies have not been as good as we wanted it to be. And so sales have suffered a little bit. So now we're setting up a warehouse in Hubli, one in Madurai. And 2 in Agra, 1 in Vijayawada, 1 in Vizag and 1 in Kapi and so on. So dividing up the states into 2 parts now. At least, our main area from where we operate and manage around 60%, 70% of the stores and a subarea from where you manage around 25% to 30% of the stores. So West Bengal, we probably have 1 warehouse in Asansol area and so on.

So this is -- we'll do 2 things. It will supply our current stores better. And with the operational heads sitting out there in that area, we probably would be able to launch new stores faster in those areas, too.

Tanmay Gandhi: Understood. So what is the plan for warehouse addition for next couple of years?

Gangadi Madhukar Reddy: So I think I pretty much told you where all we're going to put. We are adding a few more, let's say -- actually, there's one more coming up in Nagpur. In the new states, we're adding warehouse, a small one in Raipur and 1 in Indore. That will help us actually go deeper into Madhya Pradesh as we go forward.

But I would think most of these, we have already acquired. So you're probably already seeing the rents in the cost already, but we probably will add 3 or 4 more warehouses. I think a total of 10 smaller warehouses, anywhere between 30,000 to 50,000 square foot kind of leases. That includes the ones I've already talked about.

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Moderator: The next question comes from Madhav from Fidelity.

Madhav: So I had a slightly longer -term question. So now that our new MedPlus private label has been in the market for about 6 or 7 quarters, maybe 6 quarters, if I'm not wrong, so I think -- and it reached about high teens or almost 20% of our sort of volumes. So my sense is that it's kind of stabilizing within our network over the last few quarters now.

Now that some of this has been absorbed into the system, could you give us some sense on a 3 - year view if you think about network expansion, if you think about how gross margins would change for us?

And EBITDA margin has crossed 5% for the Pharmacy business, which is quite healthy. So is this a sustainable way to work with for us? As we think 2, 3 years, can it go a bit higher? Could you give us some sense because we've done a big transition in terms of the product mix, but how

are we thinking 3 years will be really, really helpful?

Gangadi Madhukar Reddy: So Madhav, as I said earlier, even if you take it for the

next 8 quarters, I fully expect that we will

be able to grow the level of 1% per quarter on the MRP basis, right? I expect -- see, this is something which we feel we can achieve without too much of a problem.

Now we could get benefited from the network efforts of these. A large number of people using it, being happy about it, maybe talking about it, one. Two, government pushing the generic products even more aggressively. And three, possibly competition also taking these up and talking up the generic -- the value of generic products out there. So all these could see, I would say, a disproportionate increase.

It is possible for it to happen that way, but I'm not -- I wouldn't bank on it. I would say, by our own thing, just by increasing the product -- increasing the range of products and also pushing it a little bit, we should be able to get that 1% for every quarter easily.

And that would mean possibly -- and if nothing else changed, I would say 1% growth in gross margin because every 1% in MRP, I think, increases our margin by around 20 basis points.

Madhav: So essentially, gross margins from here only have scope to move as the private label or MedPlus brand private label sort of improve. I understand that bit. If you're adding 600 stores, it seems like we can grow our network at, let's say, 12% -- 11%, 12% network expansion each year. So that's low double digit.

So is it fair to assume that if you have some SSSG as well, we're looking at 15%, 16% sales growth with margins expanding for us from here? Or do you think the 5.1% margin has any elements, which can bring it down? I understand seasonality of the business, which would be a different quarter would have different seasonality. But are we at a fairly sustainable base now? Or how should we think about margins going ahead? Can you be 5.5%, 6% in 3 years?

Gangadi Madhukar Reddy: No, I think we are at a fairly sustainable kind of margin level for the company out there. Now the only reason which can change it is if we add a ton of stores -- new stores. But as you said, the number of stores, which we add are going to be much lesser than the base which we have.

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So I really don't see a significant dip out there. What we can do is we can call out that number as we go forward. But the current set of stores should only be at this level or should get better -- sorry, it's not going to be a significant dip. There will always be a drag as we add stores, but it's not going to be significant.

Madhav: And if the Pharmacy GMV growth, which is 12%, which is kind of proxy for volumes, do you think that should grow in line with our store expansion? Or can it grow a bit faster? I don't know how -- is that the right way to think about the Pharmacy GMV growth?

Gangadi Madhukar Reddy: It's a little difficult to tell you, but I am sure that as the new warehouses come into play and as our supply chain also become slightly more efficient, I think we should be able to get some benefit. But I can really commit anything. We will maybe guide you in the next couple of quarters on any efficiency coming from that side.

Madhav: Sir, how much cost can these warehouses save for us? Is it any incremental cost savings which will come through or it's only sort of to kind of help the next leg of growth for the company?

Gangadi Madhukar Reddy: It is only to help the next level growth, that's all.

Moderator: The next question comes from Harith Ahamed from Avendus Spark.

Harith Ahamed: I joined the call a bit late, so my apologies if it's repetitive. The first question is on Pharma private label GMR. What exactly was the share of Pharma private label as percentage of our total GMV for the quarter.

And Madhukar, your comment that we are looking to increase this by 1 percentage point every quarter, is there a level that we're targeting and beyond which probably any further

increase

would be difficult given that branded generics will still be a significant part of our overall business and a significant part of the overall market?

Gangadi Madhukar Reddy: Yes. So the private label itself on a GMV basis is around 17.6% right now on the Pharma side. If you look at the net sales side, I think it's around 1.6%. The non-Pharma is also pretty significant now, around 8% overall. So we are a little under 20% right now for the overall private label on a net sales basis.

On our aspiration for growth, I have always been saying this. If the customer actually starts to believe that the product is same and if we can actually put in substitutes for all of products, I know there are some which we will not be able to do for a long time, insulin and some -- obviously, leave the patented products aside. Other than that, only insulin is something which we will not be able to do a substitution for.

But outside of that, if you are able to put everything out there and over a period of time, over the

next 2, 3, 4, people are not going to be, I would say, completely blind to the fact that these are all the same products being made in the same factory. And across the world, this is what actually sells. If you look at a country like U.S. and all, 90% of what they sell is generic, and that's all store generic. That's just one -- any one store basically selling their products.

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So people are not going to be completely blind to all this. I expect as we go forward, this will go all the way up to whatever numbers there are across the world. I don't see any limit to it. But over the next 7 or 8 quarters, I would still say, maybe 1% every quarter.

Harith Ahamed: Okay. Got it. My second question is on discount rates. So earlier, before we launched the branded Pharma private label, we used to share that number of around 16%, 17% as the discount rate -- blended discount rates across our various formats. Now my first question is, what is the current discount -- blended discount rate in the Pharma private label? And what is that rate for rest of the business, which used to be at that 16%, 17% level in the past?

Gangadi Madhukar Reddy: Okay. For the Pharma private label, it is around 51% right now. For the rest, I don't see any reason why it should be any different. It used to be around 17% or 18%. It will probably be the same number even today. But I'll tell you -- I'll come back to you with the numbers.

Harith Ahamed: Okay. Last one on the provision, the MRP level growth that you've shared for the quarter, it's around 8.5% and a decline versus Q2 where we had 13%, 14%. So any reason -- compared to the lower store addition could be reason that, that was the case even in 2Q? So I'm just wondering what has led to this lower growth rate on a sequential basis.

Gangadi Madhukar Reddy: A little bit of effect of seasonality, holiday season and everything else. And honestly, I can't think of any other reason out there. Possibly, some of the stuff which we're doing on the warehousing and everything else will add a little bit of efficiency and all. Possibly that our super quick addition of stores may have compromised the supply chain a little bit. And that also could have cost it, but I'm not sure. But I feel that -- I believe it's largely seasonal effect, nothing more.

Moderator: The next question comes from the line of Prateek Poddar from Bandhan Asset Management Company.

Prateek Poddar: I was just wondering if you could give me SSSG basis GMV? What could be SSSG growth basis GMV? Hello? Am I audible?

Gangadi Madhukar Reddy: Yes, yes. No, we can hear you. Outside of what is there in the slides we have given, we have to come back to you with the more granular breakdown. I think on the GMV slide you're seeing, the growth is year-on-year 8.3, but we will come back to you with the actual SSSG.

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Prateek Poddar: Yes, please. The second question is distribution of private label, right? Obviously, the average is 10.7 for the system. But maybe you could give us a sense in terms of the median number or, let's say, what's the difference between the highest store and the lowest store? Just trying to get to -- get an understanding as to where are we trending. I understand you called out 1% per quarter, but just that would help.

Gangadi Madhukar Reddy: Yes. Sure. So if you see across the board, the smaller towns and the rural areas are going to have

a higher private label than the bigger cities, one.

Second, the older states in which we have been functioning are going to be higher than the newer states in which we have gone to. For instance, Madhya Pradesh and Chhattisgarh are going to

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be at a slightly lower private label number than AP, Telangana or West Bengal or Orissa, where we've been for a long time.

To give you an idea, I think Maharashtra, which is as urban as you can get, it's probably at around 7% or 8%. I think Bombay and all are at 7% or 8% for us on the Pharma side private label or maybe slightly less. On the other hand, a smaller town in Telangana or in Orissa or West Bengal is probably close to 20%.

Prateek Poddar: 20, 20, right?

Gangadi Madhukar Reddy: Yes.

Prateek Poddar: And this is on a net sales basis, correct?

Gangadi Madhukar Reddy: Yes. It's on net sales basis.

Prateek Poddar: Okay. And the last question. Look, when I see your gross margins, a sequential improvement of 130 basis points. And when I look at your operating EBITDA margin for stores older than 12 months, only a 40 basis point of improvement, can you help me understand where does this 80 basis point go? Or where have you reinvested?

Gangadi Madhukar Reddy: Sorry. Could you just repeat the question?

Prateek Poddar: No. So when you look at -- Madhukar, when you look at your gross margins, right, on an overall basis, 130 basis points of improvement on a sequential basis, correct? But when I look at store operating EBITDA margins, which is post all the unallocated -- I mean, corporate costs, et cetera, went up from 5.2% to 5.6%, so 40 basis point improvement. I'm just trying to understand why a lot of it didn't flow down. What am I missing here?

Sujit Mahato: Sure. In terms of the movement, I would say around 60 basis points came in because of our increase in the private label Pharma share mix. 20 basis points came in because of the non - Pharma private label share mix.

And due to lower inventory provision, we had a 60 basis positive impact during the current quarter. And your observation is right. The entire thing did not flow down to EBITDA as certain costs also went up.

For example, in Tamil Nadu, the minimum wages increased by 28%. This has also eaten away some portion of our store -level EBITDA. And we also set up, as we had explained earlier, certain warehouses, which has costed us some amount of money.

Moderator: Next question comes from Prakash Kapadia from Spark BMS.

Prakash Kapadia: A couple of questions from end. What exactly is the store -level revenue growth which is reported in the PBT? Because if I take the absolute revenue growth of mature stores, it doesn't come to 4.5%.

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Typically, most of the retail companies break sales down into SSSG growth, new store addition and sales from new stores. So if we can do that, it will give a lot of clarity to investors. As you know, retail, that's the key metric. Because here, it's very difficult to understand the number, 4.3, 4.4. So if you could give some insight, that would be helpful.

And secondly, as I see net store additions were 60 during the quarter, which brings us to around 205 stores. So what has changed? Because in every quarter, we are scaling down the

expected

store addition. So those are my 2 questions.

Gangadi Madhukar Reddy: Yes. So for us, I know this year, we have been a little up and down on the new store opening and all. A lot of our bandwidth has been taken up by focusing on the private label, one. And two, on setting up the new warehouses and everything else, we have seen that some of the supplies have suffered a little bit because of the super fast growth, which we have had in the last few years. So some of that, we are already getting done.

On the EBITDA, it's -- on the same -store sales growth itself, for us, we actually -- and this is something which we have been talking about even before the IPO and all. For us, we always look at the overall store -level EBITDA. And to us, that is the most important thing.

The reason I say that is in markets like Hyderabad, Bangalore and Chennai and all, where we already have 300, 400, 450 kind of stores, we have to put in new stores because new markets keep coming up. And no matter how much of a different catchment it is, it ends up cannibalizing some of our stores anyway. And so there is always that effect.

So if you were to basically look at it, a store which is doing, let's say, INR 1 lakh per day, if we -- is probably bringing in more customers from slightly farther than what you would want. And so if you go and set up a new store in that catchment, maybe 500 -- 750 meters away, we will always see some decrease in sales. We're not really concerned about that.

When we set up a new store, we try to figure out whether it will breakeven in 3 or 4 months or not, and whether it will actually get to the expected level of EBITDA within 2 or 2.5 years, which is around 10% at the store level.

And two, on the existing stores, we -- let us say, even if the sales were to dip a little bit, our goal would be to make sure that these are -- the store is doing 10% or more.

So overall, we will continue to gain market share and everything else in that area. But for us to mention exclusively SSSG would be a little bit more tough given how we operate. So for us, it's more the market level and the 10% EBITDA kind of thing, which we actually look at.

Prakash Kapadia: I do understand the point of profitability. But typically, that's the standard retail metrics, which is the key performance indicator. And it becomes difficult to evaluate businesses and cycles over a period of time. So think about it, I think maybe the SSSG breakup, new store addition and store -- sales from new stores would really be helpful to investors. That's my suggestion.

Gangadi Madhukar Reddy: We'll definitely consider that.

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Moderator: The next question comes from Akhil Parekh from B&K Securities.

Akhil Parekh: Congratulations on a good set of numbers and healthy operating cash flow generation. On the penetration level of private label, you did highlight that the penetration levels are different across small versus larger towns. But going forward, so next 2, 3 years, how are you seeing the private labels? Will it be because of the increasing depth of private label in the existing stores? Or more by increasing the breadth of presence of private label in the stores?

Gangadi Madhukar Reddy: So both. We definitely increased the range of products, which we have out there. And we definitely try and convert more customers into our private label. But I also hope that some of the positive benefits, which the private label has on consumer population out there will get talked about and more new customers will walk in from neighboring stores to our stores. And so that will also help us in basically increasing our overall sales growth.

For us, it's going to be a combination of all 3. And I think the longer we are in the market, the more likely it is that we continue to actually grow.

Akhil Parekh: Second question on store expansion, right?

t? In answering the previous participant, you highlighted that the focus is more on the profitability rather than growth and more on gaining the market share within that particular geography.

So we have seen a reduction or decline in SSSG over the last 4 or 5 quarters over Q3 of FY '24 versus Q3 of '25. I believe it's part to do with the increasing sales contribution from private label. But are we monitoring if there is a cannibalization of sales because we are expanding aggressively within a similar cluster?

Gangadi Madhukar Reddy: So we are, but the thing which concerns us even more is that each individual store is basically sticking to the profitable -- profitability numbers we should expect to have. So that would be the first thing we should actually look at, one.

Second, one of the things which probably is showing up as less efficiency and all for us is also this, that we've always been talking about stores, which are more than 1 year. The number of stores which are between, let me say, 12 months to 24 months have been decreasing constantly. And that number -- the -- those number of stores -- the stores between 12 and 24 are going to be growing much faster. And so decrease in those number of stores would have probably shown a decreasing overall number. But I don't think the overall mature stores themselves are behaving any differently from what they were earlier.

Akhil Parekh: Okay. Sir, should we assume that SSSG of, whatever, 4%, 5%, is that a sustainable rate the next 2, 3 years perspective? Because SSSG certainly helps in terms of absorption of fixed cost, right? At one end, we want to improve the profitability. And at other end, if we're not focused on SSSG, it may not help us in terms of absorption of fixed costs, right? That's how typical retail model works. So what would be a normalized SSSG growth rate if we were to expand at 500, 600 outlets every year for next 2, 3 years?

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Gangadi Madhukar Reddy: So I think we'll be at least at the level currently where we are. I don't really see dipping any further and all. I don't think that's got to be a problem because we are not going to be adding -- so we are looking to add additional stores next year. So around ...

Akhil Parekh: Sorry. Just to clarify. You're saying 3% to 4% is the base -- new base at least.

Gangadi Madhukar Reddy: 4% to 5% is what we are looking at, actually. That is a number at least we can be at a steady state as far as the profitability is concerned.

Moderator: The next question comes from the line of Prolin B. Nandu from Edelweiss Public Alternatives.

Prolin B. Nandu: While you have talked a lot about private label, but my question is also on the same. So Madhukar, you mentioned about the difference between metros and nonmetro -s, but are you positively surprised by how fast the private labels are taking off? Can you give us some instances as to what are the products within Pharma, which are tough to crack? And have you had customers come asking for private labels in those kind of products as well?

And in some of these stores where you have introduced private label much earlier than rest of your network, are you probably surprised by, let's say, that number from a net sales point of view reaching maybe 25 -odd percent? Can you give some qualitative color on private label and whether it is surprising your internal estimates there?

Gangadi Madhukar Reddy: Not really. We expected this growth. I actually expected a slightly higher growth, honestly. But yes, this is not out of -- I would say, it's not surprising. And as I said earlier, the numbers are smaller towns to slightly better and all. And across the country now, we are almost at the same level, I would say, because it's now been more than, I would say -- we started in June of '23, it's almost 1.5 years right now. The full country started in November. So

even for the full country,

it's still more than 4 quarters right now. So not much out there.

But to give you an idea, the number of -- I would say, the private label share of chronic versus acute is exactly the same as it is for the regular brands. So I think 60% to 70% of our sales are private label comes from the chronic segment. And so people are buying all those Telmisartan, Atenolol and everything else also at the same level. The top 10 molecules probably will have at least 6 chronic and 4 acute molecules.

Prolin B. Nandu: Okay. That's clear. Now the second quest is more on competition, let's say, from quick commerce side. While what is appreciated that a typical quick commerce versus a quick commerce for pharma requires a very different kind of a setup.

But in case if the competition were to ramp up and you were to match their service levels in terms of 2 hours or whatever for all your stores, which you are doing for some part of your stores, how much of a drag will it be on margin? How soon you will be able to ramp up, right, to probably meet the standards of quick commerce?

Let's say, even if you were to launch 2 -hours delivery from your stores, right, how soon can you do it? And what kind of a drag will it create on your margins?

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Gangadi Madhukar Reddy: Yes. As long as the customer is willing to pay the delivery fee, I don't think there will be a drag at all. If the delivery were to be 100% free, today we charge around INR 20. If we shift to a 0 delivery fee and also try to kill ourselves by doing a 10 -minute delivery, then, yes, it would be a drag. But I don't foresee that happening. If we decide to do a 10 - or a 20 -minute delivery, it would bring on delivery partners. We're looking at the prices right now. The prices are not something which a customer will not pay. So I don't really expect this to be a drag.

Prolin B. Nandu: Right. But how soon will it take, Madhukar, for you to ramp up, let's say, where you are offering a 2-hour delivery all your stores?

Gangadi Madhukar Reddy: No, no. We'll never do all our stores at any point of time because customers across all the 600, 700 small towns in which we are there don't really expect this service and don't really want to pay that kind of delivery fee also. It will always be there in the urban areas, in the highly dense networks where we can actually go and supply. I don't think it's going to -- so we already have a 2-hour thing.

Now do we want to ramp it up to a much faster delivery time or not? That's something which we have not yet decided. But we are monitoring the situation right now, and we'll figure out the best way forward in the next...

Prolin B. Nandu: Sure. But Madhukar, a couple of quarters back, you had probably ramped down this, right, in the number of stores or whatever your reach was. And right now, what I understand is that you're not sensing any reason for you to ramp up, right? Am I correct in terms of competition and in terms of what the market is doing?

Gangadi Madhukar Reddy: See, we are monitoring the place -- space right now. We will see, depending on how the whole -- so it all depends. If we are able to get some delivery partners or able to do it at a very good price -- at a price at which the customer is willing to pay, then we would do it all right. But yes, we will have to figure that out.

Moderator: Ladies and gentlemen, we would take that as our last question for today. I now hand the conference over to Mr. Sujit for closing comments.

Sujit Mahato: Thank you, Sagar. Thank you, ladies and gentlemen. I thank all the participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you.

Moderator: On behalf of MedPlus Health Services Limited, that concludes this conference. Thank you for joining us. You may now

disconnect your lines.

Call: May 2025

MedPlus Health Services Limited

040-6724 6724

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May 30, 2025

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Dear Sir/ Madam,

Sub: Submission of Transcript of Earnings Call:

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the Earnings Call held with analyst / institutional investors on May 28, 2025 for the quarter and year ended March 31, 2025.

The same will be available on the website of the Company at www.medplusindia.com and also on the websites of BSE Limited and National Stock Exchange of India Ltd. viz. www.bseindia.com and www.nseindia.com respectively .

Kindly take the same on record.

Thanking You
Yours faithfully

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer

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“MedPlus Health Services Limited
Q4 FY'25 Earnings Conference Call ”
May 28, 2025

MANAGEMENT : MR. GANGADI MADHUKAR REDDY - CHIEF EXECUTIVE

OFFICER AND MANAGING DIRECTOR

MR. SUJIT KUMAR MAHATO - CHIEF FINANCIAL OFFICER ,

MR. CHETAN DIKSHIT - CHIEF STRATEGY OFFICER ,

MR. DRN SRINIVAS - SR. MANAGER FINANCE

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Moderator: Ladies and gentlemen, good day, and welcome to the MedPlus Health Services Limited Q4 FY '25 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch-tone phone.

I now hand the conference over to Mr. Srinivas. Thank you. And over to you, sir.

Srinivas: Thank you, Sagar. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Q4 FY '25 earnings conference call to discuss the financial results of MedPlus for the fourth quarter and full year FY '25, which were announced yesterday. We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, CEO and MD; and Mr. Sujit Mahato, CFO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website. I would now hand over the call to Sujit. Thank you. And over to you, Sujit.

Sujit Mahato: Thank you, Srinivas, and good evening, everyone on this call. As we prepare for the next phase of expansion, we are strategically strengthening our backend operations and infrastructure to support long-term scalability and ensure seamless execution. We remain focused on optimizing our existing network while laying the strong foundation for opening new stores across the 13 states in which we operate.

As an update, we have added around 10 additional warehouses in the last financial year to enhance our availability at our existing outlet and further supporting our endeavour in new store expansion. This disciplined approach will enable us to drive sustainable growth and enhance value for all the stakeholders.

An update on the network, we opened 113 stores during the current quarter. Over the past 12 months, we have added a net total of 305 stores, gross 398 store addition, and we closed down certain stores. Throughout Q4, there were 13 store closures. Taking into account both openings and closures, we achieved a net addition of 100 stores during the quarter compared to the 60 stores added in Q3, 108 stores added in quarter 2 and 37 stores in Q1, totaling 305 stores on YTD basis.

We expect a total of 600 net store additions in FY '26. In terms of our stores' network age, around 22% of our stores are operational for less than 2 years and the remaining 78% of our stores have been operational for 2 years or more.

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As a guardrail, we closely monitor the time frame for our new stores to reach breakeven. For stores opened between April 2024 and September 2024, approximately 58% of them achieved breakeven within 6 months of operation. As a cohort, all stores combined reached breakeven in 5 months.

In terms of our store size, as at the end of the quarter, our network has grown to 4,712 stores with 2.5 million plus square feet compared to 4,407 stores and 2.3 million square feet at the end of March 2024. The average store size is 527 SFT. In terms of the revenue mix, presently, MedPlus offers over 1,200 carefully selected SKUs spanning across pharmaceutical and nonpharmaceutical categories.

Private label sales for Q4 FY '25 constitutes 23.3%, pharma 13.6% and non-pharma 9.7% of our total revenue. The following is the impact of the launch of MedPlus branded products across our network. In Q1 FY '24,

prior to the launch, the share of private label pharma stood at 7.9% of total GMV compared to 20.9% during the current quarter.

Our consolidated revenue stands at INR15,096 million for the quarter and INR61,361 million for FY '25. Our consolidated operating EBITDA stood at INR803 million, representing 5.3% for the quarter and INR2,776 million, representing 4.5% for the full year FY '25. Around 99%

of our revenue is from our pharmacy operations.

Revenue from pharmacy operations grew by 6.2% on Y-o-Y basis on GMV terms and 1% Y-o-Y on net basis. On the full year revenue from pharmacy operations, it grew by 14.8% on GMV basis and 8.6% on net basis. The pharmacy operating EBITDA stood at INR769 million, representing 5.2%.

On the update on our store performance, I would like to update on our stores older than 12 months. Revenue from these stores in Q4 was INR14,011 million, representing 95% of pharmacy revenue. These stores had a store level EBITDA margin of 11.5%. The store level operating ROCE of these stores stood at 59.2%.

A word here on the store level EBITDA margin by age, while stores greater than 12 months had a margin of 11.5%. This was 11.7% for stores greater than 24 months and 8.2% for stores in the 13 months to 24 months age bracket. If we allocated the nonstore-related costs, then the operating EBITDA of stores greater than 12 months would be INR817 million, which translates to a margin of 5.8%.

On working capital, our net working capital for Q4 was 63 days. The inventory in our warehouse was 37 days. In Q4, the inventory level of our first year stores was 108 days. In comparison for our stores older than 12 months, the inventory was 43 days. Our Diagnostics numbers. Diagnostics revenue grew to INR280 million in Q4 FY '25 compared to INR232.4 million in quarter 4 FY '24. Diagnostics segment recorded an operating EBITDA of INR34.3 million compared to a loss of INR11.3 million in quarter 4 last year.

In January, we sold 420 gross plans per day. In Feb and March, this was 462 and 478 plans sold per day, respectively. As on 31st December, we had 152,000 active plans covering 315,000 underlying lives. As on 31st March, we had 157,000 active plans and covering

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327,000 underlying lives. Our current observed on-time renewal rate was 25% in Q4 versus 26% in the last quarter.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you very much. Our first question comes from the line of Saion Mukherjee from Nomura Securities. Please go ahead.

Saion Mukherjee: Yes. Sir, this question on GMV growth at pharmacy, which you mentioned is at 6.6%. Given that the pharma market itself is growing at around 7%, and we would expect MedPlus to grow faster, what is -- what in your assessment is impacting the overall GMV growth in pharmacy?

Madhukar Reddy: So a couple of things, Saion. The one main one would be the mix of private label and branded products. Our private label, of course, sells for a much lesser price than the branded products.

So the top line is going to be slightly lesser. That's one. Second, and I know we will come to this question going forward.

Our focus on private label and the incentives which we have set up for our employees have also probably dampened the overall or muted the sale of branded things to a small extent, right? I don't think it is significant. I think what we did was absolutely necessary to get the entire organization enthused about selling private label and everything else. So I think we have achieved what we set out to achieve. Now we're basically again aligning everyone to both private label as well as branded products. So I think we'll probably recover it again.

And the third thing, which probably muted our overall sales and maybe did not give us the SSG, which we really required was because of the rapid

id expansion, we had a few constraints,

both on manpower side on the back end and also on the warehouses, warehouses in particular.

We have now started the process of getting the warehouses. We're seeing the benefits in just West Bengal where it is launched, but the rest are still to come. So I think going forward, we'll probably see the benefits of all those things as we go forward.

Saion Mukherjee: Yes. Actually, I was mentioning about the GMV, which shouldn't have got impact. I can understand the transition and the mix change in favor of private label. But GMV growth ideally should have been sort of higher than where the pharma market growth is. I mean is there any pricing issue with respect to private label? Like, I mean -- or is there market share gain by competition or online players of quick commerce? Any such structural dynamics at play here, you think?

Madhukar Reddy: I don't think so, Saion. Actually, the GMV growth is 9%, not 6%, right, for the year. No, for the year is 9% for the year. So it is not 6%. So we have grown slightly, I mean not that it is a big thing. We expect to grow much faster. But the other reasons are also there, which basically accounted for whatever happened out there, which is our own constraints and everything else and all. And on the GMV side, it's definitely not lower for the year.

Saion Mukherjee: And sir, what's your expectation going forward? Like how should we think about this growth to play out now?

Madhukar Reddy: I think we will be back on track given that the new store numbers are going to be much less compared to the overall size of the company itself and stores also will sort of get phased out throughout the year. I don't expect that to be a big factor. So whatever growth will come in will probably come in through the existing stores just growing. And that I expect it to be, let's say, high single digits to low double -digit kind of number on the SSG.

Moderator: Our next question comes from the line of Sudarshan Agarwal from Axis Capital.

Sudarshan Agarwal: Yes. On the gross margin front, you have seen a strong expansion on a Q -on-Q basis and on a Y-on-Y basis. If I'm not wrong, every 1 percentage point of your private label expansion adds around 30 to 40 bps, right? So is there some additional benefit that kind of came into this quarter on the gross margin front? Was there some inventory provision, et cetera, that kind of was taken? Or -- and also, what would be the steady -state margin going ahead for this on the gross margin side? Yes.

Sujit Mahato: So in quarter 4, Sanjay, what happened is we had an inventory provision release of around INR3 crores. This was in connection with the provision which the company had made as part of its abundant caution accounting policy on the Wynclear or the old private label pharma products when we had transitioned to our MedPlus brand. And there has been a release only to the extent into the books on actual liquidation or sale. And during Q4, we were able to sell worth INR3 crores of this inventory, and we got a release of around INR3 crores in the gross margin. That would have contributed to around 20 bps.

But the major surge what you have seen is for the private label increase between Q3 and Q4, both on pharma as well as non -pharma. Your other question on the steady -state gross margin, we would look at in the range of 24.5% to around 24.75% going forward. And once again, the private label growth journey commence, we should again start seeing an incremental growth in the gross margin.

Sudarshan Agarwal: Okay. So my follow -up question is that given that you saw a strong ramp -up in this particular quarter for private label share, I think you had alluded to, let's say, 1% Q -on-Q expansion per quarter for, let's say, 6 to 8 quarters. Do you still see that from current levels? Should it moderate? Can you provide

some guidance on that?

Madhukar Reddy: Yes, sure. So we probably grab the Q1 growth also in the Q4 side. So I expect this quarter to be slightly flattish as far as private label growth is concerned. But going forward, I think we will definitely be able to maintain the 1% growth pretty easily. I don't think that's an issue .

Moderator: The next question comes from the line of Sanjay from Bastion Research.

Sanjay: Yes. Sir, my question would be in the morning interview, you have guided the revenue growth to be at around 15%. And I understand you have said that the older retail, older stores which we have, which commands 80% of the stores, they will grow at inflation plus. So that -- is the understanding correct that the additional 600 stores, which we are going to open in the next year, they will deliver or the remaining below 12 -month stores will deliver greater than 20% growth going forward? And how you are seeing that, how can we should see the revenue growth drivers going forward, if you can explain us?

Madhukar Reddy: See, revenue growth is going to come from regular same -store sales growth as more customers come to our store and everything else as they mature and all that. I actually said closer to, let us say, the low double -digit kind of number. I don't think the 600 stores which we add is going to really add a lot in terms of top line.

So most of it is going to come through SSG only. The 600 stores which we add at 50 per month typically will start with only around INR2 lakh to INR3 lakh or INR4 lakh per month. So it is not very significant to the overall growth of the company. So going forward, what are the drivers?

There could be several, as we continue to make the private label popular. We're not really spending any money on advertisement actually. But as word of mouth spreads and more people benefit from the savings of private label, we expect at some point, there will be people walking into a store to take advantage of that. But I don't think that time is now. I think we'll probably need to wait for a while before the commission starts happening in a major way.

Sanjay: Okay. Sir, my second question would be on -- so in Q4 FY '20, our revenue growth for stores greater than 12 months has degrown by 1%. So it is because of private label mix increasing? Or is there something else attached to that?

Madhukar Reddy: No. As I summarized in the first answer, which I gave out, 2 things. The combination of private label mix and everything else obviously drives it down a little bit. Our focus on private label also kind of, I would say, maybe put -- muted the overall growth on brands and everything else.

And finally, as I said, our growth of number of stores in the last 2, 3 years has constrained our supply chain systems and everything else. We're still in the process of easing that situation. It will take a little while. But that also has caused a little bit of, I would say, dampening of sales. So it's a combination of various things.

Sanjay: Okay, sir. So sir, since we have added 10 additional warehouses and since the ramp -up is also on the stage. So in terms of growth, you have alluded that you are seeing high single digit of the overall revenue or of the greater than 12 -month stores revenue?

Madhukar Reddy: I don't think it really is going to change that much either ways because the new stores, which we added this year are only 300. The base is going to be really low out there. So you could take it as the full set, high single digits to low double -digit kind of growth.

Moderator: Our next question comes from the line of Madhav Marda from FIL.

Madhav Marda: First, wanted to understand that, just a clarification, when we say that the pharmacy sales growth was about 9% for FY '25, how much was the GMV growth for the full year FY '25?

Madhukar Reddy: The full year FY '25 was 14.8%

on GMV.

Madhav Marda: Sir, which is a proxy for volume, right? So we can say 15% revenue growth?

Madhukar Reddy: Yes, 15%. Yes.

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Madhav Marda: And the 6%, which is sort of the difference is more of a mix impact from more private label sales, right? So that's the way we should read it very broadly?

Madhukar Reddy: Yes, absolutely.

Madhav Marda: The same thing -- could you read the similar thing for quarter 4 as well, where we reported 1% sales growth. How much is the GMV growth for quarter 4?

Madhukar Reddy: Can you hold for one second, Madhav? Can I get back to you on that regarding the growth of GMV?

Madhav Marda: Sure. Sure. Sure. Then sir, the second question was on the margin side. So I think you just guided for a gross margin of 24.5% to 24.8% going ahead. This is for the pharmacy business, right?

Madhukar Reddy: Yes.

Sujit Mahato: Yes. It will be plus 1% compared to Diagnostics.

Madhav Marda: Sorry, sir, I didn't get the last part.

Sujit Mahato: I said you can add 1% for the Diagnostics, so that comes to the consol number.

Madhav Marda: Correct. Correct. Yes. So the pharmacy business where the guidance is 24.5% to 24.8%, that number was 23.3% this year. So we are basically indicating 120 to 150 basis points of expansion of gross margin in next full financial year. That's how we should read it, right?

Sujit Mahato: So for the current year, Madhav, it's in the range of 24.1% in the -- for the full year. That's what we are saying it should move up to 24.5% to 24.8%.

Madhav Marda: Okay. At the consol level basically. Okay. Maybe I'll just check it with you offline. Maybe there's some confusion here. And sir, then on the margin side, the last follow -up was that it seems like this year, we have spent some money for expansion of our warehouses, which probably would have led to some front -ending of some cost. So is it fair to assume that next couple of years, the opex growth could be more controlled and we could see some operating leverage benefit from there? Is that the right thing?

Madhukar Reddy: The opex may or may not be controlled. We may continue to add some warehouses here and there, some automation and everything else that we will see. But definitely, the benefits of these will come in, in the next year only because of all the warehouses which we actually kicked off, only one has become functional as of now in Calcutta. Otherwise, almost everything is going to come online in the next few quarters.

Madhav Marda: Sure. Sir, any guidance for the pharmacy operating EBITDA margin, which was at 4.5% this year. Any outlook for that over the next couple of years? Do you see that crossing 5% in the next 2 years or so? Or could you give some sense there?

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Madhukar Reddy: Yes. I think so, Madhav, because the number of stores we are adding are not very significant. So the drag of the new stores is not going to be significant. But I think we'll continue to get the

benefit of private label. So that should basically help us expand the margin, I think.

Moderator: Our next question comes from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil: Yes. Madhukar, this revenue growth with all the private label shift happening, are we looking at a relatively low revenue growth increasing margin sort of strategy going forward?

Madhukar Reddy: Not as much as we have seen in the past. The main reason is I think the early adopters have basically come in and bought the private label already. So the growth from now on the private label is going to be just 1%. So the substitution of the brands is not going to be very high. So whatever growth which comes in through the brands will continue to accrete to it.

And the -- so the substitution is not going to be high. I don't expect that the drag because of substi

tution is going to be very high on the top line. But the margin will continue to grow, mainly because we will at least continue to grow every quarter on the mix.

Bino Pathiparampil: Got it. So things get stabilized now more. Second question on store addition, the 600 that you mentioned, would that be net or gross?

Madhukar Reddy: That will be net.

Bino Pathiparampil: Net 600. Okay. And yes, finally, on your tax rate, why is it below the 25% level? And how you see that moving over the next 2, 3 years?

Sujit Mahato: Yes. On the income tax, there is a deduction which the company continues to claim. It's the Section 80JJAA, which is linked with the employment created for new employees. So as we continue to expand our network and new employees get added to the network, there is an additional deduction which is available over the next 3 years for the amount spent on salaries for these new employees, and that is driving down the tax numbers. And we expect this to continue in the same manner provided this section is available in Income Tax Act.

Moderator: The next question comes from the line of Aejas Lakhani from Unifi Mutual Funds.

Aejas Lakhani: Yes. Madhukar and Sujit, nice to see the operating EBITDA number truly reflect the efforts that the team has been putting in. My first question, Madhukar, to you is that at the start of the year, we had called out for the 600 store addition, and we've closed at about half of that. So could you just walk us through what really took place for us to drop this run rate? And how are you thinking at that point of time?

Madhukar Reddy: So, Aejas, a couple of things happened actually. One, the first quarter of last year was not great because of elections and everything else. And then by the time that we went into the second quarter and all, we realized that our workers were very strained. They were not able to supply all the stores, and we decided that we would actually slow down, especially in the periphery and all. So for instance, in Tamil Nadu, Karnataka and all, we now have set up warehouses in Hubli and Madurai and likewise in other places in West Bengal and all.

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So the places which would be supplied by today's Madurai and Hubli were receiving the material very late and we were having a little bit of challenge in the main cities because of manpower attrition and everything. So because of all those reasons, we thought we would actually set up the back end properly and come back and actually start growing once again. And that's the reason we took a breather out there. We were thinking that we do around 400, but in the end, I think we guided towards a 300, 350 number, and that's where we ended. We did a total of 398 though, gross additions, but we shut down a bunch of stores, which were old and their characteristics kind of changed and renovated and all. So the net addition then ended up being only around 305 stores.

Aejas Lakhani: Got it. Madhukar, the second question actually is a leaf of what you just answered. So one is that could you just give some further color on the 10 warehouse that you have added in the year? Is this more from a densification standpoint? Or is it more expansionary towards Tier 2 and onwards?

And just double tapping into that question, I'm trying to understand how have fill rates been?

And if you can give me any color because have fill rates been affected in -- because there was supply chain from a warehousing standpoint constrained. Have fill rates been affected? And third, if you could just speak a little bit about the attrition aspect?

Madhukar Reddy: Yes, fill rates, definitely, they were affected, and they were affected largely in the remote parts of the states, which are getting supplied from the main warehouses. For instance, the Bangalore warehouse is supplying all the way up to Udipi, Mangalore and everything else. And th

at was taking a hit because of attrition at Bangalore. Attrition in the larger cities in South India has been fairly high.

So combined both warehouse which is not fully adequate and people -- and being short of people, we were barely able to supply the main cities. And so our sales -- our fill rates and then

consequently, our sales in the smaller towns took a hit. Yes, attrition fairly high.

We feel that as we go into the smaller towns, it will not be as high, and we'll probably be able to get slightly cheaper manpower, but at least the attrition will not be high even if the cost is not very different. Second, definitely, as we go forward, fill rates, we expect they will be better. And hopefully, they'll all get reflected in sales as we go forward.

Aejas Lakhani: Understood. And could you just share that the warehouse, was it more densification or expansionary towards Tier 2, Tier 3 cities?

Madhukar Reddy: Yes. Okay. By densification, you would mean, are we doing more in the, let us say, in Hyderabad or Bangalore or Chennai, no. We are definitely setting up warehouses in the, let's say, outer parts of the state, whereas in the initial stage, we basically had just 1 warehouse for one state. We're now doing anywhere from 2 to 3 for each state. So you could say this is definitely densification of the state itself. They're putting in more in the state, but away from the main warehouse and to increase the supply lines to the remote parts of the states.

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Aejas Lakhani: Understood. And just one last thing on the private label pharma as well as non-pharma. In your own mental construct, where are you from a category standpoint? Are you seeing white spaces there that you still wish to populate? Or the first wave of private label that you needed to introduce is all there, and it's just now about the push-through and conversion?

Madhukar Reddy: On the pharma side, I don't think there's that many gaps. The one big gap, I would say, is insulin, but that's not an easy one to actually fill. We will have to find a willing and a reliable partner out there for us to actually go forward on that. So there are not that many gaps. And I think right now, as we said, it is going to be more about conversion of more of the newer guys. And that's why we say 1% every quarter. We're not going to be really jumping up on those numbers anytime soon.

On the other hand, the private label and general goods, that's a wide open thing. There, we can add a lot more categories and probably a lot more -- I would say, we'd have a lot better chance of selling if the range was bigger so that the customers who walked into a store to buy the regular stuff now can see a variety of brands from us and appreciate the quality and the price difference and then start buying. So that would be all additional sales. And there we see a lot of scope, not as much in pharma, but definitely in the non-pharma side.

Aejas Lakhani: Understood. And finally, on Diagnostic, I'm sorry, Sujit, I missed the starting remark where you called out the count of paying subscribers that we have. And where are we on that journey? I know we had a number which was significantly higher. So what's the call out there from a Diagnostic standpoint? Are we just going to keep trying to increase that member base right now? And are we going to add those feeder diagnostic centers so that they feed into your larger centers? Just some color on Diagnostic.

Madhukar Reddy: Sure. The number was 157,000 active plans and 327 active plans -- underlying lives versus 152. So while we added 5,000 plans out there, it is definitely much lower than what we want. We need the number to be closer to 250,000 for us to actually take the leap of expanding to other states and all. So for now, we are going to continue to push forward and try to get this number up.

We are seeing obviously a bigger level

of acceptance out there. And one of the ways in which we expect to actually grow this number is by increasing the stand-alone collection centers across the city and increase the service level or increase the number of people available for home pickup of samples. So both these places, we will see some expansion and hopefully touch more people and increase the number of plans.

Aejas Lakhani: Understood. And Madhukar, the people who go for the home collection, so is my understanding correct that at the existing store only, you will have a technician who can double up as that and then go and collect it? Or are you talking about incremental hires and space required to execute the same?

Madhukar Reddy: No, no, no. So there is no space required. This is an online thing. You open up the slots based on the number of people you have. And typically, as the demand continues to grow, you keep hiring new people. So it is not in one shot. So rarely do we have a situation in which the people

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are not actually ready. In fact, today, we have a shortage of slots and some of the active members are actually wanting more slots to be opened up for home pickup.

So yes, there is -- and space, that's not space and that's not the people in the store don't double

up. The home service guys are different. All these orders are booked a day in advance and home service and the people in the stand-alone collection centers, they stay there between 7 to 10 to take care of any walk-in customers. They don't go out to pick up the samples.

Moderator: The next question comes from the line of Gaurav Gandhi from GCM Limited.

Gaurav Gandhi: Yes. Sir, how do you plan to reduce the drug license suspension issue? And is there any possibility of any bigger regulatory action if such suspensions happen regularly?

Madhukar Reddy: No, we are working with the drug departments and all to correct all the issues out there as much as possible. I can tell you for sure, we probably are more in line with the government's mandate than anyone else out there. It is just that we are a larger company, so we attract a little bit more attention and we probably -- and we have to actually report it. That's why it's probably in the news. But otherwise, our stores are way more compliant than anyone else out there.

Gaurav Gandhi: Okay. And the second question is, do promoters have any plans to cut down the stake in the company in the next 3 to 4 years?

Madhukar Reddy: 3 to 4 years is a long time. I'm not really thinking about it right now, but I don't expect to sell any shares in the near future at least.

Gaurav Gandhi: All right. All right, sir.

Madhukar Reddy: Yes, just to finish off that -- make that answer slightly more elaborate. If I do sell, it will be for mainly to clear the debt. And when it comes to refinancing, I may end up doing a little bit of that. But otherwise, it is not -- I'm not looking to exit any amount of my stake out there.

Moderator: The next question comes from the line of Lakshminarayanan KG from Tunga Investments.

Lakshminarayanan KG: Madhukar, a couple of things. I just want to understand what are your top 3 priorities for the next 3 years?

Madhukar Reddy: So I would say opening stores, 600 stores, that's the #1 priority. Also getting into, let us say, the -- making sure that the pilot for the franchisee store goes well. We'll try to make the franchisee thing as successful as possible. Given that the margins now have increased for us overall, we feel that this is the time for us right now where we can actually share a little bit with the franchisee and make it successful, at the same time, reduce the overall.

Let us say, burden of investment as well as the overall task of managing people, slightly less onerous for us by going to the franchisee side. So we feel if we can get the franchisee thing right, we will be able to grow

much faster. So outside of opening the stores and doing the franchisee thing, I think the third one would be to continue to grow the private label and see if we can actually continue to increase the margins for us.

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Lakshminarayanan KG: Got it. And second, I see that you have been expanding in Kerala in the last 1 year. How successful it has been, because I could see a significant ramp-up in Kerala. And what kind of potential it will have in the next 2, 3 years?

Madhukar Reddy: We are actually in comparison to other states and all, we're still going slow in Kerala. Kerala has its own set of problems out there. It's not very easy to -- there are several issues out there.

We see a lot of potential. It's very dense. People are affluent. There are a lot of, I would say, need for medication out there and all. But it had its own set of problems. So we are still kind of doing a wait and watch kind of thing. We have put in a few stores. We want to make sure that they are all profitable and we find the right model for us to expand.

Lakshminarayanan KG: Got it. Got it. And you talked about 600 stores that is over a period of -- can you just tell me in the next 3 years, how do you want to ramp up your 600 stores?

Madhukar Reddy: The 600 stores are going to be in the next 12 months, so they will spread out. So we will continue to open. Maybe Q1 is not going to be as big as the other quarters typically. But yes, otherwise it will be throughout the year.

Lakshminarayanan KG: Got it. And this revenue mix which you actually show in your Slide 14, this -- I mean, I think this is calculated at the discounted price or at the MRP?

Madhukar Reddy: It is a net price, net of discounts and everything else.

Lakshminarayanan KG: It's a net of discount. So which means that normalized thing, our -- I mean I'm just trying to understand our own brand pharma should be -- what would be the normalized price because this is net of discount.

Madhukar Reddy: So if you actually compare it MRP to MRP, which is more this thing of the volume, the pharma private label in the last quarter would be around 22% and the non-pharma in the last quarter was 13%. So bearing in mind that 80% of our overall sales comes from pharmacy, given that we are already at 22%, that would mean roughly around 28% of all medicines sold today by GMV -- in GMV terms under the MedPlus brand.

Lakshminarayanan KG: And in terms of new state addition, which are the states you are planning to go for the next 3 years?

Madhukar Reddy: So the states are Madhya Pradesh, Chhattisgarh and Kerala that remains the same, not

changed.

Moderator: Our next follow-up question comes from the line of Saion Mukherjee from Nomura Securities.

Saion Mukherjee: Madhukar, this you mentioned 28% of pharma is on private label. So what's the -- and it will keep growing, as you mentioned. So do you have any sort of optimum traction in mind that where this number can settle eventually?

Madhukar Reddy: I would like it to go all the way to 90%. But the thing is it will probably settle down at a much lower number, mainly because of 2 things. One, despite our discount, there are still some

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brands which would be cheaper. There's still some naturally really inexpensive brands, which you can't anyhow substitute. So there's going to be those.

There's also going to be products which -- for which we don't have a substitute, the insulins and all and a bunch of other things. And as we go forward, we're also seeing an increase in share of patented products, and all coming in, if we start selling a lot of those, those will not have substitutes for a long time.

So between all these things and the fact that not everyone is going to switch, we ourselves are going to be obviously pushing a lot on emphasizing a lot more on inc

reasing the top line rather

than just the private label. I think over the next 3, 4, 5, 6 quarters, a growth of not more than 1% is to be expected on the private label side.

Beyond that, Saion, I'm not sure. As more and more companies get to the generic side and they start talking about it, as the government starts talking about it, the overall word of mouth spread to an extent where people may just -- it may reach a tipping point where they just say, that's it. We will end up taking just the generics and nothing more. And then it may just change. But right now, I don't see that. I'm thinking for the next several quarters, it's going to be 1% every quarter.

Saion Mukherjee: Understood. And just one question I was thinking, this whole GLP-1 opportunity, which is going to come up next year. In terms of supply chain, is that something different which sort of benefits in any way, MedPlus? Or you think it's like one of the other products like which goes off patent? And obviously, we shouldn't expect any private label play in the short term in that, right?

Madhukar Reddy: No, why would you not expect private label? Obviously, that is a great product to launch a private label. We'll definitely try and do it as quickly as possible. We're already talking to people about that. So that will happen. Now is there anything special in the supply chain? Not really. Obviously, the cold chain and all have to be maintained and everything else, but that's about it. I don't think there's anything -- I mean it's not really a big deal. I mean it's like selling insulins anywhere.

Saion Mukherjee: And Madhukar, like private label for that injectable, like when can you -- I mean, launch? I mean, is there any timeline you have in mind?

Madhukar Reddy: So we are talking to people. I can't give you the exact timeline, Saion, but I know it is coming out of patent, I think March '26. And I'm hoping that we should be ready on day 1 or maybe if not day 1, maybe month 1.

Moderator: Our next question comes from the line of Harith Ahamed from Avendus Spark.

Harith Ahamed: The share of pharma private label, which you disclosed around 21% of GMV for the quarter.

Madhukar, can you give some color on how this number looks like across various states and across Tier 1 to Tier 4 markets, how different is the share?

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Madhukar Reddy: So I don't have the exact numbers out there, Harith, but I can tell you this for sure. Tier 4, Tier 3, obviously, are going to be much, much higher than Tier 1. But that said, Hyderabad, Bangalore and Chennai are not that bad, even Bombay, which is the lowest, I think, for us and private label is still at around 10% or 11% for us.

So smaller towns, the private label medicine could be as high as 30% or 32%. Yes. But state-wise, I don't think it really differs that much. It is more in line with our own entry into the state. So it depends on when we have actually gone to the place. So the older we are, the better the known we are, the higher the overall private label mix in the state.

Harith Ahamed: And you touched on opening stores and other franchisee models. Can you share a bit of color on the economics? Will you be booking the entire revenue? How -- what kind of share will go to the franchisees? And are these stores on top of the 600 stores that you guided for?

Madhukar Reddy: No, actually, they will be part of the 600 stores. Part of the stores which we are doing will be

franchisee. This will be like a wholesale kind of model where we sell the product to the store, and he then sells it downwards. And yes, it will be a MedPlus store where we act like a super distributor, you could say.

The only product that franchisee can sell is stuff which we sell at. So everything will be sold to them and that will be the price which we bill. So yes, the top line will come down slightly because of that, but th

e overall return on capital and everything will be much better.

Moderator: The next question comes from the line of Tarang Agrawal from Old Bridge.

Tarang Agrawal: Just quite a few questions actually. On bookkeeping, what are your number of stores in Tier 2 and beyond?

Madhukar Reddy: I don't have the exact number, but I think around 40% now of our stores probably are outside of the Tier 1.

Tarang Agrawal: Sorry, 70%, is it? I mean you used to call out this number in your opening remarks, right?

Madhukar Reddy: 40%. I think 40% of our stores, but I'll come back to you on that. So why don't you go ahead with your rest of the question.

Tarang Agrawal: Okay. In Q2, if I'm correct, I think you opened a store in UP. Is that right? I mean, is UP still there in your footprint or it's no longer there?

Madhukar Reddy: See, we opened a store in Noida, one in Gurgaon and one in Delhi. So this is largely to basically see if we could enter Delhi as a city and do online delivery out there. So we are still - that pilot is still running. Some of the stores are doing well, but it is not to enter the state of Haryana, UP or this one.

Tarang Agrawal: Okay. Got it. Got it. Got it. Madhukar, you said you guys have added about 10 warehouses in FY '25. What was this number in FY '24?

Madhukar Reddy: So 10 less.

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Tarang Agrawal: No, no. I mean, what's the overall warehouse number as on 31st March 2025.

Madhukar Reddy: It has actually doubled, I would say, close to double because we had one large warehouse for each state.

Tarang Agrawal: Okay. Got it. And in terms of working capital, as the store matures, what are the inventory days that you guys generally work with? I mean, for your reasonably matured store in your footprint, generally, what are the inventory days that, that store would work with?

Madhukar Reddy: Around 40 to 43 days.

Tarang Agrawal: But one would believe that -- I mean, that's the number that you work with for stores greater than 12 months, correct? But typically, you would make it even more efficient, right, as the store matures even further or 40 to 43 days is where sort of you hit the ceiling?

Madhukar Reddy: Not necessarily. The store sales go beyond INR2 lakh per day, it will probably come down a little bit. But anything below that, it's going to stay at around 40 to 43 days or maybe it will go down to 37. See, the reason is the store's reputation is built on fill rate. So if you're going to constantly cut the long tail, people will stop coming to us.

We have several products in our store, which are -- which we would have not sold even once in 6 months or once in 7 months, but they need to be carried because every month we see from the list of products which we sell, there are several products which we have never sold in 6 months sold as part of a customer overall basket. And if it were not for that product, maybe the customer would have walked. So the thing in pharma is not focusing a lot on inventory days in the store, but more on the fill rate and the top line, I would say.

Tarang Agrawal: Got it. Just out of curiosity, I mean, given the stock warehouse expansion that you've had, one would have presumed that your inventory days at the warehouse should have increased materially more, right? But that hasn't happened. So just wanted to understand, because logically, if you're adding a warehouse, you would typically add as much of SKUs and product that was there in the earlier warehouse, and that would have a significant drain on your overall inventory, but that hasn't played out. Is that my understanding correct, though?

Madhukar Reddy: Not really. But the thing is it is still to play out because the stores -- the warehouses are not yet functional. The only one warehouse is up right now. The others are actually getting ramped up. But in the initial stage, maybe, but afterwards,

it's not going to really change that much

because each of the warehouses is going to be taking care of at least 250 to 300 stores. So the inventory in, let's say, the Chennai warehouse is not going to be split between Madurai and Chennai.

Tarang Agrawal: Got it. Got it. And I mean, from a working capital standpoint, both on inventory and payable days, do you think there is scope of improvement or you've maxed out there?

Madhukar Reddy: I don't think there's a lot to be done out there.

Tarang Agrawal: Okay. And last, just if this number is handy with you, actually, to Madhav's question, what was the MRP rate, MRP growth rate for Q4? And second, if you could give us a sense on what were the bills cut through the quarter?

Madhukar Reddy: We don't have that number. The MRP growth, I think, was 0.4% or something.

Sujit Mahato: 6.4%.

Madhukar Reddy: 6.4%.

Tarang Agrawal: 6.4%. Okay.

Moderator: Our next question comes from the line of Vilina Jain from Perpetuity Ventures.

Vilina Jain: So I just want to understand our employee expense trend. So over the last 3 quarters, it has grown significantly. So firstly on that. Secondly, to grow private label pharma, are we using some kind of incentives in the stores?

Sujit Mahato: Yes, I think it's a great question. So it has both the components, as you rightly mentioned. As the private label sales increases, there is also the component of the employee incentive.

Technically, we would have wished that to go from the gross margin, but the way the accounting literature dictates, it goes as a selling cost, and therefore, it is shown as part of the salary expenses.

So that's the predominant thing for the increase. In addition to that, a couple of quarters ago, we had highlighted that we have brought in for the purpose of retaining employees, there is a retention plan for the store level employees. So that has also contributed to a little extent. And based on these reasons, the costs have gone up.

Vilina Jain: Sir, if you could highlight what would be this selling cost, which is like an employee incentive for private label? And how should we see this growing as we grow the private label?

Sujit Mahato: So it's again based on private label pharma and private label non-pharma and then there are -- during this year, we are also trying to align to the overall sales. So this is a dynamic thing, which we try to do it every quarter internally. So maybe offline, I can take that with you.

Moderator: Ladies and gentlemen, we will take that as the last question for today. I now hand the conference over to Mr. Sujit for closing comments.

Sujit Mahato: Thank you, Sagar. I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you.

Moderator: Thank you. On behalf of MedPlus Health Services Limited, that concludes this conference. Thank you for joining us. You may now disconnect your lines.

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NSE Symbol : MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Transcript of Earnings Call:

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the Earnings Call held with analyst/ institutional investors on November 03, 2025 for the quarter and half year ended September 30, 2025.

The same will be available on the website of the Company at www.medplusindia.com and also on the websites of BSE Limited and National Stock Exchange of India Ltd. viz. www.bseindia.com and www.nseindia.com respectively.

Thanking You
Yours faithfully

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer

Encl: a/a

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"MedPlus Health Services Limited
Q2 FY '26 Earnings Conference Call "
November 03, 2025

Management: Mr. Gangadi Madhukar Reddy - Chief Executive Officer and Managing Director
Mr. Sujit Kumar Mahato - Chief Financial Officer
Mr. DRN Srinivas - Sr. Manager Finance

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Moderator: Ladies and gentlemen, good day, and welcome to MedPlus Health Services Limited Q2 FY '26 Earnings Conference Call. As a reminder, all participant clients will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchstone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Srinivas from MedPlus Health Services Limited. Thank you. And over to you, sir.

Srinivas: Thank you, Swapnali. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to MedPlus Q2 FY '26 Earnings Conference Call to discuss the financial results of MedPlus for the second quarter of FY '26, which were announced earlier. We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, CEO and MD; and Sujit Mahato, CFO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide 1 of the Investor Presentation shared with all of you earlier.

Documents relating to our financial performance were circulated earlier and these have also been posted on our corporate website. I would now hand over the call to Sujit. Thank you. And over to you, Sujit.

Sujit Mahato: Thank you, Srinivas, and good evening, everyone, on this call. During the last 18 months, we have added 10 new warehouses across our network. We continue to strengthen our back-end operation and infrastructure to support the long-term scalability and ensure seamless execution. In terms of an update on our network, we have opened 145 new stores during the current quarter. Over the past 12 months, we have added a net total of 378, gross 469 store additions.

Throughout quarter 2, there were 28 store closures. Considering both openings and closures, we achieved a net addition of 117 stores during the quarter compared to the 101 stores added during the last quarter. We continue with the outlook for adding 600 new store additions in fiscal '26. In terms of our network age, around 22% of our stores have been operational for less than 2 years, and the remaining 78% of our stores have been operational for 2 years or more. In terms of our network size -- store size, at the end of the quarter, our network grew to 4,930 stores, with 2.6 million plus square feet compared to 4,552 stores and 2.4 million plus square feet at the end of September '24.

The average store size is 528 square feet. An update on the revenue mix. Presently, MedPlus offers over 1,450-plus carefully selected SKUs spanning across pharmaceutical and non-pharmaceutical categories. Private label sales for quarter 2 FY '26 constitute 21.5%, pharma being 12.1% and non-pharma 9.6% as a percentage of our total revenues.

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On GMV basis, during the current quarter, the share of private label pharma sales stood at 19.8% compared to 7.9% in Q1 FY '24, which was prior to the launch of MedPlus-branded pharmaceutical products.

An update on the financial numbers. Our consolidated revenue for the quarter is INR16,793 million. Our consolidated operating EBITDA for the quarter stood at INR887 million, representing 5.3%. Around 99% of our revenues is from our pharma operations. Revenue from

pharmacy operations grew by 8.8% on Y-o-Y basis on GMV basis and 6.3% Y-o-Y on net basis. The pharmacy operating EBITDA stood at INR839 million, representing 5.1%. An update on our store's performance. Stores older than 12 months, revenue from these stores in quarter 2 was INR15,3

38 million, representing 95% of pharmacy revenue. These stores had a store-level EBITDA margin of 11.8%. The store-level operating ROCE for these stores stood at 68.6%. A word here on the store-level EBITDA margin by age. While stores greater than 12 months had a margin of 11.8%, this was 12.1% for stores greater than 24 months and 8% for stores in the 13 months to 24 months age bracket. On allocating all non-store-related costs, the operating EBITDA of stores greater than 12 months would be INR877 million, which translates to a margin of 5.6%. An update on our working capital. Our net working capital for quarter 2 was 53 days.

The inventory in our warehouse was 33 days. In quarter 2, the inventory level of our first-year stores was 100 days. In comparison, for our stores older than 12 months, the inventory was 37 days. An update on our diagnostics numbers. Diagnostics revenue grew to INR332.5 million in quarter 2 FY '26 compared to INR283.1 million in quarter 2 FY '25. Diagnostics segment recorded an operating EBITDA of INR50.9 million, which is 15.3% compared to INR21 million in quarter 2 FY '25.

In July, we sold 535 gross plans a day. In August and September, this was 485 and 531 subscriptions, respectively. As on 30th September, we had 170,000 active plans, covering 351,000 underlying lives. As on 30th June, we had 167,000 active plans, covering 345,000 underlying lives. Our current observed on-time renewal rate is around 24% in Q2 versus 24% in Q1.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: The first question comes from the line of Sudarshan Agarwal from Axis Capital.

Sudarshan Agarwal: On the store addition part, right, I think H1, we are trailing a little below what half year mark we would like to be at, I would assume 300. So, we are confident of this? And on the franchisee front, I think you guys had said you would add around 100 out of the 600 stores this year from franchisee base. So, what is the update on that? Can you just let us know?

Madhukar Gangadi: Yes. We will be able to make up the balance in the next half. And franchisee also is on target for us. I don't think that's a problem.

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Sudarshan Agarwal: So can you just tell me how many have till date for this year been via franchisee model, if that is available?

Madhukar Gangadi: I don't have the exact numbers right now. We'll come back to you. But again, the 100 stores and franchisees shouldn't be a problem. We should be able to get that easily.

Sudarshan Agarwal: Okay. And your SSG growth, I think you had rejigged your private label incentives, right? So, our branded business has started seeing growth once again. And private label pharma has come down. So, what do you expect the trend to be for the quarters ahead, I mean, for the private label pharma and also on the FMCG side? FMCG actually has seen quite a big jump sequentially also.

So on that side, can you just update us?

Madhukar Gangadi: Yes. I think given the fact that we had actually grown a little more in the last quarter in franchisee -- in the pharmacy side, private label, we wanted to actually tone it down a little bit. But I think we'll be able to pick up the growth and start growing. For this quarter, it may be more or less at the same level. But after that, we should start growing at around 1% every quarter on an MRP basis.

So, that's the basic plan for us. Most likely, we will restart the -- press the accelerator on it only in the next quarter or so, not this quarter. On the private label general goods side, that is completely in our control in the sense that, that doesn't really replace any product. We don't have to actually tell the customer to buy one thing instead of the other. We're just expanding the range of products which we have.

We're taking advantage of the fact that we have

so many stores and we are able to bring in whatever products we want to without too much of a problem. So, that will continue to grow.

And even as, let us say, we kind of mute the private label pharma side, we will continue to push the accelerator on the general goods side.

Sudarshan Agarwal: Okay. And last thing. So when you say the subscription number, right, I assume that this is for the diagnostic or the pharmacy plan as well. I think you have rolled out MedPlus Advantage pharmacy plan, which allows another 2% discounts, right? So is that number -- the subscription including that?

Madhukar Gangadi: I think the 175,000-odd number, which you talked about, that is for the diagnostic side. The pharmacy subscription is a different number altogether.

Sudarshan Agarwal: Do you have that? And can you share that?

Sujit Mahato: It's upwards of 4 million.

Madhukar Gangadi: Yes. It's upwards of 4 million.

Sujit Mahato: We will connect, Sudarshan, offline on this.

Sudarshan Agarwal: Yes. Sure.

Moderator: The next question comes from the line of Jayshree Bajaj from Trinetra Asset Managers.

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Jayshree Bajaj: First of all, congratulations on a great set of numbers. My question is that diagnostics segment saw improved operating EBITDA margin, like a 15.3% in this quarter than the previous quarter, it was 13.6%. So, what are the current capex and expansion plans for the diagnostic services, including adding Level 2 and for service centers? And what is the revenue contribution we are expecting from the diagnostic centers in the next 1 year or 1.5 years?

Madhukar Gangadi: See, whether it is 15 or even 20, our plan will not change as far as the diagnostic centers are concerned, unless we cross 250,000 subscription numbers on the subscription side for the diagnostic side.

So for us, in the current model, which is capex intensive, where we put in INR120-odd crores to set up the 12 diagnostic centers, 4 of which are the large ones and all. We are not going to expand until we get the numbers we want. So that's it for that model right now. We are still looking to tweak the model to get those numbers.

And when we get them, then we will come back and basically tell you what we are planning to do. That's one. Second, given that this is a small portion of our overall business, less than 2% of our overall business, I don't think it is going to be material to us given that we are not planning to spend any more capex on this.

Moderator: The next question comes from the line of Avnish Barman, an Individual Investor.

Avnish Barman: Okay. So, this is Avnish. This is Avnish. I have one. I just wanted to understand the impact of the GST change on the business. And it will help if you can just segregate the impact on each player on the supply chain side.

So, for example, what was the impact on the manufacturer side, on the distributor side and on the retailer side? And if you also can comment on whether things are completely back to normal or there's still some time before things get back to normal?

Sujit Mahato: Sure. Sure, Avnish. Thank for the question. This is Sujit here. So, on the GST implementation, firstly, we went ahead with the broad intent of the Central Government, and we passed on the entire GST benefit to the customers right from 22nd of September when this got effective.

In terms of the impact, as you rightly said, for all stocks procured by the company prior to 22nd of September before the GST notification got implemented, we had incurred the higher input credit. So, there is an accumulation of input credit by an average of 7% for the pharma sector. And therefore, to that extent, the utilization got delayed. For example, we used to buy product at 12% GST prior to this change, and now it is reduced to 5%. So to that extent, there is a blockage of input tax credit by maybe a month or so in our case.

And on the retailer side, because all of them -- all of the stores are folding into the company, that's the only impact what we have. Additionally, what we have done, we have reached out to all our suppliers and we had negotiated an extra credit period, a one-time extra credit period, which you also see currently as a benefit in the net working capital change in our presentation, where we got some extra credit days for all the purchases made during the last couple of months.

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Because there is a blockage of our input tax credit on the inventory, which the company is currently holding. So, that's the very high-level impact on the GST change for us.

Avnish Barman: Okay. So from what I'm understanding, Sujit, it's like on one side, you've got like 1 month blockage of the ITC. And on the other hand, you've got like a one-time discount -- one-time credit leeway from the supplier. On the balance, has it helped the working capital? Or has it made transient working capital a little inferior?

And second, did you also get some discounts from the supply chain from the manufacturers?

And third, whether things are completely back to normal or there is still some time to go?

Sujit Mahato: See, from the impact on a wholesome basis, I think for a quarter, we have seen a positive impact. Going forward, it should come back to square zero because it's a zero-sum game going forward.

In terms, if you are thinking to look at what's the impact on volume, I think, yes, when the notification got implemented, we did not see any surge in volume.

It remained constant for us. And therefore, going forward, and you also mentioned, is there any discount from manufacturers? There are here and there, but it's not a standard kind of a discount

because most of them have helped us by giving us extra credit period as a one-time benefit.

Moderator: The next question comes from the line of Harith Ahamed from Avendus Spark.

Harith Ahamed: Sir, last quarter, we had talked about certain measures on the supply chain front, including addition of warehouses. So, can you talk about where exactly we are in that process? And if you can also comment on fill rates, which, again, we mentioned that there was some impact in the past as we rolled out the private label. So specifically, on fill rates for branded generics, have you seen an improvement in the last quarter or so?

Madhukar Gangadi: Yes. On the supply chain side, the new warehouses have been added and some of them are already functional, and some are still slowly coming to their full listing out there. They have all started functioning, though, most of them. On the fill rate, definitely, yes, we are slightly better, as you can see from the impact of there and all, both what we have done on rejiggering the entire incentive plan for private label versus brands.

And on the supply side, both of them have actually resulted in a slight increase in the branded generic sales. So it's a continuing process for us. I'm not sure, I don't think we are quite there on the fill rate completely, but it has improved.

Harith Ahamed: And on the pre-operative costs, there's been a marginal increase. It's higher than what we've seen in the previous quarters. So any reasons? Is this in line with the store additions planned in the second half?

Sujit Mahato: Absolutely. Absolutely, Harith. This is Sujit here. So, this is for the property bank and the number of apprentices, which we hired or the people which we put up for the initial training and those costs, we classify them as pre-operative. So, we have a property bank of 400 plus as we speak.

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So those 400 plus as and when the rental starts before we launch the stores gets accumulated under the pre-operative head. You are right.

Harith Ahamed: Okay. And last one on the margin

ins on the pharmacy retail operating EBITDA margins.

Historically, we've seen second half being stronger versus the first half. Should we expect a similar trend this year?

Madhukar Gangadi: I think where we are -- from where we are, it may not really go up by much. We'll probably be flat, maybe a slight increase, but I won't really count on it. So, a little early to tell you right now.

Moderator: The next question comes from the line of Madhav Marda from FIL.

Madhav Marda: Sir, my first question was on the -- if you could just give some sense on the growth -- top line growth versus margin metrics going ahead, if you had to think from a next, let's say, 2-, 3-year perspective, how is the sort of company thinking about driving top line growth for the pharmacy business along with the margins? If you could give some color in terms of how they could trend?

Madhukar Gangadi: Madhav, it's a little difficult for me to give you that right now. Our franchisee thing still is being piloted. Most of our growth should come from that side. Our normal growth, of course, would be to the tune of 600 to 1,000 stores, which we can put in without too much of a problem, given the overall market size and our own ability.

And the fact that we have money in the bank and everything else. So, please give us a little bit more time to really give you some proper color on what is going to happen in the next 2 to 3 years. I'd not be able to tell you right now.

Madhav Marda: Sorry, 600 to 1,000 stores is -- I mean, our own additions annually or over the next 2 years? I didn't follow that, sorry.

Madhukar Gangadi: That's annually.

Madhav Marda: Okay. Annual addition, 600 to 1,000 stores, you want to add to the network, which will be a mix of owned stores and franchisees in the next few years?

Madhukar Gangadi: And franchisee. But yes, this depends on a kind of a lukewarm kind of thing for franchisee. If franchisee really takes off, it could be much higher.

Madhav Marda: Okay. And how has the feedback been initially from some of the franchisees you've added, say, in the past year? Like if you could give some sense around the unit economics and how they are sort of faring, if you could give some sense there?

Madhukar Gangadi: A little early to speak about that, Madhav. I think we'll probably need at least another quarter or 2 for us to really draw any conclusions on that. The numbers are very low for us to really give you any meaningful kind of guidance on that or at least information on that.

Madhav Marda: Okay. Got it. And just one more question was on the opex. If you look at the pharmacy opex number, I mean, I just subtracted the EBITDA and the gross profit for the pharmacy business.

That opex number has grown a fair bit.

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So, have we front-ended some warehouse-related costs for like the people that we've hired for the new stores? Or is there some front ending of costs which has happened in 2Q for the expansions?

Sujit Mahato: Madhav, actually, the costs, if you look at are predominantly in the employee benefits line. As we had guided you earlier, the company had implemented a retention plan for the entire store-level workforce at least for the key cities.

And that's the major impact where we have seen where the first-year payment has just happened, where we had the plan was if an employee continues with us for more than 12 months, he or she is eligible for INR18,000 one-time cash bonus.

And if the employee rolls that for 1 more year and rolls it forward instead of encashing, that benefit increases to INR50,000. And if that is rolled for 1 more year, it goes up to INR1 lakh.

And that's the major chunk of increase what you're seeing other than the normal annual increment.

Madhav Marda: So, this sort of happened in 2Q. So 3Q, this number can actually come down a little bit like because then that payment is not going to

continue every quarter? Or how does that work?

Sujit Mahato: So the first year, for example, this would work more or less like an ESOP plan with various tranches. So for the first year, you have an estimate. But after a year, you start getting the true-ups or the actual catch-up depending on how many people exercise the option of roll forward versus encashment.

Once an employee encashes and the plan is still on, they go back into the queue and again start getting accrued for INR18,000 for the next year. Or if the employee rolls forward, then on a cumulative basis, you have to do an accounting for the INR50,000.

Moderator: The next question comes from the line of Tanya from Investec.

Tanya: Yes. So, congratulations for the set of numbers. My question is around the SSSG rate. So, what I can see is that it has made a turnaround in this quarter by registering a 2.2% growth as in the SSSG growth. What is your take on the same going ahead?

Sujit Mahato: Sir, we expect this to grow gradually because you saw the last couple of quarters, it did not show a positive traction. The company has changed the incentive plan and the other initiatives, adding supply chain capability. So, we expect this to grow gradually in the range and reach somewhere between the high-single digits of between 9% and 10%, but that should be a journey.

Tanya: And by what time frame do you expect it to reach the high-single digit?

Sujit Mahato: In the next 2 years, max, because this is above the entire network coverage. And as we speak, more than 80% of our network is in the greater than 24 months. So, we'll have to keep that at the back of our mind when we make this assumption.

Moderator: The next question is from the line of Amish Kanani from Knowic Investment Managers.

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Amish Kanani: Yes. Sorry. Partly my question related to GST and same-store sales have been answered. Just one question, which I just wanted to understand. Is there any benefit that we might get because of this GST rate cut on the FMCG part of our business?

And how does it work cross-sell and inherent demand, what type of products, if there is any -- some after-effects of the lower prices? Because I understand pharma, maybe the demand will not be elastic. But how do we see that?

Sujit Mahato: So on the FMCG or on the general front as well, directly, there is no impact because as you are aware, we report sales, net of GST. And also on the inventory, the inventory is accounted, net of GST. So, there is technically no impact on the P&L and balance sheet.

But as you rightly mentioned, if there are growth in the volume because of the overall uptick in the demand per se, we should get that fair share to our stores as well. But as of now, we have not seen any significant uptick on any volume.

Moderator: The next question is from the line of Saion Mukherjee from Nomura Securities.

Saion Mukherjee: Sujit, my question is on the expenses in general and employee expense in particular. So, we have seen almost 19% growth year-on-year with revenue growth at 7%. You mentioned about one-time bonuses scheme that you have launched. So, I just wanted to confirm whether this was launched in this quarter onwards?

And also, have you accounted for potential increases if the employee stays assuming some attrition rate? Has this been provided for? Basically, what I'm trying to understand is how should we think about this expense item sort of going forward?

Sujit Mahato: So, I think, on this question, Saion, we have been accounting this for the last 1 year, last 4 quarters. We have launched the scheme now close to a year ago. And I mentioned this is the first time where the payouts have happened, which means we have crossed the 12-month threshold. We have been doing an accrual basis of accounting on that. On a year-on-year, you see the impact now.

But otherwise, absolute

ly accruals have been factored with all estimates of how much do we expect will roll forward and how much we expect will encash. And only based on the due date, we are able to true up this number. But otherwise, we have factored this plan. Second, this is an ongoing plan as we speak. So it was not only a one-time, which means as and when people resign and new colleagues come in, they are also automatically getting enrolled in this plan. And therefore, the costs have been factored accordingly. So if I've answered you on this point.

Saion Mukherjee: So basically -- so there has been a structural change in the way you compensate the employee, which we have been doing it for a year now. So, going forward, this kind of a growth will come down, right? Because today, what we are seeing on the P&L is 19% growth year-on-year for employee expenses. So, the base is still adjusting.

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Sujit Mahato: Absolutely.

Saion Mukherjee: So next quarter onwards, it should start to moderate. The year-on-year growth in employee costs will start to moderate. Would that be a right assumption?

Sujit Mahato: Yes, it will start to moderate. Yes, that's the right assumption, Saion.

Saion Mukherjee: Okay. Okay. And on the pre-op expenses, so how large is that? And I mean, is it a meaningful dent on the P&L currently?

Sujit Mahato: One second. So in this quarter, it was close to around INR3.4 crores, INR3.3 crores. So, this was a factor of 2 things primarily. The employees who have been hired, not yet deployed anywhere, but are during the training program, which is for that 1-month period and rentals of premises, which we have leased, but not yet launched.

So, this should not exceed maximum of INR4 crores, Saion, because we continue to grow and therefore, there is always this number, which we classify as pre-op. But on a steady-state basis, this should continue to go down.

Saion Mukherjee: Okay. Okay. Just one more question on the employee. Have you seen any meaningful reduction in attrition after you rolled out the scheme? And if you can share the attrition rate now versus what it was a year back?

Sujit Mahato: So, I'll look into that. But at a very high level for in the major 3 cities where we launched this scheme, we have seen a significant amount of reduction because a couple of things, which we evaluated when the team looked at this model was more and more attrition pull from the, I would say, consulting model, be it a delivery point model or from a part-time store model vis-a-vis a full-time employment at our stores.

And therefore, we had upwards of 45% attrition in this section of employees. And after we implemented this, in some of the cities, we drastically saw a reduction of around 15%. So, this is being year 1. What we would like to see is how it plays out because there is also a ripple effect on the others. So, we want to see that how it plays out before we take any concrete steps on this plan.

Saion Mukherjee: Okay. And just one last question, sir, if I can ask. You mentioned about high single-digit growth rate, same-store kind of a growth rate 2 years down the line. Given that we are expecting the pharma market to grow at around 8%, don't you think you can get to double-digit growth, assuming you get market share from other retailers?

Sujit Mahato: No, I think, absolutely, but we did not want to sound so optimistic immediately. We want to go through this journey because you have seen now it's around 2.2% plus. So, I just want to be very cautious in terms of guiding. Otherwise, we would say maybe let's wait for a couple of quarters to be more optimistic on this number.

Moderator: The next question comes from the line of Aejas Lakhani from Unifi AMC.

Aejas Lakhani: Team, great job on execution and maintaining balance of margins this quarter. So, credit to you.

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Moderator: I'm so sorry to interrupt in between, sir. Your voice is not audible.

Aejas Lakhani: Yes. Is this better? Sorry. Is this better?

Moderator: Please proceed.

Aejas Lakhani: So team, I was just saying stellar performance in terms of the balance of growth and execution and margin maintenance. So, credit to the team for the same. I wanted to just understand a little bit more on the growth that you're seeing in private label, non-pharma.

So basically, your others, your branded segment and FMCG. Can you -- the growth there, I mean, if you trace it back to several quarters has been higher than that of private label pharma. So, could you tell me what exactly is driving the shift? Is it that the customer comes in, you don't have the product, maybe a branded product and they see your product?

And how do they sort of -- how is that buying decision and pattern there? Is it the price comparison that he's able to see, which helps him to pick it up? What is driving that decision?

And additionally, what are the margins on PL non-pharma and pharma? Could you just call those out?

Sujit Mahato: So on the PL non-pharma specifically, Aejas, what you rightly mentioned, couple of quarters ago, this was hovering around 7.5%, 7.8%. And we really grew steadily because one major aspect, if I can take you through is we have been guiding that the way we serve our customers in our stores, let's say, 18 months ago, we have changed that over period by bringing in the racks outside or the shelves outside wherever real estate allowed us.

So that there is some amount of display as well as buying by the customers themselves because pharma continues to be served from behind the counter, because most of the staff will have to pull out from our racks and give them based on the prescription.

But for non-pharma, since this restriction was not there, wherever real estate allowed us for more than, I think, 1,500-plus stores, we redesigned our outlets where we have now shelves outside, there's some amount of impulsive buying. And to add to that what Madhukar explained earlier that we have added a large assortment of new products on the FMCG side, which is constantly driving this growth.

To top that, there is technically no upper limit unlike in pharma, where once a doctor prescribes a regimen or a dosage for 3 days or 5 days, the customer is bound by that. But in a non-pharma, once the product is good and they see value in that, they can continue to buy those products. So, that has helped us, and I think the focus will continue on those lines.

Aejas Lakhani: Understood. And could you help on the margin build?

Sujit Mahato: Yes. So on the private label pharma, we are still close to 78% to 75%. This is before any store-level incentive is given out. And on the private label non-pharma, this would go up to 34%.

Aejas Lakhani: Okay. This is in gross terms, right?

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Sujit Mahato: This is on net revenue.

Aejas Lakhani: Okay. No, no. But in terms of an operating margin perspective, are you earning more on the non-pharma PL side versus the pharma PL side?

Sujit Mahato: No, no, absolutely.

Aejas Lakhani: That would be materially higher by, say, 5%, 10%. Could you quantify something like that?

Sujit Mahato: So, I mentioned the PL pharma would be close to 74% to 78% versus non-pharma would be in the range of up to 34%. So it's close to a 50% difference between pharma and non-pharma.

Aejas Lakhani: Okay. I'm sorry. Okay. I didn't catch that. Understood. And so could you just help me texturize this buying that is taking place in non-pharma? So let's say, it's a consumption item like, say, a toothpaste, Indians are indexed to 2 or 3 or 4 brands. So is it that the basis of purchase is non-availability? Or is it that they are able to see both the brands, compare the price and then price is the

driver of that decision? Could you help me with that?

Sujit Mahato: In terms of comparison for a customer, absolutely, for the brand, there is nothing more what MedPlus will do. But for our own products, the price and the first experience definitely drives the consumer demand.

So the pricing philosophy is very clear. It's cost-plus model rather than we doing a similar MRP model. So it is -- that is one. And then the customer experience, once they try out a product and they find good value for that, they do come back for that.

Moderator: The next question comes from the line of Ankit Singh from AB India.

Ankit Singh: Yes. Sir, my question is, sir, in this quarter comparison to last year and first quarter of this fiscal year, sir, inventory has been -- it has piled up a lot from -- I think in your P&L, it is showing approx. 150. Can you please tell me about that? Is there any change of trend are you seeing? What are you observing on that?

Sujit Mahato: I did not get your question. If you can please repeat?

Ankit Singh: My question is, sir, cost of inventories in your P&L section has been risen very significantly in this quarter compared to last year and compared to the first quarter of this fiscal year. Sir, what are you observing? Any change in trend or something else? It is going up very significantly.

Sujit Mahato: I did not understand this question. Are you talking on the inventory levels on our balance sheet?

Ankit Singh: Yes, yes, yes, yes.

Sujit Mahato: Okay. So if you see that, if you compare from a year ago, consistently, the inventory levels have been going down. So this is, again, a well-thought through plan internal monitoring where we are balancing between the private label inventory as well as in the branded, at the same time having a much sharper monitoring mechanism on our store level as well as on the warehouse

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level inventory, which I think over a period of time, the team has been able to optimize this

number. As such, there is no exception in these numbers, if I have to clarify.

Ankit Singh: Okay. Okay. Sir, what are your plans, sir? I've been listening it from last year, last 1.5 years from the listing, plans in expanding in north part of India, sir, Delhi side in North India. There is nothing you are telling about it. What are your plans now as you are listed for 2.5 -- 2 years of listing? Sir, what are your plans now north side of India?

Sujit Mahato: So currently, we have not expanded directly into the northern part of India, but we have started adding stores in Chhattisgarh and Madhya Pradesh. We are seeding our initial stores, and we will progress slowly because as you are aware, since the company is established more in the southern part of -- southern and eastern part of the Indian continent, there's still a lot of headroom to go deeper.

But at the same time, we take your point. We will internalize this because there is a plan to continue to grow. And as and when we add more states or adjacencies, we will come back and report to you.

Ankit Singh: Okay. Okay. Sir, one question more. Sir, are you getting competition from generic pharmaceuticals like I have been studying here, Knoll Private Limited in north part of India is also expanding in your cities like Hyderabad, Telangana. What is the level of competition are you getting from these drug holders?

Like they are being just selling it at very low point that I have observed in north part of India. They are selling at very cheap rate because -- by selling it generic. Are you -- because of this specific company, I know it. That's why I'm saying that it has also expanded in your Hyderabad, Telangana. Are you getting competition from these drugs?

Sujit Mahato: See, we are a retail chain. So, I think we trade or we partner with more than 2,000-plus companies. So obviously, we will not be able to comment specific

call you on any pharmaceutical

company. But we being a retailer, we would definitely try to fulfil the demand, whatever comes to our customers. And if those products are relevant, we will also sell them. That's all I can tell you.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to Mr. Sujit Mahato for closing comments. Over to you, sir.

Sujit Mahato: Thank you. I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you, and good day.

Moderator: Thank you. On behalf of MedPlus Health Services Limited, that concludes this conference. Thank you for joining us today, and you may now disconnect your lines.

Call: Feb 2026

MedPlus Health Services Limited

040-6724 6724

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February 04, 2026

The Listing Department
BSE Limited
Phiroze Jeejeebhoy Towers
Dalal Street, Fort,
Mumbai 400 001
BSE Scrip Code: 543427 The Listing Department
National Stock Exchange of India Limited
Exchange Plaza,
Bandra Kurla Complex,
Bandra (East), Mumbai – 400 051
NSE Symbol : MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Transcript of Earnings Call:

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the Earnings Call held with analyst/ institutional investors on February 02, 2026 for the quarter ended December 31, 2025.

The same will be available on the website of the Company at www.medplusindia.com and also on the websites of BSE Limited and National Stock Exchange of India Ltd. viz. www.bseindia.com and www.nseindia.com respectively.

Thanking You
Yours faithfully

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer

Encl: a/a

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"MedPlus Health Services Limited
Q3 FY '26 Earnings Conference Call "
February 02, 2026

Management: Mr. Gangadi Madhukar Reddy - Chief Executive Officer and Managing Director
Mr. Sujit Kumar Mahato - Chief Financial Officer
Mr. DRN Srinivas - Sr. Manager Finance

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Moderator: Ladies and gentlemen, good day, and welcome to the MedPlus Health Services Limited Q3 FY '26 Earnings Conference Call. As a reminder, all participant clients will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchstone phone. Please note that this conference is being recorded. I will now hand the conference over to Mr. D.R.N Srinivas from MedPlus Health Services Limited for opening remarks. Thank you, and over to you, sir.

D.R.N Srinivas: Thank you, Ryan. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to MedPlus Q3 FY '26 Earnings Conference Call to discuss the financial results of MedPlus for the third quarter of FY '26, which were announced earlier. We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, Chief Executive Officer and Managing Director; and Mr. Sujit Mahato, CFO.

Before we begin, I would like to mention that some statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website.

I would now hand over the call to Sujit. Thank you, and over to you, Sujit.

Sujit Mahato: Thank you, Srinivas, and good evening, everyone, on this call. We have opened 228 stores during the current quarter, and there were 46 store closures. Out of 46 closures, 17 stores were relocated. 7 stores are in the process of getting converted into franchisee. 10 franchisees we saw withdrawals and 12 stores are closed for other reasons.

We achieved a net addition of 182 stores during the quarter compared to 117 stores added during the last quarter. During the current financial year, we have achieved net addition of 400 stores. We continue with the outlook for adding 600 new stores in FY '26.

Update on the network. In terms of our store network age, around 23 % of our stores have been operational for less than 2 years, and the remaining 77% of our stores have been operational for 2 years or more. In terms of the network size, at the end of the quarter, our network grew to 5,112 stores with 2.6 million plus square feet compared to 4,612 stores and 2.4 million square feet at the end of December '24. The average store size is 527 square feet.

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Update on the revenue mix. Presently, MedPlus offers a large range of SKUs spanning across pharmaceutical and non-pharmaceutical categories. Private label sales for quarter 3 FY '26 constitute 22.2%, Pharma being 11.6% and FMCG being 10.6% of our total revenues.

On GMV basis, during the current quarter, the share of the private label pharma sales stood at 18.9% compared to 7.9% prior to the launch of MedPlus branded pharmaceutical products. That was in last year's quarter 1. An update on financial numbers. Our consolidated revenue for the quarter stood at 18,061 million.

Our consolidated operating EBITDA for the quarter after considering the nonrecurring charge of 70.59 million on account of the implementation of the new Labour Code stood at 96.8 crores, representing 5.4%. Revenue from pharmacy operations grew by 15.6 % year-on-year on a reported basis. The pharmacy operating EBITDA stood at 925 million, representing 5.2%.

An update on our stores performance, stores older than 12 months. Revenue from these stores in quarter 3 was 16,300 million, representing 96% of our pharmacy revenues. These stores had

a store level EBITDA margin of 12.4%. A word here on the store level EBITDA margin by age. While stores greater than 12 months had a margin of 12.4%, this was 12.6% for stores greater than 24 months and 8.8% for stores in the 13 to 24 months age bracket. On allocating all non-store-related costs, the operating EBITDA of stores greater than 12 months would be 967 million, which translates to a margin of 5.8%. On working capital, our net working capital for quarter 3 was 53 days. The inventory in our warehouse stood at 34 days. In quarter 3, the inventory level of our first-year store was 103 days. In comparison, for a store older than 12 months, the inventory was around 35 days.

An update on our diagnostic numbers. Diagnostics revenue grew to 326.7 million in quarter 3 FY '26 compared to 274.7 million in quarter 3 FY '25. Diagnostics segment recorded an operating EBITDA of 50.7 million compared to 22.1 million in quarter 3 FY '25. This translates to 15.5% as an operating EBITDA margin. In October, we sold 506 gross plans per day. In November and December, this was 529 and 528, respectively.

As on 31st December, we had 1,80,000 active plans covering 3,68,000 underlying lives. As on 30th September, we had 1,70,000 active plans covering 3,51,000 underlying lives. Our current observed on-time renewal rate is 23% in Q3 versus 24% in the previous quarter.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: We take the first question from the line of Riddhansh Chandak from Unifi AMC. Please go ahead.

Riddhansh Chandak: Hi, team. Congrats on a stellar number. My first question is that, Madhukar, could you explain that 10% plus SSSG, we've seen that after some time now. And you also spoke about in your opening comments, Sujit mentioned the 18.5% private pharma GMV sales. So could you give some more color about what's driving that?

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Sujit Mahato: So I think that's a great question. What we had articulated earlier, while we had seen a slowdown in the greater than 12-month stores. The company tweaked the incentive arrangement at the store level, and we are seeing that paying off dividends, and this is in the right direction. So at least there are 2 reasons for this SSSG growth. One, we started with a bit lower base of the previous quarter in the last year. And two, the impacts of the changes in the incentive structure and a bit of availability improvement due to the implementation of the new warehouses.

Riddhansh Chandak: Understood. Sujit, so you have alluded to the incentive structure change, but I thought that you had reached the desired level of private label -- private pharma sales and you were hoping that branded would again start to pick up. So did we make those tweaks in this quarter or the old -- or the revised incentive structure continues, which is continuing to drive the private pharma sales?

Sujit Mahato: No. As we had indicated earlier, we have tweaked the incentive structure so as to consider the total sales growth at our store level, which is paying off dividends. We are clearly seeing the improvement in the branded pharma uptick as well as the uptick in the private label non-pharma, which is helping us to achieve these numbers.

Riddhansh Chandak: Understood. Second is, could you speak about the margin profile a gain has been very strong this quarter and congrats on excellent execution. So how do you expect gross margins to continue to trend for 4Q and next year along with the operating EBITDA?

Sujit Mahato: Yes, I think we do not generally give guidance, but on the gross margin level, we expect it to remain at the same level.

Riddhansh Chandak: Understood. Thirdly, could you just speak a little bit about what's happening on your franchisees that the expansion on that? Is this an opportune time to start speaking about it? Or should we

Id we

expect more color after 4Q?

Sujit Mahato: Yes. I think after the full year, we would like to get into that. But as you rightly picked up, that initiative is ongoing. And we are working with a couple of models, a couple of partners to see which is the right fit and how does it fit into our overall strategy. But that being said, that initiative is currently ongoing.

These numbers include the impact of franchisee and distribution sales, which is captured in the presentation in the other segment, which currently, if you see the others would be in the range of 5.7% of the total revenue. And this alone is now 3.7%. As a comparison, earlier, this was 1%.

Riddhansh Chandak: Understood. So incrementally, to understand the run rates of the franchisee, you'll be reporting it under others so that we get more clearer sense. Is that broadly correct?

Sujit Mahato: Yes.

Riddhansh Chandak: Understood. And could you just call out quantitatively how many franchisees were there this quarter? How many -- any color on those things, at least if you can quantify?

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Sujit Mahato: I think going forward, we would like to do a comprehensive disclosure on that. But for the moment, I think we will continue with this, yes. We can connect offline.

Moderator: We take the next question from the line of Umakant Sharma from Viansh Ventures. Please go ahead.

Umakant Sharma: Congratulations on a fantastic set of numbers. A couple of quick questions. Could you just talk a little bit about the GMV growth that we have seen in the quarter on a consolidated basis?

Sujit Mahato: I have to take it offline. I do not have it handy.

Umakant Sharma: Okay. Sure. I can take that offline. Parallely, just on a thought process standpoint, this quarter, we saw substantial growth in your non-branded non-pharma business, right? We saw growth of about 48%. Could you just walk us through how does the margin profile look along the branded pharma versus the non-pharma on the branded side? And parallelly, the unbranded pharma and the unbranded non-pharma, what is the margin profile across the spectrum look like on a net revenue basis?

Sujit Mahato: Sure. So on the reported basis, branded pharma would give us around 13% to 14% gross margin.

Branded non-pharma would be more or less in the similar range. For us, the private label pharma, the old as well as the new MedPlus brand put together would anywhere be between 65% to 70%.

And the private label non-pharma would be around 25% to 28% gross margin.

Umakant Sharma: 25% to 30% you said?

Sujit Mahato: Yes.

Umakant Sharma: Okay, great. And after a very long time, we are seeing a very good growth in terms of the store expansion side. Could you just throw some color on what the guidance could be for next year on FY '27 as well. So you may be guiding the target of 1,000 number on a full year basis for FY '27?

Sujit Mahato: Sure. Currently, we are not guiding. But yes, once we finish our internal AOP process, maybe at some point in time or next quarter or beyond that, we will definitely let you know.

Umakant Sharma: Okay. But any ballpark numbers that you are looking at for next year, sir?

Sujit Mahato: It should at least be the similar numbers of this year.

Umakant Sharma: Okay. Got it. Sure. And structurally for next year, do we have any guidance in terms of the top line that we're expecting. How should we be seeing the mix change between the four key areas between the branded, unbranded side?

Sujit Mahato: We do not provide such guidance, I'm sorry, actually.

Moderator: We take the next question from the line of Sudarshan Agarwal from Axis Capital. Please go ahead.

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Sudarshan Agarwal Congrats on the great set of numbers. So one of my questions is regarding SSSG. So you mentioned that base year growth was kind of low. So just kind

of extrapolating Q4 was actually

a decline for you in SSSG. So that should mean that SSSG at least for the near 3 to 4 quarters will remain at current levels or kind of improve also? Is that the right kind of conclusion to be taken away?

Sujit Mahato: I think, Sudarshan, your observation is right. At least for the next 1 quarter, I have a very clear visibility, but we will see as we move forward.

Sudarshan Agarwal Okay. And on the gross margins, I understand -- so on the gross margins, I understand that the private label pharma has kind of gone down a bit. But on the other hand, you have your non-pharma private label that has gone up. So the delta in gross margins, I think we used to allude that every 50 basis points adds around 10 to 20 basis points of gross margin. That is relative to pharma private label or overall private pharma label would also add that kind of level?

Sujit Mahato: Relative to the pharma private label, but you are right, since there is no substitution being done on the private label non-pharma, the overall increase in the sales and the gross margin will be accretive to that extent. Pharma, there is a possibility that there could be a switch from the brand to the private label. But in non-pharma, we see that very low. That could be only incremental sales and incremental margin.

However, from an optics, the gross margin stood at the similar level, but that had a small impact of our franchisee as well because we sell to the franchisee at a lower margin. And that after netting of that, and the increase in the private label non-pharma, we were able to continue with a similar gross margin level.

Sudarshan Agarwal Got it. Got it. One more request. So on the private label, I understand that you are still kind of working around with the incentives, etcetera. But let's say, 2, 3 years out the line, do you have a visibility right now, you would want it to be, let's say, 25% or 30%. Is there some number that we still are looking to kind of get to either pharma or non-pharma?

Sujit Mahato: As this is a growth journey, Sudarshan, you'll also agree, our numbers will be aspirational, but we are not giving any guidance at the moment because non-pharma by design, there's no cap because at least on the pharma, there is a cap theoretically because there is a doctor's prescription,

which we need to fill.

But on the non-pharma, if the experience is good, the product is good, the availability is good and the quality is good, there is no cap per se in terms of how far this could go. And the company continues to add new assortments, new categories, which is helping the sales and the gross margin.

Moderator: We take the next question from the line of Gaurav Nigam from Tunga Investments. Please go ahead.

Gaurav Nigam: Just one question. When I was looking at non-store expenses, they seem to have gone up materially and not only on year-on-year, but also on the quarter-on-quarter basis also. Can you please explain? And is this run rate inching up going forward? How should we think about that?

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Sujit Mahato: So in corporate expense line, there includes a one-off nonrecurring expense also we'll have to be mindful of that 70 million or INR7 crores, which was the impact of the past service cost post the implementation of the new Wage Codes. So that's a one-off. Going forward, it should be more or less in the same range because, one, we'll have to be mindful of what could come, those notifications and the rules are still not yet final.

So we are looking, like others, we are keeping -- closely monitoring this space. And maybe by end of March, end of April, we should have more clarity. But as we speak, we are on a very good footing.

Gaurav Nigam: This INR7 crores is onetime? Or is it expected to continue?

Sujit Mahato: It is onetime, you're right.

Gaurav Nigam: Okay. And sir, all the warehouses a

nd the other logistics improvement that we were doing, is that all open and that cost is in the base now?

Sujit Mahato: At least 60% to 70% of the new warehouses have been operationalized. The rest will also get operationalized in the next two quarters. The manpower recruitment for all these warehouses are complete. So we do not expect any significant ramp-up on those expenses on those lines.

Moderator: We take the next question from the line of Madhav Marda from Fidelity International. Please go ahead.

Madhav Marda: On the FMCG side, could you give a little bit more color in terms of what are the new assortments or categories that we're adding? And what's sort of helping this growth which can sustain in the coming times?

Sujit Mahato: I think, Madhav, we'll take this offline. But yes, what we have been adding now is more of the food category, the wellness category, some cold pressed oils, which are in -- again in the wellness category. So these are the few categories we have started adding, and we are seeing good traction.

Madhav Marda: Understood. Got it. And sir, just on margin side for us, I think this quarter, if we adjust for that one-off of the 70 million, pharmacy operating margins are about 5.5%, 5.6%. Like you said, a lot of the cost of the warehouses is already in the base now. So are we sort of within striking distance of getting to 6% in the next couple of years? Is that a fair way to think about it, given some of the operating leverage plays out with the sales growth. And given SSSG is sort of now improving at a good pace as well?

Madhukar Gangadi: So, Madhav, our margins, whatever we actually start improving on will be largely a function of the private label. And on private label, I actually am now equally bullish about the non-pharma side. Non-pharma is just a matter of adding categories out there. We are using, let us say, getting the full benefit of the 5,000 stores, which basically means we have the minimum amount of quantity for almost any product.

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So as long as the product is something which people will buy in the regular convenience or wellness category, we are more than happy to actually put it out there. And I feel at least given the last couple of quarters' performance, I see no reason why we should not get to that number.

But yes, and again, on the pharma side, while we fully expect this to continue to grow up, I expect that to happen as a result of, let us say, the quality of the product, as a result of all the positive publicity we expect to get from customers who are already using it. So we have significantly reduced the push on the private label pharma, but we continue to be, let's say, bullish about the non-pharma side.

Madhav Marda: Okay. Okay. So, when you said we can get there, as you talked about the margins, that there is potential for us to reach the 6% over a period of time?

Madhukar Gangadi: See, I don't see any reason why not. It's a little early to say. There are a lot of pieces out there on the ground. But given that only 20% of our sales now come from general goods and most of our

competitors are in the range of around 30% to 40%, even 50 %, even a slight jump in the general route side and most of it coming from private label will not only -- will definitely increase the top line and also will improve the profitability. Yes, I feel confident about that, but I can't really give you an exact time line in which this will happen.

Moderator: We take the next question from the line of Raman KV from Sequent Investment. Please go ahead.

Raman KV: Sir, I just want to understand, you have mentioned that you have tweaked your incentive structure, which helped your SSSG growth to be in the range of 10.5 %. Can you throw some light on what are the incentive structure that you tweaked?

Sujit Mahato: So earlier, the incentive structure was predominantly working around the private label sales.

What we have tweaked is we have now included a component of total sales at the store level, which means the branded as well as the private label sales.

If the total sales are not achieved or let's say, there are targets linked to that, then the achievement percentage on the private label goes down by certain basis points, and that keeps motivating the store level employees to achieve both the private label as well as the branded total sales growth at the store level, and this is helping us in the overall journey as well.

Raman KV: Understood, sir. And sir, I just want to understand the nature of the goods or basket of goods under the non-pharma business like branded as well as private non-pharma business, what are the products which -- that you are launching? And is it the growth in this business is recurring.

Or is it because you are introducing new product and your total addressable market is expanding?

Sujit Mahato: So the product range in the non-pharma, both branded as well as private label is a large assortment of all our daily needs, right from your early morning, let's say, the needs of toothbrush, toothpaste, hair oil, cream, toilet cleaners, soap, shampoos. So there's a large assortment of, I would say, 1,300-plus SKUs, which we offer to the customers, great quality and great affordable price. So that is helping us.

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And to it, which what we mentioned earlier, we keep on adding new categories depending on what the needs are, what we identify at the store level, where there is a regular need and a good product available. So that is what is helping our incremental sales on the non-pharma products.

Moderator: We take the next question from the line of Sanjay from Bastion Research. Please go ahead.

Sanjay: Sir, I just wanted to have a bookkeeping question. So we have stopped giving store level MRP growth for stores within 12 months. Could you please share that data?

Sujit Mahato: So it was not making sense, Sanjay, because you would agree that since the change in the GST rate, there is a flat reduction of the GST by around 6.5%, 7%, which makes comparability a big issue. And that is the only reason we removed that. But otherwise, we would have to do a manual adjustment and then give you a number which is not directly comparable.

Sanjay: Understood, sir. Okay. Sir, also wanted to understand that we are going much deeper into the states which we are already present. So I see the store expansion are largely into our states which we are present and have enough presence on that specific. Do we plan on that side. Or going forward, how is the store expansion we are looking forward? I understand that we follow a cluster-based approach. So -- but I wanted to have your sense on how we are planning to new store or new geography or new city, so to say?

Sujit Mahato: So the priority always would be going deeper into the existing states and then sweating the warehouse assets, which we have created for these states and then slowly getting into adjacent states. We had started Chhattisgarh, we have started Kerala. But yes, the main new stores where you can see now is part of the densification of the existing presence where we have. So it's an ongoing strategy, Sanjay.

Sanjay: Sir, you also shared in your opening remark or somewhere you shared the revenue for stores greater than 12 months. Did you share overall for the last quarters as well because I'm not seeing that. So if you can include for the quarter-on-quarter and Y-o-Y that revenue for greater than 12 months, it would be really helpful. Thank you.

Sujit Mahato: So at the overall level, we have given. But if you're only -- the specific thing we have added is for greater than 12 months as a bucket. But overall, we have anyhow given you both in the investor deck as well as in my call. But I did not get your question

if you're asking other cohorts.

Sanjay: Sir, I'm asking for greater than 12 months only because in the previous comments or previous call, we did not spoke to revenue greater than 12 months stores. So just wanted to have your Y-o-Y and quarter-on-quarter numbers on that side?

Sujit Mahato: So we'll look at it. I'll take this offline.

Moderator: We take the next question from the line of Divyansh Gupta from Latent PM S. Please go ahead.

Divyansh Gupta: A couple of questions. With the change in the incentive structure, the previous guidance that we

Does that still hold or that will -- that should also be -- is there any new reference point you want to share?

Sujit Mahato: I think we can continue with the same assumption. There's absolutely no change on that.

Divyansh Gupta: Got it. Got it. And the second question was, I think in the last con call, we had mentioned with for that employee retention, we had introduced a scheme and probably this quarter would be the first quarter where we would actually see the results of exercise. So if you can share some insights on that part. Is it working as per plan or...

Sujit Mahato: So it's an ongoing scheme, which is going to run for 3 years, which when we launched for only three cities. So that's again one initiative which we are very closely monitoring to see the effectiveness. Based on that, the company would then take a call whether to extend that across all the regions or curtail that further.

Divyansh Gupta: But my question was that as it -- given that there was -- the exercise timeline was in this quarter, has it panned out as per our expectations even in the final?

Sujit Mahato: There is an exercise, I'll explain to you. The first exercise was post 12 months an employee getting into this plan. And we had the first exercise last quarter, wherein we saw a segment of employees exercise their right to claim that first 18,000 and the segment of employees who have postponed their rights to claim 50,000 in year 2.

And we are seeing improvement in attrition rates in the three cities which we have implemented.

I also mentioned that the company is closely monitoring this initiative to see whether it would make sense to continue this, expand this or further curtail this. So all these are still open.

Divyansh Gupta: Got it. Understood. And sir, you had mentioned the gross margins for the various product categories on a gross basis. What would be the -- what can be the margin on a net basis?

Sujit Mahato: Whatever I mentioned is on net basis. It's all on net basis.

Divyansh Gupta: I thought private firm -- it's on net basis. Got it. Got it. Understood. That's all right. Thank you.

Moderator: We take the next question from the line of Lakshminarayanan from Tunga Investments. Please go ahead.

Lakshminarayanan: A few questions. I just want to understand what's the kind of mix of the repeats you have in your pharma side? And the second question is that in your old stores, in the pharma, what kind of repeat business you actually get in a sense that, let's say, the customer comes back and buys in the last 1-year or whichever way you qualify or quantify?

And the third, since we have a significant amount of private non-pharmacy also private label, what kind of inventory write-off or damages you actually occur every quarter and how you plan to bring it down? These are the three questions.

Sujit Mahato: Sure. Thank you. Thank you for your questions. To start with, in both the entire private label pharma portfolio consists of both the acute as well as the chronic segment. I would say we are MedPlus Health Services Limited

more to 55 chronic and 40, 45 on the acute. So we are all the top five therapeutic areas as listed by WHO

, we are all in all those top five categories.

You can take a diabetics, cardiovascular and the hypertension. So all these areas are completely covered by our chronic range. And the acute, as you are aware, a gain, around 40%, 45% of our sales comes from the acute range. Coming to the private label non-pharma, absolutely, there's a large assortment.

And you also mentioned about the inventory risk. You're absolutely right. On the private label, the inventory risk is completely on our book. As a trend, we have been providing close to 0.9% to 1% of our sales of private label as a provision for deterioration in inventory.

Lakshminarayanan: Got it. And in terms of repeat business, can you just give some views on that?

Sujit Mahato: Yes. As we speak, we have more than 46 lakhs to 47 lakhs of members on the pharma side itself. That actually gives us a very strong conviction of our repeat business. Customers become members to access the trade discount, which is offered by the company on the private label product as well as a higher discount on the branded pharma product.

And once they become members, they get full access to these discounts for 1-year. What we are seeing is on the both private label, pharma and non-pharma, a very good repeat business every once in 3 months. This is the exercise what we do internally. We are seeing complete repeat business of close to 90%.

Lakshminarayanan: Got it. Got it. And last one more question. I see that these days, you are able to provide or service the online pharma very quickly. Can you just help me understand whether this is like a one-off or you have actually consciously improved your serviceability in the online purchases?

Sujit Mahato: We are also looking at this space very carefully because you would appreciate the competition or the offerings by the quick commerce. So we have also ramped up our offerings on this space. But as and when needed, we can further improve it. But I think what you have experienced is a positive story for us if you have seen that improvement. We are also consciously working on that.

Moderator: We take the next question from the line of Madhav Marda from Fidelity International. Please go ahead.

Madhav Marda: Sir, just one other question. Our working capital has come down a fair bit, almost 10 days year-over-year. Could you explain why that has come down? Is it sustainable at these lower levels going ahead?

Sujit Mahato: See, I think a couple of things. One, the -- wherever we have opened the franchisee model stores, we are not carrying inventory on our books. It is sale on first day. To that extent, roughly 15 lakhs to 18 lakhs of inventory per store, that's a reduction which we have. Otherwise, we were carrying on our books. That's one.

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And on the second hand, we have been mindful of the inventory what we are carrying both on the store level as well as at the warehouse level. And that -- those improvements are very clearly getting reflected on the number of days. On top, we have added only 400 stores. To that extent, a year-on-year comparison shows you a better benefit because cash has been conserved.

Madhav Marda: Okay. Does the higher private label mix help as well. Because I remember in one of the earlier commentary you mentioned for private label structurally, we need to carry lower inventory at the stores because the SKUs come down. So is that helping at all or not really?

Sujit Mahato: So currently, we are carrying both the inventory, Madhav. So we have not yet what we had discussed earlier. At a certain point, we can then cut the tail or the long tail of the brand. But for the moment, we have not so that we ensure full availability to our customers.

Madhav Marda: Understood. Just last question on the gross margins for the pharmacy business. I think it's at about 25% odd. Sort of this should keep gradually moving

up as the private label mix goes up,

right? That's just the way to think about this line item?

Sujit Mahato: Absolutely. That's what Madhukar explained, you're right.

Moderator: We take the next question from the line of Akhil Parekh from B&K Securities. Please go ahead.

Akhil Parekh: Many congratulations on an excellent set of numbers. My first question is, I know you're not guiding for next year, but for our business model, what is a sustainable SSSG growth rate given that we are at 5,000-plus stores and our private label business has fairly stabilized. Any guidance or any ballpark thing basically which one should look at basically whether it's high-single-digit or low-double-digit SSSG is sustainable? That's my first question.

Sujit Mahato: I think that's the aspiration which we are internally working with. But we do not give any such guidance because, again, we have seen the store network getting matured over period. And that's where we are also looking at what measures we can take at our store level, including the back-end efficiency so that we can continue this healthy SSSG and aspire for much more. So at least for this moment, we are not giving any guidance.

Akhil Parekh: Okay, sure. Second and last question, if I look at our sales contribution from metros, right, has continued to decline over the last many quarters and non-metro and Tier 2, Tier 3 has gone up.

Is it largely to do with the -- our private label story because what I understand is the adoption of private label is much better in Tier 2, Tier 3, Tier 4 towns?

And hence, as we kind of increase our sales contribution from private label, kind of -- we'll kind of continue to see that expansion more into the non-metro cities. So is that understanding correct?

Sujit Mahato: I think it's not a straight answer, but what we could also keep in mind, we need to be mindful of the net realization on our private label pharma, especially. Earlier, we were able to realize around 83, 100 minus 17 of blended discounts, we were able to realize 83 vis-a-vis now 45, 46. So that has to play out for a couple of quarters to have a like-to-like comparison.

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Moderator: Thank you. Ladies and gentlemen, with that, we conclude the question-and-answer session. I now hand the conference over to the management for their closing comments.

Sujit Mahato: I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you.

Moderator: Thank you. On behalf of MedPlus Health Services Limited, that concludes this conference call. Thank you for joining us, and you may now disconnect your lines.

Call: May 2026

MedPlus Health Services Limited

040-6724 6724

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May 26, 2026

The Listing Department
BSE Limited
Phiroze Jeejeebhoy Towers
Dalal Street, Fort,
Mumbai 400 001
BSE Scrip Code: 543427 The Listing Department
National Stock Exchange of India Limited
Exchange Plaza,
Bandra Kurla Complex,
Bandra (East), Mumbai – 400 051
NSE Symbol : MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Transcript of Earnings Call:

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the Earnings Call held with analyst/ institutional investors on May 21, 2026 for the quarter ended and financial year ended March 31, 2026.

The same will be available on the website of the Company at www.medplusindia.com and also on the websites of BSE Limited and National Stock Exchange of India Ltd. viz. www.bseindia.com and www.nseindia.com respectively.

Thanking You
Yours faithfully

For MedPlus Health Services Limited

Gangadi Madhukar Reddy
Managing Director and Chief Executive Officer
DIN: 00098097

Encl: a/a

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"MedPlus Health Services Limited
Q4 FY26 Earnings Conference Call "
May 21, 2026

Management: Mr. Sujit Kumar Mahato - Chief Financial Officer
Mr. DRN Srinivas - Sr. Manager Finance

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Moderator: Ladies and gentlemen, good day and welcome to FY26 Q4 Earnings Call of MedPlus Health Services Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchstone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. DRN. Srinivas. Thank you, and over to you, sir.

DRN Srinivas: Thank you, Iqra. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to MedPlus Q4 FY26 Earnings Conference Call to discuss the financial results of MedPlus for the financial year FY26, which were announced earlier.

We have with us today the senior management represented by our CFO, Mr. Sujit Mahato.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties.

Please note the disclaimer mentioning these risks and uncertainties on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website.

I would now hand over the call to Sujit. Thank you, and over to you, Sujit.

Sujit Mahato: Thank you, Srinivas, and good evening, everyone on this call. The update on our network with regards to openings and closures, the company opened 618 stores net additions during the year.

During the current quarter, we have opened 295 outlets and there were 77 store closures, including 18 related to relocation cases and 13 franchisee outlet closures.

We achieved a net addition of 218 stores during the quarter compared to the 182 stores added during the last quarter. For FY27, company plans to open 800 net new stores, including franchisee outlets. An update on our network. In terms of the age of our stores network, around 24% of our stores have been operational for less than 2 years and the remaining 76% of our stores have been operational for 2 years or more.

On the store size of our network, at the end of the quarter, our network grew to 5,330 stores with 2.8 million plus square feet compared to 4,712 stores and 2.4 million square feet at the end of March '25. The average store size is 528 square feet.

On the revenue mix, presently, the company offers a large range of SKUs spanning across pharmaceutical and non-pharmaceutical categories. Private label sales for Q4 FY26 constitute 22%, pharma being 11.4% and non-pharma constituting 10.6% of our total revenues. Our consolidated revenue for the quarter is INR18,644 million.

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Our consolidated operating EBITDA for the quarter stood at INR1,076 million, representing 5.8%. Revenue from pharmacy operations grew by 23.4% Y-o-Y on a reported basis. The pharmacy operating EBITDA stood at INR1,022 million, representing 5.6%.

An update on our store performance. Stores older than 12 months. Revenue from these stores in quarter 4 was INR1,606 million, representing 96% of pharmacy revenue. These stores had a store level EBITDA margin of 13.1%. A word here on the store level EBITDA margin by age.

While stores greater than 12 months had a margin of 13.1%, this was 13.3% for stores greater than 24 months and 9.3% for stores in the 13 months to 24 months age bracket.

On allocating all non-store-related costs, the operating EBITDA of stores greater than 12 months would be INR1,133 million, which translates to a margin of 6.7%. An update on working capital.

Our net working capital for quarter 4 was 53 days. The inventory in our warehouse was 30 days. In Q4, the inventory level of our

first year stores was 112 days. In comparison for our stores older than 12 months, the inventory was 36 days.

There has been a corrigendum and would like to update. In Slide 3 of our investor presentation,

request you to please read operating cash flows as INR4,956 million instead of INR917 million for the full year FY26 and OCF by operating EBITDA ratio at 135.5% instead of 85.2% for the full year FY26. On our diagnostic numbers, Diagnostics revenue for the year is INR1,309.9 million and operating EBITDA is INR196 million. Diagnostics revenue for the current quarter grew to INR347.8 million compared to INR280.8 million in quarter 4 FY25.

Diagnostics segment recorded an operating EBITDA of INR53.1 million, representing 15.3% compared to INR34.3 million in quarter 4 of FY25. In January, we sold 537 gross plans per day. In Feb and March, this was 603 and 554, respectively. As on 31st March, we had around 2 lakhs active plans. Our current observed on-time renewal rate was 21% in quarter 4 versus 23% in quarter 3.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Sanjay from Bastion Research. Please go ahead.

Sanjay: Hi, sir. Thank you for the opportunity and congratulation on a good set of numbers. Sir, my first question would be our number of store closure is continuously on an increasing trend. So how one should view this? And also our franchisee in model store withdrawal is also on uptick. So can you throw some light on this or your input on this?

Sujit Mahato: Sure. Thank you. Thank you for your question. In terms of our store closures, this quarter, we had a few stores also closed down, which relates to the performance. We waited for three-year plus stores and where we felt that we were not able to revise them to the level of the company's expectation, around 26 to 27 stores we closed down. Rest all, we have given the reasons in the investor deck.

Sanjay: Okay. Sir, my another question would be in the previous calls, we have highlighted that we are rearranging our sales due to which our private shares had significantly declined in Q4 FY26.

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So going forward, as our previous guidance of increasing private label by, you know, 20 basis points to 30 basis points every quarter-on-quarter, is that guidance intact? Or how should we view the private label shares going up going forward?

Sujit Mahato: So I think that guidance continues to be intact. What we did for the last two quarters is stabilize the overall growth where we also had to balance out between the branded products as well as the private label sales where we had informed the investors that we were restructuring the incentives. Now we have seen that it's working the way we expect it, and we'll be back to the trajectory where we had promised that every quarter, we can expect that kind of a growth on the private label as well.

Sanjay: Thank you so much. My last question would be the current growth in the revenue and the margin, can we expect in the FY27 as well? Or how should we view that margin and growth revenue growth going forward?

Sujit Mahato: Yeah, as a principle, the company does not give guidance, but there is no reason to believe that we will not continue in the same manner.

Sanjay: Thank you so much, sir. Thank you. All the best. Thank you.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee: Yeah. Thanks for taking my question. The 800 new stores that you have guided for next year, how do you split between your own stores and franchisees? And what was the split for 600-odd stores that you opened this in FY26?

Sujit Mahato: So Saion for the current year, out of the 60

0 around 30-odd stores we have done the franchisee model. Again there, we had guided that we will do an experiment on this. We continue to do that and continue to refine the model.

The second question, the second part is on the next year. As we speak, I don't have a split of that, Saion, but as we progress, maybe we will share that information. But at the moment, it is 800, which includes both the company stores as well as the franchisee outlets.

Saion Mukherjee: Understood. Just to understand the impact of this. So overall, of the 5,000-odd stores you have, the total franchisee would be now just around 300, right? Because this is the first year you had meaningfully added franchisee stores. Is that correct?

Sujit Mahato: So the old ones and the new ones all put together should be in the range of 500 now 500 plus.

Saion Mukherjee: Okay. Okay. So this new addition of franchisees should not be a drag on your profitability? Is that a right assessment because I think on the books...

Sujit Mahato: The original model was that while the company stores continue to grow and we venture into franchisee as a faster growth adjacency, we will be able to, you know, make significant or sufficient margin what we were making with the company-owned stores at the store level. And

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we are in the same, I would say, trajectory or direction. The only additional good point is in our own company-owned stores, at least for the year one, we were making losses and then we were slowly climbing up. But in the franchisee model, right from day one, we are increasing.

Saion Mukherjee: Understood. Sir, one observation that you know, your margins have been increasing. So for stores greater than 12 months, if I call them mature stores, at store level 13% margin, at you know, operating level 6.7%. You also mentioned private label, which sort of got stagnated a bit will start to grow again. From a slightly long and then now you have franchisee also adding so where should we see operating EBITDA margin over the course of the next 3 years, 5 years, if you can help us think elaborate those things will show?

Sujit Mahato: See at least for the next one year, the way we have been discussing internally is we would first try to stabilize this and maybe deliver again the same 5.7% to 5.8% and then keep growing. So the plan is to add more assortment on the private label non-pharma side as well because there we believe the size and traditionally, we have been lower on that count when compared to our other peers and competitors. So that is one area which the company would focus in the current fiscal and going forward that on a rupee absolute basis, how can we keep adding the private label non-pharma, which will definitely add to the top-line as well as to the bottom-line.

Saion Mukherjee: Just one last question, if I can, you know, ask, is there any impact of GLP-1 launches or any comments that you have on that particular market? And anything for MedPlus, which can be a positive here?

Sujit Mahato: We did not track at that level, Saion. Maybe we will do a separate deep dive and maybe we'll come back to you.

Saion Mukherjee: Okay. Thank you.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Sudarshan Agarwal from Axis Capital. Please go ahead.

Sudarshan Agarwal: Yeah. On GLP, how is your cold chain storage kind of, you know, set up? Do you have enough, you know, in terms of warehouses and transport? Do you have that setup already, kind of, built up? Yeah, that would...

Sujit Mahato: So we always had this I think, Sudarshan, we always had that infrastructure in place because we were earlier also handling the injectables, the vaccines and other products.

Sudarshan Agarwal: Yeah.

Sujit Mahato: So there was no surprise element on that. But only thing as I mentioned just now to the other person that I

have not tracked that data separately. But beyond this, I would not be able to comment momentarily.

Sudarshan Agarwal: Got it. And in terms of your store addition plans, right? So you highlighted around 800-odd stores. So a couple of years back also, we had added quite a lot of stores and then we had to add MedPlus Health Services Limited

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warehouses. So where do you see the headroom till where you will have to add more and more warehouses from the current level?

Sujit Mahato: See, it depends again on what we had explained earlier. As we move into newer states, we would for sure need new warehouses. But for this set of 800 stores and if it is predominantly we concentrate in the existing state, then I believe not many additions of warehouse would be required.

We have enough headroom. But as we progress into the existing state of MP and Chhattisgarh where we started filling stores in the previous year, there we may need to plan a few warehouses that will be much lower when compared to the current year additions what we have done.

Sudarshan Agarwal: Got it. And on the accounting side, you had a 30 bps improvement in gross margins this quarter on a Q-on-Q basis. But if I look at your product mix in terms of private label, it has gone slightly down itself. So is there some what is driving the sequential improvement in gross margin?

Sujit Mahato: So absolutely, your observation is right. The other income component, which has gone up in this quarter and also a bump up on the pharma membership income. What we have observed is there are certain contracts with suppliers where we have an arrangement upfront where we can accrue over the years based on the performance.

Sudarshan Agarwal: Yes.

Sujit Mahato: But there are suppliers where there exists no contract, but based on the overall performance of the year when we negotiate with them, and we are able to get some benefit that is only taken again at the end of the quarter, but basis the units sold. So, for example, if we have procured 1 million units during the year and we have got x amount and out of that 60% has already been sold to the extent of 60%, we get the credit in the quarter. And that's been where we have seen some positive benefit, which we had to accrue in the current fiscal year.

Sudarshan Agarwal: Got it. Got it. And one last question. So given that you are kind of exploring this franchisee model, historically, you have followed more of a densification strategy in your 4, 5 states or 7-odd states. But with franchisee model, I believe you may be able to go more into pan-India basis. So should we kind of assume as and when you pick up more franchisees, the expansion in other states will kind of pick up pace?

Sujit Mahato: So I think immediately, we have not listed our any new state, but those states where we just started filling stores in the previous 1 year or 2 years, we would definitely like to expand there to the franchisee road. And we will continue the densification program as you rightly highlighted in the existing states because there's still room to be covered. And as we go to new states, we will inform the market.

Sudarshan Agarwal: Got it, got it. Yeah, that's it from my side. Thanks.

Sujit Mahato: Thank you. Thank you, Sudarshan.

Moderator: Thank you. Next question is from the line of Ahmed Madha from Unifi Capital. Please go ahead.

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Ahmed Madha: Yeah. Thank you for the opportunity. Similar question on the franchisee model. There are a lot of moving parts in the business. I'm assuming, first of all, you'll have certain investments to be made in terms of fixed assets and our partner will be making some fixed asset investment. And then there is inventory component, which probably will be lying on with the franchisee partner already, and you have to account it and maybe clean up some of it.

So can

you give a little more clarity in terms of your franchisee business model, how you are looking at asset turns, fixed asset investments and as well as inventory management. You spoke about the margins part, which is clear that your breakeven time line will be there's no breakeven time line in terms of margins and from first year will be profitable, but balance sheet side is a little unclear.

Sujit Mahato: So when we try setting up a franchisee outlet, as you rightly highlighted, the capex is being invested by the company. So let's say, we spend INR10 Lakhs per store for setting up the complete capex, including the EBITDA. And then on the inventory, the franchisee purchases that inventory from MedPlus on day 1. So that's an outright sale for us.

When we look at margins, the way we are looking at currently, let's say, the gross margin today at the pharmacy level is 24% that is distributed between the Franchisee and MedPlus. Say, MedPlus continues to retain 10% or 9.5% and the franchisee gives 14%, 14.5%. The way it is done is if earlier we were selling a product at INR100, we would now sell that product at INR85, for example, to the franchisee and the franchisee would sell at INR100 and make that INR15 margin. And we would be making that delta margin of 9.5% to 10% when we sell the inventory to the franchisee.

Ahmed Madha: And in terms of existing inventory with the franchisee partner, we don't have any connection with that then? Is that prime...

Sujit Mahato: If it is a new franchisee, the entire inventory is purchased. If it is an old franchisee, depending on case-to-case basis, there are instances we completely acquire the inventory and then completely replenish the entire inventory by a new sale. So those are case-to-case basis where there are conversions from a mom-and-pop to MedPlus outlet. But otherwise, as you rightly said, it is if it's a new store, complete inventory sold by us, there is no other inventory which the franchisees allowed to procure from outside.

Ahmed Madha: Sure. And for the existing stores where you get into a franchisee model, you will be sort of writing-off some inventories, I'm assuming because their way of valuing and our way of valuing may have some difference. Is that a fair way to look at it?

Sujit Mahato: We have not observed any such differences, just to clarify.

Ahmed Madha: Okay. And lastly, in terms of margins, gross margin is pretty clear. It is roughly 9.5, 10 percentage. But then the translation from gross to EBITDA, I'm assuming it will be much higher in franchisee model than the existing model.

Sujit Mahato: So the way to look at it is after the store level EBITDA of 9.5%, 10, there's no other cost associated with it. So that gets accretive to the company's margin directly.

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Ahmed Madha: Okay. Got it. Sure. Thank you so much.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Divyansh Gupta from Latent PMS. Please go ahead.

Divyansh Gupta: Hi, sir. One question on the franchisee, which you just mentioned. So 9% to 10% gross margin that we have is net of supply chain cost or supply chain cost to deliver it to them is separately charged and therefore, the EBITDA becomes, let's say, 6 to 8.

Sujit Mahato: Yes, you are right. It is separately charged, which is currently the model which we present, where we say that after the store level EBITDA there is the PSD, which is the warehouse related and marketing cost and then the corporate and other costs, which gives the company level EBITDA. Similarly, whatever we now articulated is the store level EBITDA in the franchisee model less the supply-related cost of, let's say, 2.5%, 3%. And then the net is the company level EBITDA, which gets added to the company bottom-line.

Divyansh Gupta: Got it. And there is no marketing fee that we charge them because let's say we advertise...

Sujit Mahato: It's all – impact. It's included in it.

Divyansh Gupta: All?

Sujit Mahato: Yes.

Divyansh Gupta: Got it. The second question was in the deck, we have mentioned a round 13 stores of franchisee had closed in this quarter. What would be the reason for them to withdraw?

Sujit Mahato: Various reasons, you know. Some of them are new business entrepreneurs. They get into it with a certain set of expectations. Very soon within the first two months, they understand maybe it's not meeting their expectations. So it is very normal that we will see such closures.

Divyansh Gupta: But the average age that we have mentioned is about almost 2 years.

Sujit Mahato: Yes.

Divyansh Gupta: So I understand 1 or 2 might be, let's say, 2 months experiment done by people and...

Sujit Mahato: Some are also the old model franchisee stores, which have closed down.

Divyansh Gupta: Got it. So for a new franchisee model, have we seen any, let's say, quick closures.

Sujit Mahato: A quick – we have seen. Yes, yes, we have seen.

Divyansh Gupta: Got it. But not let's say, significant to.

Sujit Mahato: No, no, absolutely.

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Divyansh Gupta: Understood. The third question was that I'm a consumer of MedPlus products. So I was looking through the app. So I saw your there is a DBH biscuits and there is non-pharma, non-eatable products. And the manufacturer was mentioned as optimal. So from being a, let's say, a trading company that's typically a pharmacist is, right? From a trading company, we also are becoming a manufacturing company. So what is the thought process of, let's say, manufacturing all of these zones, especially, let's say, here, at least there is no, let's say, regulatory FDA standards that need to be met. So the cost arbitrage will also my assumption would be lesser. So what's the thought process of manufacturing these items by ourselves?

Sujit Mahato: Yes. So some of the items, absolutely, you're right. Wherever possible, we completely get into a full backward integration mode. And in optical also, we have set up, I think if you are referring to the food items of the bakery products, we are doing the own manufacturing only. And some of the non-pharma products also like cleaning liquid, some toilet stuff and other stuff is also being manufactured via our own factory. Wherever we do not have the capacity to manufacture, we do go to the best-in-class contract manufacturers for both pharma and non-pharma and get our stuff done there.

Divyansh Gupta: So is there a, let's say, thought process as you were saying, like non-pharma expansion, there is a whole blue ocean available, blue sky available that we will over a period of time aim to manufacture all non-pharma private label products in-house.

Sujit Mahato: I will not say all, but wherever there is an opportunity, company is completely open to explore that. And as and when we do that, we will keep informing the market.

Divyansh Gupta: Got it. And just last question. The Q1, I think you partially answered, but let's say, the 12-month plus stores margin EBITDA margin is pointing six point some 6.7% versus, let's say, 5.8% for Q4 FY25. So there is no one-off that is leading to a jump here because that way, it's a very steep jump.

Sujit Mahato: Yes, there is no one-off. I can confirm that.

Divyansh Gupta: Got it. Got it. Thank you. I'll join back the queue.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Jitaksh Gupta from Tikri Investments. Please go ahead.

Jitaksh Gupta: Hello. Can you hear me, sir?

Sujit Mahato: Yes. Please.

Jitaksh Gupta: Sir, so my first question is regarding private label. So sir, just wanted to understand like why we are still at 22%, 23% of the total sales. And like what is the near-term target, let's say, for next year or next 2, 3 years for private label?

Sujit Mahato: So the private label has two

two components, both the pharma and the FM CG part. So on the pharma, we have been mindful the way we are growing. On the non-pharma, absolutely, the scope is

unlimited. On the pharma also that way the scope is unlimited. But there it is also linked to the customers' behaviour and their attitude to convert to a private label rather than only the brand. Whereas in the non-pharma, that uptick is very fast, and we have slowly continuously started adding more assortment of products, which is going to help us going forward as well. So that's the only reason. But otherwise, you are right. We do not have a specific number in mind for a target, but continuously, we would focus on that and keep growing.

Jitaksh Gupta: And sir, another part. Like how do we manage unsold inventory under the private label? Do we bear the cost entirely?

Sujit Mahato: Yes, your observation is right. On the private label, the entire inventory risk of expiry and write-off is on our books.

Jitaksh Gupta: Ok, sir. That's it for now.

Moderator: Thank you. Next question is from the line of Akash Shah from Investec. Please go ahead.

Akash Shah: Hi, sir. Good evening and congratulations on the good set of numbers. A couple of questions here. Sir, we saw that the gross and the EBITDA margins for FY26 were at 26% and 8.8%. Just wanted to have a bit of a color on what should be what should we look how should we look at the trajectory going forward? That was my first question.

And on the second question, sir, we noted the SSSG growth of 17.8 % for mature stores, that is stores greater than 12-months. Going ahead, on the for FY27 on an annual basis, where do you see the SSSG rate as well? Thank you.

Sujit Mahato: Thank you. Thank you for your question. On the first question, I think, the first question, what was the reference, sorry?

Akash Shah: It was on gross and EBITDA margin trajectory going forward.

Sujit Mahato: We do not give any specific guidance, but we expect at least to maintain the same level of gross margin. And as and when we see further improvements in the share of private label that should be accretive to our gross margins.

Akash Shah: Okay, sir. And sir, on the SSSG?

Sujit Mahato: On the SSSG, very clearly, we had articulated a couple of quarters ago that we would aim for 9% to 10% on an annualized basis, while we strongly believe that SSSG is not the only metric which we need to track at least for a pharmacy kind of a setup. But from a number perspective, we still believe that we will be able to continue to clock at least 9% to 10% on an annual basis.

Akash Shah: All right, sir. Thank you. Thank you, sir.

Moderator: Thank you. Next question is from the line of Jatin Chawla from RTL Investments. Please go ahead.

Jatin Chawla: Yeah. Hi. Good evening and thanks for the opportunity. My first question is on your capital allocation, how are you thinking about it? Because what I see you are saying is, you have stores which give you fantastic ROCEs and there you are, kind of, letting the franchisee invest. And then on private label kind of non-pharma, you know, side, you are you want to manufacture yourself, where I'm assuming the ROCEs will not be as attractive as your stores. So, you know, how is the thought process kind of doing.

Sujit Mahato: So while ROCE you are right, we'll have to track at the company level and the capital allocation is a dynamic part of that. Capital allocation will continue to be at what we think is where we can do a better job vis-à-vis contract manufacturer. Company would continuously focus on where we can add more value, add more products, bring in more assortment, bring in more reliability to the supply chain. That being said, we are completely very mindful of how we allocate capital so that we get the optimum return

for the shareholders.

Jatin Chawla: Kind of continuation of the same question. A few years back, 2 years, 3 years back, you were adding 800 stores on your own. And at that time, your balance sheet or even the company level ROCE was not as strong as it is today. So why is the thought process of now kind of going aggressively with franchisees versus what you doing it yourself?

Sujit Mahato: So that's the whole reason of your evaluation between adding, let's say, another 20,000 stores where you need to manage additional manpower of, let's say, 25,000 people vis-à-vis expanding aggressively through the franchisee route, getting in micro entrepreneurs who can manage, let's say, 5 stores to 10 stores on their own and thereby continuously growing this by making the MedPlus brand stronger, getting better product offering and making it more and more affordable to the customers.

Jatin Chawla: Got it. Got it. That makes sense. Just one more question. You said your gross margins will expand going forward with as the private label contribution increases, but EBITDA margins

you are looking to largely maintain at the current rate. So are there any specific areas where you are kind of intending to spend that the gross margin improvement will not translate to EBITDA margin improvement?

Sujit Mahato: No, I've not mentioned that. Very mindfully, we'll have to only calibrate that what I mentioned is we would first, you know, ensure or at least target to maintain the current profitability. If you look at our historical numbers, we have continuously improved. So first is we need to stabilize this and then continue to grow and that will be the journey forward.

Jatin Chawla: Got it. Thanks a lot. This quarter was fantastic. Hope to see similar results going forward as well.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Amish Kanani from Knowise Investment Managers. Please go ahead.

Amish Kanani: Sir, congrats on a good operating set of numbers. Sir, partly my question on asset allocation is, you know, addressed. But then the question is, sir with free cash flow coming in since last two years, net of lease payment liability as well. You know, what are your thoughts – you

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know, about kind of sharing this via dividend, which is one of the strongest signals that company doesn't need the capital and we are generating the free cash flow. So one, why are we not still there? That's question number one.

I understand you will have a lot of growth aspiration and growth capital. You know, we are making decent ROCE, but just your thoughts there, sir, as a signaling. And second, sir you know, given our omnichannel, you did mention about how we can do a better job vis-à-vis only online kind of a store.

The question is expanding our network in Tier 3 where maybe online may not make a lot of sense versus growing growth through online. If you can give us some sense and flavour of how we are planning to grow existing store online and going Tier 2, Tier 3, which obviously we can do both. But where is the growth prospects much better and much faster? Hello. Am I audible? Hello. Yeah, hi. Am I audible?

Sujit Mahato: Yes, please. Can you please repeat the question?

Amish Kanani: Yes. So I'll just quickly summarize my question. Sir, partly the growth, you know, the capital asset allocation on own store versus franchisee you have addressed. The question is then, sir, with free cash flow coming even after payment of lease liability, how about giving a dividend signal, which is one of the strongest we as an investor believe is a good signal that even after working capital and repayment and capex need, we are generating free cash flow and there are no major debts to take care because there are lease liabilities, which you know at least Ind AS shows it as debt. How about thinking on those lines, which we

have not yet kind of probably taken action?

And second, sir, quickly on online growth through the existing channels versus growing in Tier 2, Tier 3 towns where maybe online may not be probably so strong, we'll do maybe focus on more store level thing. Where is the growth more coming from getting more out of online in the existing store versus going in Tier 2, Tier 3, sir?

Sujit Mahato: Thank you. Thank you for the question. So on the dividend, for sure, we have not yet reached that stage is what we believe, but I'll take your question. We'll discuss it internally. The current juncture at which the company stands that there is huge headroom for the company to grow and invest.

So we have not taken that call yet, but I'll take your question back and discuss with the senior management and the Board. The second on the online growth versus the physical growth. For us, we continue to grow more in the Tier 2, Tier 3, if I can answer you.

Amish Kanani: Okay. And sir, just to understand the online thing, you know, in slightly more detail in slightly more flavour, how is it going? Is it something which is just an additional? How is it as a percentage of existing store level economics? How is it adding some flavour..

Sujit Mahato: So for us, online is not a direct focus or a channel where we drive sales, but it's been there for the company since 2014 as an additional convenience to the customers where the customers can order online, pick up while going back home, going from office, they pick up from our stores

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and some of them really take the full benefit of ordering online and getting them delivered at the home. It is five-plus percent of our current total sales.

Amish Kanani: Okay. Got it. So two-hour delivery is more like giving a comfort that you will get the thing that you want, if not immediately, then maybe within two hours or something like that?

Sujit Mahato: Absolutely, absolutely. Where our stores are physically available, online service is available. So

that's how we operate online. Where we do not have in a particular pin code, let's say, there is \ or in a particular city where we do not have a physical presence, we do not offer online.

Amish Kanani: Sure, sir. All the best and thank you.

Sujit Mahato: Thank you.

Moderator: Thank you . Next question is from the line of Raman KV from Sequent Investme nts. Please go ahead.

Raman KV: Hello, sir. Can you hear me?

Sujit Mahato: Yes, please.

Raman KV: Yeah. I will just two questions one just with respect to private label. In the ope ning commentary now you mentioned that you have achieved a good mix between branded and private label growth and you are looking to focus in the coming year on the private label products. So can we expect the initial guidance of growing your private label portfoli o, which is both your pharmacy and non-pharmacy 1% every quarter?

Sujit Mahato: No, we do not give such kind of guidance. But yes, as we grow, we had earlier indicated that for every 0.3%, we will have a 0.1% impact on the gross margin. For that, I said, yes, that guidance still remains. But we do not give guidance on how much we will grow because it has a couple of dynamic factors, one being the customers' behaviour ; and two, the company's efforts in getting more and more assortment and portfolio of products, both on the pharm a as well as on the non-pharma. As we continue to progress, we will keep updating the market. But as such, we do not provide any guidance.

Raman KV: Okay. Understood. Just a follow-up on this. So about two quarters ba ck, you were like you are planning to focus on overall growth of the on the stores over the gr owing of only private label, aggressively expanding into private label products. So how is the na rrative right now for the management? Now also are you sticking towards the overall balance growth? Or are you plannin

g to grow aggressively for the Private label.

Sujit Mahato: No, we will continue to be very balanced in terms of growth because we are being a multi-brand retail outlet to continue to offer the full scale of branded products. So we do not want a customer who walks in and ask for a particular brand be it on the pharma or non-pharma to say that we do not have that. So that behaviour is what the company worke d at in the last couple of quarters.

And I think we will continue that journey saying that private label will a lso grow, but will

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continue to be completely focused and overall growth makes more sense rather than only private label because that's been what we have learned in the last couple of ye ars.

Raman KV: Understood, sir. Thank you.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Aejas Lakhani from Unifi AMC. Please go ah ead.

Aejas Lakhani: Yeah, hi. Sujit, congratulations on a strong set. Sujit, firstly, I wanted to understan d that the SSSG that we have seen this quarter has been probably the strongest in 12, 13 quarters. So is there anything that you would have done that you can call ou t? Because in the middle, you guys have tweaked the incentives at the store level to incentivize the employ ees to sell private label, then you had rolled that back a little bit. So any color here ?

Sujit Mahato: Aejas, I think while the numbers speak for themselves, we shared our views with yo u earlier as well and with all other participants that really SSSG is not the number which we track. But yes, if you see the consequences of certain initiatives which the company has been pursuing, yes, we have seen numbers in this range. And that's the reason on an annu al basis when other participants has asked me what we said at best is what we guided earlier also is 9% t o 10% when the numbers were very low, if you remember that a few quarters ago.

So I think we did not do anything special, if I can answer you that. What we made in particular ensured is the availability of the full assortment of non-pharma prod ucts and pharma products wherever possible. And we saw a good uptick in the FMCG products as well, which helped us on the overall number.

Aejas Lakhani: Understood. Second, Sujit, could you just call out about is it pos sible that you can give the franchisee revenues that you have booked for the year? And go ing forward, is it possible to give the breakdown of the store addition owned stores and franchisee stores?

Sujit Mahato: I'll share with that maybe after this call because I don't have it hand y. But yes, once we reach a critical mass, we will again reevaluate what amount of subdivisonal data we c an share absolutely.

Aejas Lakhani: Understood. And thirdly, Sujit, in terms of you spoke at the start of the call about your desires to improve the PL non-pharma bit. So could you just tell me on thi s front, three elements. One is what categories?

Second is that any visit to your existing stores feels that there is no incremental space available for storage. So if there is a few of more PL non-pharma products, how do you intend to create incremental store space to manage some of this? And third is the margin p rofile for typically the

PL non-pharma, is it could you just give some colour about how it is compared to PL pharma?

Sujit Mahato: Sure. So in terms of the categories across every category in a retail store, we are relooking at the assortment and wherever possible, either we are bringing in more branded stuff or we are parallelly evaluating and adding more and more private label new products, be it in edible oils, any health sector, any health-related food supplements. So that is on e.

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Second, what you rightly observed, there are stores where our space is constrained, but there are many other stores where

we strongly believe that there's enough room to add more and more products to the same footing. And that's where I think even during the current year, we have a project to, I would say, upgrade or modernize at least 600-plus stores to accommodate more and more. So that is smart way of storing the rack system and the pharma and non-pharma demarcation right which can be serviced from behind the counter and which can be serviced or left out in the open. So those are the certain stuff we are again carrying out this year. And the last one, what you mentioned I missed your point, what was the questions.

Aejas Lakhani: Sir, margins.

Sujit Mahato: Yeah. In terms of the margin profile, for sure, the private label pharma commands a much higher margin. The private label non-pharma margins are in the range of 23% to 25% at the gross margin level, which are significantly better than the non-pharma branded products, which are in the range of maybe 9% to 10%.

Aejas Lakhani: Understood. And broadly, just one last thing is that has there been any broad change in your acute and chronic mix, say, over the last 2 years or 3 years?

Sujit Mahato: We have not seen that change yet, Aejas.

Aejas Lakhani: Understood. Thanks, and all the best.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Divyansh Gupta from Latent PMS. Please go ahead.

Divyansh Gupta: Hi, sir. Thank you for the follow-up. First question is the inventory days that you mentioned in our slide deck, right, for more than 12 months and less than 12 months. Just a formula check.

The inventory is what is in our balance sheet, but does the sale or the COGS is only for the COCO store or it includes franchisee or how should we understand?

Sujit Mahato: The inventory on our balance sheet only relates to COCO stores. Franchisee the date is sold, it is their inventory.

Divyansh Gupta: So when we compute the number of days in the denominator, do we like-to-like inventory that COCO but the denominator...

Sujit Mahato: In COCO stores. You are right. Here, only COCO stores numbers have been taken on both sides, just to clarify.

Divyansh Gupta: Got it. That makes that is helpful. Second is you mentioned that, let's say, earlier when we were discussing that 9% to 10% is what we get as gross margin when we do a franchisee sale. Right now, let's say, 500 stores are there. And if I just take 300 that were done last year and add another 300 for next year. So about 800 stores out of 6,000-odd so some stores becomes franchisee stores.

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Now the question is franchisee margin is 9% to 10% for us, whereas COCO will be at 24%. Now as this margin for as a percentage of franchisee store increase, the margin should come down, whereas you are expecting the margin to remain similar. So does it mean that the goodness from private label increase more or less gets sort of nullified by the increase in franchisee at least at the gross margin level.

Sujit Mahato: So the just only one point I would like to add so that it is apple-to-apple. What you rightly mentioned is gross margin currently is 24% and the store level EBITDA is at the company level, 11%, right? That level you should compare with 9.5%, 10%. And as the percentage of private label also increases in my franchisee outlets, this will also continue to improve. That's what we meant.

Divyansh Gupta: Got it. So you're saying EBITDA will remain same. Maybe the gross margin might show up lesser, but you are saving on other cost?

Sujit Mahato: Yes.

Divyansh Gupta: Got it. And the last question, GLP products have been launched on our, let's say, in our stores. What would be the revenue that, let's say, contributed in this quarter? I was not there in the opening slide if you had mentioned it.

Sujit Mahato: No, I had not mentioned. I said we are not tracking that immediately. But yes, from an available

liability all the products of all the brands are available at our store s.

Divyansh Gupta: Got it. And is there a time line for store generic of these products? Or right now, it's going to be branded generics only?

Sujit Mahato: As of now, we have not launched any store level for the GLP-1. But as and when we launch, we will surely inform the market.

Divyansh Gupta: Got it. Got it. Thank you. Thank you, sir.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Rickanjeet Singh, an Individual Investor. Please go ahead.

Rickanjeet Singh: Hello. Am I audible, sir?

Moderator: Yeah.

Sujit Mahato: Yes, you are audible?

Rickanjeet Singh: Sir, my question is this, sir. We have FY26, we have paid an interest of INR120 crores, sir. That is a loan of INR1600 crores, sir. What should be the management's thinking about that? Should it be reduced or not?

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Sujit Mahato: So this interest that you are seeing in P&L, so this is all related to lease accounting. And there is no external loan in our book.

Rickanjeet Singh: Okay.

Sujit Mahato: So that is also being shown under lease liability Ind AS. So, there are no actual loans in the books. It's a zero-debt company. Yes, please say.

Rickanjeet Singh: So this is interest of INR120 crore?

Sujit Mahato: It is all about lease accounting where the lease rental is subdivided into interest and depreciation.

Rickanjeet Singh: Okay. Understood, sir. Going forward will the margin be in double-digit or how it will be, if you can give any guidance?

Sujit Mahato: So we don't provide guidance, as explained earlier, what we will ensure is to first maintain the current level of profitability and then continue to grow.

Rickanjeet Singh: Okay, sir. Thank you, sir. Thank you.

Sujit Mahato: Thank you. Thank you.

Moderator: Thank you. Next question is from the line of Govindarajan Chellappa from CSIM. Please go ahead/

Govindarajan Chellappa: Yeah, Hi. I just had one question. Your warehouse inventory has been continuously improving. What is the sustainable level of warehouse inventory, 30 days is what you reported last quarter.

That seems fairly, you know, industry best.

Sujit Mahato: I think, Govind this would be in the range of 30 days to 33 days. We have been dabbling, but we have not cut out on any tail brands as such. But at a point in time when private label really takes-off, we will relook at the entire SKUs, which we are carrying. For example, today, as you are aware, for every molecule, there are at least 200-plus brands available and depending on the physical locations of our stores, we try to fulfil the demands of those particular pin codes for the same molecules which are generated there.

And therefore, we are also forced to maintain all the tail brands plus the MedPlus brand. But a point in time, if, let's say, MedPlus reaches 40-plus percent, we would definitely relook at the tail brands and maybe reduce a few to reduce this number, which can help us also on the overall working capital. But for now, we think we will continue with the same level.

Govindarajan Chellappa: Okay. Sorry, if I could squeeze in one more question. Typically, what is the percentage of returns that you give back to pharma companies either because of expiries or damages? And what is the how long is it does it sit as inventory on your books? And how long does the payment come does it take for the payment to come back from the pharma companies?

Sujit Mahato: So generally, it is done only on expiries, returns to the pharma companies, especially on the branded products. And it's I would say, less than a month, we get reimbursed up to 95%.

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Govindarajan Chellappa: Okay. And you know, typically, how long is the inventory holding period for you? And what proportion of either value of sales or

whatever is typically return? I mean would it be 5%, 10%?

Would it be higher?

Sujit Mahato: It would be less than 1%, Govind.

Govindarajan Chellappa: It's less than 1%?

Sujit Mahato: Yes.

Govindarajan Chellappa: Okay, thank you.

Sujit Mahato: Thank you.

Moderator: Thank you. Ladies and gentlemen, we will take that as the last question for today. I would now

like to hand the conference over to Mr. Sujit Mahato for closing comments. Over to you, sir.

Sujit Mahato: I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you, and have a good day.

Moderator: Thank you very much. On behalf of MedPlus Health Services Limited, that concludes this conference. Thank you all for joining us today and you may now disconnect your lines.